Thirlwall Inquiry

Report of the Public Inquiry into events at the Countess of Chester Hospital, 2015 to 2018

Volume II

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HC 597-II

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Report of the Public Inquiry into events at the Countess of Chester Hospital, 2015 to 2018

Chair: The Rt Hon. Lady Justice Thirlwall DBE

Presented to Parliament pursuant to section 26 of the Inquiries Act 2005

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Chapter 14. Events at the end of June 2016

27 to 28 June 2016

14.1 Returning to the events of June 2016, on 27 June 2016 Ms Kelly and Mr Harvey both attended a pre-arranged meeting about the BabyGrow appeal at 10:00, along with Ms Powell, Ms Anne Murphy and Dr Jayaram. Ms Kelly made notes.[1] In evidence, she said that at the meeting Dr Jayaram had said to those present that the deaths of Baby O and Baby P the previous week were "*very worrying*".[2] This can have come as a surprise to no one. Ms Kelly said that after Dr Jayaram had raised his concerns with Ms Townsend there had been a "*significant shift in the gravity of the situation*".[3] In her note she recorded: "*Concerns over one nurse. EP* [Ms Powell] *adamant – no concerns with LL (present when babies deteriorated*)."[4] She said that she and Mr Harvey had agreed to review the information about the deaths and escalate the matter to Mr Chambers.[5] Mr Harvey said in evidence that there was no discussion about Letby until after the meeting.[6] The precise time does not matter, and Ms Kelly's note suggests it was after discussion of the BabyGrow appeal. Mr Harvey did not dispute that it took place.

14.2 There is no evidence that the review Ms Kelly and Mr Harvey had agreed to ever happened. Nor is there any evidence of their bringing the issue to the attention of Mr Chambers. It is likely that Ms Kelly and Mr Harvey did not speak to him at that stage. They were avoiding bringing to the attention of the Chief Executive and the Board the fact that the paediatricians were suspicious that babies were being harmed on the neonatal unit. This was not an active cover-up but it was, at best, the postponing of effective Board action to protect children in the hope that an explanation would be found that quashed concerns of deliberate harm.

14.3 Dr Brearey had invited Ms Kelly and Mr Harvey to the senior paediatricians' meeting at 12:00 on 27 June 2016 so they could discuss concerns about neonatal mortality.[7] Whilst Ms Powell and Ms Anne Murphy attended the meeting, neither Mr Harvey nor Ms Kelly did so. They did not explain why they did not attend.[8] It is clear that they were not interested in what the consultants had to say.

14.4 Dr Brearey spoke to Mr Harvey after the paediatricians' meeting to say that those who had attended the meeting had agreed that Letby should be removed from the unit until the causes of the deaths had been established.[9] Mr Harvey thought it unlikely that Ms Powell had agreed to that and asked Ms Kelly to check with Ms Powell[10] (who did not agree that Letby should be removed from the unit).

14.5 Mr Harvey and Ms Kelly arranged a meeting for that afternoon, to which the paediatricians were not invited. This was a regrettable approach. Given that they had ignored the request for a meeting with the paediatricians (and an email from Dr Brearey about the meeting), to arrange a meeting with nurses demonstrated that they had already decided that the way forward was to be formulated by reference to the nurses only.

14.6 Ms Powell said that she and Ms Anne Murphy were called to the meeting so that Ms Kelly and Mr Harvey could ascertain what they thought of the allegations.[11] Given that their views had been repeatedly and clearly expressed to Mr Harvey and Ms Kelly, including that morning, it is likely that the purpose of this meeting was to align those who opposed the doctors' views and to develop a plan in their absence. The plan would be presented to the doctors as a fait accompli.

14.7 Mr Harvey, Ms Kelly, Ms Powell, Ms Anne Murphy and Ms Rees were all at the meeting.[12] An action plan was formulated. Items on the plan were that Mr Harvey and Ms Kelly would meet the consultant group; Ms Powell would be mindful of staff allocation during shifts to provide support and supervision to Letby; and Mr Harvey would identify the RCPCH Lead for a review of the neonatal unit. This would be multidisciplinary. Letby was to remain on days for support, and she was going to be on annual leave the following week, for two weeks. The decision that she would stay on days was taken in spite of Dr Brearey's very clear and repeated requests on behalf of the consultant body that Letby should not return to the neonatal unit at this time. Neither Mr Harvey nor Ms Kelly sought to establish whether Letby was working that day or scheduled to work on the other days before her leave.

14.8 Ms Anne Murphy and Ms Powell agreed to the external (RCPCH) review so that nothing was missed, but in a note prepared for a grievance hearing, Ms Powell wrote that "*we did not or never had believed for an instant that LL was guilty of ANY wrong doing*".[13] Ms Rees, who was also at the meeting, was of the view that there was a 'them and us' approach between the doctors and nurses.[14] To take a them and us approach where what was required were cool heads and reason was a serious failure by senior managers.

14.9 Even without hindsight, I cannot accept that the very robust defence of Letby by the Manager of the neonatal unit (supported by others) meant that no consideration could be given even to the possibility that the doctors' concerns about Letby might be justified. The concerns could not have been more serious. It cannot reasonably have been thought that the concerns were not honestly held. If there was any possibility that even some of them might be justified, there was a duty on every person at that meeting to consider and act upon the need to protect the babies on the unit. That need was not raised. This was a serious failure by all in attendance.

14.10 When asked why she had not considered whether Letby was working that week, Ms Kelly said: "*You know, as a Director of Nursing, in that organisation I was over nearly 1,000 nurses and midwives. The last thing on my mind is that one of my nurses is -- is deliberately harming children or babies or adults.*"[15] This is a perplexing statement. Whether one of her nurses was (or at least might be) harming babies should have been uppermost in her mind; there is no question that she knew exactly what the doctors' concerns were. As to whether Letby was working that week, she said she would have relied on her senior

nursing team, and in particular Ms Powell.[16] She would have expected Ms Powell to tell her if Letby was on shift. She acknowledged that she did not ask the question and that she should have.[17]

14.11 Ms Kelly denied that it had become doctors versus nurses: "[W]*e were really keen to hear a nurse's perspective and a doctor's perspective and actually patient safety was absolutely paramount*."[18] I do not accept that, particularly in light of what she then said: "*This was a team that before all of this worked really well together and it's unfortunate that because of the events that we are now talking about … it became divisive between the nurses and the doctors.*"[19] It became divisive because of the approach taken by Ms Kelly, Mr Harvey and the nurse managers. It should never have been about nurses against doctors. It was about keeping babies safe. Had that governed the approach of the managers, they would and should have sought to work with the doctors, not against them. Characterising this as nurses versus doctors distracted from the real issue: that steps needed to be taken to protect the babies.

14.12 The fact of the deaths was incontrovertible. Something was causing them, whatever or whoever it was. The doctors had been driven to thinking that it could be Letby in the absence of any other potential cause being revealed by their investigations. If there was the slightest possibility that this might be correct, babies' lives were at risk. There was only one option: to remove Letby from the neonatal unit. This was not done nor even contemplated by Mr Harvey or Ms Kelly.

14.13 At 17:34 on 27 June 2016, Ms Powell contacted Letby to change her shifts from four night shifts starting on 27/28 June 2016 to three long day shifts on 28, 29 and 30 June 2016.[20] Baby O and Baby P had died when Letby was on day shifts. Ms Townsend said in evidence that there were concerns all around, "[b]*ut my understanding was that at that time, there was still no actual evidence or detail and the Executive decision* [by Mr Harvey and Ms Kelly] *at that time was … for Letby to remain on the unit until she went on annual leave*".[21] This was another serious failure of senior managers to discharge their responsibility to keep children safe.

14.14 At 21:04 on 27 June 2016, Ms Kelly emailed Ms Powell, Ms Anne Murphy, Ms Rees, Ms Townsend and Mr Harvey.[22] She thanked everyone for attending the meeting and set out the agreed action points, including:

> "*IH* [Mr Harvey] *and AK* [Ms Kelly] *to meet with Consultant Group re their concerns*
>
> …
>
> *IH to identify Royal College Lead to facilitate External NNU* [neonatal unit] *review*
>
> …
>
> *LL to remain on days for support*."[23]

In oral evidence, Mr Harvey confirmed that the RCPCH review was his suggestion.[24] The characterisation of Letby being on days for support reflected Ms Powell's strongly held views about Letby. No one acknowledged that since Letby's move to day shifts on 7 April 2016, there had been no deaths on the neonatal unit at night.

14.15 The next day, 28 June 2016, Dr Brearey told Ms Townsend of the outcome of the meeting of the paediatricians on 27 June 2016.[25] He was concerned that future harm might be done to babies that week. He was aware that there had been a meeting the previous day between Mr Harvey and the nurses. He asked for a meeting of nurses and doctors "*sooner rather than later*". She replied with a lengthy description of what she understood the current plan to be.[26]

14.16 Dr Brearey replied, copying in Ms Powell and Dr Jayaram with some justified exasperation in the first sentence and incredulity in the second.

> "*Thanks Karen.*
>
> *It helps to be kept informed of the plan.*
>
> *Just to confirm then, Ian and Alison are happy for LL to work on NNU in the same capacity as last week despite the paediatric consultant body expressing our concerns that this may not be safe and that we would prefer her not to have further patient contact?*
>
> *Furthermore, they are happy to wait til Friday before we can discuss this in person?*
>
> *In the meantime, I'll try to get together with Eirian and review the mortality cases. Eirian, do you have time this week? The next 2 days for me is looking very full.*"[27]

14.17 The same day, Mr Harvey contacted the RCPCH and asked about their invited review service.[28] There is no reference anywhere in the documents from the Countess to any discussion at this time between Mr Harvey and Mr Chambers about this, or indeed anything else. Mr Chambers thought it unlikely that this step would have been taken without discussion, but there is no evidence of any discussion. Since neither Mr Harvey nor Mr Chambers remembered such a discussion, and there is no record, I cannot find that it took place.

14.18 In late June, probably on 27 or 28 June, at Dr Brearey's request a meeting took place early in the morning between the consultant paediatricians, consultant obstetricians and senior neonatal nursing staff. There were two matters to be discussed: the potential downgrading of the unit and the paediatricians' concerns. There is no note of the meeting, so the two people concerned were working from memory. The first reference to what was said by Ms Powell and Dr McCormack, a consultant obstetrician, at the meeting was in Ms Powell's interview during the grievance process in November 2016, some five months after the meeting took place. As she acknowledged in evidence, the note of her interview read: "[R]*egarding the downgrading of the unit SB* [Dr Brearey] *alluded to LL being responsible. I told him not to as it wasn't his place. Jim McCormack stood up, pointed to EP* [Ms Powell] *and said 'you are harbouring a murderer'*."[29] She confirmed in evidence: "*That's what I heard him say to me.*"[30]

14.19 Dr McCormack told the Inquiry that during the meeting Dr Brearey said: "[T]*here was great concern … that a nurse was causing intentional harm to babies on the unit.*" Dr McCormack said that his response to this information was: "'*Are you saying that a nurse on the unit is a murderer?' And he* [Dr Brearey] *replied, 'Yes.' And like we were absolutely shocked at this stage, and everybody was taken aback.*"[31] Dr McCormack denied saying "*you are harbouring a murderer*". Ms Powell held her ground, "*I heard him say what he said*".[32] There is little difference of substance between the two accounts, but Ms Powell recalled that Dr McCormack's remarks were directed to her personally. Since

it was Dr Brearey who had told the obstetricians for the first time what his concerns were, it seems more likely that these powerful remarks were directed to him rather than to Ms Powell, whom Dr McCormack did not know. I accept that she believes they were directed at her and said as much in late 2016. There could be no possible doubt that everyone understood what the paediatricians thought. I deal with this incident further in Chapter 22.

14.20 At 16:25 on 28 June 2016, Ms Powell emailed Dr Brearey, copying in Ms Townsend, Ms Rees and Ms Anne Murphy. She said that she had spoken to Letby: "*LL is obviously concerned that we have issues with her performance as we are asking her to step down from the ITU* [intensive therapy unit] *environment – I have reiterated that this is primarily to protect her wellbeing.*"[33] This suggests that there had been a decision to take her away from Nursery 1.

29 June 2016

14.21 Ms Townsend did not reply to Dr Brearey's email of the afternoon of 28 June 2016 until 08:28 the next morning. She enclosed the email she had sent the previous day and said that, if Dr Brearey wanted to bring the discussion forward from 1 July 2016, he should "*speak directly with ...* [the] *Medical Director*".[34]

14.22 Dr Brearey replied shortly afterwards. He apologised that his emails the previous day had been "*a little short*" and made it plain that he was unhappy with the way matters had been managed that week. He said: "*To make decisions against the wishes and concerns of the clinicians involved without discussing it with any of us first for a week seems a little odd and disrespectful.*"[35] It was not only odd and disrespectful, it was, in my view, foolhardy. It led to the entrenching of a position where the views of the doctors were disregarded when cooperation and understanding were necessary.

14.23 In the meantime, at 08:16 on 29 June 2016 (there is some variation in the email timings on this date), Dr Saladi emailed his fellow consultants along with Ms Powell, Ms Anne Murphy, Ms Kelly and Mr Harvey. He set out the background to the paediatricians' concerns and said: "*I believe we need help from outside agencies, who can deal with suspicion. At the moment we are all under suspicion and the only agency who can investigate all of us I believe is the police.*"[36]

14.24 Dr Jayaram replied to Dr Saladi and observed that the managers did not seem to regard the issues with the same urgency as the paediatricians.[37]

14.25 The same morning, Mr Harvey consulted Mr Cross.[38] He told him that the consultants were escalating concerns about deaths on the neonatal unit and that they were concerned about the activities of a nurse. Mr Harvey showed Dr Saladi's email to Mr Cross. Mr Cross said in his witness statement, relying on his manuscript note of this meeting, that he had advised that the police be involved immediately.[39] His note of the meeting says:

> "*ADVICE: 'Police' need to be involved* now [emphasis in original].
>
> …
>
> *Death of triplets has raised concern. Nurse was on duty at deaths. Sufficient level of concern that illegal activity in neonatal*."[40]

14.26 Mr Harvey said that he did not remember the meeting at all and does not accept that Mr Cross advised that the police be called.[41]

14.27 Whilst I understand why Mr Cross inferred from his note that the advice to call the police was his, nothing he did after that supports that reading of the note. In meetings later that day, there is no evidence that he raised the point or chased action by others to call the police. The evidence is to the contrary effect. Several witnesses spoke about his description of what would happen to the hospital were the police to be called.[42] It gave no encouragement to call the police – rather, the opposite.

14.28 It is more likely that the reference in Mr Cross's note to "*ADVICE: 'Police' need to be involved now* [emphasis in original]" is, as Mr Harvey suggested in evidence, to Dr Saladi's view about what should be done.[43] The word 'Police' is in inverted commas, suggesting that it is a quote from somewhere else. It would have been more usual simply to say 'Advised contact police', were that Mr Cross's advice.

14.29 In response to Dr Jayaram's earlier email (see paragraph 14.24), Mr Harvey emailed all those on the email distribution list, saying that the matter was being treated with urgency, action was being taken and "*all emails cease forthwith*".[44] This was an ill-judged communication. In evidence, Mr Harvey said that this email to the consultants was something he got "*completely wrong*" and the email "*doesn't read as it should have done*".[45]

14.30 When giving evidence, Mr Harvey apologised for the email and emphasised that it was not intended to intimidate.[46] It is inescapable that the email reads exactly as Mr Harvey intended it to. Its tone is peremptory and dismissive. Its purpose was to stop the doctors discussing a matter of utmost concern about the safety of babies.

14.31 In the event, the paediatricians removed Mr Harvey from the distribution list and continued their discussions without copying him in. Dr Gibbs wrote: "*We are all agreed that something has to be done fairly quickly to try to ensure our neonatal patients are protected.*"[47] Something was needed immediately, not fairly quickly. Dr Gibbs is not given to using assertive language, but it would have been better had he said: 'We must do something to protect babies now.'

14.32 Dr Jayaram emailed Dr Gibbs and Dr Brearey the same day. He wrote: "*The Trust are contacting the police soon, once some information gathering has taken place, which is why Ian asked for the chit chat to stop*."[48]

14.33 Dr Jayaram gave evidence to the Inquiry that he had spoken to Mr Harvey that morning and Mr Harvey had told him that the hospital was trying to get more information and then probably contact the police.[49] Although he originally said in his written statement to the Inquiry that he could not explain why "*Dr Jayaram had said this as I cannot recall having discussed approaching the police at this stage*",[50] in oral evidence Mr Harvey accepted that he "*probably did have that conversation with Dr Jayaram*".[51] This was an inevitable concession given his own email of the same day to Ms Townsend saying that he had spoken to Dr Jayaram.[52] It follows that Mr Harvey was saying that, after gathering more information, the hospital would probably contact the police.

14.34 The same day, 29 June 2016, Ms Kelly emailed Mr Harvey to say that she had fully briefed Ms Sian Williams, and they had discussed contacting the police. She wrote: "*I know this is a big step but it is something we need to consider in light of heightened concerns.*"[53] Mr Harvey replied that he thought the police would have to be contacted. Ms Kelly agreed. When giving evidence, Ms Kelly could not remember the conversation with Ms Sian Williams but accepted that it had happened and that she was in favour of calling the police.[54]

14.35 I accept therefore that at this stage Mr Harvey and Ms Kelly were seriously considering contacting the police. In the event, the police were not contacted for almost a year.

14.36 In evidence, Mr Harvey said that he did not call the police at that point because he thought the matter was so serious it needed to be discussed by the executives.[55] By the end of 29 June 2016, having been in discussions with senior executives, Mr Harvey had changed his mind. In his evidence he said: "[O]*ne of my regrets is that having, along with Alison Kelly, come to the view that we should contact the police in June/July, that -- that we didn't.*"[56]

14.37 Ms Townsend forwarded Dr Brearey's emails of 28 and 29 June 2016 to Mr Harvey, saying: "*I wish to make you aware of a series of e-mails I have received from Steve Brearey, there appears to be some anxiety as to whether Friday* [1 July 2016] *is soon enough for you to meet with the clinicians.*" Mr Harvey replied that there was email silence, that he had spoken to Dr Jayaram and that action was being taken.[57]

14.38 Mr Harvey insisted in evidence that he took the doctors' concerns seriously, but his approach demonstrates that he did not consider them credible. Even if he honestly believed (despite the views of the consultants) that there was nothing in their concerns, he had a duty to consider whether steps should be taken to safeguard other babies on the unit. He failed to do so.

14.39 The delay in informing Mr Chambers (or the rest of the Board) supports my view that Mr Harvey and Ms Kelly were determined to keep matters quiet pending further investigation in the hope that another explanation for the deaths would be found. This was another misjudgement by both. What difference it would have made had they told the Board sooner is another matter. We know only what happened once Mr Chambers and the Board were informed.

14.40 Mr Chambers said in a written statement after he had given evidence that he first heard about the problems on the neonatal unit when Mr Cross and Mr Harvey spoke to him between about 8.30am and 9.15am on 29 June 2016.[58] That must have been after the meeting between Mr Harvey and Mr Cross to which I refer above.

14.41 The CQC report of the February 2016 inspection was due to be published that day, 29 June 2016. Mr Chambers phoned Ms Kelly shortly after 9am to tell her to delay the publication of the report. Ms Kelly noted that it would not be good for the Trust to present as 'good' in light of the current concerns.[59] She spoke to CQC and was informed that the process of publication was automated and was now in train.[60] It could not be stopped.

14.42 At 10:00 there was a meeting of the Executive Team. Amongst those present were Mr Chambers, Mr Holden, Ms Burnett, Mr Harvey, Mr Cross and Ms Kelly. The notes of this meeting record that there was to be an update on the neonatal unit at 13:00.[61]

14.43 The meeting at 13:00 was attended by Mr Harvey, Ms Kelly, Mr Chambers, Mr Cross, Ms Millward and, for part of the meeting, Ms Burnett. There are two sets of notes of this meeting, one by Ms Kelly[62] and the other by Mr Cross.[63] There are no formal minutes of this meeting (or of most of the meetings). Sir Duncan Nichol said that the notes should have been typed up and a clear chronology and action plan set out, with "*clear individual responsibility*".[64] It is not apparent that any thought was given to any of this at the time.

14.44 I infer from the contents of the notes that Ms Millward was one of the principal contributors to the meeting, presumably because of her role as Head of Risk and Patient Safety and the nature of some of what was said. The discussions included the fact that the neonatal unit was a small unit and, she said, reporting of incidents was low. The following is written: "'*Staffing' is a trend but not near what you would expect. Red herring = incidents.*"[65] That seems to suggest that the presence of a member of staff ('staffing' being code for Letby, as I said earlier) was regarded as a red herring. These opening remarks would have set the tone for the meeting. They suggest a mind closed to the possibility that there might be a link between the member of staff and the unexpected collapses and deaths.

14.45 The discussion was wide-ranging. Ms Millward summarised the reviews in June 2015 of four (in fact three) deaths, which found that they were unexpected but not necessarily suspicious.[66] The Thematic Review of ten deaths was also discussed, with Ms Millward noting that people were "*uncomfortable*" following the review; that a member of staff had been identified; and that Ms Lawrence had identified this nurse.[67] Ironically, Ms Millward had dismissed Ms Lawrence's views only a month before this meeting (see paragraph 6.43, Chapter 6).

14.46 The following matters were also discussed: closure of the unit, referral to the police, exclusion of a nurse and reputational issues for the Trust. The notes refer to the "*HR file*" for Letby.[68] Ms Millward is recorded as saying: "[Ms Powell] *has made point. Some babies did not respond to* [resuscitation] *as she* [would have] *expected.*"[69] This was a point supportive of the consultants' concerns. It does not seem to have been understood. Ms Millward is then recorded as giving a summary that begins "*Concerned witch hunt for Nurse*", then continuing to say that Letby worked full time, with additional hours, and so "*she is on duty more*".[70] Ms Millward said that there was a "[c]*losed culture in* [the department]".[71] This was untrue. Dr Brearey had sought help from Dr Subhedar at Liverpool Women's Hospital from the Risk Team and elsewhere. Further, and contrary to the neonatal unit being a "*closed culture*", Dr Brearey had complained that it was not getting adequate support from the Risk Team (a point Ms Lawrence accepted in evidence).[72] Ms Millward also failed to refer to Dr Subhedar's observations on the face of the Thematic Review that "*there was a clear and strong governance culture in CoCH* [the Countess] *which was evident at the meeting and that the number of PMs* [post-mortems] *undertaken was impressive and indicated a willingness to learn and improve*".[73]

14.47 Ms Burnett said in evidence that in the meeting "*it was a shock at the number of deaths that we were being informed of, there was shock that that hadn't been brought out sooner; and there was a shock that there was some concern that somebody was undertaking*

something malicious".[74] Ms Burnett was new to this issue. She understood in that meeting exactly what the concerns were. If further evidence were needed that Ms Kelly was simply wrong when she said that the consultants' concerns were never clearly articulated, this is it.

14.48 Much of the discussion was focused on finding an explanation other than criminal behaviour for the deaths. There was nothing wrong with that in principle, but it was not accompanied by any discussion, still less practical action to protect babies on the neonatal unit should the concerns be well founded. The danger Letby might pose had to be confronted and dealt with. She should have been removed from the unit. It is not clear whether those at the meeting (other than Ms Kelly and Mr Harvey) even knew that Letby was working that week.

14.49 Mr Chambers is recorded as saying: "*Why shut unit*?", then "*Press*."[75] Mr Harvey suggested something along the following lines: "*Consultants concern – series of deaths – investigated. No* [individual] *concerns but collectively CONCERN. Cannot explain sufficiently why our deaths higher than normal*."[76] This passage is about giving an explanation to the press about the series of deaths that protected the reputation of the hospital.

14.50 Mr Chambers said repeatedly in evidence that during this meeting (on 29 June 2016) his principal concern was patient safety.[77] I do not accept this. He made no reference to patient safety at all. He did not even ascertain whether Letby was working that day.

14.51 Mr Chambers was to comment in later meetings that there were doubts about admitting twins and triplets.[78] In evidence he said from his experience (presumably as a manager) he did not expect twins and triplets to be in a Level 2 neonatal unit. Neither Ms Kelly nor Mr Chambers had any knowledge or expertise in obstetrics or neonatology. Dr McCormack, the experienced obstetrician responsible for Mother O, P and R's antenatal care, had no concerns about the babies being born in the Countess and, as I have said already, he said that they were born in excellent condition at good weights. This issue was a true red herring.

14.52 There was no basis for the assertion that this was a 'witch hunt'. Babies were dying, infection and other causes had been ruled out (see Chapter 11), reviews of staffing competencies were taking place and appraisals revealed no performance concerns. Lots of issues were being considered. The doctors' concerns were plainly relevant. Although the meeting referred only to Dr Brearey, these concerns were held by all the consultants, as the executives knew by now. Declaring the expression of their honestly held beliefs a witch hunt was offensive but, far more important than that, it was a distraction from what the doctors were saying. Ms Millward and the others in the meeting knew that the deaths were unexpected and more numerous than in previous years. The imperative was to rule nothing out. To rule out the very person whom the doctors thought may be the cause took an unnecessary, serious risk with the future safety of babies on the unit.

14.53 Ms Millward was not asked about the notes made by Mr Cross when giving evidence and was sent them afterwards. She was asked about her summary, and in particular the reference to a witch hunt. She said in a further statement that she did not remember making those remarks, and Mr Chambers (who was also asked for his comments on the

notes) did not remember her saying anything about a witch hunt either.[79] I doubt that Mr Cross invented the summary, and the phrase 'witch hunt' was repeated by others, including Mr Chambers. I am satisfied that Ms Millward said it.

14.54 Mr Cross recorded that, as the meeting finished, consideration was given to calling the police, shutting the unit and excluding Letby.[80] None of these three actions was taken.

14.55 After those observations, the note concludes: "*David S, Ravi, Steve B, Murthy Saladi all say YES to police. If police: unit closed, forensic examination,* [interview] *of all staff, arrest of nurse. Reputational issues for trust – link to CQC report.*"[81]

14.56 By this stage, Dr Brearey had sought a meeting on 27 June 2016 but he had been ignored. Instead, Mr Harvey organised a meeting with the nurse managers. The same day, Dr Brearey asked for a meeting of nurses and doctors. He received no reply. The meeting of the executives took place without anyone asking to hear from the doctors. There is a note saying that they need "[to manage communications] *with consultants*".[82] This sounds like something said by Mr Chambers. It means, I think, that the executives ought to tell the doctors what the plan is. It would have been far better had all the executives heard directly from the doctors what they were worried about and why, and then planned accordingly.

14.57 In his witness statement, Mr Cross said that at this meeting he reiterated his view that if there were serious concerns the matter should be reported to the police.[83] He said he did not have sufficient information to assess whether there had been criminal activity and that his notes record uncertainty around the cause of the deaths. There could have been no doubt that serious concerns had been raised. No more than suspicion was required for action. Mr Cross's description of the effect on the unit were the police to be called was not designed to reassure those present that this would not be traumatic. As it happens, Mr Cross's prediction of what would happen were the police to be called was completely wrong.

14.58 Mr Cross's assertion that the nurse would be arrested is revealing. He had been a police officer for years. He knew that a person may not be arrested unless there are reasonable grounds to suspect the person of committing an offence. I suspect this is something that Mr Cross reflected upon in the nine years following these events. That may explain why his belief at the date of the Inquiry was that he must have advised Mr Harvey on 29 June 2016 to contact the police.

14.59 Mr Chambers is recorded as saying in the notes of the executives' meeting on 29 June 2016 that calling the police was "*absolute*" and he wanted to explore "*other options*".[84]

Meeting with the consultants

14.60 A meeting with the consultant paediatricians eventually took place at 17:10 on 29 June 2016.[85] In attendance were Mr Harvey, Mr Chambers, Ms Kelly, Dr David Semple (a consultant in obstetrics and gynaecology), Ms Burnett, Mr Cross, Dr Brearey, Dr Jayaram and Dr Saladi. During the meeting, the consultants set out in detail the concerns they had about the deaths and about Letby. Those present discussed next steps. Ms Burnett said, correctly, that the unit was unsafe.[86]

14.61 Mr Harvey said when giving evidence that at this meeting the consultants were "*highlighting their concerns that she's associated, that there was -- at no point did they say in their view she was murdering them*".[87] Generally speaking, he said that the consultants' concerns were "*not fully voiced and were difficult to follow*".[88] I do not accept this. The reason the meeting was taking place was because the consultants did not think Letby should be on the neonatal unit, as there was a risk that she was a danger to patients. The following notes appear in the record of the meeting:

> *Dr Brearey:* "*Some PM reports but not all. Inconclusive. Some not satisfactory giving answers … unexplained collapses – perhaps* [should] *Datix … Met July 2015. 3 cases common theme was nurse. Discussed at thematic with Liverpool … Disturbing thing – twin survived and got better in Arrowe Park* [Hospital]. *Babies coming back to COCH. Babies deteriorate. Nurse 7 out of 9* [between] *12 noon* [should be midnight] *and 4 am … More than just an association with this nurse.*"[89]
>
> *Dr Jayaram:* "*entirely subjective. Staff member – almost always Nurse in charge. Babies were stable and then deteriorated. Why always this nurse? Babies were unwell but getting better … Babies did not respond as they should … How?* [Cannula?] *air embolism? Crystal ball. Unquestionably got something going on at COCH. But what?*"[90]

14.62 Ms Kelly accepted in evidence that the consultants spoke about the possibility of deliberate harm.[91] She also accepted that she should have initiated safeguarding procedures at that time.[92] The same must apply to Ms Burnett. These were serious and genuine concerns that required immediate action to protect babies.

14.63 Mr Harvey accepted in evidence that Dr Jayaram had raised air embolism at this meeting, but he asserted that it was not clear whether he was talking about an accidental or deliberate act.[93] From the perspective of patient safety it makes no difference. If babies are dying as a result of air embolism, action must be taken to protect other babies. This evidence, like Mr Harvey's evidence about Dr Brearey in the meeting of 11 May 2016, seeks to put responsibility on the doctors for the fact that he failed to respond appropriately to their concerns.

14.64 In my view, it is plain that all in the room, including Mr Harvey, understood that the doctors were talking about deliberately harming babies. It is overwhelmingly likely that he did not accept the concerns being expressed by experienced doctors. This was a very ill-judged approach.

14.65 The meeting notes also record Mr Chambers making the comment "*can we explore more* [before] *police*?".[94] Mr Harvey gave evidence that he did not think Mr Chambers was leaning towards any particular outcome with that comment. He described it as Mr Chambers "*putting in a challenge*" and a "*request to explore the full range of the options*", which he would expect him to do as Chief Executive.[95] I do not accept that. This was not a time for "*putting in a challenge*". Mr Chambers was in charge. He knew what was under discussion. He wanted to do anything other than call the police.

14.66 Mr Chambers had already challenged those at the meeting, "*Why did we call the police?*"[96] (the word 'not' has been inadvertently omitted). The question is an interesting one. It was not one he asked in the meeting of the executives; it was asked in the meeting that included the doctors. Responsibility for calling the police over such a serious matter lay with the senior executives. He was the Chief Executive; no one would bring in the

police without reference to him. It seems that Mr Chambers considered that the fact that the doctors had not themselves called the police undermined what they were saying. It was at one of the meetings at this time that both Dr Brearey and Dr Jayaram said that, when presented with their concerns about Letby being responsible for collapses, Mr Chambers said: "[T]*hat would be convenient.*"[97] There is no note of that remark, but it is consistent with Mr Chambers' approach to the doctors generally. When it was put to Mr Chambers that he had said words to the effect, 'that would be convenient', he said he could not remember saying that.[98] I find that he did say it.

14.67 Dr Brearey asked if Letby could be moved but was told that she could not be excluded. This was ridiculous. Of course she could have been moved from the neonatal unit, just as she was moved two weeks later. The advice to the contrary was simply wrong. It is not clear who gave it but whoever it came from, it should have been challenged and a plan developed to remove Letby from the neonatal unit. Consideration was given to downgrading the unit and conducting a further review before going to the police. Mr Harvey was in favour of an early review.[99]

14.68 At the end of the meeting, it is recorded that the risk was accepted.[100] This is puzzling. I infer that it was the executives who were accepting the risk. Even if the executives thought that the doctors' concerns about Letby were unlikely to be borne out, they could not be sure of that. The risk being accepted was the risk of harm and death to more babies. Everything should have been done to remove it. Carrying out an investigation could not do that; neither could downgrading the unit. If the risk was from Letby, however unlikely the executives thought that to be, that had to be dealt with by removing her from the neonatal unit while the other investigations were being carried out, as eventually occurred.

14.69 Mr Cross notes Mr Chambers saying towards the end of this meeting: "*Nurse cannot be excluded.*"[101] I have dealt with that approach already. Mr Chambers is then noted to have concluded the meeting by saying: "*Ty* [thank you] *to clinicians.*"[102] This was window dressing (see paragraph 39.24, Chapter 39).

14.70 It is striking that Mr Harvey and Ms Kelly were both satisfied early on 29 June 2016 that the police should be called. It is not clear that they ever said that directly to Mr Chambers. They should have done. The police were not called that day because Mr Chambers was against it. I infer that by late afternoon Ms Kelly and Mr Harvey had changed their minds.

Effect on relationships on the neonatal unit

14.71 Ms Powell said that as at 28 June 2016, when Dr Brearey made it clear that the consultants did not want Letby back on the unit, she felt that the working environment was untenable because, in contrast to the consultants' position, "[t]*he majority of the* [nursing] *staff*" felt that Letby was "*not responsible*".[103]

14.72 In contrast to the positive relationships between nurses and doctors working with babies on the unit, Nurse T described the relationship between the neonatal unit managers and the consultants, following the raising of concerns about Letby, as "*adversarial*". In particular, the relationship with Dr Jayaram and Dr Brearey, in her view, "*didn't feel like a cohesive, co-operative working relationship*".[104]

14.73 Nurse T said that, when the consultants raised concerns about Letby, the divide of opinion felt like "*nurses against doctors*",[105] but the overwhelming balance of the evidence from nurses and doctors was that this was not a feature of the day-to-day working of the neonatal unit, where nurses and doctors worked well together. Rather, any acrimony or sense of division between nurses and doctors appears to have resulted from the manner in which the Executive Team responded to consultants' concerns and the handling of the subsequent grievance process, including the deliberate misleading of nurses as to the reasons for Letby's removal from the neonatal unit.

30 June 2016

14.74 At 08:25, the morning after the meeting with the executives where he had raised air embolism as a concern, Dr Jayaram sent an email to the consultants, Ms Powell and Ms Anne Murphy. He attached an article titled 'Pulmonary vascular air embolism in the newborn' by S.K. Lee and Alan Tanswell.[106] By this point, Dr Jayaram had seen a similar rash on Baby A and Baby M. This was only days after the deaths of Baby O and Baby P.

14.75 On that same morning of 30 June 2016, the senior executives met Sir Duncan.[107] The purpose of the meeting, according to Mr Harvey, was to update Sir Duncan.[108] As set out earlier in the Report, Sir Duncan had read the Brigham Review in December 2015. It is not apparent that he remembered it at the time of this meeting. Sir Duncan was told that, if the proposed RCPCH review were conducted within two weeks and if Letby were removed from the neonatal unit with no contact with patients, the doctors might be persuaded not to go to the police.[109] This was odd for two reasons. First, there is no reference in any of the notes of any of the meetings of the doctors to them saying that they were going to the police. There is a record of them having been asked whether they favoured going to the police and they all said they did (see paragraph 14.55). But there is no suggestion that they were intending to take that course unilaterally. It was a matter for the senior executives. Second, the issue here was the safety of babies. At the meeting it was agreed that the unit would be downgraded to Level 1 and the RCPCH review would be commissioned.[110]

14.76 In the afternoon of 30 June 2016, there was a further meeting. Present were Sir Duncan, Mr Chambers, Dr Semple, Ms Fogarty, Dr Davies, Mr Harvey, Ms Kelly, Ms Burnett, Ms Hodkinson, Mr Cross, Dr Jayaram, Dr McCormack, Dr Brearey, Dr Saladi and Dr Brigham.[111] By this stage the executive plan was in place. The handwritten notes of the meeting (by Mr Cross, Ms Kelly and Ms Hodkinson) recorded that Mr Chambers and Mr Harvey had set out their plans to downgrade the unit and referred to the plan for the RCPCH to carry out a review.[112] Mr Harvey spoke about the timing of the proposed RCPCH review. Mr Cross records him saying:

> "*Review can be done in August 16. 2 Neonatologists, 2 nurses, 1 lay. Immediate feedback. Full report 2/3 weeks. College drafting proposal this pm. IH* [Mr Harvey] *to draft ToR* [Terms of Reference]*, visits and* [interviews] *and pulling data to feed into review.*"[113]

14.77 The meeting was intended to persuade the clinicians of the wisdom of those two paths over a referral to the police. In his email to Ms Townsend at 12:08 on 28 June 2016, Dr Brearey had already said that the senior paediatricians entirely agreed with "*Ian's suggestion for an external peer review and the RCPCH have undertaken these in other*

units recently. However, it does not address our immediate concerns regarding patient safety."[114] Dr Brearey accepted that care on the unit was not perfect, but he also sought to defend the quality of care generally and pointed to how well it was regarded across the network.[115] It appears that no one paid any attention to that. At the same meeting, Dr Brearey is recorded as saying that it "[d]*oes not matter what level with concerns about a member of staff! Can reduce cots, HDU* [high dependency unit], *gestation BUT still not safe* [because] *of staffing*".[116]

14.78 There was no doubt about the nature of the concerns held by the clinicians: that a nurse was deliberately harming babies. The notes record references to the cases of Beverly Allitt and Harold Shipman.[117] Having heard the evidence from those present at the meeting, I am satisfied that these names were raised as examples of situations where a doctor and a nurse murdered patients. This should have served as a reminder that what seems unthinkable does sometimes happen. Sir Duncan was Chief Executive of the NHS at the time of the Clothier report into the Beverly Allitt murders. He wrote to all health authorities on 11 February 1994 to draw their attention to the inquiry report.[118]

14.79 The paediatricians agreed to work with the executives on plans for the regrade of the unit but remained concerned that the risk that Letby presented to patients would not be altered by the unit being downgraded.[119] Nor did they consider that the proposed review by the RCPCH could properly investigate their concerns.[120] They would be proved right about that. They pointed out (again) that the pattern of collapses had changed when Letby was moved from nights to days in April 2016.[121] This was very significant information, which was repeatedly ignored by the Executive Directors. Like the paediatricians, Dr McCormack predicted that the RCPCH review would not be adequate for the task being suggested.[122] These views, subsequently proved to be right, were also ignored. He is recorded as saying: "*I don't think it's fair to ask College to do forensic/ nurse issue. Team will come in – maybe minor issues … Police* [have] *better investigation. Difficult*."[123]

14.80 Mr Chambers said that, if the Trust brought in the police, this would have severe consequences for the unit, the hospital, staff, patients and other neonatal units in the region.[124] The clinicians were assured that a forensic review of Letby would be carried out and her position would be discussed again in two weeks' time. There was here a false equivalence between the consequences of bringing in the police (which in the event bore no relation to the truth) and the consequences of not doing so. If the latter course was chosen (and Letby was left on the neonatal unit) there was, in light of the doctors' concerns, a risk of the sudden and unexpected death of a baby. Risks to reputation and disruption are not of the same order. The assertion that bringing in the police would have severe consequences for the unit, the hospital, staff, patients and other neonatal units in the region, as before, was wrong. In light of later events, the references in the meetings to what would be upsetting for the families seem to be convenient arguments to justify the decision not to bring in the police. The executives had no idea of how the families would feel. As Mr Harvey and Ms Kelly acknowledged, the distress caused to the families by delays has been significant.

14.81 In their opening statement, the executives acknowledged that at the end of June 2016 they had been told that there were concerns Letby was directly involved with the deaths.[125] And yet by the end of this meeting the decision was, exactly as planned earlier

in the day, not to call the police, to conduct an internal review (which would be badged Silver Command in the style of police investigations) and to engage the RCPCH's invited review service.

14.82 Also on 30 June 2016, there was a series of meetings involving various combinations of senior executives, Ms Rees and senior figures from HR, Risk and Communications. At one such meeting it was noted that this was Letby's last day at work before her annual leave.[126]

14.83 Ms Kelly emailed CQC and referred to her telephone call earlier that day.[127] She explained that the hospital had identified an increase in neonatal deaths in respect of which "*the reviews* [had] *failed to identify any cause or common theme*". Ms Kelly informed CQC that an internal review and a peer review had been conducted and (wrongly) that the two reviews were included in the recent CQC inspection data packs. She told CQC that the hospital had commissioned an independent RCPCH review and set out a long list of actions that were being taken, including a deep dive into staffing rotas and a "*Robust Comms plan*" for (amongst others) patients in the unit and the families of babies who had died.

14.84 During that year, nurses on the neonatal unit were misled about what was going on. The RCPCH took on a task for which it was ill-equipped, the product of which was to be relied on much later as exonerating Letby when it did nothing of the sort. Letby brought a grievance which was badly handled and ultimately led to the doctors being asked to apologise and/or mediate. The police were not called until Mr Chambers could see that he had no choice nearly a year later. In the meantime, his focus was on bringing the doctors to heel, moving on, and drawing a line under the concerns about Letby.

Endnotes

1 INQ0015537/4

2 Witness statement of Alison Kelly INQ0107704/91/para 300

3 Alison Kelly 25 November 2024 132/16-20; Witness statement of Alison Kelly INQ0107704/91/para 300

4 INQ0015537/4

5 Witness statement of Alison Kelly INQ0107704/90/para 299

6 Ian Harvey 28 November 2024 152/7-18

7 INQ0003142/2

8 Alison Kelly 25 November 2024 132/1 to 133/8; Ian Harvey 28 November 2024 149/3-19

9 Ian Harvey 28 November 2024 151/14-16

10 INQ0005727

11 Eirian Powell 17 October 2024 140/14-22

12 INQ0005745

13 INQ0002879/64

14 Karen Rees 21 October 2024 151/15-17

15 Alison Kelly 25 November 2024 137/13-17

16 Alison Kelly 25 November 2024 138/3-7

17 Alison Kelly 25 November 2024 136/9-25

18 Alison Kelly 25 November 2024 139/7-10

19 Alison Kelly 25 November 2024 139/10-14

20 INQ0001333/1

21 Karen Townsend 4 November 2024 44/5-9

22 INQ0005745

23 INQ0005745

24 Ian Harvey 28 November 2024 154/13-15

25 INQ0005749/3

26 INQ0005749/2-3

27 INQ0005749/1-2

28 INQ0009615/5

29 INQ0002879/38

30 Eirian Powell 17 October 2024 163/14

31 Dr Jim McCormack 8 October 2024 52/14-25

32 Eirian Powell 17 October 2024 164/3

33 INQ0014306/1

34 INQ0005749/1

35 INQ0005749/1

36 INQ0003112/9

37 INQ0003112/8

38 INQ0003360

39 Witness statement of Stephen Cross INQ0107707/13/para 47

40 INQ0003360

41 Ian Harvey 28 November 2024 156/3 to 158/24

42 Anne Murphy 21 October 2024 88/3-8; Dr Jim McCormack 8 October 2024 66/13-22; Dr Stephen Brearey 19 November 2024 135/16 to 136/1

43 Ian Harvey 28 November 2024 156/12-17

44 INQ0003112/2

45 Ian Harvey 28 November 2024 159/19-20

46 Ian Harvey 29 November 2024 47/19-21

47 INQ0003112/1

48 INQ0003112/1

49 Dr Ravi Jayaram 13 November 2024 103/5-10

50 Witness statement of Ian Harvey INQ0107653/51/para 208

51 Ian Harvey 28 November 2024 159/10-11

52 INQ0014249/1

53 INQ0047571/1

54 Alison Kelly 25 November 2024 140/24 to 141/3

55 Ian Harvey 28 November 2024 159/24 to 160/4

56 Ian Harvey 28 November 2024 195/7-9

57 INQ0014249/1

58 Witness statement of Tony Chambers INQ0108723/3/para 11

59 INQ0015537/5

60 Alison Kelly 25 November 2024 84/14-18

61 INQ0003364

62 INQ0015537/5-6

63 INQ0106816/5-7

64 Sir Duncan Nichol CBE 2 December 2024 32/13-17

65 INQ0106816/5

66 INQ0106816/5

67 INQ0106816/5

68 INQ0015537/6

69 INQ0106816/6

70 INQ0106816/6

71 INQ0106816/6

72 INQ0006769/1-2; Annemarie Lawrence 22 October 2024 252/5-23

73 INQ0003251/2

74 Lorraine Burnett 5 November 2024 135/15-19

75 INQ0106816/6

76 INQ0106816/6

77 Tony Chambers 27 November 2024 30/12-24

78 INQ0003371/2

79 Witness statement of Ruth Millward INQ0108715/4/para 16; Witness statement of Tony Chambers INQ0108723/8/para 22

80 INQ0106816/7

81 INQ0106816/7

82 INQ0106816/7

83 Witness statement of Stephen Cross INQ0107707/14/para 49

84 INQ0106816/7

85 INQ0003371/1

86 INQ0003371/2

87 Ian Harvey 28 November 2024 160/17-19

88 Ian Harvey 28 November 2024 90/23

89 INQ0003371/1-2

90 INQ0003371/1-2

91 Alison Kelly 25 November 2024 22/7-8

92 Alison Kelly 25 November 2024 22/14-16 and 45/19-22

93 Ian Harvey 28 November 2024 160/22 to 161/9

94 INQ0003371/2

95 Ian Harvey 29 November 2024 167/14 to 168/20

96 INQ0003371/2

97 Dr Ravi Jayaram 13 November 2024 108/13-23; Dr Stephen Brearey 19 November 2024 140/7-22

98 Tony Chambers 27 November 2024 22/23 to 23/10

99 INQ0003371/3

100 INQ0003371/3

101 INQ0003371/3

102 INQ0003371/3

103 Eirian Powell 17 October 2024 143/12 to 144/13

104 Nurse T 14 October 2024 27/3-13

105 Nurse T 14 October 2024 66/16-17

106 INQ0102065/2

107 INQ0003361/1

108 Ian Harvey 28 November 2024 161/16-18

109 INQ0015639/51

110 INQ0003361/2

111 INQ0015639/54; INQ0003362/1

112 INQ0003362/1

113 INQ0003362/1

114 INQ0003116/2

115 INQ0003362/3; INQ0015639/55

116 INQ0003362/1

117 INQ0015639/55

118 *Hansard*, HC, Deb 11 February 1994, vol. 237, col. 587 (https://hansard.parliament.uk/commons/1994-02-11/debates/8f6d3c4b-520e-4ed0-a7a8-42030bd0fe79/BeverlyAllitt(Report)): "*The chief executive of the national health service, Sir Duncan Nichol, has also written today to all health authorities and trusts to draw the report to their attention*."

119 INQ0003362/2-4; INQ0015639/55 and 57

120 INQ0015639/55

121 INQ0003362/4

122 INQ0015639/59

123 INQ0003362/4

124 INQ0015639/59

125 Written Opening Statement on Behalf of the Senior Management Team 30 August 2024 4/para 13

126 INQ0003361/3

127 INQ0017411/1-2

Chapter 15. Early July 2016: Discussions; downgrade of the neonatal unit; Silver Command; risk and reputation

15.1 On 4 July 2016, there were a number of discussions between the Chair, executives and consultant paediatricians and obstetricians at the Countess about plans to downgrade the neonatal unit to Level 1. Part-way through the meeting, Dr Brearey is noted as saying: "*Difficult issue re:comms for parents whose babies have died … Team going over to Liverpool* [where Baby R and his parents remained]*. Previous families – up to a year ago and beyond.*" He asked: "[Could] *these families be involved in the review. Duty of candour links to this*."[1]

15.2 In a box to the right on the page is written "*Action, TOR* [Terms of Reference]". Dr Brearey was unaware that the Terms of Reference for the review to be undertaken by the RCPCH had already been drafted, without reference to the paediatricians or the parents. The parents were never involved. The paediatricians were involved only in the sense that they were interviewed by the review team.

15.3 The decision to downgrade the unit was taken by the Board of Directors at the Countess. After coordination between the hospital, NHS West Cheshire Clinical Commissioning Group, NHS England Specialised Commissioners and the Cheshire and Merseyside Neonatal Network, on 7 July 2016 the hospital's neonatal unit was downgraded so that babies were now to be admitted from 32 weeks' gestation instead of from 27 weeks'. This was thought likely to reduce the risk of complications due to prematurity. The number of cots on the neonatal unit was reduced from 16 to 13 (with 3 intensive care cots being closed), and so the overall number of potential admissions was reduced. Ms Millward informed CQC.

15.4 A press announcement was made that day informing the public that the neonatal unit was being downgraded after "*an increase in neonatal mortality rates in 2015 and 2016*" and that the RCPCH had been invited in to review the unit.[2] It is lamentable that the Trust was able to contact only some four or five parents prior to the public announcement.[3]

15.5 Mother C found out about the increased mortality rate on the neonatal unit and the RCPCH report via a family friend who had seen the press announcement in a local newspaper. She told the Inquiry:

> "[I]*t was July 2016 that my husband's friend sent him a WhatsApp message which contained a picture of an article from the Chester Chronicle, and the message was to ask whether we were involved in the investigation that was going on at the Countess of Chester, which covered the period of time during which Child C had died, and it was relating to an investigation of increased deaths on the neonatal unit. And up until this point, we had absolutely no idea that there was any kind of concern or investigation taking place, or even that there had been an increased number of deaths, so this was extremely distressing to find out, and especially to find out in this way.*"[4]

15.6 Mother C considered "*we should have been told* [about the RCPCH review] *as soon as it was decided that our son's death was going to be looked into by anybody, and definitely before any press release was made*".[5]

15.7 After learning about the RCPCH investigation via the press, Mother C told the Inquiry she went to the Bereavement Office at the Countess. She informed the staff that she:

> "*needed to speak to somebody as a matter of urgency about the article that had gone in the Chester Chronicle, and the first question that I was asked was whether I was from the press, which was an awful question to be asked at that point. And I said no, that I was a bereaved parent, and I wasn't leaving until somebody had the decency to talk to me about the article that had gone in the newspaper.*"[6]

15.8 Mother C described what happened next as follows:

> "*So this lady went and got Sian Williams and Alison Kelly, who came down and spoke to me. It was a fairly short meeting, to my recollection, where I was told by them that there was an investigation being done by the Royal College that was more of a formality because there'd been a very small increase in number of deaths, that it was looking at various sort of logistical things like staffing levels and that sort of thing, and that they weren't really expecting anything to come from it, that they had tried to contact me, which I challenged because, you know, I'd gone in there and said, 'How can you let me read this in the newspaper? I think that's absolutely outrageous.' I was really upset. They said that they had tried me on my landline once, and I challenged this as well. I said, you know, 'Do you really think that that is acceptable to just try somebody once?' And the response was that they didn't know whether parents would want to know, so they didn't know how far to take the attempts to contact parents to let them know that this was happening. And I was quite taken aback. I didn't see any sort of malice in the way that they'd behaved, but I saw absolute breathtaking ignorance. And I said, you know, 'That is not your choice to make, who would want to know, when you're talking about the death of people's children.'*"[7]

15.9 Ms Sian Williams stated she had no recollection of this meeting. However, she accepted that, if Mother C's memory was correct, at the point in the timeline that Mother C said they met, Ms Sian Williams would have been aware that the consultants had concerns of deliberate harm and she thought the police should have been called. Ms Sian Williams accepted that, if she had said to Mother C that there was a small increase in deaths which they did not think was significant and that the review was a 'formality', then this was

misleading.[8] She told the Inquiry that she wanted Mother C to know how "*desperately sorry I am for the lack of communication, for the whole situation, that on reflection how much it could have been so different*".[9]

15.10 Ms Kelly did not dispute Mother C's evidence that a meeting took place; however, she could not recall meeting any of the families of the babies that had died. It was put to her that, if Mother C was right about what she had been told, it was misleading. She replied in general terms: "*I think one of the things that one of the key elements and it's in my reflections of -- of this case is that we didn't get the communication right with Families and we didn't get the balance right and I think that is an example of where we didn't get it right.*"[10] Not for the first time the language falls short of the reality. This was not a question of balance. Mother C was being misled. It was not a question of not getting the communication right; it was about not being straight with parents who had the right to know what was going on.

15.11 As set out in greater detail in Chapter 20, by early July 2016, Mr Harvey was in discussions with Ms Sue Eardley, Head of Invited Reviews for the RCPCH and a member of the review team. On 12 July 2016, Ms Eardley asked whether the parents of the babies who had died would be expecting the RCPCH to meet them, adding that it would not be usual practice for them to do so. She also sought confirmation that the duty of candour arrangements were in place.[11] This was consistent with Dr Brearey's observations on 4 July 2016.

15.12 In his reply of 13 July 2016, Mr Harvey said that the hospital had made every effort to contact the parents of every baby who had died, before the story was in the local newspaper, but address and phone-number changes meant that they were unable to contact all. He added that part of the conversation was that they would share the findings of the RCPCH report with them. He said that, to his knowledge, none had requested to see the review team. He did not add, as he should have done, that the parents had not been given that option. I deal further with communication from the hospital to the families in Chapter 20 from paragraph 20.145.

Mr Harvey's analysis of neonatal mortality

15.13 In early July, Mr Harvey produced a document headed 'Analysis of NNU Mortality Rates'.[12] He had prepared this over the preceding days without reference to clinicians on the unit. The aim of the analysis was threefold:

a. To review the level of any increase in mortality levels in the neonatal unit during 2015/16 and whether this represented normal variation or a significant change that broke with long-term trends.

b. To evaluate activity levels during 2015/16 as a possible contributory factor.

c. To evaluate certain measures of acuity in the neonatal unit during 2015/16. Was the condition of neonates admitted to the neonatal unit more acute than previously?[13]

15.14 The following facts do not appear in the analysis: the admissions criteria had not changed since the unit was opened in 2010, and the consultant paediatrician team had remained the same throughout the period.

15.15 In the background section to his document, Mr Harvey recorded that consultants had raised concerns about an apparent increase in mortality rates in the neonatal unit.[14] Mr Harvey had sent amended Terms of Reference to the RCPCH on 7 July. Instead of the original draft, which had read "*Are there any possible common factors linking the recent neonatal deaths?*", he suggested: "*Are there any identifiable common factors or failings that might in part, or in whole, explain the apparent increase in mortality in 2015 and 2016*?"[15] These two occasions in July were the first to see a description of the increase in the number of deaths or the increased mortality rate as 'apparent'. Whichever way it was looked at – as raw numbers or as a mortality rate – it was much higher than it had been in previous years. Every person working on the neonatal unit knew that. So did all the executives. Mr Harvey said, in evidence, that these were small numbers and that it was difficult to know whether they were statistically significant. That is true, but what mattered was that more babies had died in that period than had died in comparable periods before. The deaths needed individual and collective attention.

15.16 The fact of the increase in the number of deaths was reflected in the analysis. In addition, Mr Harvey presented a graph on neonatal admissions showing that, between 2013 and 2015, the average number of admissions to the unit each month was 40.[16] On the question of whether the neonatal unit had been under more pressure (i.e. subject to higher activity levels), the analysis showed that it experienced higher than average admissions for five out of six months in the second half of 2015. In July, August, October, November and December 2015, the admissions were higher than average, ranging between approximately 40 and 55 in those months. The chart also shows admissions for the first half of 2015 (including the month of June, when Baby A, Baby C and Baby D died) were significantly below average.[17] The analysis concludes, on that issue, that higher admissions "*may have been a contributing factor*", but it should be noted that 2014 saw higher admission numbers, including a significant peak in December, without a similar increase in the number of mortalities.[18] For example, there were just under 60 admissions in November 2014 and a peak of 65 admissions in December 2014. In short, this meant that a higher-than-average number of admissions did not, on its own, explain a higher number of deaths. The number of deaths did not rise when admissions rose. Activity was irrelevant to the deaths of Baby A, Baby C and Baby D.

15.17 The question whether increased acuity (the complexity of the condition of the baby) had contributed to the number of deaths was considered at some length. There was an increase in high acuity days, when they looked at care days (the number of days a baby spends in the neonatal unit) per month in Level 1 (intensive treatment) cots and Level 2 (high dependency) cots. Between May 2015 and March 2016, care days dropped below the long-term average in only one month. Dr Brearey pointed out to Mr Harvey, and later to the Inquiry, that more care was required because of the actions of Letby during that period. Mr Harvey concluded that "*an increased and sustained acuity level may be a contributory factor*" to the increased number of deaths.[19] Unsurprisingly, he did not seek to attribute any single death to the acuity level.

15.18 Mr Harvey's analysis was incorporated into a document for the Executive Team by Ms Kelly and Ms Millward in July 2016: 'Position Paper: Neonatal Unit Mortality', which was considered later in the month.[20]

Silver Command

15.19 The executives decided to conduct an investigatory exercise under the banner Silver Command or (sometimes) Silver Control, both more usually found in policing. Mr Harvey explained the purpose of the Silver Command exercise: "[W]*e were trying to find explanations for the increase in mortality … we envisaged feeding the data into the subsequent Royal College review*."[21]

15.20 The investigation took place over about ten days. It involved staff at middle and senior level. There were extensive briefings. Over 30 people attended a briefing on 8 July 2016. They included Sir Duncan Nichol, and members of all the following teams: Executive, Risk and Patient Safety, Midwifery, HR, Communications and Operations. Only Dr Gibbs was invited from the team of paediatricians. There was no one from the safeguarding team, save for Ms Kelly, who, as I have said earlier, did not consider this a safeguarding matter.[22] Sir Duncan was to say in due course that the police should have been informed about the doctors' concerns at this time.[23]

15.21 The activity planned/carried out included:

a. Meetings between data analysts, Ms Kelly, Dr Gibbs, Ms Anne McGlade (née Martyn; Manager, Children's Ward), Ms Sian Williams, Ms Millward, Ms Lawrence and others.[24]

b. A review of non-fatal unexpected collapses, where the baby was transferred out of the neonatal unit, carried out by Dr Gibbs and Ms McGlade, a paediatric (but not neonatal) nurse.[25]

c. Ms June Henderson, Bereavement/Patient Services Coordinator, compiled two spreadsheets of deaths that were reported to her by the dedicated nurse or midwife between 2014 and 2016.[26]

d. Ms Sian Williams conducted a staff matrix analysis, at the time, of identified incidents. This was completed with Ms Fogarty.[27]

e. Ms Appleton-Cairns, Deputy Director of HR, and the HR team reviewed the personnel files of staff on the neonatal unit.[28] They found nothing relevant.

f. Ms Millward and Ms Kelly collated the details from various tasks into a position paper titled 'Neonatal Unit Mortality'.[29] This was to be reviewed at the Executive Director Group meeting on 13 July 2016.[30]

g. Ms Sian Williams to ring bereaved families.[31]

h. Briefing of the media and external stakeholders. Creating helplines for patients, families and the public in relation to the increased neonatal mortality and downgrade of the unit.[32]

i. Ms Kathryn De Beger, Occupational Health Manager, had offered occupational health support to the neonatal unit.[33]

j. Mr Harvey, along with data analysts, produced a PowerPoint presentation containing data about patient acuity, staffing, admissions and activity on the neonatal unit.[34]

15.22 Mr Harvey confirmed in evidence that the Silver Command investigation did not provide the answer to the question of whether deliberate harm had been inflicted.[35] Nor did it provide any reason for the rise in deaths either. Reflecting on Silver Command, Mr Harvey told the Inquiry: "[I] *regret that I didn't stick with my original view that we should have gone to the police*."[36]

15.23 In his written evidence, speaking of his review with Ms McGlade, Dr Gibbs explained that: "*Concentrating on the cohort of babies who required transfer from the NNU would identify some of the babies who had suffered non-fatal collapses. It had been my impression, and that of my consultant Paediatric colleagues, that Letby had been involved in many of the non-fatal collapses*."[37] The Silver Command action log records that cases of concern from Dr Gibbs' and Ms McGlade's review would be shared with Ms Sian Williams, who, along with Ms Fogarty, was conducting a staff matrix analysis.[38] Dr Gibbs said he was told that "*a staffing analysis would be done afterwards*".[39]

15.24 Dr Gibbs said in evidence that there were six collapses that were unexpected or unusual in some way and which required the transfer to another hospital of the baby.[40] Dr Gibbs also conceded it was not a forensic review and that the babies' notes were reviewed quickly.[41]

15.25 Ms McGlade considered the identification of six unexpected deteriorations or collapses in their review as "*quite a lot*".[42] Like Dr Gibbs, she understood that her review was to be analysed alongside a staffing grid. Ms McGlade was expecting feedback on the review and on how it fitted into the larger picture; she never received it.[43] Both Dr Gibbs and Ms McGlade accepted that their review was unfinished. Nevertheless, information about non-fatal (as well as fatal) collapses was used in the position paper prepared by Ms Kelly and Ms Millward. The position paper included the information that:

> "*Nurse and medical staffing had been matched to all 32 cases within this review, some staff do appear more frequently on the rota in respect of being allocated to those babies who deteriorated but there are no reported competency issues with nursing or medical staff and neonatal skill training is seen as a priority.*"[44]

Dr Gibbs said that the findings of Silver Command were presented on 13 July 2016 at a meeting between the executives and consultant paediatricians. He expected his review to feature in the presentation, but it did not.[45]

15.26 Ms Sian Williams and Ms Fogarty had been tasked with looking at the Meditech electronic patient records of babies who had unexpectedly died or collapsed.[46] Having done so, they concluded that Letby was "*80% more likely to be on duty either during or before a baby collapsed*".[47] Ms Sian Williams presented their findings to the executives. She also approached Mr Harvey privately to tell him about the findings and raised concerns that there was one baby who was stable during the day and then suddenly collapsed on three occasions during the night when Letby was on shift. In response to her concerns, Mr Harvey told her that he was going to check himself.[48] Mr Harvey had no memory of this exchange.[49] I accept that it happened. Ms Sian Williams, who was a straightforward witness, had told Ms Fogarty of her intentions and later said to her that she had spoken to Mr Harvey. Had Mr Harvey checked Letby's shifts, he would have seen that she was on shift at the time of many of the collapses, and of course he knew that already. I accept

he has forgotten the conversation with Ms Sian Williams, as he has forgotten much else. I acknowledge that this was a ferociously busy time for the executives, including Mr Harvey, and he would not be expected to remember everything.

15.27 The Silver Command exercise had absorbed time and energy to no effect. The aim – to find an explanation for the deaths – was not achieved. This should have reinforced the need, now, to involve the police.

15.28 Around this time, Ms Sian Williams, who was anxious about her findings, told the executives about an occasion on which she contacted the police in relation to an incident in adult medicine. She explained that the police handled the matter in a low-key way. She told them about this experience because she felt the executives "*needed to consider the police*". She stated: "*I spoke to Alison Kelly on a number of occasions, one I remember with Karen Rees in my office saying that you need to go to the police and she said 'I have taken advice' and that was it and she wouldn't listen*."[50]

Risk registers

15.29 Until 11 July 2016, there was no entry on any of the risk registers in the hospital setting out the increase in neonatal mortality, which had, by then, been known about for a year. This was a major failure by the Risk and Patient Safety Department, and by Ms Millward in particular.

Policy

15.30 The Risk Management Strategy and Operational Policy set out that the risk registers were intended to be live documents, actively managed and reviewed[51] (see also Chapter 4).

15.31 Ms Millward explained the compilation of a risk register was a relatively new process in 2015. The management of risk was intended to be identified at a local level and escalated first to a divisional risk register and then upwards, dependent on the risk score. A risk with a score of 16 to 25 (or a risk that carried significant concern but had a lower risk score) was to be placed on the Executive Risk Register, and the Divisional Director responsible for the relevant risk would present it to the Corporate Directors Group. The Executive Risk Register was intended to be a list of the most significant patient safety risks facing the hospital. The Risk Scoring Matrix is below in Figure 3.

Figure 3: Risk Scoring Matrix

Likelihood	SEVERITY/IMPACT RATING 1 Insignificant	2 Minor	3 Moderate	4 Major	5 Catastrophic
5 – Almost Certain 1/10	5	10	15	20	25
4 – Likely 1/100	4	8	12	16	20
3 – Possible 1/1,000	3	6	9	12	15
2 – Unlikely 1/10,000	2	4	6	8	10
1 – Rare 1/100,000	1	2	3	4	5

RISK **Very low** **Low** **Moderate** **High**

Source: Countess of Chester Hospital NHS Foundation Trust, Risk Management Strategy & Operational Policy, 11 December 2015[52]

In evidence Ms Millward said that the avoidable death of a child would be graded as catastrophic.[53]

15.32 The Corporate Directors Group had the delegated responsibility of reviewing the Executive Risk Register. On behalf of the Clinical Directors Group, Ms Millward was responsible for preparing the Executive Risk Register and ensuring the risks were reviewed.

What was the risk?

15.33 The mortality rate at the neonatal unit (then a Level 2 unit), between 2010 and 2014, was two to three deaths a year. By the end of 2015, that figure had risen to eight.[54]

15.34 The evidence of Sir David Spiegelhalter was that such an increase in mortality "*would generally be considered sufficient to trigger an alert signal, someone should look at this locally*".[55]

15.35 Whilst all the deaths in 2015 featured on the indictment were reported on the Datix system,[56] and some deaths (Baby A and Baby E) were described on the Datix report as "*unexpected*", no risk was entered on the Urgent Care Risk Register (which included the neonatal unit) to reflect the increase in neonatal mortality during 2015.

15.36 The first reference to an increase in neonatal mortality on the Executive Risk Register was made on 11 July 2016. This was over two weeks after the death of Baby P, the seventh baby that Letby was convicted of murdering, and four months after Dr Brearey

had produced his March 2016 Thematic Review of neonatal deaths and had concluded: "*Some of the babies suddenly and unexpectedly deteriorated and there was no clear cause of the deterioration/death identified at PM* [post-mortem]."[57]

15.37 The entry on the Executive Risk Register was made by Ms Townsend, on 11 July 2016, and it records "*Apparent Increased Mortality within the Neonatal Unit* [for] *2015/16*", with the risk level being rated as 'moderate'.[58]

15.38 A further entry on the Executive Risk Register, made on the same date in July 2016, was: "*Potential Damage to Reputation of Neonatal Service and Wider Trust due to Apparent Increased Mortality within the Neonatal Unit*". Ms Townsend was recorded as the "*handler*" of this reputational risk, which was graded as a 'high' level risk.[59] She said the script for the entry had come from the communications team under the instructions of Ms Kelly and Ms Hodkinson.

15.39 Ms Millward asserted that it was an "*executive decision*" to phrase the risk as "*potential damage to reputation of neonatal service and wider Trust*".* [60] Her understanding of the rationale behind this decision was that, at the time the risk was first created, "*the unit was being downgraded*" and "*that had been put into the public arena and there will have been concern whether, you know, how that would have been received and potentially reported back on, you know, through the press*".[61] Her recollection was that it was Ms Kelly and Ms Hodkinson who had been involved in the decision to put these entries on the Executive Risk Register.†

15.40 When asked why the "*apparent increased mortality within the neonatal unit*" had a lower score of 15 when the reputational risk was graded 20, Ms Millward's evidence was that the risk was scored lower because of the mitigation measures the Trust had taken: "[T]*he unit had already been downgraded at that point and therefore that would reduce, you know, the -- the challenges around those … more high acuity patients coming into the unit … I think that's why that particular risk is scored slightly lower than the one around reputational damage*."[62]

15.41 She added: "[R]*eputational damage and media coverage and so on would be outside the Trust's control so they wouldn't have the same level of assurances around the effectiveness of the controls and that would have meant that would be reflected in the risk always being slightly different*."[63]

15.42 Ms Millward was somewhat defensive when asked why the Trust was so concerned with its reputation, retorting: "[W]*ith respect, why wouldn't the Trust be concerned with its reputation?*"[64] In her view, the Trust's approach to the risk was reasonable.

15.43 Ms Millward accepted in her evidence that the risk of increased mortality on the neonatal unit should have been put on the Executive Risk Register by, at the latest, mid-March 2016, following the finalisation of Dr Brearey's Thematic Review on 2 March 2016.[65]

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* Ms Townsend's evidence was that it was either Ms Kelly or Ms Hodkinson's decision: **Karen Townsend 4 November 2024 61/4-12**.

† Ms Kelly was unable to recall the risk register but accepted the possibility of her involvement: **Alison Kelly 25 November 2024 167/7 to 168/3**; Ms Hodkinson did not remember scripting the risk: **Sue Hodkinson 26 November 2024 105/3-14**.

Had such an entry been made, the Executive Team would have been required to consider what control measure should be put in place to address the risk, prior to the collapses of Baby L, Baby M, Baby N and Baby Q and before the deaths of Baby O and Baby P.

15.44 The hospital's Executive Risk Register of July 2016 noted: "*The trust has a robust process for managing risk and escalating concerns via the risk registers at Divisional and Executive level: this is integrated within the DATIX risk management system which enables a comparison of risk and incident reporting data*."[66] In fact, the failure to place the increased risk of neonatal mortality on the risk register months earlier, when the increase in mortality was first identified, demonstrated that the process was neither robust, nor integrated.

15.45 Not only was no entry made regarding increased mortality rates until July 2016, when the entry was made, it was inaccurate. The recorded risk on the register referred to "*Apparent Increased Mortality*", despite there being no doubt that there was an increase in the number of children dying on the neonatal unit.[67] Further, the recorded risk made no reference to the issue pointed out by Dr Brearey's Thematic Review, that: "*Some of the babies suddenly and unexpectedly deteriorated and there was no clear cause for the deterioration/death identified at PM*."[68]

15.46 Asked to consider the wording of the Executive Risk Register, Sir David suggested in his evidence to the Inquiry that a more accurate characterisation of the number of deaths might be "*increased mortality of undetermined cause*".[69] This is the risk that the executives should have applied their minds to, at the latest following receipt of Dr Brearey's Thematic Review.

15.47 The July 2016 Executive Risk Register, having pointed out what was inaccurately described as an "*Apparent Increased Mortality within the Neonatal Unit*", went on to set out the "*Controls and Actions required*". Under the heading 'Controls', it was noted that the "*Care Quality Commission inspection in Feb 2016 did not highlight any concerns regarding neonatal mortality*".[70]

15.48 This reassurance might have carried some weight had CQC considered the issue of the raised neonatal mortality, or been alerted to, and shown, Dr Brearey's Thematic Review report and dismissed the concerns about the rise in neonatal deaths. However, as Ms Millward accepted in her evidence, as far as she was aware, CQC did not see Dr Brearey's Thematic Review in draft or final form.[71] Neither did any CQC inspectors recall any discussion of an increase in neonatal mortality or concerns about sudden and unexpected neonatal deaths during their inspection visits (see Chapter 7). As such, the apparent reassurance documented on the Executive Risk Register, that CQC "*did not highlight any concerns regarding neonatal mortality*", was misleading.[72] The reassurance was false.

15.49 As a method of alerting the Executive Team to a serious risk relating to patient safety, the system of Datix reporting and risk registers at the hospital failed. Neither did the entry of risks on the register lead to proper consideration of the appropriate controls to be put in place. Notably, it did not lead to a decision by the Executive Team to consider what steps were needed to investigate the cause of the increase in deaths and whether, in view of the concerns that a member of staff may be responsible for causing deliberate harm, the police should be called.

15.50 In the view of Dr Gilby, there was "*no oversight*" of risk by the Executive Team.[73] Her view was that members of the Executive Team should have been more involved with the risk registers at divisional level and asked questions in order to identify the most significant risks.

15.51 No risk register can be completely comprehensive. Risk registers can never be a replacement for a board working with divisions to understand their concerns and questioning whether there are risks to which they need to be alert. As Dr Gilby said, even when presented with a risk register, a board should still ask: "*Have we really captured the issue here?*" and "*Are we talking about the wrong thing?*"[74]

15.52 On 30 January 2017, the consultant paediatricians Dr Gibbs, Dr Jayaram, Dr V, Dr Brearey, Dr Saladi, Dr ZA and Dr Holt posed a question, requesting "[t]*he Board's understanding of the reason for the increased number of unexpected and unexplained deaths on the neonatal unit between June 2015 and July 2016*".[75] The board did not provide a response to this question. It was a question that should have been addressed much earlier. It was also the question that should have been considered by the Board, had the Executive Risk Register properly recorded the risk of an increased mortality rate of undetermined cause on the neonatal unit.

Conclusions on risk and reputation

15.53 Mr Harvey having introduced the word 'apparent', it was then used by others. The increased mortality within the neonatal unit should not have been described on the Executive Risk Register or anywhere else as 'apparent'. It was real. To describe it as 'apparent' was to downplay its seriousness and so justify a lack of effective action.

15.54 It is important that the public has confidence in NHS hospitals. It is imperative that the confidence is well placed. Where, as here, executives focus on the effect of a situation upon reputation rather than the effect of the situation on patient safety, the strong impression given is that reputation has become more important than patient safety.

15.55 Some of the parents perceived that the Countess was focused on its reputation. Mother G commented: "*To my mind, the Countess of Chester Hospital was more concerned about their reputation than about our daughter's life*."[76]

15.56 Mother D was referred to a case review of Baby D's death, which contained the words: "*The Trust also recognises the potential psychological impact to the staff directly involved, and this, in conjunction with the potential impact to the reputation of the Trust, is considered severe harm.*"[77] Mother D commented:

> "*That troubled me. I didn't understand why this is what was concerned -- this -- it was out of order for me to mention anything to do with the reputation and that's one of my concerns because that's when exactly I was trying to push and ask questions, where I felt there was resistance, where things were trying to be played down.*"[78]

15.57 Mother I said: "*They shouldn't have been concentrating on saving their own skins and jobs and reputations. Babies died because someone in an office being paid hundreds of thousands of pounds didn't want the hospital to look bad if they shut*."[79]

15.58 Mother O, P and R said: "[H]*ospital management should have been much more responsive when concerns were raised, rather than ignoring or covering up concerns to protect the reputation of the hospital and the Neonatal Unit*."[80]

15.59 No one could disagree with any of these observations.

Endnotes

1 INQ0004314/3
2 INQ0103147/1
3 INQ0004319/5
4 Mother C 16 September 2024 79/3-15
5 Mother C 16 September 2024 114/21-25
6 Mother C 16 September 2024 80/10-17
7 Mother C 16 September 2024 80/18 to 81/18
8 Sian Williams 5 November 2024 97/9 to 98/7
9 Sian Williams 5 November 2024 100/9-11
10 Alison Kelly 25 November 2024 249/1-7
11 INQ0009615/2
12 INQ0002837/1
13 INQ0002837/1
14 INQ0002837/1
15 INQ0010256
16 INQ0002837/3
17 INQ0002837/3
18 INQ0002837/3
19 Witness statement of Ian Harvey INQ0107653/68/para 276
20 INQ0001888
21 Ian Harvey 28 November 2024 169/22 to 170/2
22 INQ0003174/3
23 Sir Duncan Nichol CBE 2 December 2024 81/12-14
24 INQ0003336/8
25 INQ0006955; INQ0003336/8
26 INQ0003174/3; Witness statement of June Henderson INQ0107817/2/para 4
27 INQ0003336/8
28 INQ0003336/3-4
29 INQ0001888
30 INQ0003336/8-9
31 INQ0003336/2
32 INQ0003336/2 and 7
33 INQ0003336/2

34 INQ0002837

35 Ian Harvey 29 November 2024 178/19 to 179/2

36 Ian Harvey 28 November 2024 170/5-6

37 Witness statement of Dr John Gibbs INQ0102740/105/para 370

38 INQ0003336/8

39 Dr John Gibbs 1 October 2024 145/22-23

40 Dr John Gibbs 1 October 2024 146/8-10

41 Dr John Gibbs 1 October 2024 146/11-23

42 Anne McGlade 16 October 2024 220/18-21

43 Anne McGlade 16 October 2024 221/1-7

44 INQ0004593/10

45 Dr John Gibbs 1 October 2024 147/20-24

46 Sian Williams 5 November 2024 39/7-12

47 Sian Williams 5 November 2024 39/13-16

48 Sian Williams 5 November 2024 41/13 to 42/9

49 Ian Harvey 28 November 2024 169/10

50 Sian Williams 5 November 2024 42/20 to 44/19

51 INQ0014962/3

52 INQ0014962/14

53 Ruth Millward 4 November 2024 108/21-23

54 INQ0005697

55 Prof. Sir David Spiegelhalter OBE 15 January 2025 43/18-20

56 Baby A INQ0000016/1-2; Baby C INQ0000111/1-2; Baby D INQ0000766/1-2; Baby E INQ0000194/1-2; Baby I INQ0000457/1-3; Baby O INQ0008615/1-2; Baby P INQ0008624/1-2

57 INQ0003251/7

58 INQ0049845/2

59 INQ0049845/2

60 Ruth Millward 4 November 2024 121/1-2 and 127/20

61 Ruth Millward 4 November 2024 124/2-6

62 Ruth Millward 4 November 2024 121/7-9 and 124/21 to 125/2

63 Ruth Millward 4 November 2024 125/23 to 126/3

64 Ruth Millward 4 November 2024 127/8-9

65 Ruth Millward 4 November 2024 131/22 to 132/8

66 INQ0049845/1

67 INQ0049845/2

68 INQ0003251/7

69 Prof. Sir David Spiegelhalter OBE 15 January 2025 51/9-10

70 INQ0049845/8

71 Ruth Millward 4 November 2024 128/3 to 130/9

72 INQ0049845/8

73 Dr Susan Gilby 24 February 2025 53/1

74 Dr Susan Gilby 24 February 2025 52/12-19

75 INQ0003095

76 Mother G 18 September 2024 87/11-13

77 Mother D 17 September 2024 48/20-24

78 Mother D 17 September 2024 49/3-8

79 Mother I 17 September 2024 136/21-25

80 Mother O, P and R 25 September 2024 28/9-12

Chapter 16. Meetings on 13 and 14 July 2016

Executives' meeting with paediatricians, 13 July 2016

16.1 On 13 July 2016, there was a meeting between Mr Chambers, Mr Harvey, Ms Sian Williams, Mr Cross, Ms Hodkinson, Dr Saladi, Dr Jayaram, Dr Gibbs and Dr ZA.[1] At this meeting, the executives persuaded the consultants not to go to the police, but to agree instead to further internal investigation; to the RCPCH review; and to Letby being allowed to remain on the neonatal unit under supervision. At the same time, the possibility of CCTV was being investigated.

16.2 The meeting discussed the deaths of babies in 2015 to 2016. Mr Chambers accepted there was a correlation with Letby's presence on duty, but said they had now identified possible contributory causes, including a change in activity and acuity, plus staffing level challenges and a culture of coping.[2] As I have said earlier, no deaths could be attributed to staff shortages, nor could they have been caused by a culture of coping, something which characterises many NHS wards. The nursing staffing levels at the Countess were slightly better than in all other comparable hospital neonatal units in the region. The Board were aware from the business case that more consultants had been requested and refused. I infer that it was not then suggested this posed a risk to patients. As to the increase in activity and acuity, there was no good evidence that either of these had contributed to a single death, still less that they had caused some or all of them.

16.3 Mr Harvey was recorded as saying, in respect of the correlation with Letby, that if this involved a doctor there would be a process of assessment and supervision, and they "*would not jump straight to police*".[3] This reveals, again, that Mr Harvey did not seriously consider the possibility that Letby was harming babies deliberately. Were a doctor suspected of deliberate harm, safeguarding processes would be initiated, and the police would be called as a result, or the police would be called immediately. Mr Chambers was recorded as saying that, a week earlier, the only option was going to the police, but now they had more information, it could be managed differently.[4] Two things arise. First, up until that point, Mr Chambers was reluctant to go to the police. He had already persuaded Ms Kelly and Mr Harvey to this view on 29 June 2016, and was intent on persuading the consultants. Second, he did not seem to have understood that the 'more information' did not explain any of the deaths.

16.4 Dr Jayaram said that he did not think the information explained the causes for the babies' deteriorations, and that they must not be blinkered to "*the unspeakable*".[5] The consultants sought reassurance as to the supervision which would be given to Letby, if she were to remain working with patients, and were given a "*cast iron*" assurance that there would be "*total supervision*".[6] On that basis, the consultants agreed to the

executives' proposals, with Dr Gibbs saying he did not think there was a need for whistleblowing, and Mr Chambers concluding with the remark: "[P]*ressing the doomsday button would probably cause more harm.*"[7] The "*more harm*" was not explained. Had Mr Chambers reflected on his remark, he would have had to accept that, when weighing up the harm in, on the one hand, the risk of serious injury or death and, on the other hand, a report to the police that came to nothing, the balance would surely come down in favour of the latter. Calling the police was not pressing the doomsday button. On the contrary, that description would apply to leaving on the neonatal unit a person who was suspected (in good faith) of having harmed babies. In the end, the consultants agreed to the RCPCH review, and to Letby remaining on the neonatal unit, as long as she was fully supervised.

Executives' meeting, 14 July 2016

16.5 On 14 July 2016, the Executive Team met.[8] Mr Chambers, Ms O'Neill, Ms Hodkinson, Ms Burnett, Ms Kelly, Mr Ian Bett (Programme Director, Model Hospital), Mr Holden and Mr Cross were all there. Mr Chambers and Ms Hodkinson provided an update on the position regarding the neonatal unit and Letby. This included that they would not be going to the police at present, but there would be enhanced security (which included CCTV), supervision of the nurse, and continuous review of the neonatal unit, with weekly dashboards which highlighted any concerns or incidents to the Executive Team, including monitoring of transfers.

16.6 Ms Hodkinson reported on the need for complete supervision of Letby. Ms Burnett noted that relationships had broken down between nurses and doctors, adopting the narrative from Ms Rees and Ms Powell. That the nurse managers did not agree with the doctors was clear. As for the nurses on the neonatal unit, Dr Gibbs said some of them were upset, but they remained courteous and professional with the consultants at all times.[9] Ms Powell said that the nurses did not know about the allegations.[10] This is highly unlikely. Even if Ms Powell had not told them, and I infer that she had not, they all knew about the increase in mortality, and many were discussing the reasons for it. I bear in mind the evidence of Dr Lambie about the nurses gathered round the computer (see Chapter 5). This was the sort of information that would be the subject of gossip and speculation. That the nurses were upset is not in dispute. They must have had some idea of what was going on.

Meeting with Letby, 14 July 2016

16.7 Ms Hodkinson, Ms Sian Williams and Ms Powell met on 14 July 2016 and agreed that Letby was to be supervised at all times and not allowed to care for a baby unsupervised until 31 August 2016. They also discussed a proposal to install cameras in the neonatal unit.[11]

16.8 Later that day, Ms Sian Williams and Ms Powell met Letby. After the meeting with Letby, Ms Sian Williams sent a letter to Letby, drafted by Ms Hodkinson, confirming what had been said in the meeting.[12] Letby was already aware of the increase in mortality rates, as she confirmed, and the fact that some of the cot spaces had been closed. Ms Sian Williams and Ms Powell said that the collapse and deterioration of babies had not been explained, and this was a serious concern. Letby was told that a review had revealed "*that a small number of staff were regularly involved in the care of the babies concerned.*

Their involvement was either on the shift, or the shift before, a baby had unexpectedly collapsed or deteriorated … the review has identified you as being more regularly involved in the care of babies concerned."[13]

16.9 Ms Sian Williams and Ms Powell explained that:

> "*a decision has been made to provide additional support to all of the staff, including you, who have been identified in the review. The Trust will provide all staff who had been identified in the review as having had* [regular] *involvement in the care of the babies concerned, with a period of supervised practice. This will involve a review of all clinical competencies.*"[14]

Letby was informed that she would be the first nurse to undergo this process "*due to you being identified in the review as having been most regularly involved in the care of babies involved*".[15] She was also told that the RCPCH would be undertaking an external review of the neonatal unit from 18 August 2016. The Trust had decided that Letby would "*remain subject to clinical supervision until the Trust has received feedback from the external review*".[16]

16.10 Ms Sian Williams and Ms Powell went on to say that they recognised that Letby was:

> "*understandably upset and distressed by this decision and said that you* [Letby] *would be very upset and could not live with yourself if any of your care had harmed a baby. I* [Ms Sian Williams] *reassured you that the decision to provide you with clinical supervision should not be interpreted as apportioning blame but was intended to be an additional supportive measure, pending completion of the external review.*"[17]

16.11 "*During the meeting,*" the letter read, "*we discussed an alternative to clinical supervision, being the option to take a short break from the NNU* [neonatal unit]*. However, you confirmed that you would prefer to continue supervised practice.*"[18] The letter also set out arrangements for support by Occupational Health. Ms Sian Williams said in evidence that it was the executives' decision not to set out the nature of the allegations against Letby; she did not think that the letter was transparent.[19] When asked why the executives took this approach to the letter, she replied: "[T]*he Executives were still of the belief it wasn't a single person.*"[20]

Extraordinary Board meeting, 14 July 2016

16.12 An Extraordinary Board meeting took place to consider the neonatal unit on 14 July 2016.[21] It was chaired by Sir Duncan Nichol. Also present were Mr Wilkie, Ms Hopwood, Ms Fallon, Mr Chambers, Mr Harvey, Mr Holden, Ms Kelly, Mr Cross, Ms Burnett, Dr Brearey and Dr Jayaram.

16.13 The executives' aims for the Extraordinary Board meeting can be inferred from the meeting with clinicians on 13 July 2016. Here, they presented the data from the 'Position Paper: Neonatal Unit Mortality' and persuaded the clinicians not to go to the police but to agree to further internal investigation and an external review by the RCPCH, and to Letby remaining on the neonatal unit, with a "*cast iron*" promise of full supervision.[22] I am satisfied that the executives' aims for the Extraordinary Board meeting were to:

a. inform the Board that the Trust had decided to downgrade the neonatal unit

b. persuade the Board that the increase in mortality was not due to the criminal activity of Letby, but to multifactorial issues, such as increased acuity, increased activity, staffing, taking in babies who were Level 3, gestation, and lower weight, etc

c. get the agreement of the Board for further internal investigation and an external review of the neonatal unit by the RCPCH

d. persuade the Board not to call the police at this point

e. get the agreement of the Board to allow Letby to remain on clinical day duties under supervision.

16.14 Prior to this, on 5 July 2016,[23] Sir Duncan had informed the Non-Executive Directors that an Extraordinary Board meeting was being called, to discuss the proposal for an external review by the RCPCH into unexpected and unexplained deaths in the neonatal unit, the imminent downgrading of the neonatal unit to Level 1, which would mean reducing intensive care cots and not admitting babies of less than 32 weeks' gestation, an internal review, and difficult messages arising from this.

16.15 In oral evidence, Ms Fallon said that Sir Duncan did not tell the Executive Directors on 5 July 2016 that concerns about deliberate harm to babies had been raised. Ms Fallon said that she first heard of the concerns in a chance meeting with Mr Oliver on 12 July 2016.[24] Ms Hopwood and Mr Wilkie said that the Board meeting on 14 July 2016 was the first time they heard about the concerns about Letby.[25]

16.16 The minutes[26] of the Board meeting record that Mr Chambers reported:

> "[T]*he Trust has noticed a change in mortality rates in the neonatal unit. This rise could not be explained and following on from concerns raised by the clinical team, the Trust is sufficiently motivated to do an in-depth review into the deaths.* [Dr] *Brearey, lead for the neonatal unit, had asked for a peer review from colleagues in Liverpool into the cases to see if anything had been missed. This review had proved inconclusive.* [Dr] *Brearey and the team decided that there was a need to escalate further to understand what had changed, what this meant in terms of staffing, to understand the clinical context of all the baby deaths over a longer period and to also commission an independent review on the neonatal services."*[27]

16.17 This was the beginning of an exercise in spin. It was Dr Brearey who had brought to the attention of the executives the increase in the number of deaths. He had also brought to their attention his concerns, and those of his colleagues, about Letby, and the reasons for their concerns, which Mr Chambers omitted from his opening remarks. Mr Chambers also gave the impression that Dr Brearey had been instrumental in commissioning the independent review, when he knew that Dr Brearey had said repeatedly that the RCPCH report would not answer the question of whether a nurse was causing harm to babies. This introduction was not made carelessly by Mr Chambers. As was discussed in evidence, his degree was in media and communications; he was adept at communicating precisely what he wanted the listener to hear.

16.18 Mr Chambers informed the Board of the decision to change the admission criteria for the neonatal unit. He told them about the internal review which the Trust was conducting (see Chapter 14), and the proposal for an external review (RCPCH).[28]

16.19 Mr Harvey told the Board that further work would be done on the internal review and a detailed report produced; that care was needed when discussing the numbers; that the number of deaths was small and not significant in neonatal mortality figures, but that higher than expected mortality rates had been found; and that some of the deaths were unexpected. He said that it was important to understand how busy the neonatal unit was, and how it compared on a local and national level. He referred to "*an apparent step change*" in the number of deaths in 2015 to 2016.[29] In fact, it was not an 'apparent' step change, it was real. The question was why it had occurred.

16.20 Mr Harvey spoke of an upward trend in care days (see Chapter 15), and referred to the gestational age of babies, birth weight and issues of staffing.[30] He advised that the baby deaths had been reviewed, and said that there was also a need to look into babies who had suddenly deteriorated. He gave figures about cots, and said that there were normally 14 cots, but that at one point there were 19 cots in use, and he questioned whether there were enough skilled staff, particularly at night.[31] His information was incorrect. There were 16 cots in the neonatal unit, and there was no suggestion then or since that there were not enough skilled staff at night.

16.21 Mr Harvey then noted an issue around the rota allocation of medical and nursing staff on shifts before and when babies deteriorated, saying that there were a number of staff who appeared more frequently, and one member of staff in particular. The member of staff was said to be one of the unit's highest trained staff.[32] This was not accurate. Letby was an entry-level band 5 nurse, with a degree and QIS training. In fact, there were approximately seven band 5 nurses, all with degrees. Letby was one of two band 5 nurses with QIS training. The majority of nurses on the neonatal unit (approximately 18) were more senior, at band 6 level, all qualified to at least QIS standard.[33] There were also a number of junior doctors.

16.22 Dr Jayaram made it plain that the consultants' concerns were not only the number of deaths, but that the babies who died were not expected to die. They were premature but stable, with no reason for their collapse followed by a failure to respond to resuscitation. These factors, plus the numbers, were what concerned them.[34] He did not include poorly babies with abnormalities, who were expected to die, within the group referred to. He and Dr Brearey made it clear that the data provided to the Board had had no input from neonatal clinicians or the Neonatal Network, and had only been presented to them the previous day. Whilst they recognised there were strains in the system, they believed the Countess was no worse than other comparable areas, and better than some. Nor did they believe that changes in acuity and staffing could account for the deaths, or that these factors had a direct effect on each baby.[35]

16.23 At this point, Dr Jayaram asked for his next remarks not to be minuted.[36] There was no objection taken to that, and he told the Board about his suspicions. In evidence, he stated this was a reference to the explicit concerns that he raised about Letby. He explained: "*I had increasing discomfort here … there seemed to be a pattern emerging that they didn't want to listen and I was already becoming concerned that this, if minuted, could potentially come back and … and be used against me.*"[37] His reluctance to have

something so important minuted should have been discussed with him. He should have been reassured about the consequences, by the Chief Executive, the Medical Director and/or the Chair. In due course, his anxieties proved to be well founded.

16.24 Mr Chambers repeated the point about pressures on staffing and asked the Board whether it considered the Trust was taking this seriously: "*The Board agreed that the Trust was taking these issues seriously and agreed that the proposed actions were proportionate.*" He added that the Trust would continue their investigations while an external review was being conducted, which would look at the competence of all staff. Again, no consideration was given to safeguarding. He also promised increased security arrangements, including CCTV. Dr Brearey intervened to say that he did not consider competence to be the issue, as the medical observations of the babies had not indicated that earlier intervention was needed. He explained about the deaths occurring on night shifts, so they should look at events in the 12 hours prior to that.[38]

16.25 Dr Jayaram acknowledged that these steps were adequate as a holding measure, but he was concerned about what would happen if there was no conclusion at the end of the external review.[39] Mr Harvey said he had approached the RCPCH and that they would undertake the external review on 18 August 2016.[40] In fact, that turned out not to be possible.

16.26 At this point, the Non-Executive Directors spoke up. Mr Wilkie challenged the executives, saying that he accepted that there was no evidence to say that the increase in deaths was due to an individual, but neither was there any evidence to say the contrary. He wanted to know what had changed since the earlier conversation, when it had become known that there "*was considerable disquiet about an individual*".[41] He pointed out: "*We are saying there is something wrong here as we are now supervising that person … he* [Mr Wilkie] *wanted to better understand what are the critical issues that mean it is not appropriate to engage the police as he could see disquiet*."[42] Dr Brearey agreed that there was considerable discomfort regarding the nurse, and it was felt that supervising her would not solve the problem.[43]

16.27 Ms Hopwood asked about the practicalities of the nurse working under supervision.[44] Dr Brearey informed her that there had been no unexplained collapses on night shifts when Letby was working on day shifts for three months, but that the recent collapses during the day had caused the clinicians to raise concerns again, and they needed to correlate the collapses and deaths against staffing rotas.[45]

16.28 Sir Duncan made the point that: "[T]*he situation is that on the back of the staffing and the acuity this does not link to the mortality rates.*" This was an important point, which was clearly correct. He went on: "*What still needs to be informed is the rotas, competencies and situation all to be combined.*"[46] Contrary to Dr Jayaram's observation of the view of the doctors, Sir Duncan added: "*This next stage is not a holding measure.*"[47] Whilst this may not have been his intention, the effect of the next stage was to delay a decision to call the police.

16.29 Mr Chambers added the external review (RCPCH) was necessary, and would look at data, people, context and environment. He went on to say that, as the Trust could not determine that harm was happening through lack of competence, it thought that the proposed approach of supervision, ongoing review of data and an external review was

proportionate.[48] The question arises: proportionate to what? Without removing the nurse, the risk was of death or serious injury to a baby. Supervision, ongoing review of data and external review were not proportionate to that risk.

16.30 Mr Wilkie said he did not know how effective the measures would be, and asked how confident the Trust was that "*we are removing all risk*".[49] In short, he wanted to know why they should not call the police now. Mr Chambers answered that there would be weekly monitoring of the neonatal unit. Ms Hopwood questioned the feasibility of supervising a nurse at all times, and both Ms Hopwood and Mr Wilkie remained somewhat unconvinced that the measures proposed would be effective and remove all risk.[50]

16.31 Mr Wilkie asked about the member of staff. Ms Kelly said that Ms Sian Williams and Ms Powell "*had an extremely difficult conversation with the individual* [that is, Letby] *who is devastated. It was a very sensitively worded conversation and the staff members understood about the review. The individual knew about the review, there are a lot of upset staff on the unit*."[51] The Board may have gained the impression that Letby had been told about the fact that she was suspected of deliberately harming babies. As I said earlier (see paragraphs 16.7 to 16.11), I am satisfied, on the evidence, that she had not.

16.32 Mr Wilkie remained troubled, and asked repeatedly whether supervision and checking the nurse's skills would abate any possibility of further issues. Dr Brearey said that they would not do so completely. Ms Fallon asked if the individual would have access to babies without being supervised. Mr Chambers replied: "[I]*f we believe that this is the only explanation, then we phone the police. The most plausible option is competency and when the individual has interaction with babies they will be supervised.*"[52] It is striking that, given the information that he had repeatedly received about Letby's high level of competence, Mr Chambers persisted with the view that the most plausible explanation was lack of competence.

16.33 Ms Hopwood asked about the mortality rate over recent years, and this was explained.[53]

16.34 Sir Duncan sought to pull the discussion together:

> "*In the meantime, the previously expressed concerns about the individual, actions are being taken and it is agreed that these are reasonable as we cannot see a single hypothesis. We have to move forward in this way if the majority agrees, in the meantime there are no guarantees however this is what we would do in these circumstances or are there any alternative propositions.*"[54]

16.35 Dr Jayaram replied that the only alternative would be to go straight to the police, and they would want hard evidence.[55] This reflected the information Mr Cross had provided on several occasions. In evidence, Sir Duncan explained that what he meant by "*a single hypothesis*" was that the Board had been told that there could be multiple other factors causing the deaths (as set out by Mr Harvey at the start of the meeting – acuity, activity, staffing), as well as the hypothesis of suspicion of criminality.[56] The reference to the absence of a single hypothesis was subsequently repeated by Mr Chambers in several meetings. He was asked about this on behalf of the Inquiry by Mr Nicholas de la Poer KC. In oral evidence, Mr Chambers confirmed his position at the meeting that two hypotheses would be tested: (a) the suggestion that one nurse was causing harm (this could be tested by her absence from the unit); and (b) that the babies were very

poorly (this could be tested by downgrading the unit and taking in babies who were less vulnerable).[57] He went on to add that other factors may be behind the increase in deaths, rather than the suggestion of harm by one nurse.[58]

16.36 Mr Harvey told the Board that he had approached the RCPCH to conduct the review, and they had agreed to do so quickly. Dr Jayaram made the point that the Terms of Reference would need to be very clear.[59] Mr Harvey did not mention that he had received the Terms of Reference and had suggested amendments. Mr Chambers informed the Board that further work would be done on the internal data and that there would be weekly monitoring of the neonatal unit.

16.37 Ms Fallon asked whether, if there was a direct correlation, the review would uncover it. Mr Harvey did not reply directly, saying that any issues would "*be outed*", and the reviewers would be told about the supervision of Letby as part of the measures the hospital had taken.[60] Sir Duncan asked whether the review team would be briefed on the explicit concerns. Mr Harvey confirmed that they would be. He said he would discuss the concerns during his interview with them.[61]

16.38 Ms Hopwood asked how the ongoing discussions and fact-finding would be brought to the Board. Mr Cross said that the Board would be updated as necessary, and Mr Chambers said the dashboard would give clear oversight. Mr Chambers added that the Trust "*will be proactive on communications for all staff and the media*".[62] Sir Duncan said that he and Mr Higgins would keep in close contact with the review. Ms Hopwood said that another Board meeting should be held post-review as a minimum, unless there was a need to get together sooner.[63] The issue did not come back to the Board until January 2017.

16.39 The day after this meeting, on 15 July 2016, Mr Wilkie went to see Ms Kelly, because he was unhappy with the decision that Letby should return to work with supervision. He said that his impression from speaking with Ms Kelly was that she was concerned about the impact that removal from the unit would have on Letby.[64] Mr Wilkie said: "*I took the view that patient safety trumped any concern of an individual member of staff's feelings*."[65] His position was correct. It was the case that patient safety could not be guaranteed with the proposed arrangements in place. This was obvious to Mr Wilkie, and should have been obvious to the rest of the Board. In fact, the conclusion of the Board, after a very lengthy meeting, was superseded within days. It turned out that there were not enough people to supervise Letby's practice, or anyone else's. The plan for her to return to the neonatal unit was therefore abandoned. Given everything that had been said about staffing in the meeting, mainly by Mr Chambers, the decision to impose close supervision without any additional staff was impractical, as well as wrong in principle.

16.40 Three days later, on 18 July 2016, Ms Rees wrote to Letby, informing her that, due to staffing constraints, it was not possible for her to be clinically supervised. It was decided that she would be seconded to the Risk and Patient Safety Department.[66]

Endnotes

1 INQ0003365/4-8
2 INQ0003365/5
3 INQ0003365/5
4 INQ0003365/6
5 INQ0003365/7
6 INQ0003365/8
7 INQ0003365/7-8
8 INQ0004327
9 INQ0102740/2-13
10 Eirian Powell 17 October 2024 148/16 to 150/13
11 INQ0005769/2 and 4
12 INQ0002839
13 INQ0002839/1-2
14 INQ0002839/2
15 INQ0002839/2
16 INQ0002839/2
17 INQ0002839/2
18 INQ0002839/2
19 Sian Williams 5 November 2024 53/13-24
20 Sian Williams 5 November 2024 54/3-4
21 INQ0004216
22 INQ0003365/8
23 INQ0102040/2
24 Rosalind Fallon 3 December 2024 181/2 to 182/22
25 Rachel Hopwood 3 December 2024 128/15 to 129/19; James Wilkie 2 December 2024 162/1-12
26 INQ0004216
27 INQ0004216/1-2
28 INQ0004216/2
29 INQ0004216/2
30 INQ0004216/2
31 INQ0004216/3
32 INQ0004216/3
33 INQ0003835

34 INQ0004216/3
35 INQ0004216/3-4
36 INQ0004216/4
37 Dr Ravi Jayaram 13 November 2024 119/1-13
38 INQ0004216/4
39 INQ0004216/5
40 INQ0004216/5
41 INQ0004216/5
42 INQ0004216/5
43 INQ0004216/5
44 INQ0004216/6
45 INQ0004216/6
46 INQ0004216/6
47 INQ0004216/6
48 INQ0004216/6
49 INQ0004216/7
50 INQ0004216/6-7
51 INQ0004216/7
52 INQ0004216/7
53 INQ0004216/7
54 INQ0004216/7
55 INQ0004216/7-8
56 Sir Duncan Nichol CBE 2 December 2024 58/24 to 59/8
57 Tony Chambers 27 November 2024 32/22 to 34/16
58 Tony Chambers 27 November 2024 205/8-14
59 INQ0004216/8
60 INQ0004216/8
61 INQ0004216/9
62 INQ0004216/9
63 INQ0004216/9
64 James Wilkie 2 December 2024 175/13 to 177/5
65 James Wilkie 2 December 2024 177/5-7
66 INQ0002458

Chapter 17. July 2016: Redeployment of and support for Letby

Letby's redeployment

17.1 As set out in the previous chapter, Ms Powell and Ms Sian Williams had met Letby on 14 July 2016 and explained that she had been identified as being "*more regularly involved in the care*" of those babies who had collapsed and deteriorated on the neonatal unit. She and all those staff who had regular involvement in the care of the babies concerned were to be provided with a period of supervised practice.[1] The period of supervised practice was due to commence on 18 July 2016, until which date it was agreed that Letby would go on authorised leave. This would continue pending the completion of an external review by the RCPCH.[2]

17.2 Ms Sian Williams told the Inquiry she felt like she was the messenger between Letby, Ms Hodkinson and Ms Kelly. She thought that the Trust should have been more transparent with Letby and that "*with the benefit of hindsight, I should have stood up a bit more to her* [Ms Hodkinson]".[3]

17.3 Ms Hodkinson agreed that the hospital was not transparent with Letby and that this was a subsequent finding in the grievance.[4]

17.4 On 15 July 2016, Ms Sian Williams approved an email drafted by Ms Powell and Ms Griffiths which was sent to all nurses on the neonatal unit:

> "*In preparation for the external review it has been decided that all members of staff need to undertake a period of clinical supervision … we have decided that it would be useful to commence with staff who have been involved in many of the acute events … Therefore Lucy has agreed to undergo this supervision first.*"[5]

17.5 Ms Hodkinson admitted that it was only ever Letby who was going to undergo supervision.[6] Ms Sian Williams was unaware of this. She thought that all the nurses on the unit were to undergo supervision. She acknowledged that, in failing to mention the concerns about the increased mortality and harm to babies, the email misled the nurses on the neonatal unit and they remained in the dark as to what was going on.[7] Ms Sian Williams – the Deputy Director of Nursing – was misled. All the nurses on the unit were misled as to the reason for supervision. This was bound to and did lead to nurses feeling one of their number was being singled out for unfair treatment.

17.6 A meeting was held on 18 July 2016 which included Letby, Ms Rees, Ms Griffiths, Ms Farmer and Mr Tony Millea (RCN). Ms Rees sent a letter to Letby summarising the meeting. The letter set out that it was not possible to provide her with full-time

supervised practice because of staffing levels on the neonatal unit. As a result, a decision had been taken to redeploy Letby to the Risk and Patient Safety Department instead, where she would be line-managed by Ms Millward.[8]

17.7 The letter referred to the redeployment as "*temporary*" and a "*neutral act … taken in the best interest of all parties and in the interests of patient care, pending completion of the external review*". Letby was told that she could maintain social contact with her colleagues on the neonatal unit but that she "*should be mindful of discussing any matters which may be sensitive in nature, relating to the* [RCPCH] *review of the NNU* [neonatal unit]".[9] Letby was later to say to the RCPCH and others that she had been told not to contact her colleagues on the neonatal unit. This was not true.

17.8 Although Ms Rees informed Letby about the redeployment, it was not her decision that Letby should be redeployed. She told the Inquiry that she returned from annual leave shortly before 18 July 2016 and was told that management had decided to remove Letby from the unit.[10] Like Ms Sian Williams and Ms Hodkinson, Ms Rees considered that the hospital was not transparent with Letby about what was actually going on.[11] They were all right about that.

17.9 In oral evidence, Ms Millward informed the Inquiry that it was her idea for Letby to be redeployed to the Risk and Patient Safety Department. She explained that the management of Letby was discussed in an Executive Team meeting she attended. She perceived that the executives were looking for, and struggling to find, answers because the consultants had made clear that they did not want Letby to work on the unit when she returned from annual leave. Ms Millward stated: "*I am absolutely confident that Mrs Hodkinson said something along the lines of 'there's not enough', meaning not enough evidence 'there is not enough to exclude her, suspend her'*."[12]

17.10 Ms Millward explained her rationale for suggesting Letby be redeployed to the Risk and Patient Safety Department: "*I felt I wanted to help … I also felt that, you know, given the fact that somebody was being moved with -- with no real explanation I felt that she did warrant to have some support in place.*"[13]

17.11 Ms Millward's understanding was that Letby's redeployment would be a temporary move while the RCPCH review was carried out (which she thought would be completed within eight weeks). She stated that Letby joined the complaints team and was based in an office with administrative staff.[14] She reflected that, in hindsight, it was not appropriate for Letby to join her team and that, once the RCPCH report was complete and Letby's grievance had commenced, "[A]*t that point I really should have said 'I think we need to move her now' because there was no end in sight at that point*."[15]

17.12 On reflection, Ms Sian Williams considered the redeployment to the Risk and Patient Safety Department "*was probably not appropriate*".[16] At the time, she raised a concern that Letby may have had access to material about babies on the neonatal unit via the computer system.[17]

Lack of transparency with legal advisers (DAC Beachcroft)

17.13 In July 2016, the hospital obtained legal advice from Mr Ian Pace and Ms Corinne Slingo at DAC Beachcroft (Associate, Employment and Pensions, and Head of Healthcare Regulatory, respectively). The advice concerned two issues: Letby's redeployment; and whether the police should be called. Ms Hodkinson confirmed that it was her decision to obtain legal advice regarding matters related to Letby.[18]

17.14 Ms Appleton-Cairns told the Inquiry that, on 5 July 2016, she contacted Mr Pace because there was consideration of removing Letby from her role, but the evidence and grounds against her were quite vague. She explained:

> "*I was just checking if we were to remove Lucy Letby from the unit, then what would be our risk from another direction, which is the direction of Lucy Letby who was being heavily backed by the RCN and what that risk would be if we were to move her … what we came to in the end was that obviously the -- that risk was not as big as the risk that she may be harming babies and in which case we had to move her*."[19]

17.15 Ms Appleton-Cairns was taken through key sections of the file note that documented her call with Mr Pace.[20] She confirmed that she made reference to Beverly Allitt and that, in 2015 to 2016, she was aware of that case.[21] She was asked about the passage in the file note that read: "*Dee is satisfied that there are no malicious issues involved*." She explained that she was satisfied of this because Mr Harvey and Ms Kelly had given her "*assurances*" that there was nothing malicious.[22] Whatever assurances she may have received, she knew the doctors were concerned that Letby was harming babies. It was not for her to declare herself satisfied that there were no malicious issues involved. Her approach suggests that she gave no credence to the concerns, not least because she had not troubled to learn the details.

17.16 Ms Appleton-Cairns denied that she was minimising the problem in her call to Mr Pace. When asked in evidence about "*a large number of unexplained, unexpected deaths on the neonatal unit*", she responded: "*At that point … it wasn't that many*."[23] This was a most unfortunate observation. There had been 13 deaths in little over a year. She informed the Inquiry that she was not aware there had been 13 neonatal deaths on the neonatal unit between June 2015 and June 2016 and she had not seen the Thematic Review when she called Mr Pace.[24] If this is true, and I see no reason why it would not be, it shows at best a lack of application to her role and no appreciation of the seriousness of the situation. It is likely that she gave no credence to the doctors' concerns. She asserted in evidence that there was "*no commonality on the -- on the spreadsheets*".[25] This was not true. She said: "*The only Consultant that I knew of that was expressing any kind of concern for a long, long time was Dr Brearey*." Again, this was not true. She went on: "*So for me there was -- there was nothing here other than Dr Brearey saying he had some concerns about a nurse, a specific nurse*."[26]

17.17 Mr Pace said in evidence that he received a call from Ms Appleton-Cairns seeking advice on the breakdown of working relationships and concerns about an individual member of staff's involvement in neonatal deaths. In the call, she told him that a consultant had referred to the individual as 'Beverly Allitt'. Mr Pace regarded this as a "*red flag*".[27] He was right to do so. So should every other person involved.

17.18 Mr Pace perceived that Ms Appleton-Cairns lacked "*recognition, perhaps, of the issue*".[28] He stated: "*I wanted to effectively switch the focus from effectively what was a relatively minor issue of breakdown in working relationships to clearly a very serious -- potentially very serious matter*."[29] The file note reflects that he advised Ms Appleton-Cairns that the employment considerations paled into insignificance.[30] He was right to do so. It is dispiriting that Ms Appleton-Cairns needed that to be explained to her. It reinforces my view that she gave no credence to the concerns.

17.19 The file note also records that Mr Pace advised Ms Appleton-Cairns that there was a potential risk of a constructive unfair dismissal claim if she resigned. He advised that the Trust should put in place steps to justify a satisfied position that there was a suggestion or evidence to link the individual to the deaths so that the Trust would have a defence if it decided to suspend Letby.[31] Mr Pace emphasised that he was acting in his capacity as an employment solicitor and that the advice about suspension was in line with Acas guidance.[32]

17.20 Mr Pace explained he did not suggest an immediate suspension or that a safeguarding referral should be made because Ms Appleton-Cairns provided him with limited information and presented "*a confusing picture*".[33] Thus, he thought it was best to obtain specialist regulatory advice before suggesting those steps.

17.21 Following the call, Mr Pace spoke to Ms Kirsty MacDonald, Head of Employment at DAC Beachcroft, who advised him to speak to Ms Slingo. Mr Pace thought that he told Ms Slingo everything Ms Appleton-Cairns told him.[34]

17.22 At some point between 5 July and 18 July 2016, Ms Slingo read Mr Pace's note of his call with Ms Appleton-Cairns from 5 July 2016.[35] She noted that a consultant had made reference to Beverly Allitt. She commented: "[F]*or any healthcare lawyer at that time, one would regard a reference to* [Beverly] *Allitt as suggesting that there was a deliberate harm element to the patient safety concern*."[36] However, she emphasised that the file note also recorded that Ms Appleton-Cairns was satisfied that there were no malicious issues.[37] Both Ms Slingo and Mr Pace accepted what Ms Appleton-Cairns said at face value. I repeat, it is not easy to see how a doctor referring to Beverly Allitt was consistent with an absence of malicious issues.

17.23 At 17:00 on 18 July 2016, Ms Slingo spoke to Ms Hodkinson. This was the first time she had spoken to someone at the Trust. Her evidence was that the call lasted 42 minutes and that at 18:15 she sent Ms Hodkinson and Mr Pace an email outlining her advice.[38] In Ms Slingo's email she sets out her understanding of the consultants' concerns and the context to her advice:

> "*Based on the fact that:*
>
> - *the only current evidence of any clinical concern is the (potentially circumstantial) fact that one particular nurse was on shift on more occasions than others at the point when neonatal deaths arose, and*
> - *there are also deaths/deteriorations that occurred when she was not on shift, and*
> - *No incidents have been linked to her practice, and*
> - *No previous concerns or whistleblowing has arisen in respect of the individual or the unit, and*
> - *There is currently no cause of death or thematic clinical basis to suggest the deaths are connected to each other, or connected by a common intervention (deliberate or not), and*
> - *Approximately 75% of the deaths have also been through the coronial system, with no common feature or issue arising from the individual's care.*"[39]

17.24 Ms Slingo was asked whether she felt misled by the incomplete information that was provided to her in the calls with Ms Hodkinson. She responded: "*I do think that I didn't have all the information that you suggest was available by the July period in 2016, because it doesn't feature in my note as part of the narrative and the history given by Sue … if I wasn't told everything that was known, then that would be disappointing to find out and that appears to be the case*."[40]

17.25 Ms Slingo confirmed that she was not aware that the majority of the deaths had occurred between midnight and 4am when Letby was on night shift, or that they stopped at night when Letby was moved to days and then occurred during the day, nor was she aware that the doctors were concerned that the deaths were unexpected and unexplained. She said that she did not know the consultants were concerned that Letby was deliberately harming babies.[41] It is not clear how that fits with the earlier reference by one of the doctors to Beverly Allitt but Ms Slingo did not ask about this. She explained: "[T]*he entire tone of this call wasn't that there is a deliberate harm situation*."[42] She added: "*I did not take away from that call the idea of there is a nurse potentially killing babies*."[43]

17.26 Ms Slingo explained that what she understood from the call was that the Trust had experienced some anomalies with mortality data and there was a concern about an individual's involvement with the deaths, which they were trying to understand.[44] The impact of Ms Slingo being provided with incomplete information is borne out in her advice:

> "*There does not currently appear to be any reason to formally alert the police to these issues, as there is nothing upon which one might reasonably base a suspicion of a criminal offence having been committed. We advised that this fine balance of decision making be kept under very close review, with a very low threshold for moving this to a decision to notify the police, in the event any clinical evidence comes to light in the ongoing investigations that tips the balance in favour of alerting them.*"[45]

17.27 Ms Slingo's evidence was that, given what she now knows, she wished she had advised the Trust to contact the police; however, at the time deliberate harm was not at the forefront of her mind. She said, had there been any concern that there was deliberate harm or killing of babies, "*there would be no question that my advice would have been very different*".[46]

17.28 At the end of her email, Ms Slingo wrote that it would be helpful to see the Terms of Reference for the RCPCH report; however, neither the Terms of Reference nor the report were ever provided to her.[47]

17.29 Ms Hodkinson confirmed in evidence that she did not send Ms Slingo the Thematic Review.[48] She was not sure whether she had mentioned that the Thematic Review noted a pattern of deaths in the night-time and Letby's presence on those shifts.[49] She initially stated that she could not recall whether she had mentioned that the consultants had concerns about an Allitt/Shipman type situation. She then appeared to admit that she did not say this, when she reflected "*potentially maybe I should have done but I -- it wasn't around a misleading point or any aspect there*".[50] If she did not mention it, she should have done.

17.30 Ms Hodkinson was invited to comment on Ms Slingo's evidence. She was not particularly reflective, telling the Inquiry: "[Ms Slingo] *should have asked me more questions … I felt that I was giving a clear and honest and truthful overview at that time*."[51] Her account was partial. I find that she did not tell Ms Slingo about the pattern of deaths and the doctors' concerns that babies were being deliberately harmed. She should have done so. No more than that needed to be said to cause a radical change in the legal advice being given. Ms Hodkinson did not give Ms Slingo crucial information about the doctors' concerns.

Support for Letby

17.31 On 30 June 2016, after the deaths of Baby O and Baby P, Ms Powell referred Letby to the Occupational Health Department for support.[52]

17.32 Ms Kathryn De Beger was the Occupational Health Manager. She qualified as a nurse in 1985 and began working as an occupational health nurse in 1995. In 2009, she joined the Countess in that role and became the Occupational Health Manager in 2010.[53] She was line-managed by the Director of HR, Ms Hodkinson, but Ms De Beger and her team were independent of the HR department. She explained that the team was there "*to support and give advice to members of staff going through those policies* [HR policies, management policies] *in a very independent, impartial manner and non-judgmental* [way]".[54]

17.33 In her referral, Ms Powell wrote: "*As I mentioned … on the phone … I have requested that Lucy come to the occupational health department for support especially in view of the proposed allegation that will evidently come to light*."[55] Ms De Beger said that when she received the written referral she was unaware of the nature of the allegation.[56] She was aware of an increase in deaths on the neonatal unit and that there was an investigation. She also knew that Letby had been redeployed to a non-clinical role at about the same time the investigation was announced and had made the connection that Letby may be involved in some way. However, she was not aware that she was accused of

deliberately harming patients.[57] The role given to her by the Trust was "*to support her, to support her mental health, to support her well-being*" and "*to support her through this investigation*".*[58]

17.34 Ms De Beger met Letby for the first time in mid-July 2016, when Ms Powell accompanied Letby to the Occupational Health Department.[59] She learnt of the nature of the allegations at some point during her meetings with Letby.[60] In addition to formal weekly meetings with her, she also had very frequent email and, later, text message contact with Letby. She kept in touch with Letby even when she (Ms De Beger) was on holiday. During a 15-month period between March 2017 and July 2018 (when Letby was arrested), they exchanged over 400 WhatsApp messages. It was clear from her evidence that she discharged her responsibilities with great care and was an excellent support to Letby. Contrary to some reporting, Ms De Beger was not a counsellor and there is no evidence that counselling or any form of therapy was provided to Letby.

17.35 In addition to the support from Occupational Health, Letby also had weekly or fortnightly meetings with Ms Hodkinson, Ms Kelly and Ms Rees. This was a very senior team, which supported Letby throughout (with Ms Rees as mentor).[61] They made it plain to her that the expectation was that she would be returning to the neonatal unit. They provided her with updates of all the management meetings that concerned her and they ensured she had a copy of the RCPCH report before the consultants did.[62]

17.36 As of April 2017, a WhatsApp group was set up which included Letby, Ms De Beger, Ms Cooper and Ms Rees. There were over 750 messages in that group between April 2017 and July 2018, in addition to those between Ms De Beger and Letby only. Ms De Beger said that she had never – before or since – been in WhatsApp contact with a person she was supporting.[63] She explained that she took this different approach because Letby was in a distressing situation and because, as she stated: "*I felt that at that time I was the only support that Lucy Letby had*."[64]

17.37 During her evidence, Ms De Beger was referred to a passage in her police witness statement, which read: "*Lucy did ask for meetings with me on anniversaries of some of the babies' deaths as she was particularly distressed*."[65] In evidence to the Inquiry, Ms De Beger modified that and said it had happened only once. It was not something that any other nurse had asked for.[66] The distress shown by Letby on the anniversary contrasts with her response to the deaths at the time they occurred.

17.38 Ms De Beger was not alone in believing she was Letby's only support, and she became very close to her. This echoed what was said by Ms Cooper and Dr U (see Chapter 11). Further support came from Dr U. He was an experienced registrar. He worked at the Countess from September 2015 to September 2016 as the most senior of the junior doctors. He then worked as a locum consultant at Alder Hey.[67] Dr U said in evidence that he began Facebook messaging with Letby in June 2016.[68] Between June 2016 and September 2016, they exchanged approximately 1,355 messages.[69] When asked to explain the reason for such frequent contact, he said: "*Letby was struggling with her mental health and I think I picked up on that and I'd offered some support, and that support, it grew, and I understand that she slept very poorly because of worry and anxiety, and there were often messages that were passed throughout the day and*

* This is in reference to the RCPCH report.

sometimes late at night, earlier in the morning."[70] I have referred (see Chapter 11) to their messages at the time of the collapses and deaths of some of the babies. I do not repeat them. Like Ms Cooper, Ms Rees and Ms De Beger, Dr U was content to be a close support of Letby.

17.39 The effect of Letby's displays of distress was that all those supporting her (Ms De Beger, Ms Cooper and Ms Rees) were genuinely sympathetic towards her and gave her a great deal of attention and support. The intensity of the situation brought all the people in the WhatsApp group together. Whilst Ms Cooper and Ms De Beger were responsible for supporting Letby, Ms Rees's position was different. She was the Divisional Head of Nursing. She should have maintained appropriate distance. She accepted in evidence that she had become too close to Letby, as a result of the frequent contact: "[Y]*es, because I was tasked to meet with her on a near weekly basis for nearly two years*."[71] In evidence, Ms Rees said that she was given a management instruction from Ms Kelly and Ms Hodkinson "*to meet with her* [Letby] *on a weekly basis with my two other senior colleagues to support her health and well-being during all of this and to give her feedback from any investigations*".[72] It is likely that this made it impossible for her to be objective about the issues that had been raised by the doctors. Ms Kelly's participation in the meetings was ill judged. As a senior manager, she should not have been involved in supporting Letby. She acknowledged that her involvement in supporting Letby and direct conversations with her were inappropriate.[73]

17.40 Letby also had the support of both her parents, although Ms De Beger recalled that there was a difference of opinion between Letby and her father about how to proceed.[74] Mr Letby said that the police should be called, a view he also expressed in meetings with the executives (see Chapter 23). Letby did not want this. Ms De Beger denied that Letby's father had ever telephoned her or harassed her, as had been asserted by another witness to the Inquiry.[75]

17.41 There were good reasons to support Letby, and the hospital was right to do so, but every person who supported her stepped very quickly from professional support to personal friendship. They were all relatively senior people. They should have maintained an appropriate professional distance and not become involved.

17.42 On 23 June 2016 at 09:55, Letby messaged Dr U saying: "*I lost my handover sheet. Found it in the donor milk freezer. Clearly I should still be in Ibiza.*" Ms Powell was asked about the contents of this message in oral evidence. She said she did not know how it had happened. She also did not know that Letby had taken 241 handover sheets home in 2015 to 2016. Ms Powell confirmed that handover sheets should not leave the unit and she was very surprised to learn that Letby had taken handover sheets off the unit.[76]

17.43 Letby kept 241 handover sheets stored at her home. Twenty-one of those sheets related to babies named on the indictment.[77] There is no evidence that any nurse, doctor or manager was aware of Letby doing this at the time, nor could anyone explain this practice. Whilst I can understand that sometimes a nurse or doctor may (whatever the rules may be) take home a handover sheet to complete their notes, and it may well be that sometimes this is done absent-mindedly, the taking and retaining of such large numbers of handover sheets is deliberate and worrying behaviour and obviously contrary to good nursing practice.

17.44 A handover sheet for Baby H was found in a plastic bag under Letby's bed at her home after her arrest. Handover sheets for Baby G and Baby I were also found. She took and retained information about Baby E and Baby F. Mother E and F's evidence was that she learnt at the criminal trial that Letby "*had Child E's and Child F's sensitive data stored at her home and she actually moved house and actually took it with her in a Morrison's carrier bag*".[78]

Visits to Alder Hey

17.45 In addition to support by text message and while working with Letby, Dr U later helped her to secure theatre observations at Alder Hey in Liverpool, where he was working as a locum consultant. Dr U wrote the following email to colleagues:

> "*Dear colleagues …*
>
> *Lucy Letby is a band 5 staff nurse* [from the Countess] *with an interest in NICU* [neonatal intensive care unit] *nursing of post-operative babies. In order to facilitate her personal development, she would like to have the opportunity to observe some theatre sessions. Mr Lamont feels that there is no problem from a surgical perspective.*"[79]

17.46 In evidence, Dr U said he had assumed that, if Letby's visits to Alder Hey were not appropriate, HR or the nursing management team would have said 'no'.[80] However, he conceded that he did not actually have confirmation that this had been formally approved by the Countess: "*I, again in hindsight, was remiss not to complete the loop and check that that was correct. But my assumption here was that in offering a letter of access, with contact with the Countess of Chester, that had all been signed off as appropriate.*"[81]

17.47 In a meeting on 31 January 2017, between Letby, Ms Kelly, Ms Rees, Ms Cooper and Ms Hodkinson, the meeting summary notes: "[Y]*ou* [Letby] *advised how you have been liaising with a colleague based at Alder Hey to view theatres lists and to have observational contact. We agreed that you would work with Karen to come back with a plan around this within the next week*."[82] Ms Rees was referred to this note in oral evidence and was asked about her knowledge of the Alder Hey visits. Her response is set out below:

> "*If I recall, Lucy Letby had a friend, a medical colleague that worked -- she had obviously worked with him prior and he was currently based at Alder Hey Hospital and I recall her arranging to have this observational contract with this particular medic, so she could sit in his clinic and just observe. There was no hands-on clinical contact as far as I was aware, made aware. But I do recall once that had been -- Lucy had set that up with this particular medic, that Alison Kelly then on hindsight retracted it and stopped it.*"[83]

17.48 Ms Rees confirmed that she did not contact Alder Hey at any time to either confirm or prevent that placement and did not get involved in this arrangement.[84]

17.49 Dr U accepted that he did not inform Alder Hey that Letby was not looking after babies on the neonatal unit.[85] He informed the Inquiry that "*she was still having patient contact at that time … in the clinic rooms*", as part of her non-clinical role conducting clinical audits in the Obstetric and Gynaecology Outpatient Department.[86] This was not something mentioned by any of the managers but I see no reason why Dr U would have

invented it. He said that he understood that Letby was later moved to an administrative role, while she retrained. This was a reference to her redeployment to the Risk and Patient Safety Department.

17.50 Dr U succeeded in organising the observations at Alder Hey. The details appear in a feedback form he prepared for Letby's application to the Nursing and Midwifery Council (NMC) for revalidation (see Chapter 18). He said that he had worked with Letby between September 2015 and September 2016 on the neonatal unit at the Countess. As with Alder Hey, he did not mention to the NMC that Letby had not worked on the neonatal unit after the end of June 2016. His summary of Letby's observations at Alder Hey between December 2016 and August 2017 read:

> "*22/04/2017*
>
> *Attendance at Insulin Pump roadshow*
>
> *Lucy attended the Alder Hey Insulin pump road show along with* [patients] *and the* [families] *to be educated on the pros and cons of Insulin pump therapy and the relative benefits of the supported Insulin pumps used by our service.*
>
> *Lucy met with the various pump* [manufacturers] *and participated in demonstrations of subcutaneous cannula insertion.*
>
> *Lucy demonstrated a keen interest in acquiring new skills and knowledge and I hope she felt this road show was beneficial.*
>
> *18/08/2017(Dec 2016 – April 2017)*
>
> *Clinical observation at Alder Hey Children's NHS Foundation Trust*
>
> *For a period of 4 months Lucy has been attending Alder Hey Children's Hospital in order to experience the breadth of secondary and tertiary paediatric services provided.*
>
> *Lucy has observed General Paediatric outpatient and diabetes Clinics. She has observed renal dialysis and been taught about peritoneal dialysis, as well as sitting in on a complex discharge planning meeting.*
>
> *Lucy has spent a day observing nursing practice on the Paediatric Intensive care Unit.*
>
> *All of the teams that participated in offering the sessions to Lucy have commented on how keen she has been to acquire new skills and to gain knowledge experience outside of her normal working field.*"[87]

17.51 He added at the bottom of the form: "*I have worked with Lucy between September 2015 and September 2016 on the CoCH* [the Countess] *Neonatal Unit*." In fact, the period finished in early July 2016. He then wrote: "*We have worked together on babies with acute admissions, labour ward transfers and a range of gestations from 26-40 weeks*."[88]

17.52 Dr U was writing this in late August 2017. By then, he knew that Letby had not looked after babies on the neonatal unit for over a year. He knew the police were investigating and that there were concerns about Letby. He did not mention that on his form. The form did not require it. Nor did he mention that she had been moved from the neonatal unit to a non-clinical role. Again, there was no requirement upon him to do so.

17.53 At the end of his evidence, Dr U said: "*I think I've become more aware that I wasn't aware of the full clinical picture, and I provided support by being misled and maybe manipulated, and for that … I'm really sorry that things have come to end as they have. I have a lot of regrets about how that period of time took place.*"[89]

Use of mobile phones

17.54 Some parents were understandably distressed to hear of the text messages between Letby and other staff about them and their children.[90] Concerns were expressed about the use of mobile phones by nurses and doctors to communicate about patients when they were not on the neonatal unit. Parents were shocked that they and their children were being discussed with no thought for their privacy. Mother E and F's concerns extended to the distraction that mobile phones presented:

> "*I think for me, they need the full concentration on the babies, and if phones are going to be used, have them in their break rooms. They shouldn't be in their pockets. They shouldn't be able to be accessed in non-clinical areas such as the nurses' station … I think having a phone is a big distraction, and I think that they should be left in their lockers, and I think that should become common practice.*"[91]

17.55 Ms Powell's evidence was that, in 2015 to 2016, whilst there were discussions about midwives using translation apps on mobile phones to overcome language barriers, she was clear that a nurse's mobile phone "*was not to be used for personal use*".[92]

17.56 Doctors and nurses are not provided with mobile phones for use at work. They are expected to, and do, use their own devices for clinical and related calls and messaging. In many places mobile phones have replaced bleeps or pagers. In the ten years since the events I am describing, the use of mobile phones has become almost universal. The principal use is as a computer with many apps – search engines, medical and nursing websites, banking, shopping, livestreaming and so on. WhatsApp groups were used at the Countess and I have commented on some of them. I have no doubt that they (or some other encrypted messaging services) are still used between staff across the NHS. I agree with Mother E and F that there is a risk of a phone becoming a distraction, but it is my impression that for most doctors and nurses the phone is an essential connection to, for example, information about procedures/symptoms via search engines and medical websites, as well as to communicate appropriately about patients and other work-related matters. Like all other working people, nurses and doctors have to discipline themselves during long working hours not to be distracted by personal matters, on the phone or otherwise. I do not think it right to recommend removing mobile phones from doctors and nurses while at work.

17.57 One of the dangers of WhatsApp or similar apps generally is the forwarding of messages from within a closed group to another person. At that point, control of the information is lost. That does not happen with a telephone call. If someone receives information and wants to pass it on, they have to make another call or send a message. That makes the unthinking passing on of the information less likely. The ease of communication via messaging apps leads, as email does, to a less cautious approach to communication.

17.58 The NMC now has guidance on using social media responsibly, which covers issues such as sharing confidential information inappropriately or posting inappropriate comments about patients.[93] The guidance emphasises that sharing confidential information online can have the potential to be more damaging than sharing it verbally, due to the speed at which it can be shared and the size of the potential audience. Similarly, the General Medical Council (GMC) has issued guidance, with effect from 30 January 2024, entitled *Using Social Media as a Medical Professional*.[94] Social media is defined as including private messaging. The guidance makes clear that "*when using social media of any kind, you must maintain patient confidentiality and recognise and respect patients' dignity and their right to privacy*".[95]

Endnotes

1 INQ0003147/2
2 INQ0003147/2
3 Sian Williams 5 November 2024 68/19-25
4 Sue Hodkinson 26 November 2024 86/3-6
5 INQ0002879/91
6 Sue Hodkinson 26 November 2024 86/13-18
7 Sian Williams 5 November 2024 55/17-25
8 INQ0002458
9 INQ0002458
10 Karen Rees 21 October 2024 154/2-15
11 Karen Rees 21 October 2024 153/23-25
12 Ruth Millward 4 November 2024 201/2-5
13 Ruth Millward 4 November 2024 201/20-25
14 Ruth Millward 4 November 2024 201/12-18
15 Ruth Millward 4 November 2024 202/7-9
16 Sian Williams 5 November 2024 58/19-20
17 Sian Williams 5 November 2024 59/6-15
18 Sue Hodkinson 26 November 2024 60/3-7
19 Dee Appleton-Cairns 5 November 2024 191/8 to 192/21
20 INQ0101934/1
21 Dee Appleton-Cairns 5 November 2024 195/1-3
22 Dee Appleton-Cairns 5 November 2024 195/24 to 196/8
23 Dee Appleton-Cairns 5 November 2024 196/14-18
24 Dee Appleton-Cairns 5 November 2024 202/9 to 204/1
25 Dee Appleton-Cairns 5 November 2024 196/22
26 Dee Appleton-Cairns 5 November 2024 196/17 to 198/16
27 Ian Pace 21 November 2024 70/13 to 75/1
28 Ian Pace 21 November 2024 76/1-2
29 Ian Pace 21 November 2024 73/25 to 74/4
30 INQ0101934/1
31 INQ0101934/1
32 Ian Pace 21 November 2024 77/20 to 78/9
33 Ian Pace 21 November 2024 77/14

34 Ian Pace 21 November 2024 80/22 to 81/14
35 INQ0101934/1
36 Corinne Slingo 21 November 2024 115/9-12
37 INQ0101934/1
38 Corinne Slingo 21 November 2024 126/21 to 127/3
39 INQ0003397/2-3
40 Corinne Slingo 21 November 2024 157/18 to 158/12
41 Corinne Slingo 21 November 2024 130/3 to 132/25
42 Corinne Slingo 21 November 2024 132/23-25
43 Corinne Slingo 21 November 2024 156/20-22
44 Corinne Slingo 21 November 2024 124/6-11
45 INQ0003397/2-3
46 Corinne Slingo 21 November 2024 136/17-18
47 INQ0003397/3; Corinne Slingo 21 November 2024 138/12-25
48 Sue Hodkinson 26 November 2024 61/4-9
49 Sue Hodkinson 26 November 2024 62/8-17
50 Sue Hodkinson 26 November 2024 64/15-17
51 Sue Hodkinson 26 November 2024 64/22-24
52 INQ0018046/1
53 Kathryn De Beger 9 October 2024 40/23 to 41/10
54 Kathryn De Beger 9 October 2024 41/23 to 42/4
55 INQ0018046/1; Kathryn De Beger 9 October 2024 43/5-15
56 Kathryn De Beger 9 October 2024 43/16 to 45/4
57 Kathryn De Beger 9 October 2024 46/12 to 47/12
58 Kathryn De Beger 9 October 2024 51/24 to 54/21
59 Kathryn De Beger 9 October 2024 44/6-19
60 Kathryn De Beger 9 October 2024 48/7-12
61 INQ0003611/4
62 INQ0003471/2
63 Kathryn De Beger 9 October 2024 50/21 to 51/23
64 Kathryn De Beger 9 October 2024 51/22-23
65 Kathryn De Beger 9 October 2024 57/1-8
66 Kathryn De Beger 9 October 2024 57/9-16
67 Dr U 7 October 2024 167/24 to 168/6
68 Dr U 7 October 2024 183/15-19

69 Dr U 7 October 2024 183/20-24

70 Dr U 7 October 2024 184/1-7

71 Karen Rees 21 October 2024 178/2-9

72 Karen Rees 21 October 2024 159/8-14

73 Alison Kelly 25 November 2024 197/13-22

74 Kathryn De Beger 9 October 2024 55/18-22

75 Kathryn De Beger 9 October 2024 56/12-22

76 Eirian Powell 17 October 2024 117/17 to 118/16

77 Court of Appeal (Criminal Division) Judgement [2024] EWCA Crim 748 8/27 to 9/31

78 Mother E and F 18 September 2024 49/18-21

79 Dr U 7 October 2024 215/2-12

80 Dr U 7 October 2024 219/3-16

81 Dr U 7 October 2024 219/10-16

82 INQ0003471/5

83 Karen Rees 21 October 2024 170/3-13

84 Karen Rees 21 October 2024 170/14-17

85 Dr U 7 October 2024 217/17-23

86 Dr U 7 October 2024 217/23 to 218/6

87 INQ0100851/2

88 INQ0100851/3

89 Dr U 7 October 2024 222/4-10

90 Mother E and F 18 September 2024 50/13-23; Mother D 17 September 2024 6/17-21

91 Mother E and F 18 September 2024 50/24 to 51/9

92 Eirian Powell 17 October 2024 35/12 to 36/4

93 NMC, *Guidance on Using Social Media Responsibly*, 2 July 2015 (https://www.nmc.org.uk/globalassets/sitedocuments/nmc-publications/social-media-guidance-2016.pdf)

94 GMC, *Using Social Media as a Medical Professional*, 30 January 2024 (https://www.gmc-uk.org/cdn/documents/using-social-media-as-a-medical-professional-final-version_pdf-105395775.pdf)

95 GMC, *Using Social Media as a Medical Professional*, 30 January 2024, page 6, para 18 (https://www.gmc-uk.org/cdn/documents/using-social-media-as-a-medical-professional-final-version_pdf-105395775.pdf#page=6)

Chapter 18. Nursing and Midwifery Council

Introduction

18.1 The Nursing and Midwifery Council (NMC) has four broad areas of responsibility in respect of the 854,000 nurses, midwives and nurse associates on its register:*

a. education, training and standards

b. registration

c. revalidation

d. fitness to practise.

Its activities are funded by the annual subscriptions of its members, currently £120 per annum.†

18.2 By the time of the Inquiry hearings, there had been a change of senior staff at the NMC. Evidence was given by the Acting Chief Executive and Registrar, Ms Helen Herniman. Ms Herniman is an accountant and until 2021 her career had been in finance. She was for some years the Chief Finance Officer for the Law Society. In 2021, she joined the NMC as the Interim Executive Director of Resources and Technology Services. From 4 July 2024, she served as the Acting Chief Executive and Registrar. She was due to return to her former role in January 2025 on the appointment of an Interim Chief Executive. I am grateful to Ms Herniman for shouldering much of the burden of the NMC's response to the Inquiry. She relied, understandably, on the written statement of Ms Andrea Sutcliffe CBE, former Chief Executive and Registrar of the NMC, who resigned due to ill health on 4 July 2024.

Employer Link Service

18.3 The NMC's Employer Link Service was developed in response to Sir Robert Francis KC's report of his Inquiry into Mid Staffordshire NHS Foundation Trust. Its purpose is to advise and support employers with concerns about nurses, midwives and nursing associates on the NMC register. In her written statement, Ms Sutcliffe wrote and Ms Herniman

* As at the end of March 2025.

† Fee correct at the time of writing.

confirmed that the Employer Link Service aims to encourage robust local investigation where there is concern relating to nurses or midwives. Where that was successful, the number of fitness to practise referrals would reduce, with a clear benefit to the NMC.

18.4 The Employer Link Service was piloted between September 2015 and operated substantively from April 2016. There were six Regulation Advisers, who reported to an Assistant Director who reported to the Director of Fitness to Practise. The Employer Link Service has access to lawyers and other professionals within the wider NMC.

18.5 On 22 July 2015, the NMC published a manual for the Regulation Advisers (Employer Link Service Regulation Adviser Manual). The Manual describes the Regulation Adviser as:

> "*the first point of contact for the DoNs/CNs or HoMs* [Directors of Nursing / Chief Nurses or Heads of Midwifery] *when they wish to discuss urgent issues, escalate concerns, or seek advice and guidance. If an RA* [Regulation Adviser] *is unable to assist the DoNs/CNs or HoMs they should discuss the matter with colleagues internally before providing the DoNs/CNs or HoMs with a response, or sign post the DoNs/CNs or HoMs to an appropriate member of staff who can deal with their query*."[1]

18.6 In 2016, Regulation Advisers were, and still are, expected to hold regular meetings with the Directors of Nursing on ongoing cases and raise awareness about the range of enquiries they can help with. The Regulation Advisers were also available "*as needed*" to provide expert advice on how to deal with patient safety risks, fitness to practise thresholds for referral and queries about revalidation.[2]

18.7 Where an employer makes a referral of a nurse to the NMC (on advice from the Employer Link Service or otherwise), the NMC considers whether to restrict the practice of the individual subject to the referral. An interim order may be imposed, removing the individual from practice or placing restrictions on their practice while an investigation takes place.[3] Those powers are in place only once a referral has been made.

Involvement with the Countess in 2016

18.8 When Ms Kelly made contact with the service in July 2016, the Employer Link Service had only been operating for three months. Ms Herniman described it as an "*inexperienced function*" at the time.[4]

18.9 Mr Tony Newman, a Regulation Adviser, gave evidence to the Inquiry. He joined the NMC in April 2015 as a senior project manager after 33 years working for the Post Office in a security role. He was appointed to the role of Regulation Adviser in May 2016.

18.10 Mr Newman explained that, at that time, when he received a call from an employer he had three broad options:

a. Advise the employer to make a referral to the NMC.

b. Advise the employer not to make a referral to the NMC.

c. Advise the employer to investigate and/or manage the concern locally.[5]

18.11 This was a gloss on the provisions of the Manual, which set out the three outcomes thus:

> "1. *refer the nurse* [or] *midwife to the NMC for investigation. The RA* [Regulation Adviser] *should encourage the provision of appropriate evidence, remind the DoN/ CN or HoM* [Directors of Nursing / Chief Nurses or Heads of Midwifery] *of the importance of continuing local action and manage expectations around outcomes*
> 2. *deal with the issue locally, with monitoring and review, for future update with the RA*
> 3. *deal with the issue locally*"[6]

As Mr Newman acknowledged, the advice given to employers about the need to refer can be important in protecting patients.

Mr Newman's initial contact with Ms Kelly, July 2016

18.12 Ms Kelly sought a meeting with Mr Newman in early July 2016. This was shortly after the deaths of Baby O and Baby P and at a time of intense activity in the Countess when the consultants were pressing for Letby to be removed from nursing duties and for the police to investigate. The hospital was also about to downgrade the neonatal unit. A telephone meeting was arranged, which took place on 6 July 2016, between Mr Newman, Ms Kelly and Ms Hodkinson. In accordance with the NMC guidance, Mr Newman summarised the conversation in an email he sent to Ms Kelly, copying in Ms Hodkinson. At 14:04 the same day, Ms Kelly made some corrections that Mr Newman accepted.

18.13 The email, as corrected by Ms Kelly, recorded that there was an increase in mortality of babies on the neonatal unit, and continued:

> "• *Each death has been the subject of a clinical team case review.*
> • *The reviews have produced no evidence as to a lack of competence by individuals or the team.*
> • *…*
> • *… one registrant … has been present at nearly all these incidents.*
> • ~~*The trust*~~ *Some clinicians are concerned that the registrant may present a serious risk to public safety although no evidence is available at this time.*
> • *The* ~~*trust board*~~ *executive team are due to meet today (6/7/16) to decide if this registrant will be reported to the Police to investigate.*"[7]

18.14 Mr Newman agreed in evidence that the totality of the information amounted to a potential concern about a connection between deaths and an individual. He pointed out: "[T]*here was never an allegation that she could be causing deliberate harm or that there was serious negligence*."[8] He agreed that there was an inference to be drawn that some clinicians were concerned that the registrant had killed babies. He said: "*Yes, which is why I took the time to actually record the conversation … it crossed my mind*." Mr Newman told the Inquiry that it was the only time in eight and a half years in the role that he has ever recorded a conversation in a follow-up email in this way.[9]

18.15 Mr Newman considered, correctly in my view, that his inexperience affected how he handled the call: "*At the time, I think this was probably only the second call that I ever took on the advice line … this could even have been the first … So it was my uncertainty as to the level of risk at the time which probably caused me to follow up in writing*."[10] His instinct to do so was undoubtedly right and it happened to be consistent with the Manual, which required Regional Advisers in all such cases to send an email to Directors of Nursing enclosing a record of the telephone meeting and inviting confirmation of content and errors to be corrected; this is what Mr Newman did on this occasion.[11]

18.16 Mr Newman said that during the call he advised Ms Kelly to manage and investigate the case locally. This was in keeping with the NMC approach, as expressed by Ms Herniman. He explained that he did not advise Ms Kelly to refer the case to the NMC because "[w]e *wouldn't be able to take any action. We wouldn't be able to consider it for an interim order without further information or evidence of deliberate harm. So we were in a really difficult situation*."[12] Mr Newman said that, if he had advised Ms Kelly to refer the case, it would have sat in the screening section until evidence was presented.[13] He explained that he advised local management and investigation of the case because "*there was no allegation and there was no evidence to support* [it]".[14]

18.17 Mr Newman did not sufficiently probe the information he was being given. The Manual provided a useful Regulation Adviser Case Referral Checklist, prompting the Regulation Adviser to ask relevant questions, such as "*What is the risk to patients/public in allowing the nurse or midwife to continue to practise unfettered?*" and "*Does the local investigation and management satisfy you that the issue is being appropriately managed?*"[15]

18.18 Ms Herniman reflected: "*Had we have asked further questions, shown more curiosity, … we may have received information which would have potentially led us to give different advice regarding the referral*."[16] In evidence, she agreed that it would be helpful for the Employer Link Service to have a checklist with concise pertinent questions.[17] I hope this is now in place. If it is not, urgent steps should be taken to provide it to the Regulation Advisers. She agreed that the handling of the call underlines the importance of the recommendation in the Allitt Inquiry that awareness of deliberate harm should be heightened in the minds of staff. Ms Herniman said, "*I can understand why it would be beneficial to always have that in -- at the forefront when we are receiving any information into us as the regulator*"[18] because it may lead to a conversation developing about deliberate harm. That reminder should be embedded in the training and checklists provided to all Regulation Advisers.

18.19 At the very least, Mr Newman should have referred the matter to his line manager, the Assistant Director, before providing a response, or he should have referred Ms Kelly to someone with experience in screening. Had he referred the matter to his line manager, it is likely that he would have been asked to call Ms Kelly and obtain more information immediately to allow the NMC to assess the risk. Instead, Mr Newman decided, based on little information, that the matter need not be referred and that local investigation should continue. The Manual says that a matter may be left for local investigation "*if local processes are robust and there is no presenting patient risk*".[19] Mr Newman was not in a position to assess the first point and the Manual makes clear that it is not the Regulation Adviser's role to do so. Quality assurance of the local governance or investigation process is left firmly with the organisation conducting the process.[20] As to the second, I infer that he did not think there was a potential risk to other patients.

That may be because the significance of the information that there was no issue about the nurse's competence escaped him (just as it seems to have escaped so many others). In any event, if he had asked those questions, I am not confident that he would have been provided with all of the relevant information and concerns.

Peer review and benchmarking

18.20 Mr Newman explained that after the call he spoke to an experienced lawyer in the Fitness to Practise Directorate and informed them of the information he had received. He said that the lawyer agreed that, without evidence or further information to suggest the registrant posed a risk, the correct advice was to manage locally or investigate further. He added that his advice went through a peer review (in which other Regulation Advisers consider the call and the advice given) about a week after the call, on 19 August 2016, and that the team agreed with his advice.[21]

18.21 The notes from that meeting (at which Mr Newman was present) record that, "*following a conversation with KG* [Mr Kristian Garsed, Regulation Adviser], *this should be discussed further at the benchmarking meeting. This is potentially really serious. TN* [Mr Newman] *to follow-up by emailing the DoN* [Director of Nursing]."[22] At the end of the record, there is a note to the effect that, "[c]*onsidering the above circumstances* [as set out in Mr Newman's email summarising his discussion with Ms Kelly], *the NMC would need to be advised of both the trust board decision to report to the Police and any subsequent action taken by the Police … I would also recommend another* [advice] *call to take place following confirmation of any Police action.*"[23] It is not clear who made these remarks. The peer review meeting concluded that this call and the advice given should be referred to the following benchmarking meeting and that Mr Newman should follow up by emailing Ms Kelly.

18.22 Mr Newman emailed Ms Kelly on 23 August 2016, asking for an update. Ms Kelly did not reply until 31 August 2016. In the meantime, on 24 August 2016, Mr Newman's advice was discussed at the benchmarking meeting with senior lawyers, at which he was present. Mr Newman said that this meeting also agreed with the advice he had given.[24] The comments of the reviewers (not exhaustive) refer to "*looking into the actions of the midwife, cannot find action or conduct that the midwife is accountable. This is a matter for police investigation.*" That is followed by "*we do not have a plausible allegation*". The comments continue that, "*unless there is medical negligence – this is a matter for the police … the clinical governance risk manager should be assessing the risks in relation to this … did not find clinical failing therefore need to look into the chain of causation*". And "*feels like the Trust has this matter in their hands*". Finally, it is agreed that Mr Newman will "*follow-up*".[25]

18.23 Mr Newman said: "*Looking back at it now, it seemed like* [the call from Ms Kelly on 6 July 2016] *was an initial enquiry to maybe test the waters with the regulator.*"[26] By the time he gave his evidence to the Inquiry, Mr Newman knew what Ms Kelly had said in her evidence, and he said he now understood that certain information had been withheld from him during the call. He said that, if Ms Kelly had said directly that the hospital suspected the individual was harming babies, then "*that would have set off all sorts of alarm bells and we probably would have said, even with a lack of evidence because we would recommend that they contact the police, we would recommend a referral at that time so that we could contact the police*".[27] In truth, Ms Kelly would never have said 'the

hospital suspects' – hence her correction of Mr Newman's first email, changing the words "*the trust*" to "*some clinicians*". This relatively subtle correction underlined the fact that the executive did not suspect anything. At most, if asked, she would have said 'the consultants suspect'.

18.24 Mr Newman (and, more importantly, all those who were said to have approved his advice) knew that "*some clinicians are concerned that the registrant may present a serious risk to public safety*".[28] This was said in the context of deaths on a neonatal unit and, it was said, in the absence of concerns about competence. I do not follow why, in those circumstances, the fact that clinicians thought the nurse presented a serious risk to public safety was not sufficient to set off alarm bells with Mr Newman and all those who reviewed his advice. Where it was thought that a nurse may be harming babies, whether deliberately or not, there was cause for concern at the NMC.

18.25 In the call of 6 July 2016, Ms Kelly had told Mr Newman that there was to be consideration of whether to call the police that afternoon. This may have given the impression that the hospital was dealing with the matter in a responsible way but the reference to calling the police should have rung alarm bells in an experienced (or at least adequately trained) Regulation Adviser and in those who were reviewing and/or benchmarking. Such a person should have asked why consideration was being given to calling the police and asked to be informed of the decision the following day in case it affected his view. In fact, calling the police was not discussed that day or the following day. Nothing more was said about it at that stage.

18.26 I bear in mind that Ms Kelly consistently said to others that there was no evidence against Letby, so she would have said the same to Mr Newman. She did not tell him what was known about the unexpected and unexplained nature of the deaths nor did she mention the fact that, when Letby had been moved off the night shift, the unexpected deaths stopped happening at night. Nor did she mention that the most recent deaths had occurred during the day when Letby was on shift. Mr Newman's lack of experience meant that he asked no probing questions about the nature of the doctors' concerns, nor did he ask whether the nurse was currently working on the unit. Whilst the NMC does not have a corporate responsibility for safeguarding, its members have individual responsibility.[29] Mr Newman did not ask about safeguarding steps, nor did his colleagues. This was an important omission. As Ms Herniman accepted, the NMC should have been prompting the hospital to consider its safeguarding responsibilities.

Follow-up

18.27 On 23 August 2016, Mr Newman emailed Ms Kelly to seek an update. This was the day before the benchmarking meeting. Ms Kelly responded on 31 August 2016 and told Mr Newman that: (a) the Trust had undertaken a thorough internal review and nothing of significance was identified; (b) "*on receiving views from our clinicians*", the step was taken to remove Letby from clinical duties; (c) there had been "*no indication to discuss*" the matter with the police; (d) an external review had been commissioned; and (e) Letby would remain redeployed until the outcome of the review.[30] As with her initial call to Mr Newman, this was an economical approach to the facts.

18.28 This was the first time Mr Newman had heard that the nurse was being moved from the unit to non-clinical duties in light of the clinicians' views. It was a clear sign that there was a need to protect patients. He said that it looked as though the Trust was taking the right steps to protect against future risk. That was true, as far as it went. But it did not answer the question why this was necessary, nor did it protect patients in other settings (should there be any). Mr Newman accepted this and acknowledged that he did not ask the Trust to alert the NMC were Letby to move to work elsewhere. It did not occur to him. Ms Herniman accepted that the NMC should have asked Ms Kelly questions about why the nurse had been moved from clinical duties. The answer would probably have been that it was for her protection or because the consultants had insisted on it. Either response should have prompted more questions and the answers should have prompted advice to the Trust to consider their safeguarding responsibilities.

18.29 Mr Newman subsequently met Ms Hodkinson and Ms Kelly on 29 November 2016. He was told, for the first time (over four months after the event), that the neonatal unit had been downgraded because of the increase in mortality and that the independent review (the RCPCH report; see Chapter 20) had provided initial feedback that there was no risk to patient safety.[31] The note of the meeting records that the plan was to have a phased approach to Letby's return to the neonatal unit and that there were no grounds for referral of Letby to the NMC. The note of the meeting also records that the reviewers recommended a further, more detailed review of the cases by an external independent consultant.[32] Mr Newman said he was aware that the clinicians still had concerns at the time. Significantly, he said he was not given any further detail about the nature of the allegations against Letby or why the clinicians were concerned.[33] He conceded: "*I didn't at the time ask specifically what their concern was*."[34] He was asked whether he should have done and replied: "*Yes, and I would now given the same call*."[35]

18.30 Mr Newman conceded that, at the time, he had no idea of the quality of the investigation at the Countess. He was candid that the emphasis was put on the Trust's investigation. "*So I understand what you are saying, how can we leave the burden or responsibility down to NHS Trusts? But that's simply the way it was then and it still is, to a certain extent*."[36] This echoed what was in the Manual and what Ms Sutcliffe and Ms Herniman said about the aim of the service being to encourage robust local investigation. This keeps responsibility (and therefore all the resource implications) within the hospital, locally, rather than inviting a referral on fitness to practise, which would require the NMC to investigate. I do not doubt that local investigation is a good first principle but the NMC was faced with matters of real gravity. A local investigation of unknown quality and scope could never be the right response to those concerns.

NMC actions after the arrest of Letby, May 2017 to December 2023

18.31 On 17 May 2017, Ms Kelly requested a call with the NMC in relation to the RCPCH report. She sent a press release announcing a police investigation into the neonatal deaths. Mr Kristian Garsed, Regulation Adviser in the Employer Link Service, spoke to her on 18 May 2017. Mr Garsed records Ms Kelly saying:

> "[T]*here was a view held by several medical colleagues that a registrant (Lucy Letby) may be the common denominator … based on an identification of her as having been present on most, but not all of the occasions, when infants collapsed and or died. However, as is noted in the RCPCH review there was no certain picture of who was*

> *present on these occasions, and the most in depth analysis undertaken of the staffing situation, did not extend to non-clinical staff. In addition, the registrant has apparently a very good professional history and a high degree of clinical credibility and was not present on all of the relevant occasions. Other staff were present on a similar number of relevant occasions.*"[37]

I have set out on a number of occasions what was known by this stage. I will not repeat it here. It is sufficient to say that much of this account was misleading.

18.32 Mr Garsed's note of the call included his advice that: "[A]*t this stage … there is nothing which could amount to an identifiable or sustainable allegation of impaired fitness to practise*, *however the outcome of the police investigation has the potential to be very significant, and if this individual or any other registrant is identified as having been involved … then plainly referral/referrals would be necessary*."[38] Ms Herniman accepted on behalf of the NMC that it should have insisted that the police kept it up to date with their investigation.[39] I agree.

18.33 On 3 July 2018, the NMC learned through its media monitoring service that an individual had been arrested in connection with deaths at the Trust.[40] Ms Kelly was contacted by the Employer Link Service and, on 5 July 2018, submitted the referral about Letby.[41] In the referral, Ms Kelly set out a chronology, noting that on 27 June 2016 concerns had been raised verbally by paediatricians regarding increased neonatal mortality and that Letby was named as "*allegedly being on duty a number of times when incidents had taken place*", but adding that there was "*no evidence to suggest that this was a contributing factor to increased mortality*".[42] The NMC Chief Executive, as part of an email discussion on 6 July 2018, said: "*Of course we can't and shouldn't comment on the police investigation but we are able to say that for us to consider whether we should stop someone from working requires more than the fact of an arrest*."[43] The media manager replied: "*Yes there is no reason to be defensive, just explain why we are not in a position to restrict her practice*."[44] It does not seem to have been appreciated that Letby had not been working as a nurse for nearly two years by this stage. An application for an interim order should have been made.

18.34 For months, the legal department of the NMC debated whether or not to apply for an interim order. Views were divided. The view that prevailed was that an arrest was not sufficient evidence to justify an application for an interim order suspending a nurse's registration. The fact that there was other evidence was either not appreciated or ignored. This underlined and reinforced the absence of probing in the early conduct of this case – but the early conduct was by an inexperienced Regulation Adviser. The lawyers should have done better.

18.35 In evidence, Ms Herniman said:

> "[T]*he NMC has reflected and has already changed the interim order guidance because it was felt on reflection of the Lucy Letby matter, as well as another case the GMC were involved in, as well as comparing our guidance to that of other regulators, we felt that the way the guidance was written didn't give flexibility to, in extraordinary circumstances,* [so] *that you don't have to have an individual being charged in order to make the application.*"[45]

18.36 The updated interim order guidance published in March 2024 refers to a document titled 'Decision making factors for interim orders'. This currently states:

> "*A criminal charge may well provide cogent evidence of a concern, particularly if we are satisfied that the decision to charge and prosecute the person was made following a robust consideration of the evidence. However, we do not always need to wait until a person has been charged before applying for an interim order, and in appropriate cases we may decide that a case should be put before a panel for interim order consideration prior to charge.*"[46]

18.37 The NMC's position is that, had this guidance been in place at the time, a decision to suspend Letby on an interim basis **may** have been made sooner (for instance, at the point of arrest rather than charge).[47] I am surprised by the reference to the NMC being "*satisfied that the decision to charge and prosecute the person was made following a robust consideration of the evidence*".[48] It is not at all clear in what circumstances the NMC would consider it appropriate to assess such a decision nor how they would do it. The NMC also refers to a new practice of holding case conferences in complex or sensitive cases, which includes expertise from a range of areas (legal, clinical and safeguarding), and notes that a relevant regulatory decision-making process document is being actively developed.[49] Such a 'decision-making process document' must include a direction to establish what, precisely, is being referred and why. It should never again be possible for the NMC to be fobbed off, as it was here.

18.38 Letby was charged on 11 November 2020. On 12 November 2020, the NMC decided to apply for an interim order. On 20 November 2020, she was made the subject of an interim order of suspension on the grounds of public protection and public interest. This was reviewed and maintained.

18.39 It is the NMC's view that – in light of changes they are making – were a similar call made to the Employer Link Service today, questions would be asked that would elicit information that would lead to the advice to refer the matter to the Fitness to Practise Directorate. The assertion that a similar call would lead to advice to refer, which was included in the written opening statement on behalf of the NMC, caused some consternation within the current team of Regulation Advisers. They wrote to the Inquiry to say that they thought the outcome would be the same now as it had been in 2016.[50] There were discussions within the NMC, at the end of which I was informed – and I accept that the Regulation Advisers were satisfied – that the difference between then and now is the fact that further questions would be asked that would elicit information that would lead to the appropriate advice being given.[51]

18.40 On 19 January 2021, questions were asked within the NMC about the corporate memory of previous cases. The cases of Beverly Allitt, Colin Norris, Benjamin Green and Victorino Chua were listed. Ms Herniman said in evidence that she had heard of the Allitt case but was unaware of the others.[52] Ms Herniman acknowledged the need for a heightened awareness of the possibility of deliberate harm even in a hospital, as directed by Sir Cyril Clothier in the Allitt Inquiry.[53]

18.41 Letby was convicted on 18 August 2023, and on 22 August 2023 the NMC notified her that her case would be considered by the Case Examiners. She was referred to the Fitness to Practise Committee on 21 September 2023 for hearing, and on 12 December 2023 she was struck off the nursing register. She indicated in writing that she did not object to the application but restated her innocence.

18.42 In evidence and in their opening statement, the NMC rightly acknowledged a number of mistakes and failures.[54] However, these were most unfortunately undermined by the written closing submissions (unsigned and undated but not prepared by Counsel for the NMC), which reverted to the language of things that "*could have been*" done differently.[55] It should be understood by now that things **should** have been done differently. The language used, repeatedly, neither recognises failings and mistakes nor, importantly, takes responsibility for them. A particularly egregious example of language avoiding responsibility is at paragraph 35 of the written closing submissions. It reads: "*As explained in paragraph 4 above, the NMC has reflected on the events at CoCH* [the Countess] *and its own processes and has identified opportunities to learn and improve*."[56] This euphemism is unhelpful. The idea that errors should first be characterised as opportunities for learning may be appropriate in some situations. It is plainly not appropriate here. Before lessons can be learned there must be an honest recognition and acceptance of errors. What needs to be said (and what the witnesses did say in evidence) is that mistakes were made. Things should have been done differently. The NMC failed in a number of respects. It is essential to recognise the failures and then to take steps to ensure that the same mistakes do not recur. I acknowledge that some steps have been taken to improve the Employer Link Service and the NMC processes more generally. I understand that a document has been disseminated designed to bring about cultural change.[57]

Revalidation

18.43 Nurses have to renew their registration every three years. The process is described as revalidation.

18.44 The online Cambridge English Dictionary defines 'revalidation' as "*the process or act of making something or someone officially acceptable or approved again*". The dictionary gives examples of the word being used in a sentence, including: "*Doctors will be required to demonstrate their fitness to continue practising, a process known as revalidation*."[58]

18.45 The process for revalidating doctors meets that definition. On their website, the GMC say: "*Every licensed doctor must revalidate to show they're up to date and fit to practise, in line with the legislation that supports revalidation*."[59] The revalidation process for doctors is considered the 'gold standard' process. It is detailed, onerous and expensive.

18.46 Many other professions – nursing, pharmacy, the Bar – are required to renew their registration or practice certificate at given intervals. The renewal does not require revalidation. It generally requires self-certification that the applicant has completed a certain amount of work, professional development and, usually, a declaration of good character.

18.47 On their website, in a section directed at applicants, the NMC define revalidation as "*the process that all nurses and midwives in the UK and nursing associates in England need to follow to maintain their registration with the NMC*".[60] It continues: "*Revalidation is about*

promoting good practice, as well as strengthening public confidence in the nursing and midwifery professions. ***It's important to know that it's not about making an assessment of your fitness to practise*** [emphasis added]."[61] On behalf of the NMC, Ms Herniman and Ms Sutcliffe both reinforced that the process does not include an assessment of fitness to practise. That occurs only when a registrant is the subject of a specific referral.

18.48 To achieve revalidation, a nurse is required to have completed 450 hours of clinical practice over the previous three years and 35 hours of continuing professional development, as well as written accounts of practice and reflection. There is a requirement for the nurse to declare they are of good health and good character and that they have insurance.[62] Registrants are also required to declare if they have received a police charge, caution or conviction, and they must also declare whether they have been subject to a determination by a professional or regulatory body. There is no requirement to alert the NMC to any outstanding complaints or claims or even to a restriction on practice at the time of the application or in the three years under consideration. Nor is the application affected by a period away from practice.

18.49 Confirmation of the form is required.[63] The nurse seeking revalidation chooses the confirmer. The NMC suggests that it should be a line manager.[64] The current guidance to confirmers reads: "[I]*t will be your role to check whether a nurse, midwife or nursing associate has met the requirements of revalidation*."[65] That is not consistent with what follows, which informs the confirmer that most nurses keep a portfolio of evidence and use it to demonstrate how they have met the requirements. The following then appears: "*As a confirmer, you are not being asked to make an assessment of a nurse*['s] … *fitness to practise, or assess the quality of the individual's work. You are simply looking to see that they can demonstrate they have met the requirements*."[66]

18.50 The confirmer's role therefore is purely administrative. As the NMC website confirms, there is no assessment of the nurse's abilities, the quality of their work or their fitness to practise. The confirmer is not asked to confirm the truth of the contents of the form. Nor are they expected to correct errors. The sole question is – does the form demonstrate that the registrant has met the requirements for revalidation, as defined by the NMC. In my view, this is not revalidation as it is generally understood.

Letby's revalidation

18.51 Letby applied for revalidation on 30 August 2017.[67] Ms Farmer was her confirmer.[68] By this stage, Letby had not been working in a clinical capacity for over a year. Senior members of the Fitness to Practise Directorate of the NMC knew this. Notwithstanding this, Letby's application for revalidation was accepted by the NMC on 14 September 2017[69] because, the NMC explained, on the date she submitted her revalidation application Letby was not the subject of an interim suspension order. Had she been the subject of such an order, she would not have been able to revalidate. The fact that she had not been in practice for a year was, it seems, not relevant. Letby also confirmed that she was not the subject of any police charges, cautions or convictions. Had she been, she may not have been revalidated. Ms Farmer had confirmed that the information on the form met the requirements of the process.

18.52 Ms Herniman acknowledged, rightly, that improvements are needed to the revalidation process, which was introduced nearly ten years ago. A decision was made not to introduce a process in which the nurse is appraised and their practice examined as a doctor's would be. It is not suggested that this compromises patient safety. In other professions, as with nurses, fitness to practise is scrutinised only when a complaint is made and a referral is made to the fitness to practise panel. There are significant numbers of such referrals and there is a backlog. A process that considers experience, professional development and insight, if properly used, should strike the right balance between protecting the public and regulating nurses. I have recently been informed that the NMC is currently reviewing revalidation and consulting on changes, and that this includes consideration of strengthening the role of confirmers. This is a welcome step.

18.53 For this process to be more than a box-ticking exercise, both the nurse and the confirmer should be required to confirm the truth and accuracy of the contents of the form and any other written documentation forming part of the application. Consideration should also be given by the NMC to a requirement to declare any complaints or warnings, restrictions on practice or gaps in practice, so that they form part of the information to be considered when renewing registration. Often a gap in a nurse's practice is for parental leave. Sometimes it is the result of a health issue or, much more rarely, of some reservation about the nurse's practice. The nature and extent of the gap in practice, and its consequences, would be for the NMC to consider but in my judgement such gaps should be declared and considered. I bear in mind the evidence from Ms Sutcliffe: "*When a concern is raised about a professional on our register's conduct, health, or competence we investigate through our fitness to practise process*."[70] In the examples I have given above, it is possible that a complaint or a health issue or a reservation about practice might be relevant to the nurse's fitness to practise, in which case that process could then be initiated. This would be a question for the NMC to consider on a case-by-case basis. What is not acceptable is for people to slip through the regulatory net because there are two processes – the first of which does not require transparency, the second of which requires a referral.

18.54 It is striking that Letby received revalidation when she had not worked on a ward for over a year, was not permitted to work on a ward and a police investigation was taking place. She had been on observational work experience placements at Alder Hey when she had no patient-facing role in the hospital that was employing her. No questions seem to have been asked. All the boxes were ticked.

18.55 That there was no sharing of information between the fitness to practise team, Employer Link Service and the revalidation team at the NMC in this case is regrettable. The NMC draw attention to the fact that from 2022 the Employer Link Service, fitness to practise and registration teams have developed an intelligence sharing hub, as a forum for colleagues to share any potential regulatory risk, including emerging risks where it is felt a fitness to practise referral may be required. I also note the NMC announcement on their website in September 2025 about their increased use of technology.[71] I would expect this sort of information to be easily shared electronically across teams.

Name change

18.56 Revalidation gives false reassurance about the level of scrutiny that has taken place. Renewal of registration would be more accurate. The fact that Ms Sutcliffe and Ms Herniman (and, for that matter, the NMC website) spent so long explaining what revalidation is not, is a clear indicator that the use of the term revalidation is at best unhelpful and at worst misleading.

Endnotes

1 INQ0108447/35

2 INQ0108447/35

3 Nursing and Midwifery Order 2001, article 29 and 31 (https://www.legislation.gov.uk/uksi/2002/253/article/29; https://www.legislation.gov.uk/uksi/2002/253/article/31)

4 Helen Herniman 8 January 2025 90/14

5 Tony Newman 12 December 2024 207/14-23

6 INQ0108447/42

7 INQ0002951; the strikethroughs and yellow highlights in the email copy are Ms Kelly's corrections.

8 Tony Newman 12 December 2024 213/16-18

9 Tony Newman 12 December 2024 214/4-21

10 Tony Newman 12 December 2024 214/23 to 215/7

11 INQ0108447/69

12 Tony Newman 12 December 2024 216/15-18

13 Tony Newman 12 December 2024 216/12-14

14 Tony Newman 12 December 2024 219/24-25

15 INQ0108447/64

16 Helen Herniman 8 January 2025 93/21-24

17 Helen Herniman 8 January 2025 128/22 to 129/9

18 Helen Herniman 8 January 2025 93/9-12

19 INQ0108447/38

20 INQ0108447/42

21 Tony Newman 12 December 2024 215/11 to 216/14

22 INQ0108434/38

23 INQ0108434/38

24 Tony Newman 12 December 2024 229/17-22

25 INQ0108439

26 Tony Newman 12 December 2024 218/2-4

27 Tony Newman 12 December 2024 218/17-22

28 Tony Newman 12 December 2024 212/22-24

29 INQ0002419/17

30 INQ0002964/1

31 INQ0002447/6

32 INQ0002447/6

33 Tony Newman 12 December 2024 234/17-21

34 Tony Newman 12 December 2024 235/25 to 236/1

35 Tony Newman 12 December 2024 236/6

36 Tony Newman 12 December 2024 223/4-21

37 INQ0002449/1

38 INQ0002449/2

39 Helen Herniman 8 January 2025 103/25 to 104/5

40 Witness statement of Helen Herniman INQ0107926/8/para 33(h)

41 Witness statement of Helen Herniman INQ0107926/8/paras 33(h) to 33(i)

42 INQ0002455/7

43 INQ0108595/6

44 INQ0108595/5-6

45 Helen Herniman 8 January 2025 112/5-13

46 NMC, 'Decision making factors for interim orders', last updated 25 March 2026 (https://www.nmc.org.uk/ftp-library/interim-orders/applying-the-interim-order-test)

47 Written Closing Submissions on Behalf of the Nursing and Midwifery Council 4 March 2025 14/para 41a

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Chapter 19. Speak Out Safely

The Speak Out Safely Policy

19.1 During the period the Inquiry is examining, paediatricians were raising and reporting patient safety concerns that demanded safeguarding measures be taken and further investigation carried out. The Speak Out Safely (Raising Concerns about Patient Care) and Whistle Blowing Policy being used in the hospital in 2015 (the Speak Out Safely Policy) was written before Sir Robert Francis's *Freedom to Speak Up Report* was published in February 2015.[1] The Speak Out Safely Policy was designed to prevent recriminations against members of staff who reported patient safety concerns. Designated Officers were the initial points of contact for anyone wishing to make a disclosure about patient safety. Mr Harvey, Ms Kelly and Ms Hodkinson were Designated Officers, as was Mr Higgins and Ms Cooper (RCN Representative).[2]

19.2 The Freedom to Speak Up Review took place because there were serious concerns about the way that staff within the NHS were treated when they reported concerns about patient safety. Sir Robert Francis made several recommendations dealing with this issue, including the appointment of Freedom to Speak Up Guardians in NHS organisations. In light of the *Freedom to Speak Up Report*, the hospital re-issued its Speak Out Safely Policy in January 2016. The same people were identified as named points of contact for disclosures.[3] A number of them held roles which would potentially conflict with the role of Designated Officer, as they duly did.

19.3 The Speak Out Safely Policy specified that it applied to situations in which there was a reasonable belief, that is to say, an honestly held belief which was objectively reasonable, that a criminal offence had been committed, or that there was a danger to the health and safety of any individual (see paragraph 12.18, Chapter 12).[4]

19.4 Ms Kelly confirmed in evidence that the responsibility of a Designated Officer was to make sure staff who reported concerns in good faith did not experience recriminations.[5] Mr Harvey said: "*I never doubted that Dr Brearey, in particular, had concerns.*"[6] Ms Hodkinson also told the Inquiry that she understood the consultants "*were genuinely concerned*".[7] No executive suggested that they did not think the consultants' concerns were genuine at the time, nor that the Speak Out Safely Policy did not apply. Nevertheless, it is striking that, when they became aware of the paediatricians' suspicions and concerns about Letby, none of the Designated Officers took any steps to ensure that the Speak Out Safely Policy was used to protect the doctors from recriminations.

19.5 Ms Hodkinson confirmed that no substantive evidence was required to raise concerns under the Speak Out Safely Policy. It provided a route for dealing with concerns regarding criminal acts, and concerns did not need to be raised in a formal manner to be dealt with under the policy.[8] Ms Kelly also agreed that the Speak Out Safely Policy had provisions for making referrals to the LADO if concerns were raised, or there was an allegation that a person may have harmed a child.[9]

19.6 The Speak Out Safely Policy set out that the Deputy Director of HR, Ms Appleton-Cairns, was responsible for undertaking an annual audit to ensure compliance with the policy. Ms Appleton-Cairns confirmed she had ultimate responsibility for this, but she did not personally undertake the audit.[10] It was not clear what form her responsibility took. Her evidence was that Ms Hodkinson and Mr Harvey "*chose not to*" use the Speak Out Safely Policy in relation to the consultants' concerns.[11]

19.7 Ms Hodkinson, meanwhile, did not agree in evidence that the Speak Out Safely Policy was not used. She told the Inquiry: "[I]*n 2017 I know we formally recorded it as well but it was absolutely badged under this policy.*"[12] Speak Out Safely is not just about attaching a badge; it is about making sure that the policy is followed.

The Speak Out Safely Committee and records

19.8 The Speak Out Safely Committee, which later became the Freedom to Speak Up Steering Group, dealt with concerns raised by members of staff. In 2016, the group met monthly. In 2017, the group met every two months. Its members comprised the Designated Officers – Ms Kelly, Ms Hodkinson, Mr Higgins, Ms Cooper and Mr Cross.[13]

19.9 Ms Hodkinson told the Inquiry that the Speak Out Safely Policy was addressed in relation to the consultants' concerns at a meeting on 27 June 2016.[14] However, by September 2016, their concerns had still not been logged in any record of the committee. On 21 September 2016, Ms Appleton-Cairns emailed Ms Kelly and Ms Hodkinson and said: "[W]*e were going to ask Ian* [Harvey] *to speak to SB* [Dr Brearey] *and ask him to formally voice his concerns under Speak Out Safely. I think we need to do this in parallel – any thoughts?*"[15] Ms Hodkinson and Ms Kelly both replied, stating that they were unsure whether this had happened.[16]

19.10 On 28 September 2016, Ms Appleton-Cairns again emailed Ms Hodkinson about logging the consultants' concerns under Speak Out Safely. She wrote as follows: "*I understand you were considering asking Ian Harvey to speak to* [Stephen Brearey] *and the other Consultants and ask them to explain their concerns in writing under Speak Out Safely.*"[17]

19.11 Ms Appleton-Cairns told the Inquiry that Ms Hodkinson responded to say that she would speak to Mr Harvey about it, and they subsequently decided not to speak to the consultants.[18] Ms Appleton-Cairns also said she told Ms Hodkinson that Letby's grievance, lodged on 7 September, should run "*in tandem*" with the Speak Out Safely Policy for Dr Brearey and Dr Jayaram (see Chapter 22).[19]

19.12 In oral evidence, Ms Hodkinson accepted: "*I did speak to Ian Harvey at that stage. I don't know what Ian did with that.*"[20] She denied that she and Mr Harvey chose not to follow the policy: "*I can see why it, it's perceived that we didn't choose to use this policy. But that was not the case, it was considered under this policy.*"[21] I do not accept this; there is

no evidence, other than Ms Hodkinson's assertion, that the consultants' concerns were considered under the policy. Ms Hodkinson accepted that the consultants' concerns were not formally logged under the policy until the following year.[22]

19.13 Mr Harvey conceded the Speak Out Safely process was not followed for some time. He said, "*I don't recall a conversation with Mrs Hodkinson about whether it* [the concerns] *would constitute Speak Out Safely or not. But I would accept that we were late*" recording the concerns under the policy.[23]

19.14 Like Ms Hodkinson, Ms Kelly said in evidence that she considered the Speak Out Safely process on 27 June 2016. On that date, Ms Kelly, Mr Harvey and Ms Powell had a meeting. A handwritten note of the meeting reads: "*Consider SOS* [Speak Out Safely] *process*".[24] Despite considering the process, Ms Kelly told the Inquiry she did not consult the policy and that Speak out Safely processes were "*not fully embedded in the organisation*".[25] In the case of the concerns raised by the paediatricians, they were not used.

19.15 Ms Kelly's handwritten notes of an executives' meeting on 8 September 2016 contain the following: "*SOS process – IH* [Mr Harvey] *to discuss SB* [Dr Brearey] *as he initially raised concerns*."[26] Ms Kelly said that a further conversation needed to take place with Dr Brearey to gain clarity in respect of the consultants' concerns (by this time, Baby A, Baby C, Baby D, Baby E, Baby I, Baby K, Baby O and Baby P had died, and Baby B, Baby G, Baby H, Baby J, Baby M, Baby N and Baby Q had collapsed). Ms Kelly agreed that, "[o]*n reflection, looking back, yes*", they should have logged the consultants' concerns under Speak Out Safely at that stage.[27]

19.16 On 18 November 2016, Ms Hodkinson had spoken with Mr Pace from DAC Beachcroft, where he is noted as explaining that the consultant had raised whistleblowing concerns that needed to be investigated.[28] Mr Pace had advised the Trust to deal with the concerns under the Speak Out Safely Policy, and that there was no requirement to put concerns in writing to trigger the policy (see Chapter 22).[29]

19.17 The concerns were not, however, formally logged until 24 April 2017. Before then, and in various meetings (and in the investigation report of Letby's grievance), it was stated, wrongly, that the consultants' concerns were handled under Speak Out Safely. For example, on 26 January 2017, in a meeting between the consultant paediatricians and the executives, including Ms Kelly and Ms Hodkinson, Mr Chambers stated that the "*Speak Out Safely process had been professionally managed*".[30] This was not true. Ms Kelly accepted: "*I didn't challenge Mr Chambers at that meeting about that comment.*"[31] She agreed that she should have challenged this assertion because it was not true, and she knew it.[32] The same applies to Ms Hodkinson. Dr Brearey described the consultants as "*stunned*"[33] by what they were told and labelled the executives "*incompetent*". He said: "*How you can start a meeting saying you followed Speak Out Safely practices and then tell seven Consultants* [in fact eight] *who all have significant concerns like this that they are to apologise to the person and that she would be going back to work or else there will be consequences.*"[34] Dr Brearey's observations were justified. Contrary to Mr Chambers' assertion, the Speak Out Safely process had not even been started. Despite Mr Harvey's assertions that it was fine to raise concerns, experience had shown the consultants that it was not fine, far from it.

19.18 In the minutes of a Speak Out Safely Committee meeting, dated 20 February 2017, Ms Kelly's contribution is recorded as follows: "*AK said that we need to consider whether the concerns raised by paediatricians in NNU* [the neonatal unit] *need to be formally logged.*"[35] As Ms Kelly confirmed, this was the first Speak Out Safely meeting where the concerns were discussed.[36] This was at least seven months after every senior executive knew of the consultants' concerns, and a stage at which the views of senior executives had become entrenched. After discussion, the committee decided the concerns would not be logged and would instead be managed by other routes. Such other routes would not include protection for the consultants. Ms Kelly's evidence was that the committee found it difficult because the nature of the concerns did not fit solely within one policy. This is not a reason not to log concerns.[37] However, on reflection, she conceded: "*I think we should have just put it on the list as per* [the] *policy.*"[38] Reflection was not needed. It should have been done when the consultants spoke up; in any event, immediately after the discussions on 24 June 2016 (see Chapter 11).

19.19 On 16 March 2017, there was an executives' meeting. Ms Hodkinson informed the Executive Team that Dr Jayaram had told her that the consultants felt bullied, intimidated and victimised, and that they did not feel reassured by how Mr Chambers and Mr Harvey were managing the process.[39] In Ms Hodkinson's meeting notes, Mr Chambers is quoted as saying: "*New comments – deal with … Speak Out Safely*".[40] Yet no action was taken under the policy. See paragraphs 26.14 to 26.19, Chapter 26.

19.20 The concerns were not formally logged under the policy until the meeting on 24 April 2017.[41] There are two versions of these minutes. In the original version, it is noted that:

> "*Members did not recall agreeing* [at the February meeting] *not to formally log the concerns raised by the paediatricians about NNU … Ian Harvey had also had a conversation with one of the Consultants, who requested it be logged under SoS* [Speak Out Safely]."[42]

And that:

> "*Members agreed conversation with IH* [Ian Harvey] *should be logged and issue cross referenced on the SoS spreadsheet.*"[43]

19.21 In the amended version of the minutes, the section about the members not recalling that they agreed not to formally log the concerns is removed.[44] This appears to have occurred during the meeting of 6 June 2017. On that occasion, the group again expressed confusion as to any logging of the consultants' concerns. The group decided to update the meeting minutes from 24 April 2017 to remove this sentence: "*Members did not recall agreeing not to formally log the concerns raised by the paediatricians about NNU.*"[45]

19.22 The committee had got themselves into a difficult situation. Ms Kelly and Mr Harvey both knew that the consultants had been raising serious concerns for more than a year. They had received no protection. The minutes of the February meeting were clear. The attempt to rewrite the narrative in April was pointless. The concerns had not been logged in February 2017. There were only two potential reasons for that: either the concerns were not considered at all, or they decided not to log them. It does not matter which it was. The concerns were not logged until late April 2017. This was not just a question of form. It reflected how the committee was thinking about the concerns and

the consultants raising them. I see no explanation for the committee not logging these concerns, other than that they did not regard them as credible, whatever they might have said about believing them to be genuine.

19.23 Ms Kelly was asked whether the committee was trying to rewrite the past following the police involvement on 2 May 2017. Ms Kelly denied this and asserted that it was a "*genuine oversight that we didn't remember what we had agreed and the notes had been amended but there was nothing to say that anything suspicious was done around those notes*".[46] Whatever the reason for the rewrite of the minutes, the central point does not change. The concerns were not logged and the consultants were not protected – not at any stage. This was a serious failure. It arose in part from the absence of any understanding of the obvious conflict between, for example, Ms Kelly as the Director of Nursing, and her position on the committee. The same applied to Ms Hodkinson, who was advising the Board about employment and other matters. Ms Cooper's position as Staff-Side Chair and RCN representative and support for Letby was impossible. Mr Higgins was on the Board where decisions were being taken in the context of the consultants' concerns. Mr Cross was advising the Board (and the executives) about how to manage the situation. All members of the committee were conflicted.

19.24 Mr Higgins opined that the 'back and forth' recorded in the minutes reflected the "*indecisive conversations*" about how to deal with the consultants' concerns.[47] I accept Mr Higgins' evidence that there was indecision amongst the group on this issue. It was the inevitable result of conflicts between roles held by the same person. Ms Cooper's evidence was that the consultants' concerns were "*glossed over*" by the Speak Out Safely Committee.[48] She told the Inquiry: "[M]*y recollection is nothing was ever really discussed in those meetings about the neonatal unit.*"[49] In his evidence, Mr Higgins was invited to comment on Ms Cooper's assertion. He said: "*I think that's an accurate way of describing it … I think yes, the group did gloss over them.*"[50]

19.25 Standing back and looking at the concerns raised by the doctors, there was no reason for indecision (or, rather, a decision not to act, which was what happened). The committee knew that the concerns raised by the doctors involved criminal acts, safeguarding issues for babies on the neonatal unit, and that protection was required for the doctors from recriminations for raising such concerns. The policy should have been effectively deployed. The contrary was the case.

19.26 Ms Anne Murphy commented on the executives' inaction in her evidence:

> "*I personally feel that rather than Ian Harvey and Tony Chambers being confrontational and threatening towards the Consultants, the Consultants should have been listened to earlier. External agencies could have been involved earlier and had the Executive agreed to involve the police earlier, such as in March* [2016]*, when the Consultants first voiced their concerns about Lucy Letby, the outcome could have been a little different in the fact that several of the babies may not have died, but I think this with hindsight.*"[51]

19.27 Ms Kelly conceded: "*I accept that formalising those concerns through the Speak Out Safely process didn't happen in a timely way.*"[52] It was worse than that. The process was never effective in protecting the doctors. Ms Kelly denied intentionally depriving the consultants of protection via the Speak Out Safely process. Her explanation for

the failure to log concerns was that the issue got lost amidst all the other actions that were ongoing at the time.[53] That does not excuse a complete failure of protection for whistleblowers.

19.28 Despite raising concerns in good faith, the consultants were deprived of protection from recriminations under the Speak Out Safely Policy. This lack of protection was clear from the way the consultants were subsequently treated. Ms Kelly accepted that formalising the concerns would have been helpful to the consultants, and would have provided them with protection from the grievance process outcomes. Instead, they were subject to criticism for voicing concerns about a member of staff; required to apologise to Letby and engage in mediation with her; and reference was made to avoiding a GMC referral.[54] This is further supported by Ms Hodkinson's note of the meeting between her and Mr Chambers on 12 May 2017 (see also paragraphs 26.87 to 26.92, Chapter 26).

Patient safety and employment law

19.29 A number of employment law issues arose in respect of the management of Letby and the treatment of those who raised concerns about her. The Inquiry heard evidence from Professor John Bowers KC, an employment law expert.

19.30 Professor Bowers KC asserted that, in the NHS: "[T]*here's a tendency to consider employment issues separate to the issues of patient safety.*"[55] He explained:

> "[A]*s far as the employer is concerned they are looking at the risks that there may be a constructive dismissal case,* [potentially] *costing tens of thousands of pounds, ten days in an Employment Tribunal.*
>
> *So they are looking at those risks rather than perhaps the wider risks to patient safety.*"[56]

19.31 Professor Bowers KC agreed that, if a staff member posed a risk to others, an appropriate safeguarding measure would be to immediately remove that person from the situation. However, he sought to point out that, "*from an employment point of view, there's usually not a right to suspend. So suspension would be a potential breach of contract, could lead to a constructive dismissal.*"[57] As a result, he explained Trusts often opt to exercise their right to redeploy the staff member, as set out in the staff member's contract, to another unit. In his experience, Trusts commonly use redeployment to avoid a disciplinary process, because it is the "*easy option*".[58] He explained disciplinary action can take a long time, be financially costly and affect staff morale.

19.32 He agreed that, if an employment process is followed, which involves redeployment of the staff member, as opposed to immediately suspending them via a safeguarding process, the individual may still pose a risk to patient safety. Professor Bowers KC described this as a "*danger*" and a "*real problem*".[59]

19.33 Additionally, Professor Bowers KC cautioned about the use of grievances. He said that grievances are "*often used as a defensive manoeuvre*" in response to criticism or disciplinary action.[60] He explained that, when this occurs, Trusts tend to hear the grievance before the disciplinary on the basis that "*if the grievance is upheld then*

perhaps you either don't go ahead with the [disciplinary] *or you do it in a different way*". Professor Bowers KC considered that the people hearing the grievance should put less validity on it if it is clearly a counter manoeuvre to a disciplinary process.[61]

19.34 Professor Dixon-Woods shared her expertise on the tension between the competing priorities of an HR process and managing patient safety issues. She explained that HR departments must operate in accordance with Acas, and as such are typically:

> "*focused on essentially ensuring employment -- compliance with the expectations governing that process.*
>
> *The Trust may have very legitimate patient safety concerns or concerns about bullying, harassment …, but they may become subordinated to the handling of this process.*"[62]

Professor Dixon-Woods pointed out the power of unions: "*The unions may get involved … and the Trust may become extremely focused on avoiding progression to an Employment Tribunal*."[63]

19.35 Professor Dixon-Woods' evidence aligned with that of Professor Bowers KC in relation to grievances being used as a defensive manoeuvre. She told the Inquiry:

> "[P]*eople who are actually behaving badly may be able to engage in all kinds of counterclaims, grievances, they may be strategically advised by their union representatives on what to do in order that they essentially don't end up with a disciplinary outcome. So these are very complex problems that NHS organisations are having to deal with.*"[64]

19.36 Ms Appleton-Cairns was informed of Professor Dixon-Woods' evidence and was candid that this captured the "*essence*" of the challenge she was confronted with when Letby initiated a grievance.[65] She described Letby and the RCN filing separate, but simultaneous, grievances as a "*pincer movement*" that made her feel "*under pressure to hear the grievance*".[66]

19.37 Ms Hodkinson informed the Inquiry that, "*sadly, yes*", she had experience of staff using a grievance as a defensive manoeuvre in response to concerns being raised about them.[67] She stated that the RCN, who supported Letby to bring the grievance, "*put myself and Alison Kelly under significant pressure*".[68] Whilst that may well be true, the RCN were supporting a nurse. That is what they were there for. It was for Ms Kelly and Ms Hodkinson to do what was right. Instead, they and the other senior managers avoided involving the police and put pressure on the doctors not to do so either.

19.38 Ms Hodkinson explained her conflicting priorities as follows: "[N]*ot only are you considering the patient safety implications, the workforce implications, you are also considering the potential legal risk … we were at risk of a potential constructive unfair dismissal claim … I was trying to make sure that we were mitigating* [that risk]*.*"[69]

19.39 Professor Bowers KC recommended that there should be "*an overriding objective of some sort to take into account patient safety in all the employment decisions*".[70] An overriding objective of this type would have proved useful at the hospital. When Letby learnt she was being redeployed, and there were concerns about her involvement in the neonatal

deaths, she filed a grievance. The grievance was not recognised as a counter manoeuvre, and despite the subsequent RCPCH recommendation, no disciplinary investigation was ever attempted or undertaken by the hospital in respect of Letby until 2021, when she was in custody. Instead, the grievance procedure focused in part on what the doctors had or had not said. This was directly contrary to the Speak out Safely Policy.

Endnotes

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3 INQ0003012/4-5

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5 Alison Kelly 25 November 2024 43/23 to 44/1

6 Ian Harvey 28 November 2024 90/21-22

7 Sue Hodkinson 26 November 2024 40/2-6

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9 Alison Kelly 25 November 2024 43/10 to 45/16

10 Dee Appleton-Cairns 5 November 2024 173/21 to 175/21; INQ0003012/11

11 Dee Appleton-Cairns 5 November 2024 222/7-10

12 Sue Hodkinson 26 November 2024 13/15-16

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14 Sue Hodkinson 26 November 2024 12/19-22

15 INQ0002976/2

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18 Dee Appleton-Cairns 5 November 2024 222/7-21

19 Dee Appleton-Cairns 5 November 2024 221/21 to 222/1

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23 Ian Harvey 28 November 2024 96/8-11

24 INQ0015537/4

25 Alison Kelly 25 November 2024 47/25

26 INQ0015537/19

27 Alison Kelly 25 November 2024 50/15

28 INQ0102269

29 Ian Pace 21 November 2024 97/3 to 98/25

30 INQ0003241/2

31 Alison Kelly 25 November 2024 58/18-19

32 Alison Kelly 25 November 2024 59/2-14

33 Dr Stephen Brearey 19 November 2024 170/11
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35 INQ0098375/3
36 Alison Kelly 25 November 2024 60/4-6
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41 INQ0098434/4
42 INQ0098434/2
43 INQ0098434/4
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46 Alison Kelly 25 November 2024 67/8-11
47 Andrew Higgins 3 December 2024 37/22-23
48 Hayley Cooper 6 November 2024 14/1
49 Hayley Cooper 6 November 2024 13/24-25
50 Andrew Higgins 3 December 2024 38/13-21
51 INQ0101325/16/para 55
52 Alison Kelly 25 November 2024 50/25 to 51/2
53 Alison Kelly 25 November 2024 60/18-22
54 Alison Kelly 25 November 2024 61/1 to 63/16
55 Prof. John Bowers KC 5 December 2024 63/5-7
56 Prof. John Bowers KC 5 December 2024 73/20-25
57 Prof. John Bowers KC 5 December 2024 65/3-6
58 Prof. John Bowers KC 5 December 2024 63/20
59 Prof. John Bowers KC 5 December 2024 64/25 to 65/25
60 Prof. John Bowers KC 5 December 2024 90/13-21
61 Prof. John Bowers KC 5 December 2024 91/14-16 and 19-22
62 Prof. Mary Dixon-Woods 26 September 2024 51/2-8
63 Prof. Mary Dixon-Woods 26 September 2024 50/3-6
64 Prof. Mary Dixon-Woods 26 September 2024 53/6-12
65 Dee Appleton-Cairns 5 November 2024 193/14 to 194/1
66 Dee Appleton-Cairns 5 November 2024 249/3-9
67 Sue Hodkinson 26 November 2024 18/20-22

68 Sue Hodkinson 26 November 2024 102/19-21

69 Sue Hodkinson 26 November 2024 101/14 to 102/3

70 Prof. John Bowers KC 5 December 2024 63/12-14

Chapter 20. The RCPCH review

20.1 The Royal College of Paediatrics and Child Health (RCPCH), a charity, was established by Royal Charter in 1996. Its charitable objectives are: to advance the art and science of paediatrics; to raise the standard of medical care provided to children; to educate and examine those concerned with the health of children; and to advance the education of the public (and in particular medical practitioners) in child health.[1] It is the representative body (but not a trade union) for paediatricians, with over 22,000 members.[2]

20.2 The RCPCH oversees training in paediatrics.[3] It sets the paediatric curricula. It runs the Membership of the Royal College of Paediatrics and Child Health (MRCPCH) exam, a mandatory part of the training pathway for paediatricians.[4] It is responsible for the Specialty Trainee Assessment of Readiness for Tenure and oversees the training and assessment of paediatricians leading to the Certificate of Completion of Training, which marks the end of formal training in paediatrics. After that, the doctor is able to apply for a consultant role.

The Invited Review Service

20.3 In 2012, the RCPCH set up an Invited Review Service.[5] For a fee (in this case £19,000 plus VAT),[6] a small team of RCPCH-identified experts visit a paediatric service in a hospital, interviewing relevant individuals, reviewing evidence and providing a report with recommendations.[7] The aim is to assess how a service complies with accepted standards. It is not a regulatory process or a forensic investigation.[8] Its purpose is to provide a professional, external, independent peer review to help improve a paediatric service with a focus on patient safety.[9] The RCPCH's August 2016 'Invited reviews – A guide' described a service review at paragraph 2.2 as "*an invitation to visit and comment upon a current service*". It continued:

> "*This may be the whole paediatric service or a specific element such as safeguarding, neonates or emergency care. It will include meeting the paediatricians, nurses, managers and others who have links with the service. The terms of reference will usually be rooted in the quality, safety and efficiency of that service.*"[10]

20.4 In 2021, the Crisp Review,[11] commissioned by the RCPCH, found that the service rarely turned away work and there was no risk-based approach to selecting which requests would be accepted. The evidence to this Inquiry provided strong support for those findings. The review has led to important changes in the service, to which I will refer later in this chapter.

20.5 Ms Sue Eardley was Head of Invited Reviews for the RCPCH between 2012 and 2019. Her academic background was in electrical and electronic engineering. She was not a clinician but had worked in project management and had significant experience in healthcare. She had been a Non-Executive Director, and subsequently board Chair, at Mayday Healthcare NHS Trust. In 2005, she joined CQC, leading on children, maternity and child safeguarding policy. In 2010, she joined the RCPCH as the Head of Health Policy. Dr David Shortland, a consultant paediatrician, was Clinical Lead for the Royal College of Paediatrics and Child Health Invited Review Programme.[12]

20.6 Ms Eardley had developed the RCPCH review team process flowchart.[13] The first step is the request for assistance. In the second, the clinical adviser decides the most appropriate action, service, design or individual performance review. Alternatively, they can advise "*to explore internal or external processes eg NCAS* [National Clinical Assessment Service]*, GMC, BMA* [British Medical Association]*, MHPS* [Maintaining High Professional Standards]", with "[n]*o further RCPCH action*".[14]

20.7 Ms Eardley was asked whether the police ought to have been included in that list. She replied: "*With hindsight of course the police should be there. At the time, there was such an unlikely occurrence it had not crossed our mind.*" She added: "*Again, with hindsight, we would have expected the Medical Director would have gone straight to the police had they had suspicions.*"[15]

Mr Harvey contacts the RCPCH

20.8 On 28 June 2016, Mr Harvey contacted the RCPCH by email, asking whether they provided an independent review service "*for individuals practice or for departments where there are concerns*".[16] Ms Eardley replied the same day, explaining she was happy to have a confidential conversation. This probably took place by phone on 28 June 2016. She was referred in her oral evidence to a briefing sheet that she had prepared about the Countess review, in which she summarised the key issue as: "*Outlier for neonatal deaths over last 12-18 months. Done a thematic review and nothing highlighted – no pattern. Neonatologists say they were not expected (although some might have been).*"† [17] Ms Eardley stated she would not have written "*no pattern*" if she had been told about the nurse commonality or the deliberate harm allegations in the initial call with Mr Harvey.[18]

20.9 Although Ms Eardley could not remember exactly when, she said that, at some point before she assembled the review team, Mr Harvey told her, she thought on the phone, about the doctors' suspicions of deliberate harm and the correlation of a nurse being "*on shift at the same time as some of those deaths had occurred*". She said Mr Harvey informed her of this with "*almost a passing remark*".[19] Mr Harvey informed her that there was a rise in neonatal mortality and that there were concerns about a staff member. He also said no one had witnessed anything and there was no other evidence. Ms Eardley's impression was that Mr Harvey did not consider the concerns were significant.[20] She explained: "[I]*t was not intimated to me that that was a serious allegation taken seriously by the Medical Director. That's my inference at the time.*"[21] She was asked whether she ever thought Mr Harvey thought it was a serious allegation.

† The form has a date of 27 June 2016; however, Ms Eardley confirmed in evidence that it was 28 June 2016: **Sue Eardley 7 November 2024 143/17-19**.

She replied (using a phrase used by many in this Inquiry): "*I think if he had thought it was a serious allegation he would have called the police sooner.*"[22] The result was that she did not think this was a serious allegation either.

20.10 Ms Eardley was also asked whether she ever thought Ms Kelly thought it was a serious allegation. She replied that Ms Kelly was very supportive of Letby "*and in my recollection quite dismissive of the allegation*".[23]

20.11 Of the early telephone exchange, Mr Harvey said:

> "*I would have said that the paediatricians had raised concerns about an association of one member of staff. But that there was no other supportive evidence to go with that; that her managers and colleagues felt that it was related to her increased level of duty and that she was qualified in specialty. I probably wouldn't have been any more specific than that.*"[24]

This way of describing the paediatricians' concerns downplayed that which was known. It ignored the contents of the Thematic Review, in particular that deaths were unexpected and unexplained, and that there had been a pattern of deaths at night that had stopped when the nurse was moved to day shifts. Further unexpected and unexplained deaths then occurred during the day shifts.

20.12 Whatever Mr Harvey said left Ms Eardley with the impression that he did not consider the concerns were significant. Mr Harvey gave that impression, even though, at 29 June 2016, he and Ms Kelly had considered it necessary to call the police, as had others.[25] That information was not given to the RCPCH. Part of the purpose of engaging the RCPCH was as an alternative to contacting the police. Mr Harvey giving the impression that the concerns were not significant affected Ms Eardley's approach to the review. She thought it could go ahead.

20.13 Ms Eardley acknowledged that, "[l]*ooking at it now yes, we should not have proceeded. We should have gone back and said no this is not for us.*"[26] In her closing submissions on behalf of the RCPCH, Ms Scolding KC said it was "*singularly inapposite and wrong*"[27] to try and undertake a review of a clinical service when criminal allegations had been raised about a nurse. I accept that submission. Ms Scolding KC also confirmed in her written closing submissions[28] that the review was never going to answer the question of why there was an increase in unexplained and unexpected deaths. She also acknowledged that the review did not provide those answers.

20.14 At this stage, Letby was still working on the neonatal unit, although on holiday. Given that she knew the concerns of deliberate harm related to a spate of deaths, Ms Eardley should immediately have raised the question of safeguarding and asked what steps were being taken about that. She had been appropriately trained and had experience.[29] Her failure to act on safeguarding was serious. It is likely she did not even think of it, as she said when giving evidence.[30]

20.15 That Mr Harvey was keen for a speedy outcome is set out in notes taken by Mr Cross at a meeting between Mr Harvey and other executives on 30 June 2016. Mr Cross noted Mr Harvey saying that the review could be done in August 2016. There would be two neonatologists, two nurses, one lay. There would be immediate feedback and the full

report in two or three weeks. Mr Cross further noted: "*College drafting proposal this PM. IH* [Mr Harvey] *to draft ToR* [Terms of Reference]*, visits and i/vs & pulling data to feed into review*."[31]

20.16 By 30 June 2016, Ms Eardley had prepared a proposal document containing draft Terms of Reference.[32] It does not mention what Mr Harvey had told her about the doctors' concerns about the possible link with a nurse. As a result, before drafting the Terms of Reference, she had excluded the information that should have brought this process to an immediate halt.

20.17 Ms Eardley accepted that the process was rushed and the proposal lacked her usual care.[33] By way of example, the proposal stated: "*The concerns outlined in the client brief are not uncommon.*"[34] That sentence was from the standard template and Ms Eardley had left it in the proposal for the Countess review in error. It was obviously wrong. In her evidence, she said: "*That sentence was an error. The concerns outlined were uncommon.*"[35] Leaving aside the doctors' concerns, Ms Eardley accepted she had never before (nor has she since) been asked to undertake an invited review to identify why there had been an increase in mortality.[36] That of itself was enough to ring alarm bells. Instead, she carried on without reference to anyone other than Mr Harvey. Including the standard template sentence was likely to give false reassurance about the nature of the concerns and their seriousness, as she accepted in her oral evidence.[37]

20.18 Guidance published by the RCPCH envisaged the following process (after the initial contact seeking assistance):

- The clinical adviser, Dr Shortland, would determine whether there should be an invited review and be responsible for agreeing to it.[38]
- Dr Shortland would then decide whether a pre-review visit should be arranged.[39]
- Such a visit would usually be attended by either Dr Shortland or the Lead Reviewer from the RCPCH (Dr David Milligan) along with Ms Eardley.[40]
- Ms Eardley would then work with either Dr Shortland or Dr Milligan in preparing the Terms of Reference.[41]

20.19 Ms Eardley accepted that a pre-review visit to discuss the review with Mr Harvey in person should have taken place.[42] It did not. She accepted it was likely she did not speak to Dr Milligan about the formulation of the Terms of Reference.[43] I find that she did not and she should have done. Nor, as she accepted, did she speak to Dr Shortland about the Terms of Reference or the scope of the review.[44] She should have done both. Her evidence was that the Invited Review Service at the time was particularly busy, which made it challenging to follow procedure.[45] Being busy should not have led to ignoring the RCPCH's own guidance, particularly in respect of clinical input. It ought to have alerted Ms Eardley to the fact that the Invited Review Service was overloaded.

20.20 Mr Robert Okunnu, the Chief Executive of the RCPCH until December 2025, said that the Terms of Reference of an invited review can often take weeks to agree.[46] It was Ms Eardley's experience that a proposal would normally be put together within seven to ten days because most proposals were quite straightforward.[47] That may be right, but this review was not straightforward, as she should have realised early in the process. It may be that the pressure of time upon her was the reason she cut corners.

Terms of Reference

20.21 The initial draft Terms of Reference were set out in the review proposal of 30 June 2016 by Ms Eardley:

> "*The RCPCH will conduct a review of the COCH* [the Countess] *Neonatal unit against standards, evidence and guidance to examine*
>
> - *Is the service provision compliant with current professional standards?*
> - *Are staffing numbers and competencies appropriate for the acuity of the infants cared for?*
> - *Does the unit have?*
>   - *clear and engaged leadership*
>   - *good team working*
>   - *a culture of safety and proactive risk assessment*
>   - *sound governance processes*
>   - *a positive relationship with the neonatal network and transport service*
> - *Are there any possible common factors linking the recent neonatal deaths?*
> - *Are there any areas of concern of potential development which would improve outcomes?*"[48]

20.22 Paragraph 7.5 of the RCPCH document 'Invited reviews – A guide' provided that the RCPCH would not take on cases where "*the expected scope includes behavioural, misconduct, bullying, harassment or possible mental health concerns*" or "*the Police or Counter Fraud Service are involved*".[49] Ms Eardley did not consider that.

20.23 She accepted that the fourth Term of Reference above could be interpreted as including a review into the consultants' suspicions about Letby's possible involvement in the deaths. When asked why she did not make it clear in the Terms of Reference that they were not investigating these concerns, she replied: "*I think we were in a pressured environment. Had I had the time to sit back, reflect, discuss with people it may have been different.*"[50] She was leading the review. It was up to her to take the time she needed to take each of those steps. This was too important to approach in a hurry. If there was insufficient time, the review should not have been started.

20.24 On 7 July 2016, Mr Harvey emailed Ms Eardley[51] with a counterproposal in which the fourth Term of Reference was amended to:

> "*To consider concerns about the Neonatal Unit with specific reference to:*
>
> - *Are there any identifiable common factors or failings that might in part, or in whole, explain the apparent increase in mortality in 2015 and 2016?*"[52]

20.25 At the time, Ms Eardley did not think the change made a material difference to the investigation.[53] The "*recent neonatal deaths*" had been replaced by the "*apparent increase in mortality*". She did not notice the use of the word 'apparent'.[54] It should have caused her to ask a question. The increase in mortality was not apparent, it was a matter of fact. As described elsewhere in this Report, the use of the word 'apparent'

was deliberate. Mr Harvey endeavoured to explain that he had used the word 'apparent' because he did not know whether or not the increase was statistically significant. That is a different issue. What is not in dispute is that, in the year 2015, there was, in fact, a marked increase in the number of baby deaths. That increase was real, and continued into 2016.

20.26 Ms Eardley said in oral evidence that the RCPCH review was not investigating Letby. Thus it would not exclude the consultants' concerns.[55] She did not raise this with Mr Harvey. She should have done, given that he had raised the doctors' concerns with her.

20.27 The Terms of Reference did not include a case note review nor to look for or to exclude criminal conduct. The RCPCH review team were offering a service review, not a criminal investigation.

20.28 There was a failure of communication between Mr Harvey and Ms Eardley. Both are responsible for that.

20.29 Mr Peter Skelton KC, on behalf of Family Group 1, asked Ms Eardley why – given she knew the RCPCH could not exclude deliberate harm and the review team were not tasked with doing so – this was not reflected in the Terms of Reference. She stated: "*I can't say. I can only think it was too awful to contemplate.*"[56] This does not withstand scrutiny. The doctors had raised the issue. That having been done, it was being contemplated. It was not open to the RCPCH to decide it was too awful.

20.30 It is likely that the failure to make this explicit was the result of three factors: first, a mindset that meant that deliberate harm was unthinkable; second, Ms Eardley's belief, arising from the telephone conversation with Mr Harvey, that he did not think that the concerns were significant; and third, that she was working in a hurry. Even taken together, these were not good reasons to fail to make explicit that the RCPCH review could not exclude deliberate harm. As Ms Eardley said in evidence, the purpose of the review was to look at other potential causes of the deaths.[57]

20.31 The Terms of Reference were agreed too quickly. Haste led to predictable results, including an assertion by Mr Harvey that a high powered team had not called out a criminal act, when they could never have done so.

The review team and what they knew prior to the service review

20.32 In addition to Ms Eardley, the following were members of the review team:

a. Dr Milligan – Lead Reviewer

b. Dr Graham Stewart – Second Reviewer

c. Ms Claire McLaughlan – Lay Reviewer

d. Ms Alexandra Mancini – Nursing Reviewer nominated by the RCN.

It was Ms Eardley's role to support the review team, administer the project and ensure the timely delivery of the report.[58]

20.33 Dr Milligan, Lead Reviewer, was a consultant paediatrician and neonatologist at the Royal Victoria Infirmary and Great North Children's Hospital in Newcastle. He had retired in 2013.[59] Prior to this inspection, he had led several other reviews for the RCPCH. I note that, during the subsequent review of the RCPCH's Invited Review Service (resulting in the Crisp QI report), Dr Milligan was noted to have felt "*somewhat unprepared for the review*", having not been involved in the scoping of the review or any pre-visit meeting.[60] This was foreseeable. It would have been better had he raised this at the time. It may (but I cannot say would) have caused Ms Eardley to pause.

20.34 Dr Stewart, Second Reviewer, was a consultant paediatrician with a special interest in neonatology. He had over 15 years' experience in clinical leadership and management posts.[61] He had contributed to two service reviews for the RCPCH prior to the Countess review.[62] Dr Stewart died during the course of this Inquiry.

20.35 Ms McLaughlan, Lay Reviewer, qualified as a nurse in 1983, gaining qualifications in intensive care and teaching in 1998.[63] She remained on the NMC register until 2016 but had ceased practising as a nurse in 2004.[64] She had completed a law degree and the Bar vocational course and was called to the Bar in 2005. She had not undertaken pupillage and so was never entitled to practise as a barrister.[65] She was a former associate director of the National Clinical Assessment Service and a former Head of Fitness to Practise at the NMC.[66] Since 2014, she had worked as an independent consultant.[67] She had worked as a Lay Reviewer for the RCPCH since 2014.[68] As the RCPCH pointed out in their closing submissions, the fact that Ms McLaughlan had the title 'barrister' may have led people to believe she would be bringing a forensic approach (I note Dr Gibbs and others referred to her title). Her role was that of a lay person, not a lawyer.

20.36 Ms Mancini, Nurse Reviewer, was a senior neonatal nurse with over 25 years' experience. In 2016, she had been appointed Pan London Regional Lead Nurse for Neonatal Palliative Care.[69] She was not a member of the RCPCH; she was nominated for the invited review by the RCN.[70] She had not previously participated in any invited review.[71] She said in evidence that she had raised her lack of experience with the RCN and with Ms Eardley.[72] No doubt she was reassured that she should continue. That was reasonable. Lack of experience is not a reason for a senior nurse not to take on a first review as part of an experienced team.

20.37 Dr Shortland (then the Programme Lead for the Invited Review Service) recalls Ms Eardley speaking to him a few days before the review team was to go to the hospital. She told him that a nurse had been suspended but that the primary purpose of the review was to look at other factors on the neonatal unit that could have led to an increase in mortality. He did not remember the telephone call very clearly, but, on the basis of what he was told, he believed the review could go ahead. He acknowledged in evidence that, if there were any concerns about criminality or potential criminality, the police should be called and a service review could not investigate.[73]

20.38 Ms Eardley accepted that the other reviewers are correct in their recollection that she did not share her knowledge of the consultants' concerns with them.[74] Professor Stephen Turner (current President of the RCPCH) said in evidence that she should have done so.[75] I agree. This would have led to an early discussion of whether an invited review was appropriate at all.

20.39 Dr Milligan's evidence was that the early documentation the review team received (which included files on each of the babies) must have mentioned that doctors had raised the possibility that at least some of the deaths could have been attributable to poor practice or even malign interference. This was based on his recollection of seeing a list of seven or eight unexplained deaths together with the names of nursing staff who had been on shift at the time. He recalled observing that Letby was present for all but one or two of them.[76] The document he was referring to may be the Thematic Review, which was included on a list of the documents received by the RCPCH ahead of the review visit.[77] Alternatively, it could be a table listing nursing staff on duty at the time of 11 deaths or on duty during the shift prior to each death. That table shows that Letby had been on duty at the time of eight deaths, and on the shift before for two deaths.[78] The 11 deaths included those of Baby A, Baby C, Baby D, Baby E and Baby I. The other six babies did not feature on the indictment. Ms Eardley had no recollection of seeing that document.[79] Ms Mancini's recollection is that she saw this document while preparing for the review visit,[80] but she could not recall whether she saw the Thematic Review.[81]

20.40 Dr Milligan's evidence is that he wrote to Ms Eardley with his observation on the table he saw.[82] That is likely to be the entry contained in the RCPCH chronology dated 26 August 2016:

> "*I have had a look at most of the documentation (but not yet all the individual baby files) and we have much of the workload data I was looking for plus a more in-depth analysis of what happened with the index cases but a number of questions arise from that, not least that one individual appears to have been present for all but one of them.*"[83]

20.41 Ms Eardley accepted in her evidence that, by the time she received Dr Milligan's email, she knew about the doctors' concerns.[84] Dr Milligan had independently noted the presence of a nurse.

20.42 In sending to the review team the Thematic Review and files in respect of babies who had died, Mr Harvey was plainly expecting a review of the deaths. Ms Eardley did not discuss this with him. Mr Harvey said he had expected the RCPCH to carry out a case note review. Ms Eardley had never said that, but it was his expectation. He acknowledged that the "*Casenote Review wasn't explicit in the Terms of Reference*";[85] however, he had "*assumed that, in commissioning them to review on the back of concerns about increased mortality, it was inherent in that review that they would actually be reviewing the cases*".[86] Had Ms Eardley and Mr Harvey discussed the review as they should have done, the differences in their expectations would have been clear.

20.43 Despite Dr Milligan's evidence that he recognised the possibility that the deaths could be attributed to malign interference, he does not recall discussing or escalating that within the RCPCH. He accepts, with hindsight, that this might have been the correct course of action prior to the review proceeding.[87] Given what he knew at the time, hindsight is not necessary to come to that conclusion. Dr Milligan accepts that he should have recognised the potential hazards of conducting a review where potential criminality was raised. He accepts, in light of that, that he should have taken a more proactive stance in ensuring that conducting the review had been cleared by the RCPCH Invited Review Board.[88]

20.44 Dr Stewart's evidence is that he was not told that clinicians had raised concerns about Letby prior to the visit to the Countess on 1 September 2016.[89] Whilst he considered the staff rotas, he did not identify the correlation between Letby and the deaths.[90] He did not see Dr Milligan's comment identifying that one individual appeared to be present for all but one or two deaths, and he did not discuss that issue with Dr Milligan.[91] If he had been aware of those concerns, he would have questioned whether the review process was appropriate.[92] He would have been surprised if the RCPCH would have agreed to an invited review in such circumstances; it is not an appropriate means of investigating an increase in unexpected or unexplained deaths with an associated suspicion of criminality.[93]

20.45 Ms Mancini was unaware of the suspicions until she was at the hospital for the review. Ms Eardley had not informed her of the concerns and she had no discussion about them with other members of the review team.[94] In her preparation for the invited review, she too had identified that Letby was on duty during the shift, or the shift before, for 10 out of the 11 deaths shown in the table referred to above. She did not think at the time that there was anything particularly unusual about that.[95] She did not consider that an explanation for the unexpected and unexplained deaths might be deliberate harm.[96] I accept her evidence about that.

20.46 The evidence shows that there were differing levels of knowledge amongst the review team as to the concerns about deliberate harm. That deliberate harm might be suspected was appreciated by Dr Milligan on his review of documents ahead of the review, and an inference was drawn by Dr Shortland that there might be a relationship between the nurse and the deaths. Ms Eardley was aware of the concerns of possible deliberate harm and that the doctors were suspicious. That information should have been shared with the rest of the review team. It is information that ought to have led to the RCPCH not taking on the review in accordance with paragraph 7.5 of 'Invited reviews – A guide'.[97]

The review

20.47 Concerns about deliberate harm dominated the first day of the invited review on 1 September 2016. All the reviewers were present at the interview of Mr Harvey and Ms Kelly. Dr Milligan began the meeting by saying that the RCPCH "*may not be able to explore the detail of the deaths*".[98] In evidence, Mr Harvey confirmed: "[T]*hey told us that they may not be able to do a detailed case report.*" He described this information as "*unexpected*".[99]

20.48 In evidence, Mr Harvey said he understood the review team's position. He added: "*I anticipated that the College would be reviewing the individual cases … we had prepared all the documentation for them to do that to be told, 'Well, no actually that's not part of it' … hence the subsequent Jane Hawdon review.*"[100]

20.49 Nonetheless, Ms Eardley noted the following from Mr Harvey:

> "[C]*orrelation of one nurse – paediatricians see as elephant in the room, Lucy Letby. Pattern of babies collapse don't seem to follow normal pattern & respond to resuscitation in normal way. Multifactorial. Want to think the worst – but nothing else*

is pointing[?] to it. Director of Corporate affairs was DCI [Detective Chief Inspector] *before he retired. Rely on him not police[?] Huge nettle to grasp … Need to pull together before we press the nuclear button.*"[101]

20.50 Pressing the nuclear button was Mr Harvey's characterisation of calling the police. It echoed Mr Chambers' earlier description of "*pressing the doomsday button*" (see paragraph 16.4, Chapter 16). By this stage, the paediatricians had investigated the deaths. The post-mortems had been reviewed, the Silver Command exercise had been conducted. No explanation had been found for the increase in deaths or for the fact that the babies had not responded to resuscitation as expected.

20.51 By this stage, if not before, the review team were aware that Letby was no longer on the neonatal unit. The notes included: "*Lucy has full OH* [Occupational Health] *support + weekly catchups with her manager. Friends on the unit. Rest of the team – OH team are going to the unit & having regular visits*."[102] Ms Mancini asked to see post-mortem reports, and the notes suggest that these were to be sent through. Ms Kelly said that, at the time Letby was taken off the unit, it was way over capacity, so that they were not able to ensure safety. There is no evidence that the unit was way over capacity. The unit had been able to accommodate the triplets (Baby O, Baby P and Baby R) at the end of June 2016. Ms Kelly is recorded as saying that she had thought about supervised practice for Letby. The notes added: "*Clinicians threatened[?] to go to the police* [emphasis in original]."[103] This was not the case. As the evidence shows (see Chapter 16), the clinicians had agreed to supervised practice for Letby, but there were not enough staff to supervise her, so the plan was abandoned.[104] It was at that point that the clinicians insisted that she remain off the neonatal unit. As I have already written, when asked their view about whether the police should be called, they had all said they should be. The clinicians did not threaten to go to the police.

20.52 The notes go on to record that the executives said: "*Not excluded or suspended*[.] *Just taken out of duties*[.] *How to get her back in again? Need executives to get round the stem of the nettle.*"[105] Mr Harvey is recorded as saying that he "*had to intervene with the neonatal lead as junior doctors had been referring to her as 'nurse death'*" and that he could not see "*how* [the matter] *is concluded without calling the police*. *Unless there is something to satisfy the medical staff they can call the police.*"[106]

20.53 The review team observed that the executives had only realised how busy the unit was when they got analysts to have a look. The executives informed the review team about the difficulties with transfers and transports and said staff "[g]*et used to working to overcapacity and swallow their own smoke*".[107] This was a theme that had arisen in the meetings at the end of June 2016 as the executives sought reasons for the increase in the number of deaths.

20.54 The review team next interviewed Dr Brearey and Dr Jayaram. The notes of that interview record the paediatricians giving a detailed account of events:

a. Dr Brearey said that, having previously been comparable to other units and not an outlier, they had experienced three neonatal deaths in June 2015, had reviewed them in detail, and had met Ms Kelly and the Serious Incident Panel to discuss them. The notes then record him saying:

"*Learning from every case but no overarching deficiency in practice. Identified one nurse present @ all collapses[?]. Didn't think it was significant. Agreed to keep an eye on things. As the year progressed each subsequent mortality not huge concern but by end 2015 numbers stacked up a little. Nothing til August – then small IUG[?] bloodstains aspirate persistently*."[108]

b. Dr Jayaram said the babies had collapsed with no indication and "[d]*idn't respond physiologically how they should have done*".[109] Ms Eardley understood this to mean that the babies did not respond to medical interventions and medication as would be expected.[110] All seven consultants "*so not always the same one Talked to each other*".[111] Ms Eardley agreed when answering questions from Mr Skelton KC that each consultant came to his or her conclusion individually, in dealing with different cases.[112] It was not a question of groupthink.

c. Dr Brearey said they had invited Dr Subhedar, for a half day. Dr Brearey had done mortality meetings. They discussed everything with Dr Subhedar (this was clearly a reference to the Thematic Review that the review team had seen).[113]

d. Dr Brearey is noted as saying: "*Even after PM.* [post-mortem] *unexplained.*"[114] The post-mortems "g*ave no cause*"[115] for the deaths.

e. Both consultants wondered if there was something they were missing in the review of all the cases. There was nothing consistent except that Letby had been on shift during all the collapses and deaths.[116]

f. At the time of discussions with Ms Kelly and Mr Harvey (probably the meeting of 11 May 2016), Letby had been moved from night to day shifts. Six out of nine deaths had occurred between midnight and 4am. Her manager (Ms Powell) was concerned that she had been on duty for harrowing events and she was moved for pastoral reasons. The unexpected collapses then occurred in the day rather than the night.[117] In fact, as set out in Chapter 8, Ms Powell had moved Letby in April 2016; Mr Harvey, Ms Kelly and Dr Brearey learnt of this shortly before the meeting on 11 May 2016.

g. They said that the nurses were all happy with Letby's practice, record keeping and competence. She was "*young*[,] *no family*[,] *partner* [or] *children so always stepped up*". They also said: "*There because does more shifts than other nurses.*" They added: "*On since 2012 perfectly nice good in a crisis. Always there.*" They then asked: "*Thinking about it – what could she be doing? PMs gave no cause.*"[118] They then looked again at Baby A and the discussions with Dr Subhedar about the UVC and the long line.[119]

h. They said they then began thinking forensically: "*What happens with air embolism.*" They had looked at case studies, considered air embolism. "*Last [observations?] chilling*".[120] Ms Eardley explained this was a reference to what was observed on the babies: "[U]*nresponsive to any inputs… Odd skin discolouration. Blue with eyelids of pink.* [Query] … *?injecting air into babies?*"[121] Surely that was the moment for the review team to withdraw.

20.55 Ms Mancini recalled that, in a subsequent meeting with the rest of the consultants, they made plain to the review team that they similarly had concerns that Letby was doing something deliberate to harm babies.[122]

20.56 Thus, during the morning of the first day of the invited review, there was no doubt that all the paediatricians were concerned that Letby was deliberately harming babies and made their concerns plain. There had been an increase in neonatal deaths and those deaths were unexpected and unexplained. The consultants considered that the babies had not responded physiologically in the way that they would be expected to. There was a correlation with Letby that persisted even after she was moved from night to day shifts.

20.57 The nature of the interviews with the executives and the doctors on the morning of 1 September 2016 gave the clear impression that the RCPCH review team were looking at, or at the very least were interested in, the concerns raised about Letby.

20.58 Dr Stewart recorded in his notes of the review that members of the obstetric and neonatal teams thought that there was "*foul play*" in the deaths of babies.[123] He could not recall if that term was used or if it was his interpretation of what Dr Brearey and Dr Jayaram told the review team.[124] It does not matter. The message was crystal clear.

20.59 Ms McLaughlan was unaware of the allegations about a nurse harming babies until the first day of the review visit.[125] She said that, in their interview, Dr Brearey and Dr Jayaram provided a mixed picture.[126] Although she found the consultants to be sincere in their concerns, she said she was given false reassurance because they also described Letby as a good nurse and had not contacted the police about their allegations. This does not bear analysis. First, none of the doctors gave any assurance or reassurance, false or otherwise, to Ms McLaughlan. They were expressing their detailed concerns about Letby, sincerely, notwithstanding their high opinion of her competency. That was not a reason to be reassured. It was a reason to be worried. They had raised their concerns with the most senior managers in the hospital, whose responsibility it was to contact the police. If Ms McLaughlan or any of the reviewers really thought that the fact that the doctors had not themselves called the police undermined the validity of their concerns, they were bound to ask the doctors why they had not done so.

20.60 Ms McLaughlan was taken to her witness statement, in which she had stated that, if she had known about the concerns about Letby prior to the visit, she would not have participated in the review and would have advised the RCPCH that the Terms of Reference were misleading and a service review inappropriate.[127] She confirmed that she would have given such advice right up until the night before the review started.[128] Ms McLaughlan was asked why, when given such information some 12 hours later, she did not in fact advise that the review should be stopped.[129] She responded that the reviewers were "*given a false level of assurance*", this time from the Executive Team.[130] She recalled that the executives "*sort of dropped* [the allegations] *into the conversation as a 'by the way'*". She added: "[I]*t wasn't given any level of importance or credence and it was given to us as part of almost a breakdown in relations*."[131] She seems to have accepted the executives' approach without question. Her impression was that Mr Harvey did not want to contact the police and Ms Kelly was supportive of Letby.[132] Her impression is likely to have been correct. That was not a reason to continue; taken with the information from the paediatricians and the obvious difference in positions, it was a reason to stop.

Discussions about methods of killing babies

20.61 At lunchtime on 1 September 2016, the notes record that the review team discussed amongst themselves different methods that might be used to kill babies, including insulin injection and air embolism.[133] (Some of the methods have been redacted from the record for reasons of public protection.) This shocking interlude demonstrates that the team knew how serious the doctors' concerns were and were considering how a killing could be carried out without leaving obvious traces. Ms McLaughlan and Ms Mancini had no recollection of this discussion. Given its contents, it would not have been forgotten. It may be that neither of them was present.

20.62 It was plain that this service review should not proceed. There were discussions amongst the review team about whether or not to continue. Regrettably, there is no contemporaneous note of any of the discussions or of the reasons for the decision to continue.

20.63 A note from a further meeting the following morning with Ms Kelly and Mr Harvey includes the following: "*Not sure if the review will give you the answers you are looking for. Considered aborting* [and] *starting again but TOR* [Terms of Reference] *to be important to get the background.*"[134]

20.64 Ms McLaughlan and Ms Mancini had interviewed Ms Powell, Ms Rees and other nurses. They were all clearly of the view that the review team were looking at whether Letby was responsible for the deaths. All robustly defended her. No examples of behaviour that was other than exemplary were given at the interview. Ms Eardley pointed out that the senior nurses were very supportive of Letby.[135] She said nurses had described the concerns of the consultants as a "*gut feeling*".[136] It was accepted by Ms Eardley that the discussions regarding Letby with the other nurses should have been shut down but the lines had, by that stage, become quite substantially blurred.[137] The nurses (like the doctors) were not told that the review team were not investigating the concerns about Letby. The review team accepted, without challenge, the nurses' views and left the nurses with the impression that they were there to investigate the consultants' concerns about Letby.

20.65 In her witness statement, written in 2024, Ms McLaughlan said that the consultants' concerns about Letby were based on "*the personal view/feelings/interpretations of one person regarding Ms Letby*, *it was not based on fact and was uncorroborated*".[138] This was not correct. Ms McLaughlan had simply accepted, and then, years later, repeated, the views of the senior nurses. She did this notwithstanding the fact that she knew all the other doctors had the same view as Dr Brearey and had heard the detailed concerns of the doctors on that first morning. They had imparted a number of facts; I will not repeat them. When giving evidence, Ms McLaughlan eventually accepted that Dr Brearey had been expressing his professional opinion and that the other consultants shared his concerns.[139] She also indicated that, instead of reading the Thematic Review (with its appendix), she had read a spreadsheet showing staffing prepared by Dr Brearey.[140] She had concerns about the document because Dr Brearey may have "*manipulated the data*". When asked to clarify what she meant by manipulation, she explained that, if Dr Brearey had made a mistake, the spreadsheet could be misleading.[141] That is not manipulation. The suggestion that Dr Brearey may have manipulated data was a casual slur for which she had no foundation. This too was regrettable and was clear evidence of her hostility towards Dr Brearey and towards the doctors more generally.

20.66 Dr Stewart's evidence is that he was shocked when he learnt about the allegations concerning Letby. He thought the review team were being expected to assist in resolving an extremely complex situation.[142] At the discussion as to whether to abort the review, his evidence is that he expressed the view that the Trust had not been honest and transparent, and it may be better to leave. His recollection is that the opinion of Dr Milligan and Ms Eardley was that the review could continue by "*strictly adhering to the terms of reference*".[143] He could not recall if it was during that discussion, or subsequently, that he considered whether the review might prejudice a future police investigation.[144] It is not at all clear what Dr Milligan and Ms Eardley thought the review could achieve by "*strictly adhering to the terms of reference*". Nor is it clear what they meant by this. It was never clear why they elicited so much information from the managers and the doctors about the concerns about Letby, when they were not in a position to deal with them.

20.67 Ms Mancini said in evidence that, at the time, she did not think the review should be aborted. Nor did she recall that any other member of the review team had thought or said that it should. Her view was that the review should continue because it was still a very valuable fact-finding exercise.[145] She ultimately accepted that the position that emerged on the first day of the review – that the invited review was being used as a means of investigating unexpected, unexplained deaths where clinicians suspected a nurse of criminality – did mean that the review should have been stopped.[146]

20.68 Mr Okunnu said: "[W]*e do not know why the Invited Review team did not stop the review after learning this information.*"[147] He recognised as an inadequacy of the review the fact there was no documentation about the discussion between the reviewers as to whether to proceed or regarding the rationale for continuing.[148] I agree. This discussion should have been minuted.

20.69 Dr Milligan's evidence was that any steps to abort the review should have been taken at an earlier stage. Whilst that point is well made, it does not follow that, because it was not aborted when it should have been, it should continue. He considered that there were important issues, which may have contributed to increased mortality, to explore at the review, and the danger posed by Letby had been removed, as she had been suspended from clinical work.[149] Whatever he thought those issues were, the review ultimately found nothing that explained any of the deaths.

20.70 The RCPCH accepts that the invited review should have been aborted when concerns were raised at the beginning of the first day of the visit and that there was a failure by the review team to seek advice from the Invited Review Board when considering whether to abort it.[150] These concessions were properly made.

20.71 Had Dr Shortland been contacted by the reviewers for guidance, as he should have been, his advice would have been that the allegations were so concerning that the police should have been involved.[151] The issue being raised was a safeguarding one and it should have been escalated. In situations where there may be criminal allegations, a review should not be undertaken by the RCPCH.

20.72 I referred earlier to paragraph 7.5 of 'Invited reviews – A guide'. Whilst that provision should have prevented the RCPCH from taking on this review, paragraph 7.7 addressed the situation where such issues "*come to light during an IR* [invited review]". On one

view, these issues did not come to light during the review. They were there all the time. Nevertheless, it is instructive to see what the guidance is where something comes to light. Paragraph 7.7 provides:

> "[T]*he review should be completed in relation to its original remit unless advised to the contrary in order to avoid prejudicing other investigations by a public authority or regulator, but the reviewers cannot investigate or suggest solutions for any of the above. Clear scope boundaries should be agreed before further work takes place in order to avoid prejudicing other investigations, which should be undertaken if required under existing internal or NHS mechanisms.*"[152]

20.73 Accordingly, the process envisaged is that a review could be completed in relation to its original remit. If it were to proceed, clear scope boundaries should be agreed before further work took place. The review should not be completed where there is advice not to do so, given the risk of prejudicing other investigations. The guidance does not make clear who is responsible for giving such advice. Professor Turner's evidence was that the guidance could be interpreted as meaning advice from the Invited Review Board.[153]

20.74 As was largely accepted by the review team, and is the position of the RCPCH now, the review should have stopped when the concerns of deliberate harm by Letby were raised. These were allegations of the utmost severity. They were far beyond the scope of an invited review.

Interview of Letby

20.75 It is difficult to believe that a team, who at lunchtime had been discussing how a nurse might kill babies, decided to continue the review and then, knowing what had been alleged, agreed that Letby ought to be interviewed that afternoon. That is what happened.

20.76 Letby was not one of the scheduled interviewees. Her name was added to the list of interviewees during the course of the first day.[154]

20.77 Ms Eardley's evidence was that Letby was added to the list of interviewees following the (unrecorded) discussion by the review team as to whether or not to continue with the invited review.[155] Members of the review team felt it was "*very important to interview her*" and collectively agreed to do so.[156] Ms Eardley thought it was Ms McLaughlan who proposed the interview. Ms McLaughlan could not remember who proposed it. It is likely that it was Ms McLaughlan. She was very supportive of Letby and hostile to the doctors. Ms Eardley said that the team "*felt strongly*" that Letby should have the opportunity to give her perspective,[157] and, notwithstanding the risk of prejudicing any police investigation, the team considered "*that proceeding with the interview in a limited way would not be unhelpful*".[158] Ms Eardley accepted in her evidence that the only reason it was decided to interview Letby was because the consultants suspected her of murder.[159] This was an error, as she accepted.[160] I am satisfied that it reflected the view of the nurses on the review team (Ms McLaughlan and Ms Mancini) that this was doctors against a nurse and they wanted to hear the nurse's point of view.

20.78 Dr Stewart cannot recall who decided Letby should be interviewed and does not recall expressing a view on the appropriateness of doing so.[161] Members of the review team were concerned she had been moved from clinical duties without a clear HR process and believed she was potentially vulnerable.[162]

20.79 Ms McLaughlan accepted, at the time of giving evidence, that interviewing Letby was a wrong turn.[163] Her evidence was that, at the time, interviewing Letby "*felt like the right thing to do*".[164] Ms Mancini accepted in her evidence that it was a mistake to interview Letby.[165]

20.80 Mr Okunnu stated that the interview of Letby was "*highly unusual*", as she had been suspended from clinical duties at the time.[166] He stated that the view of the RCPCH in 2023 was that the interview should not have taken place.[167] Dr Shortland's evidence was that it was wrong to interview Letby.[168] The RCPCH's position is that Letby should not have been interviewed.[169] Professor Turner confirmed that it was wrong and should not have happened.[170]

20.81 Ms McLaughlan and Ms Mancini were considered to be the appropriate interviewers, as they were both women and had a nursing background. In their written statements, they disagreed with the RCPCH view that they should not have interviewed Letby, as did Ms Eardley. In evidence, Ms Eardley said that, at the time the interview with Letby took place, they did not consider that Letby's behaviour was misconduct, as all they knew was that she had been taken off clinical duty without an HR process having taken place. That there had not been a proper HR process was true. I cannot understand why they did not consider that the doctors' concerns raised misconduct. The paediatricians on the review team seem to have done so, as evidenced in the lunchtime conversation. The whole review team had heard all the same information. The lack of HR process paled into insignificance next to the seriousness of the concerns.

20.82 There is a further reason the interview should not have taken place: Letby was to be interviewed without the benefit of legal advice and none of the protections of the criminal process. What she said could be used in evidence against her in due course. This was irresponsible, as Ms McLaughlan should have realised. Be that as it may, the reason the interview was thought desirable was in the interests of fairness. It is likely that neither Ms McLaughlan nor Ms Mancini thought there was anything in the doctors' concerns. Were it otherwise, their decision to interview Letby is inexplicable.

20.83 Letby (accompanied by Ms Cooper in her capacity as an RCN representative) was interviewed by Ms McLaughlan and Ms Mancini at the end of the first day of the invited review, on 1 September 2016. Recollections of the interview differ. Ms Cooper remembers that Letby left part way through and she followed her out. Ms Cooper then returned to collect her own coat, at which point the reviewers asked her whether she felt that Letby understood the seriousness of the allegations being made against her.[171]

20.84 Letby messaged Dr U shortly after the interview. At 18:14 on 1 September 2016, she wrote: "*The two members were nice. They didn't ask much about the babies, it was more about the unit as a whole etc. In brief, it looks as though there is potential for this to go further over a long period of time. H thinks we need to look at taking out a grievance case.*"[172]

20.85 At 18:33, Letby wrote to Dr U: "*The report will take a minimum of 6wks with a preliminary tomorrow. They 'off the record' told me they think an investigation into the deaths will be a recommendation & I need to prepare myself as I would play a big part in that over due to being a common factor & it could take several months.*"[173]

20.86 Ms McLaughlan accepted, in evidence, that she did have in mind that a further investigation into the deaths was required on day one of the investigation (which is when Letby was interviewed).[174] However, she could not recall an "*off the record*" conversation between herself, Ms Mancini and Letby.[175] Further, she could not recall a time when Letby left the meeting and the reviewers had a conversation with Ms Cooper alone. She was informed of Ms Cooper's evidence that after the interview Ms Cooper had re-entered the room by herself to get her coat and that either Ms McLaughlan or Ms Mancini had said something along the lines of "*does she understand the severity of the allegations against her?*" Ms McLaughlan could not recall this.[176] I accept Ms Cooper's evidence about this. Ms McLaughlan was unable to remember much of the detail of events, when asked. It may be because she has conducted a lot of interviews. For Ms Cooper, this was a most unusual event, and it is more likely that she has remembered it accurately. It is also the case that Ms Mancini and Ms McLaughlan were sympathetic to Letby, whom they believed to have been unfairly treated by being moved without proper process. It is likely that they sought, off the record, to be supportive of her.

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20.87 During her oral evidence, Ms McLaughlan was referred to a note of an interview between herself and Dr Milligan, Ms Eardley, Ms Kelly and Mr Harvey on 2 September 2016 and a line which read "*gave Claire's number*" next to Ms Eardley's name.[177] Ms McLaughlan said she thought it was Ms Eardley who had given Letby her number.[178] It is not apparent that Letby ever used it. In the interview, an unidentified member of the RCPCH team is noted as saying: "*Suspect there will be a grievance. If nothing happens good case for constructive dismissal. She knows it'll be horrid.*"[179] This is consistent with Letby's messages to Dr U. It suggests that Ms McLaughlan or Ms Mancini had told Letby or her RCN representative (Ms Cooper) what was likely to be coming Letby's way and that she understood it was going to be very difficult. Such a conversation was misguided. The reviewers were conflating their personal desire to support a fellow nurse with their role as reviewers of a service. I make clear they had asked general questions about the neonatal unit and not about the concerns expressed by the doctors, and it was Letby who had raised with them, unprompted, the fact she had been redeployed without explanation. That this occurred is further evidence of the foolishness of carrying out this interview. I also note that Letby told the interviewers that she felt "*everyone* [had] *turned their back on her*".[180] This was not true. She had a significant number of supporters, as I have explained elsewhere.

20.88 I referred earlier to a note made during the meeting with the executives on 1 September 2016 regarding Letby: "*Just taken out of duties. How to get her back in again?*"[181] Ms McLaughlan's evidence was that this comment accorded with the impression she had that the executives "*hadn't given any credence to the allegations*".[182]

20.89 Also on 2 September 2016, Mr Millea (Regional Officer, RCN) emailed Ms Rees stating that he believed that Letby "*has grounds to action a Grievance*".[183] In a detailed letter, Mr Millea set out two principal concerns:

a. First, it had become apparent in the course of Letby's interview by the RCPCH the previous day that, contrary to the indication given in the meetings in July 2016, "*the terms of reference for this investigation does not seem to address the … concerns … in relation to the unacceptable high mortality rate on the NNU* [neonatal unit] *and our* [member's] *involvement*". Instead, the review appeared to be around more general matters such as procedure, culture and staffing levels, and therefore it would "*not solve the issues for Lucy personally*".[184]

b. Second, Letby's redeployment was "*a result of the* [Trust's] *response to …* [consultants'] *comments about our* [members'] *practice*". Mr Millea therefore requested "*to see the* [Trust's] *evidence to substantiate their actions*".[185]

20.90 In due course, the grievance was lodged (see Chapter 22).

Patients and parents

20.91 Ms McLaughlan described the role of the Lay Reviewer as representing patient and public interest, stopping too much jargon being used and ensuring amongst professionals who know each other that proper boundaries are maintained.[186] She described the role as being a "*critical friend*".[187] She accepted that her role on the review team involved representing the interests of the families of the children who had died and patients of the hospital more generally.[188] Her role was to ensure the process would be patient first.[189] She could not say she gave any thought to any invasion of their privacy when requests were made by the review team for post-mortem reports in respect of children who had died.[190] She accepted that "*perhaps*" she did not put the parents of the babies at the centre of her thinking while conducting the review, despite the fact that, of all the people at the review, it was her role to ensure that happened.[191] In reality, her focus was on protecting Letby and ensuring there was a proper HR process.

20.92 Some of the parents of the babies who featured in the RCPCH review found out about the investigation as a consequence of the press announcement in July 2016 (see Chapter 15). Some parents were not made aware that the review had taken place or that a report was available until February 2017, when Mr Harvey wrote to them (see Chapter 21). There were some parents who learnt about the RCPCH report for the first time at the criminal trial or as part of disclosure for this Inquiry.

20.93 Mother A and B first heard there was an independent review (by the RCPCH) at Baby A's inquest hearing on 10 October 2016 when Dr Jayaram mentioned it. She gave evidence that "*I'd like to have been more involved. I'd like them to have spoken to us. Because maybe we could have shed some more light on it* [Baby A's death]."[192]

20.94 The review team had requested and received copies of post-mortem reports and "*details of the nurses who looked after the babies at the time*".[193] Professor Turner accepted in evidence that the review team did not have the capacity to be carrying out the type of analysis they had embarked upon.[194] The review team should have appreciated that they were not in a position to assess and report on evidence of criminality.[195] They were out of their depth.

Safeguarding

20.95 Safeguarding was a specific element that could form part of a service review in accordance with paragraph 2.2 of the RCPCH's 'Invited reviews – A guide'.[196] It was one of the areas being reviewed by the RCPCH review team at the Countess in September 2016. The review team met members of the hospital's safeguarding team. There was an opportunity that would not trespass beyond appropriately clear scope boundaries, to test the effectiveness of safeguarding procedures by enquiring whether the safeguarders were aware of the concerns in the neonatal unit and, if so, what steps had or were being taken. There is no evidence that such questions were asked or that the reviewers explored if and how concerns were being managed from a safeguarding perspective, in accordance with the statutory guidance, *Working Together to Safeguard Children* (pursuant to which a referral to the LADO should already have been made by the Trust) (see Chapter 12). Ms McLaughlan and Ms Mancini rightly accepted in evidence that such questions ought to have been asked.[197] Ms Eardley's evidence was that she did not treat the allegations in respect of Letby as a safeguarding issue, though she recognised that she should have.[198] This was another failure by the RCPCH review team.

20.96 Dr Stewart's evidence is that, at the feedback session on 2 September 2016, the review team advised the management to involve the police *if* there was suspicion of criminality.[199] This is odd, since he knew there was suspicion of criminality – all the RCPCH review team knew that; and Dr Stewart's own notes refer to "*foul play*".[200] The suspicion had been there for months and the police had not been called. Dr Milligan describes a meeting with Mr Harvey at the end of the interview process (probably the one Dr Stewart refers to) in which the review team "*asked*" Mr Harvey if he thought the police should be involved and he responded with words to the effect that he preferred to handle the issue internally for the time being.[201] It is surprising that neither Dr Stewart nor Dr Milligan challenged him on the wisdom of that position. All that was necessary was to say we think you should call the police. The RCPCH team knew that Mr Harvey considered the police a nuclear option that he wanted to avoid. Unless someone said something direct, he would continue with his linear approach. First, eradicate all other possibilities. If no alternative, call the police. In their letter of 5 September 2016, the review team would recommend a detailed case review.[202] This was something else to be done instead of calling the police. More than two months had already passed since the deaths of Baby O and Baby P.

Letter of 5 September 2016 with recommended actions

20.97 Following the invited review visit, Ms Eardley sent Mr Harvey a letter dated 5 September 2016.[203] The letter was to confirm "*the short term advice which the team shared* with *you, Alison* [Kelly] *and then Tony* [Chambers] *on Friday*. *This is to enable you to move forward swiftly towards resolving the concerns and issues which have been raised, being just and fair to all involved and restoring confidence in the service as a whole.*"[204] Ms Eardley said the full report would be ready in draft for checking in four to six weeks, and sooner if possible. According to the letter, the RCPCH were aware that the neonatal unit had been downgraded so as to operate as a Level 1 (Special Care) unit for babies over 32 weeks' gestation and that "*one of our terms of reference* [was] *to explore whether there were any common factors that might explain the apparent increase in mortality in 2015 and 2016*".[205]

20.98 Ms Eardley went on to say that the review team were not aware until the meeting on 1 September 2016 that action had also been taken "*in early July to move one of your nurses from the unit to other duties, with a requirement that she did not contact colleagues from the neonatal unit*". She wrote: "[T]*his took place without a formal process nor clear notification to her of the reasons for so doing. These steps appear to have been taken on the basis of an allegation made by one member of medical staff, supported by his medical colleagues. Some staff were aware of this and the reasons, others were not.*"[206]

20.99 Ms Eardley pointed out that, when they interviewed Letby, she was under the impression that the RCPCH review "*would resolve the situation and enable her to resume duties on the unit*". Ms Eardley added: "*She appeared to be distressed that there was very little information as to the reasons for her move, and appeared isolated and vulnerable.*"[207] This is an interesting observation. As the letter of 18 July 2016 from Ms Rees to Letby makes clear, she was not prevented from contacting colleagues.[208] When Ms Rees gave evidence, she initially said it was her fault that Letby had misunderstood the position, but, as she acknowledged later in evidence, the clear terms of the letter make it plain that there was no requirement to that effect.[209]

20.100 Letby was well supported by Ms De Beger, Dr U, Ms Rees and a number of colleagues. She knew that she had been moved because she had been on duty for more of the deaths of babies than other staff – because Ms Powell and Ms Sian Williams had told her. It was not a great leap to work out that she may have been suspected of being responsible for what had happened. That may explain why, when, also in September 2016, Ms Cooper (her RCN representative) told her (because she too had believed Letby was unaware), Letby responded in a very low-key way, just as she did at a later date when Dr Green told her during the grievance process.[210] Ms Cooper thought it "*was a little strange*".[211]

20.101 There was no excuse for the RCPCH downplaying the nature of the concerns in this letter, asserting that there was "*an allegation made by one member of medical staff, supported by his medical colleagues*".[212] Even the reference to "*one member of medical staff*" understated the position and experience of the Lead Clinician for the unit and ignored the fact that seven consultant paediatricians were expressing concerns, not just supporting a colleague. That assertion persisted in Ms McLaughlan's statement to the Inquiry. It reflected the views of, at least, Ms Mancini and Ms McLaughlan, who were quite clear that Letby had been unfairly treated. They accepted the views of the senior nurses and the senior executives without even asking the paediatricians about those views.

20.102 Ms Mancini accepted in evidence that describing the allegation as being made by one member of staff was not an accurate description of the level of concern expressed to the review team.[213] The review team knew that the concerns were not just of one doctor but were held by the whole of the consultant team.

20.103 Under the heading 'Action required – HR Investigation', the letter required the Trust to take immediate steps to formalise the actions already being taken with Letby.[214] The letter read: "*It is important that the Trust takes immediate steps to formalize the actions you are taking with the nurse. Our understanding is that an allegation has been made and therefore a process of investigation needs to be put in place which set out the nature of the allegation and the process you will follow to investigate it*."[215] It goes on to remind them that they should provide appropriate support to the nurse "*and an effective communication strategy for the unit*".[216] To recommend an HR investigation into concerns

that babies were being deliberately harmed and dying was misconceived. The review team should have made plain to the executives that, whilst it was not for the RCPCH to investigate the doctors' concerns, it was a matter for the police, either directly or via the LADO.

20.104 The RCPCH accepted that recommending an HR investigation into allegations of murder was wrong and inappropriate.[217] Ms Eardley accepted that the recommendation was inappropriate, and the language used could give a false reassurance that a disciplinary process was sufficient.[218] It was worse than inappropriate, it was egregious.

20.105 The only explanation is that the RCPCH review team accepted the view that there was nothing in the allegations. The suggestion that an HR process would get to the bottom of the concerns was predicated on that view – that is, the cart was before the horse. Ms McLaughlan could not explain why they had gone down the path of HR rather than recommending calling the police.[219]

20.106 Ms McLaughlan accepted that a disciplinary process to get to the bottom of the consultants' allegations was inappropriate. Her evidence was that a process needed to be started.[220] She described a process involving suspending Letby from practice, referring her to the NMC and then referring her to the police, rather than calling the police to start the process.[221] She was not able to provide any satisfactory answer to the question "*why not call the police first?*"[222]

20.107 The second recommended action was a case note review. The letter from Ms Eardley said that the review team agreed that the pattern of recent deaths, and mode of deterioration prior to death in some cases, appeared unusual and needed further enquiry. A detailed forensic case note review of each of the deaths since July 2015 using at least two senior doctors with expertise in neonatology/pathology to determine all the factors around the deaths was recommended.[223] The reference to July 2015 was an error. The deaths to be reviewed began in June 2015. That error was to be repeated in other documents. The RCPCH team advised that the investigation should include five elements as a minimum:

> "*a) a full systematic chronology for each case including all interventions, and details of nursing and medical observations and activity*
>
> *b) a view on whether escalation of each case at an earlier stage to involve more senior opinion locally or more expert opinion from a regional centre would have potentially made a difference to the outcome*
>
> *c) examination (with the relevant paediatric pathologist) of the post mortem findings and any additional information available on their files which might identify cause of death, including rare conditions such as air embolism and severe metabolic derangement*
>
> *d) details of all staff with access to the unit from 4 hours before the death of each infant. Ancillary and facilities staff should be included*
>
> *e)* [c]*onsideration of any other 'near miss' cases with similar chronology / presentation where the child survived.*"[224]

20.108 The letter concluded by informing Mr Harvey that the review team had identified four individuals "*with appropriate expertise and experience who may be prepared to take on the casenote work swiftly for you on a private basis. I will advise these separately and continue to seek alternatives.*"[225]

20.109 Ms Eardley contacted Dr Jane Hawdon, a consultant neonatologist in London, and asked whether she would be content for her name to be put forward to undertake an independent case note review.[226] She did not explain why the case note review was necessary.[227] This was unfair on Dr Hawdon. Dr Hawdon agreed to take on the review (see Chapter 21).

20.110 The recommendation for a case note review extending to a neonatologist or pathologist considering all staff, including ancillary and facilities staff, with access to the unit was clearly outside the area of medical expertise of those disciplines. It was not a matter that a medical expert ought properly to be asked to report upon. Ms Eardley accepted that such an investigation was a matter for a police investigation.[228] Ms McLaughlan also accepted that this was an inappropriate recommendation for the RCPCH to make because it was a matter for the police.[229]

20.111 In his oral evidence, Mr Wilkie said that the Board should have been informed of the contents of the 5 September 2016 letter from the RCPCH.[230] Mr Wilkie stated that, had he been provided with that letter and the unredacted version of the RCPCH report and been informed of Dr Hawdon's conclusions, this would have prompted him to reconsider the decision not to contact the police.[231] In her oral evidence, Ms Hopwood conceded: "[C]*learly in hindsight, we should have had a board meeting before January to review the report.*"[232]

20.112 On 19 September 2016, a QSPEC meeting took place, attended by Sir Duncan, Mr Harvey, Ms Sian Williams, Dr Green, Ms Fogarty, Ms Rees and Non-Executive Directors Mr Higgins, Ms Fallon and Ms Hopwood. Mr Harvey gave an update about the RCPCH review, telling them that the review team had not raised any immediate concerns and that the Trust was awaiting their report; that the review team had recommended that the Trust commission a forensic review by two independent paediatricians of the cases that had "*sparked the external review in the first place*"; and that Mr Harvey was currently contacting candidates to undertake this to fit with the timing of the final report being received. Once the reports were received, and depending on the results, the Trust would "*need to discuss how the findings will be shared, particularly with the parents concerned*".[233] The recommendation from the RCPCH that there be at least two independent paediatricians involved in the case note review fell by the wayside.

Receipt of the draft RCPCH report

20.113 On 18 October 2016, Ms Eardley sent the RCPCH draft report to Mr Harvey. She informed him that the report provided "*some fairly strong recommendations*" and suggested that he and Ms Kelly read it first and then share it with a few people, such as Dr Jayaram, Dr Brearey and Ms Powell, to check for factual inaccuracies prior to the final version being issued. She asked for an update on how the other investigations were going.[234] Ms Eardley confirmed that she sent the "*full report*" – that is, the 'confidential' version. The original version of the report contained, she explained, all the information that the

review team had gleaned. However, she "*wrote it in a way that those paragraphs that are in green could be removed for wider dissemination because of the sensitivity of the allegations that had been made*".[235]

20.114 On 19 October 2016, at the executives' meeting, Mr Harvey gave an update on the draft RCPCH report. Once they had read it, a decision would be taken on its further distribution.[236]

20.115 On 20 October 2016, at a meeting with Ms Hodkinson, Ms Kelly, Ms Rees and Ms Cooper,[237] Letby was told that the draft report had been received and that there was to be a clinical review of the cases. The meeting was followed by a letter to Letby confirming that she was "*not under investigation but … temporarily redeployed … as a supportive measure as it was a vulnerable environment with some of the comments we had been made aware of*".[238]

20.116 On 1 November 2016, Dr Brearey asked Mr Harvey when he could see the RCPCH report. Mr Harvey replied two days later confirming that they had received the draft and that Dr Brearey, Dr Jayaram and Ms Anne Murphy would be given an opportunity to review the report before it was finalised by the RCPCH.[239]

20.117 On 2 November 2016, there was an executives' meeting. The review by the RCPCH and Dr Hawdon's report were discussed. The minutes noted that a "*redacted*" version of the RCPCH report would be shared with Dr Jayaram, Dr Brearey and a "*senior nurse*" (Ms Anne Murphy).[240] Letby met Ms Hodkinson, Ms Rees, Ms Kelly and Ms Cooper. Letby was told by Ms Kelly that the RCPCH review would be shared with her when finalised.[241]

20.118 On 10 November 2016, Dr Brearey, Dr Jayaram and Ms Anne Murphy were allowed to read a redacted version of the RCPCH report in the presence of Mr Harvey.[242] In this version, the passages written in green ink – that is, those about Letby – were absent.

20.119 On 11 November 2016, Dr Brearey emailed Mr Harvey with suggested revisions to the draft RCPCH report. These were all clinical – there was no reference to anything said about Letby. Dr Brearey also emailed Ms Eardley reiterating his amendments and informing her that he had been shown a "*redacted*" version of the report. Ms Eardley replied, making no reference to the 'redacted' comment.[243] In oral evidence, she said that Dr Brearey's email mentioning the 'redacted' version "*did concern*" her.[244] Reports were expected to be shared with all those involved or interviewed (not families).[245] She said: "*I had thought the report … the full report had been shared in the middle of November with the senior team, with the senior doctors, with Alison Kelly, with Eirian* [Powell]*. I thought they had all seen the report with the green sections in it.*"[246] I am satisfied that the senior doctors had not seen the full report.

20.120 On 23 November 2016, Letby met Ms Hodkinson, Ms Kelly, Ms Rees and Ms Cooper to complain about a doctor discussing the RCPCH report. Ms Hodkinson agreed this was inappropriate and told Letby that she and Mr Harvey would meet with Dr Brearey to discuss. This meeting took place on 24 November 2016, when Dr Brearey was warned that he must not discuss the report until it had been finalised.[247]

RCPCH report – final versions

20.121 The report was subject to a quality assurance process from two senior clinicians: Dr Nicholas Wilson (a consultant neonatologist) and Dr Shortland. Having read the report, Dr Shortland wrote, in November 2016, that the review was both interesting and complex but "*almost felt a bit like the Grantham situation 30 years ago and my only question was why they didn't involve the police if they had those suspicions. Otherwise looks like a good report with very clear recommendations.*"[248] In 1988 and 1989, Dr Shortland was a registrar in Nottingham. He was involved in retrieving sick babies from the unit in Grantham where Beverly Allitt worked.[249] He was, in these comments, drawing a parallel between what he had read in the invited review report and the crimes of Beverly Allitt. He accepted that it was extremely unusual for newborn infants to die without a clear diagnosis or evidence of a clinical deterioration. A cluster of unexpected and unexplained deaths would therefore be a matter of concern.[250] He described the review as a complicated one and said that he thought the police should be involved. He was raising a query as to why the police had not been contacted.[251] He did not receive an answer.[252] As the RCPCH accepted in their closing submissions, such a question should have prompted serious discussion about escalating the concerns of the consultant paediatricians.[253] This was another occasion when the RCPCH should have advised the Trust to contact the police or to take steps to ensure that they were contacted.

20.122 The final reports, confidential (unredacted) and dissemination (redacted) respectively,[254] were sent to the Countess by a letter from Dr Shortland dated 28 November 2016.[255] There was an appendix to the confidential report summarising some of the medical information about individual babies, including a summary chronology of Baby A, Baby C, Baby D, Baby E, Baby I, Baby O and Baby P. The review team agreed there were no obvious factors which linked the deaths and that the circumstances in the unit were not materially different from those that may be found in many other neonatal units within the UK.[256] This important observation underlines the fact that none of the findings began to explain any of the deaths.

20.123 The final (confidential) report states at paragraph 3.12:

> "*The neonatal lead, in an effort to be thorough and explore all possibilities had identified that one nurse (Nurse L)* [Letby] *had been rostered on shift for all the deaths although the nurse had not always been assigned to care for that specific infant. Subsequently the paediatric lead and all the consultant paediatricians had become convinced by the link. Although this was a subjective view with no other evidence or reports of clinical concerns about the nurse beyond this simple correlation an allegation was made to the Medical Director and Director of Nursing.*"[257]

20.124 As in the RCPCH letter of 5 September 2016, the description of the concerns at paragraph 3.12 was not a proper or accurate reflection of the level of concern or the details that had been conveyed to the review team. The review team knew that the paediatricians were not concerned that she was harming the babies through incompetence. The evidence included that a correlation with Letby had persisted when Letby had moved from night shifts to day shifts (with collapses also moving from night to day). There were concerns that the deaths were unexpected and unexplained, and

babies had not responded physiologically, as they should have, to resuscitation. These were matters that appear in the section of the RCPCH report headed 'Concerns raised' but were not part of the review findings. This was unfortunate.

20.125 Under the heading 'Findings', the confidential report set out first that the review team had learnt of Letby's move away from the neonatal unit when they arrived at the hospital. It then explained that she was an enthusiastic, capable and committed nurse. She had told the team that she was passionate about her career and keen to progress. She said she regularly volunteered to work extra shifts and to change shifts when asked. Her colleagues were said to think highly of her and she demonstrated an enthusiasm for learning. The review team went on to say that the Director of Nursing had considered supervised practice but "*the consultants would not accept this and required the nurse be removed from the unit*".[258] This was not true, as I set out above in paragraph 20.51. The review teams had not asked the doctors about it. Had they done so, they would have heard their version of events. The report then read: "*The consultants explained that their allegation was based on Nurse L* [Letby] *being on shift on each occasion an infant died (although not necessarily caring for the infant) combined with 'gut feeling'. There was no other evidence or history to link Nurse L* [Letby] *to the deaths and her colleagues had expressed no concerns about her practice*."[259] The notes of interviews showed, and Ms Eardley accepted in evidence, that the term "*gut feeling*" was a term used by the nurses when describing the suspicions of the paediatricians.[260] By adopting it, the review team undermined the seriousness of the doctors' concerns. In short, the doctors' concerns were accorded no more serious consideration by the RCPCH than they had received from the executives at the Countess.

20.126 Two of the recommendations in the full report were relevant to Letby. They reflected the advice given by the RCPCH in the letter of 5 September 2016 (to conduct an external independent review and to ensure "*there are clear, swift and equitable Trust processes for investigating allegations of concerns which are followed by everyone*").[261] In the dissemination report, under the heading 'Findings', there are no findings. Instead, there are two recommendations that correspond with the recommendations in the confidential report, in respect of Letby.

20.127 The review found problems with staffing, although it acknowledged that the nursing staffing levels were closer to target levels than the average across the network. There was also a shortage of consultants, as the consultants had argued. Interestingly, in answer to the question of whether the unit had clear and engaged leadership and good team working, the response was as follows:

> "*Yes, generally but there were some areas where communications could be strengthened.*"

20.128 The final report made 21 recommendations: four including the two in respect of Letby related to managing or investigating deaths, near misses and other obstetric and paediatric incidents; four related to staffing; five related to management and governance; six concerned with the wider Neonatal Network and others; one on managing allegations or concerns; and one on the mechanism for recording, management and reporting across IT systems.

Distribution of the report

20.129 The RCPCH's 'Invited reviews – A guide' provided that the responsibility for receiving, disseminating and acting upon invited review reports lay with the Medical Director or Chief Executive who commissioned the review.[262] The expectation is that the final report will be shared amongst as many of those who contributed as possible.[263] The initial proposal for the invited review at the Countess stated that the tested principles of a review include "*a commitment that findings will be shared as far as possible with those involved*".[264]

20.130 The guide provided for separate reporting on sensitive issues: "*Occasionally where there are sensitive findings or concerns relating to an individual the RCPCH will write separately in confidence to the Medical Director or their nominee about those issues*."[265] The 2023 escalation procedure similarly has provision for a letter separate from the report to be provided in relation to issues concerning patient safety or relating to staff "*which are outside the Terms of Reference*".[266]

20.131 An email from Ms Eardley to Mr Harvey, dated 28 November 2016, enclosed the reports and a letter from Dr Shortland that explained the rationale: "*Aware of the personnel issues, we have provided two reports, one including full details of actions taken and one omitting the confidential HR issues.*"[267] The letter describes the report with references to Letby removed as the "*dissemination copy*",[268] and expresses the hope that Mr Harvey will share the dissemination copy with those who contributed. It gives no guidance about who should see the confidential version. The confidential report was described as the RCPCH's formal version. The dissemination copy was described as a version that was perhaps suitable for wider dissemination "*amongst those who contributed*".[269]

Dissemination version

20.132 Careful readers would notice obvious signs that the report had been edited. There were unexplained references to "*this allegation*" and "*the response to the concerns subsequently identified*" (paragraphs 3.12 and 3.13 in the dissemination version).

20.133 Both the letter from Dr Shortland and the email from Ms Eardley suggest, as is the interpretation in Mr Okunnu's statement, that the dissemination report would be shared with the clinicians and the full report would likely be held as confidential within the hospital.[270]

20.134 Dr Stewart's view was that all the paediatric consultant team should have seen both versions of the report. He accepted that a review requiring a second redacted report may have overstepped its brief.[271] He was right on both points.

20.135 Professor Turner said in evidence that the RCPCH considered that those paediatricians raising concerns should have seen the full version of the report addressing those concerns.[272] He considered that, for the majority of external organisations, it should also have been the full version of the report that was shared.[273] I agree.

20.136 Six weeks later, on 6 January 2017, Mr Harvey contacted Ms Eardley, expressing concern that there were two reports and stating that the Trust did not want to be seen as concealing anything. He asked whether there was anything the RCPCH did not want published or whether the two reports were "*purely to protect the nurse and the Trust*".[274]

Ms Eardley confirmed that the two reports were purely to protect "*individuals for the HR issues*". She confirmed that, from the RCPCH's perspective, either report could be published.[275] It was the dissemination report that was published. In considering how the report was to be published, the focus of Mr Harvey and Ms Eardley on the nurse and the Trust and the omission of the parents of the children who had died is striking.

20.137 On 31 January 2017, Ms Eardley emailed Mr Harvey and Ms Kelly to check "*where things are with dissemination of the review report*", commenting that it was important that staff had a chance to read and discuss the report and its recommendations before it was published.[276] It is not clear from this email whether at that stage Ms Eardley was recommending that the dissemination or the confidential report be shared with staff.

20.138 The issues that were omitted from the dissemination version of the report had dominated the initial interviews at the review visit and were of the utmost seriousness. They were so important that they were included in the full report. Providing a dissemination version to be shared with the paediatricians in circumstances where it was silent on the principal concerns raised was wrong. The paediatricians ought to have been provided with the full report.

20.139 Neither the letter from the RCPCH of 5 September 2016 nor the RCPCH review team's final reports sent on 28 November 2016 advise external escalation, whether to the police, the NMC or the LADO. In fact, there is no evidence that the RCPCH review team ever advised the executives to take their concerns to any external body. They should have done. A referral to the LADO should already have been made by the time of the invited review visit. The review team should have explored whether the hospital had complied with safeguarding procedure. They failed to do so.

20.140 Ms Eardley explained in evidence that the review team would not refer matters to external bodies, since that was the responsibility of the hospital, having commissioned the service.[277] 'Invited reviews – A guide', at paragraph 9.7, sets out the approach to be taken.[278]

20.141 Ms Eardley accepted in her evidence that the Trust should have been told that they needed to contact the police.[279] Ms Mancini accepted in her evidence that the Trust should have been advised to contact the police, as the appropriate agency to investigate the concerns that had emerged.[280] She accepted that contacting the police should have been recommended in strong terms.[281]

20.142 On 6 February 2017, Mr Chambers wrote to staff at the Countess to inform them that the RCPCH report would be published on 9 February 2017. Mr Chambers stated: "*There is no single cause or factor identified as a means of explaining the increase we have seen in our mortality numbers.*"[282] See Chapter 23.

Follow-up by the review team

20.143 The letter of 5 September 2016 and the guide to the review service anticipated a follow-up with the Countess at either three or six months from the report being provided. The purpose was to review the Trust's implementation. By this stage, the RCPCH knew that the review by Dr Hawdon was in hand. There was no further follow-up by the RCPCH. That said, I doubt that it would have made much difference, given the approach taken up to that point.

Effect of the reports

20.144 When the dissemination report was eventually published, none of its findings explained any of the deaths; nor did the findings in the confidential report. This important point was accepted by the RCPCH in its closing submissions. That this would be the case was foreshadowed by Dr Brearey and Dr McCormack seven months earlier, in the meeting on 30 June 2016 (see Chapter 14 for further details).[283]

The parents' experience

20.145 The RCPCH report (dissemination version) was not provided to the parents until after it was published online. Ms Sian Williams spoke to some parents by telephone before the publication date. She told them about the report and arranged for them to receive a copy and letter on 8 February 2017. Parents who were not spoken to by telephone received a letter from Mr Harvey notifying them of the publication and confirming that the report was available on the hospital website.[284]

20.146 In a vivid and reflective part of her evidence, Mother E and F said:

> "*I received that letter by a black taxi knocking on my door about 30 minutes before that report was due to go live online … I had absolutely no warning whatsoever. I was absolutely mortified. I was panicking. I didn't know what was going on … it beggars belief that a black taxi could turn up at my door with a letter about something that I had no idea about, I had no knowledge that there was any assessment or anything that was going on.*"[285]

20.147 Mother A and B said in her evidence: "*We had a right to know. As I explained before, they are our babies. We had a right to know. And we were being -- well, I was made to believe that that report was the report. I never knew that there was another version of it that explained concerns.*"[286] Whilst the other report set out the doctors' concerns, it contained no answers either.

20.148 Speaking further about the dissemination version of the report, Mother A and B explained: "*We thought the report was going to give us some answers, and we were very, very disappointed when we were given a copy of it because it left us no better off. There was no answers in it at all.*"[287]

20.149 Mother C was asked about Mr Harvey's decision to share the redacted report with the families. She said:

> "*I felt at the time that we were being misled, that we were being kept in the dark. I feel very strongly now that Ian Harvey was desperately trying to stop us from asking further questions by providing a whitewash gloss-over of a report and hoping that we would just take his word for it and not ask any more questions. I feel that we were treated extremely disrespectfully, and I think it's added hugely to our distress at what was already a distressing time.*"[288]

20.150 Mother C added: "[W]*e were in a very vulnerable position as bereaved parents who were very much being kept in the dark about what had happened to our child … these are serious suggestions in that report, that there was serious concern about this nurse, that were being hidden from us. They were deliberately removed from the report that we were given.*"[289]

20.151 Mother E and F told the Inquiry: "[T]*hat's my son. I think any information that they have about him should have been shared openly with us. I don't think it should have been held back. I think to do that is, I think it's quite hideous.*"[290]

20.152 The families were failed by the Countess and by the RCPCH, from the commissioning of the report to its publication.

20.153 As Dr Brearey was to observe in a meeting with the RCPCH in 2019, the information shared with the review team was the same as was shared with the CDOP in 2017.[291] That led to the police investigation two weeks later.

RCPCH apology

20.154 At the beginning of his evidence, Professor Turner apologised to the parents on behalf of the RCPCH for the role it played in contributing to the delay in their finding out what had happened to their children. He also apologised to the paediatricians at the Countess for the way they had been treated by their professional body and informed the Inquiry that the RCPCH were determined to learn and had already learnt from the mistakes they had made.[292] Those apologies were repeated in the closing submissions of the RCPCH.[293] Unlike other organisations, the starting point for the RCPCH was an uncomplicated recognition that their review team had made serious mistakes, for which the RCPCH acknowledged accountability and responsibility, including its contribution to the delay in the police being called to investigate. The apologies were genuine and reflective. The changes made since then by the RCPCH have been far reaching. It is to be hoped that they are long-lasting and effective.[294]

Endnotes

1 Witness statement of Robert Okunnu INQ0017463/2/para 8
2 Witness statement of Robert Okunnu INQ0017463/3/para 9
3 Witness statement of Robert Okunnu INQ0017463/5/para 16
4 Prof. Stephen Turner 12 December 2024 76/3-6
5 Witness statement of Robert Okunnu INQ0017463/6/para 18
6 INQ0003330/2
7 Witness statement of Robert Okunnu INQ0017463/4/para 11 and 6/para 18
8 Prof. Stephen Turner 12 December 2024 77/14-23
9 Prof. Stephen Turner 12 December 2024 113/22 to 114/2
10 INQ0010214/2/para 2.2
11 Crisp QI's 'External Review of the RCPCH Invited Reviews Service'.
12 Sue Eardley 7 November 2024 95/25 to 98/6 and 99/3-9
13 Sue Eardley 7 November 2024 99/21 to 100/12
14 INQ0012822/4
15 Sue Eardley 7 November 2024 99/21 to 103/4
16 INQ0012748/1
17 INQ0009590/1
18 Sue Eardley 7 November 2024 144/4 to 145/12
19 Sue Eardley 7 November 2024 136/3-6 and 142/8
20 Sue Eardley 7 November 2024 135/24 to 136/11
21 Sue Eardley 7 November 2024 142/10-12
22 Sue Eardley 7 November 2024 143/20-22
23 Sue Eardley 7 November 2024 142/20-22
24 Ian Harvey 28 November 2024 193/13-19
25 Ian Harvey 28 November 2024 195/1-25
26 Sue Eardley 7 November 2024 140/13-15
27 Fiona Scolding KC 17 March 2025 112/17-20
28 Written Closing Submissions on Behalf of the Royal College of Paediatrics and Child Health 17 March 2025 2-3/para 6
29 Sue Eardley 7 November 2024 105/5-10
30 Sue Eardley 7 November 2024 105/23 to 106/12
31 INQ0006023/1
32 INQ0009595/5; INQ0012748/1
33 Sue Eardley 7 November 2024 159/12 to 160/1

34 INQ0009595/4/para 4.1
35 Sue Eardley 7 November 2024 149/16-21
36 Sue Eardley 7 November 2024 111/5-12
37 Sue Eardley 7 November 2024 149/25 to 150/4
38 INQ0012822/4; INQ0010214/9/para 8.1
39 INQ0012822/4
40 INQ0010214/9/para 8.1
41 INQ0010214/9/para 8.2
42 Sue Eardley 7 November 2024 117/25 to 118/10
43 Sue Eardley 7 November 2024 130/7 to 131/7
44 Sue Eardley 7 November 2024 140/17-25
45 Sue Eardley 7 November 2024 141/5-12
46 Witness statement of Robert Okunnu INQ0017463/18/para 53
47 Sue Eardley 7 November 2024 158/9-13
48 INQ0009595/5/para 5
49 INQ0010214/8/para 7.5
50 Sue Eardley 7 November 2024 154/6-9
51 INQ0009599/1-2
52 INQ0010256/1
53 Sue Eardley 7 November 2024 154/23 to 155/6
54 Sue Eardley 7 November 2024 157/3-15
55 Sue Eardley 7 November 2024 152/20 to 157/2
56 Sue Eardley 7 November 2024 236/8-9
57 Sue Eardley 7 November 2024 109/21-23
58 Sue Eardley 7 November 2024 128/12 to 130/1
59 Witness statement of Robert Okunnu INQ0017463/17/para 49
60 Witness statement of Robert Okunnu INQ0017463/58/para 143a
61 Witness statement of Robert Okunnu INQ0017463/17/para 49
62 Witness statement of Dr Graham Stewart INQ0101347/8/para 45
63 Claire McLaughlan 11 November 2024 2/10-14
64 Claire McLaughlan 11 November 2024 2/20-25
65 Claire McLaughlan 11 November 2024 3/6-22
66 Witness statement of Robert Okunnu INQ0017463/17/para 49
67 Claire McLaughlan 11 November 2024 4/22 to 5/23
68 Claire McLaughlan 11 November 2024 6/8-11

69 Witness statement of Robert Okunnu INQ0017463/16/para 49
70 Alexandra Mancini 11 November 2024 101/11-22
71 Alexandra Mancini 11 November 2024 105/21-23
72 Alexandra Mancini 11 November 2024 106/7-10
73 Dr David Shortland 11 November 2024 195/15 to 199/1
74 Sue Eardley 7 November 2024 169/13-21
75 Prof. Stephen Turner 12 December 2024 91/6-19
76 Witness statement of Dr David Milligan INQ0102061/1/para 4
77 INQ0012847/4
78 INQ0010072
79 Sue Eardley 7 November 2024 168/14-24
80 Alexandra Mancini 11 November 2024 118/10-23
81 Alexandra Mancini 11 November 2024 121/21 to 122/1
82 Witness statement of Dr David Milligan INQ0102061/1/para 4
83 INQ0012748/3
84 Sue Eardley 7 November 2024 161/17 to 162/13
85 Ian Harvey 29 November 2024 3/24-25
86 Ian Harvey 29 November 2024 4/1-4
87 Witness statement of Dr David Milligan INQ0102061/1/para 4
88 Witness statement of Dr David Milligan INQ0102061/3/para 12
89 Witness statement of Dr Graham Stewart INQ0101347/11/para 73
90 Witness statement of Dr Graham Stewart INQ0101347/11/para 75
91 Witness statement of Dr Graham Stewart INQ0101347/12/para 76
92 Witness statement of Dr Graham Stewart INQ0101347/11/para 74
93 Witness statement of Dr Graham Stewart INQ0101347/26/paras 172-173
94 Alexandra Mancini 11 November 2024 117/2-14
95 Alexandra Mancini 11 November 2024 118/4 to 119/11
96 Alexandra Mancini 11 November 2024 119/8-22
97 INQ0010214/8/para 7.5
98 INQ0014604/1
99 Ian Harvey 29 November 2024 3/21-23
100 Ian Harvey 28 November 2024 198/25 to 199/10
101 INQ0014604/1
102 INQ0014604/2
103 INQ0014604/2

104 INQ0003365/4-8
105 INQ0014604/3-4
106 INQ0014604/4
107 INQ0014604/4
108 INQ0014604/7
109 INQ0014604/9
110 Sue Eardley 7 November 2024 246/12-20
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121 Sue Eardley 7 November 2024 251/1-4
122 Alexandra Mancini 11 November 2024 164/19 to 166/1
123 Witness statement of Robert Okunnu INQ0017463/32/para 84
124 Witness statement of Dr Graham Stewart INQ0101347/15/para 91
125 Claire McLaughlan 11 November 2024 22/2-10
126 Claire McLaughlan 11 November 2024 31/3-7
127 Claire McLaughlan 11 November 2024 26/11 to 27/19
128 Claire McLaughlan 11 November 2024 27/20 to 28/5
129 Claire McLaughlan 11 November 2024 28/21 to 29/12
130 Claire McLaughlan 11 November 2024 29/20
131 Claire McLaughlan 11 November 2024 29/21-24
132 Claire McLaughlan 11 November 2024 53/7-9 and 53/18-19
133 INQ0010124/23
134 INQ0014605/6
135 Sue Eardley 7 November 2024 193/20-24
136 Sue Eardley 7 November 2024 210/22-23
137 Sue Eardley 7 November 2024 194/6-19
138 Witness statement of Claire McLaughlan INQ0100895/36/para 108

139 Claire McLaughlan 11 November 2024 35/10 to 37/25 and 65/24 to 66/8

140 Claire McLaughlan 11 November 2024 41/20 to 42/6

141 Claire McLaughlan 11 November 2024 47/3-9

142 Witness statement of Dr Graham Stewart INQ0101347/16/para 95

143 Witness statement of Dr Graham Stewart INQ0101347/16-17/para 97

144 Witness statement of Dr Graham Stewart INQ0101347/16/para 96

145 Alexandra Mancini 11 November 2024 141/22 to 142/21

146 Alexandra Mancini 11 November 2024 147/1-10

147 Witness statement of Robert Okunnu INQ0017463/32/para 85

148 Witness statement of Robert Okunnu INQ0017463/56/para 139f

149 Witness statement of Dr David Milligan INQ0102061/3/para 12

150 Prof. Stephen Turner 12 December 2024 95/12-19

151 Dr David Shortland 11 November 2024 200/6 to 201/6

152 INQ0010214/9/para 7.7

153 Prof. Stephen Turner 12 December 2024 84/10-13

154 Sue Eardley 7 November 2024 186/2-16; Claire McLaughlan 11 November 2024 72/1-24

155 Sue Eardley 7 November 2024 175/16-23

156 Sue Eardley 7 November 2024 176/2-4

157 Sue Eardley 7 November 2024 186/5-6

158 Sue Eardley 7 November 2024 176/7-10

159 Sue Eardley 7 November 2024 190/3-7

160 Sue Eardley 7 November 2024 188/10-15

161 Witness statement of Dr Graham Stewart INQ0101347/17/paras 99-101

162 Witness statement of Dr Graham Stewart INQ0101347/17/paras 98 and 102

163 Claire McLaughlan 11 November 2024 72/6-10

164 Claire McLaughlan 11 November 2024 74/13-14

165 Alexandra Mancini 11 November 2024 151/8-13

166 Witness statement of Robert Okunnu INQ0017463/23/para 69

167 Witness statement of Robert Okunnu INQ0017463/33/para 88

168 Dr David Shortland 11 November 2024 202/20-24

169 Written Opening Statement on Behalf of the Royal College of Paediatrics and Child Health 12 September 2024 7/para 24

170 Prof. Stephen Turner 12 December 2024 96/13 to 97/2

171 Hayley Cooper 6 November 2024 39/10 to 40/15 and 67/5-14

172 INQ0000569/34

173 INQ0000569/34

174 Claire McLaughlan 11 November 2024 76/21-24

175 Claire McLaughlan 11 November 2024 76/13-16

176 Claire McLaughlan 11 November 2024 93/24 to 94/2

177 Claire McLaughlan 11 November 2024 79/10; INQ0014605/6

178 Claire McLaughlan 11 November 2024 77/15-21

179 INQ0014605/6

180 INQ0014602/3

181 INQ0014604/4

182 Claire McLaughlan 11 November 2024 59/5-6

183 INQ0002966/2

184 INQ0002966/2

185 INQ0002966/2

186 Claire McLaughlan 11 November 2024 11/12-25

187 Claire McLaughlan 11 November 2024 12/1

188 Claire McLaughlan 11 November 2024 12/11-18

189 Claire McLaughlan 11 November 2024 13/16-18

190 Claire McLaughlan 11 November 2024 60/5-25

191 Claire McLaughlan 11 November 2024 61/8-9

192 Mother A and B 16 September 2024 52/19-21

193 INQ0014604/3

194 Prof. Stephen Turner 12 December 2024 103/4-7

195 Prof. Stephen Turner 12 December 2024 102/18-25

196 INQ0010214/4/para 2.2

197 Claire McLaughlan 11 November 2024 71/1-7; Alexandra Mancini 11 November 2024 167/18 to 168/4

198 Sue Eardley 7 November 2024 105/23 to 107/13

199 Witness statement of Dr Graham Stewart INQ0101347/18/para 107 and 27/para 177

200 Witness statement of Dr Graham Stewart INQ0101347/15/para 91

201 Witness statement of Dr David Milligan INQ0102061/2/para 7

202 INQ0009611/2

203 INQ0003120

204 INQ0003120/1

205 INQ0003120/1

206 INQ0003120/1

207 INQ0003120/2

208 INQ0002458/2

209 Karen Rees 21 October 2024 155/21 to 157/5

210 Dr Christopher Green 6 November 2024 191/9 to 192/5

211 Hayley Cooper 6 November 2024 68/13 to 69/19

212 INQ0003120/1

213 Alexandra Mancini 11 November 2024 171/1-7

214 INQ0003120/2

215 INQ0003120/2

216 INQ0003120/2

217 Prof. Stephen Turner 12 December 2024 101/21-24

218 Sue Eardley 7 November 2024 203/6 to 204/2

219 Claire McLaughlan 11 November 2024 24/1-18 and 81/19 to 82/5

220 Claire McLaughlan 11 November 2024 81/7-11

221 Claire McLaughlan 11 November 2024 81/14-18

222 Claire McLaughlan 11 November 2024 81/19 to 82/5

223 INQ0003120/2

224 INQ0003120/2

225 INQ0003120/3

226 Dr Jane Hawdon 12 November 2024 2/17 to 3/15

227 Dr Jane Hawdon 12 November 2024 3/19-24

228 Sue Eardley 7 November 2024 207/21 to 208/23

229 Claire McLaughlan 11 November 2024 85/7-12

230 James Wilkie 2 December 2024 179/19 to 180/21

231 James Wilkie 2 December 2024 179/19 to 180/21

232 Rachel Hopwood 3 December 2024 136/10-19

233 INQ0003178/2

234 INQ0003403/1

235 Sue Eardley 7 November 2024 214/19 to 215/4

236 INQ0003370

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238 INQ0003447/2

239 INQ0003111/8

240 INQ0003215/1

241 INQ0008964/79

242 INQ0003111/1-2

243 INQ0003111/1-2

244 Sue Eardley 7 November 2024 224/4-16

245 Sue Eardley 7 November 2024 119/12 to 120/24

246 Sue Eardley 7 November 2024 218/10-14

247 Dr Stephen Brearey 19 November 2024 154/4 to 156/8

248 INQ0012748/4

249 Dr David Shortland 11 November 2024 205/23 to 206/7

250 Dr David Shortland 11 November 2024 207/19-23

251 Dr David Shortland 11 November 2024 211/25 to 212/12

252 Dr David Shortland 11 November 2024 212/13-25

253 Written Closing Submissions on Behalf of the Royal College of Paediatrics and Child Health 26 February 2025 16/para 36

254 INQ0009619

255 INQ0009620/1-2

256 Witness statement of Dr Graham Stewart INQ0101347/22/para 135

257 INQ0009618/8/para 3.12

258 INQ0009618/9

259 INQ0009618/9

260 Sue Eardley 7 November 2024 210/20 to 211/8

261 INQ0009618/10

262 INQ0010214/6/para 4.2

263 INQ0010214/12/para 9.5

264 INQ0009595/4/para 3.3

265 INQ0010214/12/para 9.5

266 INQ0012813/10/para 6.2.2

267 INQ0009620/1

268 INQ0009620/1

269 INQ0009617/1

270 Witness statement of Robert Okunnu INQ0017463/48/para 120

271 Witness statement of Dr Graham Stewart INQ0101347/26/para 165

272 Prof. Stephen Turner 12 December 2024 106/1-8

273 Prof. Stephen Turner 12 December 2024 106/9-15

274 INQ0005390/2; INQ0012748/4

275 INQ0005390/1; INQ0012748/4

276 INQ0003132/1

277 Sue Eardley 7 November 2024 103/2-4

278 INQ0010214/12/para 9.7

279 Sue Eardley 7 November 2024 203/3-5

280 Alexandra Mancini 11 November 2024 147/11-14

281 Alexandra Mancini 11 November 2024 172/12-16

282 INQ0003060

283 INQ0003362/3-4

284 INQ0012628/3

285 Mother E and F 18 September 2024 25/18 to 26/8

286 Mother A and B 16 September 2024 53/23 to 54/2

287 Mother A and B 16 September 2024 28/16-19

288 Mother C 16 September 2024 125/22 to 126/5

289 Mother C 16 September 2024 88/11-19

290 Mother E and F 18 September 2024 31/13-17

291 INQ0012742/2

292 Prof. Stephen Turner 12 December 2024 73/22 to 74/6

293 Written Closing Submissions on Behalf of the Royal College of Paediatrics and Child Health 26 February 2025 1; Fiona Scolding KC 17 March 2025 112/17 to 113/8

294 Prof. Stephen Turner 12 December 2024 112/4 to 113/14

Chapter 21. Reports: Dr Hawdon and Dr McPartland

Dr Hawdon

21.1 After receiving the RCPCH letter of 5 September 2016, Mr Harvey commissioned a review from Dr Jane Hawdon, consultant neonatologist at Barts Health NHS Trust (who joined the Royal Free London NHS Foundation Trust as Medical Director in January 2017). In December 2016, on Dr Hawdon's advice, he contacted Dr Jo McPartland, consultant paediatric pathologist at Alder Hey.

21.2 Mr Harvey emailed Dr Hawdon on 8 September 2016, and explained that, following a recommendation from the RCPCH, he wanted her to conduct a detailed case note review that was likely to extend to 13 neonatal deaths, and between 4 and 6 near misses.[1] He said in his email that clinicians had observed that: (a) there was an increase in the number of neonatal deaths in 2015 and 2016; and (b) some of the babies did not respond to resuscitation. He said that Dr Hawdon's review and examination was "*to be carried out together with a relevant paediatric pathologist by two independent specialists reporting separately*".[2] Mr Harvey stated there was a degree of urgency to the review. Mr Harvey did not mention any concerns about deliberate harm to babies by a staff member, nor that the deaths were unexpected or unexplained. He should have done.

21.3 On 5 October 2016, Mr Harvey sent Dr Hawdon a letter of instruction to review 13 neonatal deaths and 4 near-miss cases.[3] Again, he failed to mention any concerns about deliberate harm to babies by a staff member, or that the deaths were unexpected or unexplained. This time, the letter specified that the RCPCH recommended a detailed **forensic** case note review with five minimum elements: these were items (a) to (e) of the RCPCH letter of 5 September 2016.[4] In evidence, Dr Hawdon said that a detailed 'forensic' case note review, as opposed to a case note review, includes a broader inquiry into factors that are not in the case notes, such as unit records, staffing records and equipment records. In her view, the word 'forensic' did not have criminal connotations. She explained: "*The suspicion of criminal intent was certainly not raised to me and I did not read criminal intent into the detailed forensic case note expression.*"[5] There was no reason why she should have done so.

21.4 Dr Hawdon was not provided with the letter from the RCPCH to Mr Harvey, dated 5 September 2016. She did not see it until 2024, when it was sent to her by the Inquiry team. In evidence to the Inquiry, she said: "*I now feel misled … if those details had been made available to me the process I would have followed would have been very different*."[6] She stated she would not have conducted a case note review. She added: "*Had I been told that there was a suspicion about a member of staff, I would have had a much more*

detailed conversation with Mr Harvey as to whether it was or was not appropriate for me to proceed and the basis on which I would be proceeding."[7] And: "*I would have asked what safeguarding processes were being employed.*"[8]

21.5 Dr Hawdon added that, had she been in Mr Harvey's position, "*I would not as a neonatologist or as a Medical Director have proceeded without the involvement and guidance of the Head of Safeguarding*".* [9] This approach contrasted markedly with the approach of Mr Harvey and the senior executives at the Countess.

21.6 Before beginning her review, Dr Hawdon asked Mr Harvey via email whether he was seeking parental consent to release the records of the babies.[10] Mr Harvey responded: "*Re parental consent, we had informed* [parents] *ahead of the review that it was occurring*."[11] Dr Hawdon's evidence was that, from Mr Harvey's response, she understood the parents had been informed about her review. She stressed it is "*vital*" that parents know that their baby is being reviewed by an external person.[12] The parents had not been informed about Dr Hawdon's review. The email from Mr Harvey may have been referring to the RCPCH review, but none of the parents had been informed about that either. Mr Harvey knew consent was needed for both reviews. When giving evidence, Mr Harvey said: "*If I didn't* [obtain consent]*, then that is a significant error on my part and I'm -- I'm very sorry for that.*"[13]

21.7 On 14 October 2016, medical records of Baby A, Baby C, Baby D, Baby E, Baby H, Baby I, Baby O, Baby P and Baby Q were sent to Dr Hawdon.[14] Mr Harvey had not contacted the parents to discuss this, nor to obtain their consent.

21.8 Dr Hawdon's unchallenged evidence was that the case papers she received from the Countess "*were loosely filed, they were not well ordered and I don't believe they were complete in terms of they were not full sets of records … and in the second box there were some loosely filed papers that didn't apply to any of the babies at all*".[15] She remarked it was "*very unusual*" to receive papers in that state.[16] This was inexcusable. The RCPCH had made it clear what was required for an effective review to take place. Mr Harvey should have made sure the papers were properly prepared. Whatever Mr Harvey's views were about the validity of the paediatricians' concerns, there was no benefit to excluding them from the preparation of the papers for Dr Hawdon, who was entitled to, and should have asked for, complete and accurate papers. She may have carried on without them because she knew Mr Harvey was in a hurry, and waiting for papers would have slowed her down; Mr Harvey had told her that there was a "*significant degree of urgency*" to the investigation.[17] Whatever the urgency, it is unlikely that she would have taken that course had she been aware there were concerns about a nurse harming babies.

Receipt of Dr Hawdon's report, 29 October 2016

21.9 Dr Hawdon did not speak to any of the doctors or nurses at the Countess. All of her communication was with Mr Harvey. At no point did Mr Harvey offer her the chance to speak to the clinicians. Nor did she ask. Nor did she point out that she was not

* Dr Hawdon stated that the CDOP process was aligned with safeguarding and that she would expect them to have been contacted.

in a position to fulfil the requirements of paragraphs (d) and (e) of her instructions. She should have done. It is surprising that she did not ask why she was being asked to establish the details of all staff in (d).[18]

21.10 On 29 October 2016, Dr Hawdon wrote to Mr Harvey, informing him she had completed a review of the cases.[19] However, she set out various limitations to what she was able to do. She followed the minimum elements set out in the letter of instruction:[20]

a. She explained that she did not have the capacity to carry out a full systematic chronology for each case, as this would take between 10 and 12 hours per case. She determined there needed to be a "*balance between the sense of urgency and the level of detail that could go into a review*".[21] She completed a synopsis of key events and issues preceding, and during, episodes of collapse or death.

   Knowing how important this review was, Mr Harvey should have instructed Dr Hawdon to take the time necessary to prepare a full systematic chronology. He knew this was what the RCPCH had recommended. It was not for Dr Hawdon to carry out a balancing exercise between urgency and detail, and so dilute the quality of the exercise. If she did not have time to prepare a full systematic chronology, she should have said so before going ahead. Had she done that, Mr Harvey would either have given her more time, or he would have had to find someone else to carry out the review.

   Having decided to prepare a synopsis, Dr Hawdon did so by applying the MBRRACE-UK methodology. This tool scores identified shortcomings in care by (i) severity and (ii) relevance. Dr Hawdon accepted in her evidence that a case note review by its nature, being based only on the information provided, is a fairly superficial exercise.[22] Writing the report, having made the decision to provide only a synopsis of key events, meant that she had not been able to fulfil her instructions and, although she was not to know this, allowed Mr Harvey to rely on it, as he duly did. I accept that, had Dr Hawdon been told that a number of deaths were unexpected and unexplained, she would not have taken that approach.

b. Where relevant, Dr Hawdon commented on cases where escalation at an earlier stage to senior opinion locally, or at a regional centre, would have potentially made a difference to the outcome.

c. Dr Hawdon pointed out to Mr Harvey that some post-mortems were not made available to her; thus, she was not in a position to consult with the relevant paediatric pathologist. She should have asked for the post-mortems before reporting. She should have informed Mr Harvey immediately that she was not able to consult with a paediatric pathologist and, since this was her view, that the hospital should instruct one so that she could report effectively on the issues identified. Instead, she suggested that, once Mr Harvey was in receipt of her report and the post-mortems, he instruct an independent perinatal pathologist.† Again, this gives the impression of a hurried approach to what Dr Hawdon initially seems to have considered a reasonably straightforward exercise, but which she came to acknowledge was nothing of the sort. In evidence, she said it would

† Please note that Dr Hawdon used perinatal pathologist and paediatric pathologist interchangeably.

have been helpful to complete her review "*in tandem or in close sequence*" with a paediatric pathologist.[23] She said, correctly, that "*triangulation is essential*"[24] in a forensic review, because one person reviewing an aspect of the case will not have the full picture. Dr Hawdon was asked why she did not raise this from the outset. She responded that she saw the two tasks separately; it was for the Trust to organise, and it would have been an unnecessary delay given the degree of urgency.[25]

d. Dr Hawdon informed Mr Harvey that she was not in a position to perform an analysis of which staff had access to the unit four hours before the death of each infant. In oral evidence, she explained this was not within her expertise, and she had never been asked to do this before. She said in evidence that she did not think the request to review staff access was due to suspicions of criminal intent, because it could have related to staff competency.[26]

e. Dr Hawdon informed Mr Harvey she could only consider the near-miss cases that she was supplied with. However, she suspected that there could be more cases, on the basis that one of the babies not on the indictment had a twin who had "*a similar course*".[27] Mr Harvey never followed up on this. He should have done. Dr Hawdon was not provided with the case notes of Baby F or Baby L, and so did not identify the abnormal insulin/C-peptide result. Had she seen them she would have been extremely concerned.[28]

Conclusions to Dr Hawdon's report and amendments without her knowledge

21.11 Dr Hawdon produced an Advisory Medical Report dated October 2016. She concluded that the 17 cases she had considered could be divided into two groups.‡[29] In Group 1 were those cases where the death/collapse was explained, but may have been prevented with different care. The following babies on the indictment were included in that group: Baby C, Baby E, Baby H and Baby Q.

21.12 In Group 2 were cases where the death/collapse was unexplained. In her report, Dr Hawdon concluded that these cases could benefit from a broader forensic review regarding the circumstances and personnel involved, because the deaths remained unexpected and unexplained. This group included Baby A, Baby D, Baby I, Baby O and Baby P.

21.13 In response to Dr Hawdon informing Mr Harvey that she did not have some post-mortems, he sent her the post-mortems of Baby A, Baby D, Baby O and Baby P.§ It appears that Dr Hawdon had seen the post-mortem report for Baby I and commented on it in her report.[30]

21.14 On 25 November 2016, Dr Hawdon provided her conclusions on deaths via email:[31]

- Baby A: Dr Hawdon agreed with the post-mortem result that the cause of death was 'unascertained'.

‡ Please note that not all the babies on the indictment were included in Dr Hawdon's review.

§ Baby E did not have a post-mortem. Baby K was not considered by Dr Hawdon.

- Baby O and Baby P: After reviewing the post-mortems for Baby O and Baby P, Dr Hawdon concluded the collapses and deaths remained unexplained.
- Baby D: Dr Hawdon agreed with the post-mortem cause of death – pneumonia.
- Baby I was not included in the email.

21.15 Following Dr Hawdon's email of 25 November 2016, Mr Harvey amended her report by moving Baby D from Group 2 to Group 1.[32] When giving evidence, Dr Hawdon informed the Inquiry she was not consulted by Mr Harvey before he did this, and that she was not aware of any of Mr Harvey's amendments until she was informed by the Inquiry.[33]

21.16 Dr Hawdon was informed of Dr McPartland's evidence that death 'with' pneumonia, as opposed to death 'from' pneumonia, is context dependent, and a difficult conclusion to reach. Dr Hawdon agreed with Dr McPartland and stated that, in these circumstances, the clinical course leading up to the death is important.[34] Dr Hawdon explained she had initially identified there was a backing-off of respiratory support for Baby D on the day she deteriorated. Thus, at the time, it was plausible that the change in respiratory management contributed to Baby D's subsequent deterioration. However, now that she had the fuller picture of the circumstances surrounding the death, this was no longer a plausible contribution to the death. Thus, it was her view that Mr Harvey should not have moved Baby D from Group 2 to Group 1.[35]

Re-categorisation of Baby C and Baby E

21.17 Mr Baker KC put to Dr Hawdon that it was arguable that Baby C should have been placed in Group 2 as an unexplained death.[36] He pointed out that Baby C's post-mortem report showed a cause of death at 1b. as 'Immaturity of lung', which did not fit with Baby C's gestation.¶ Dr Hawdon agreed with him: "[W]*e would not expect immaturity of the lungs at 30 weeks gestation to be a cause of death.*"[37] Mr Baker KC also informed Dr Hawdon of Dr Gibbs' evidence that, in the days leading up to Baby C's death, his oxygen requirements reduced substantially, and that Dr Gibbs did not anticipate he would have any significant problems.[38] Further, on the day of death, Baby C's respiratory rate was in the upper limit of the normal range. Dr Hawdon agreed that Baby C's respiratory function prior to death had a pattern of improvement, and thus it would not be expected that immaturity of the lungs was the cause of the collapse.[39] Dr Hawdon concluded it was "*perfectly reasonable*" to place Baby C in Group 2.[40]

21.18 As set out above, Dr Hawdon placed Baby E in Group 1 as an explained death. Mr Baker KC put to Dr Hawdon that Baby E should have been in Group 2. She was referred to a comment in her report about Baby E that there was delayed intubation, which amounted to significant suboptimal care, and that this was possibly relevant. Dr Hawdon agreed with Mr Baker KC that, in the absence of a recorded cause of death, she could not say whether aspects of care were relevant to Baby E's death or not.[41] She explained that she placed Baby E in Group 1 because her understanding was that his gastric bleed had caused the collapse.[42] She accepted the cause of the gastric bleed was unknown, and thus it could be argued Baby E belonged in Group 2.[43]

¶ Baby C's other causes of death in the post-mortem were '1a. Widespread hypoxic/ischamic damage to heart/myocardium' and '1c. Severe maternal vascular under perfusion'. The oral evidence from Dr Gibbs in relation to 1a. was that he was uncertain whether the damage to the heart arose after Baby C's resuscitation, or before.

21.19 Dr Hawdon was taken to an updated version of her report, which had the words 'Necrotising Enterocolitis' next to Baby E. She said that she did not make this addition.** [44] Dr Hawdon's evidence was that NEC did not fit well with the pattern of health issues that preceded Baby E's collapse.[45]

Events following the receipt of Dr Hawdon's report

21.20 On 2 November 2016, there was an Executive Directors' meeting, at which both the draft RCPCH report and Dr Hawdon's report were discussed. Her findings were summarised thus: "[A]*ll bar five deaths could have been avoided with better care.*" Dr Hawdon had not gone that far. She had said that some deaths **may** have been avoided with different care.

21.21 The note of the meeting continues: "*5* [deaths] *unexplained; 1 of concern.*" Mr Harvey is recorded as advising that these cases needed a secondary review.[46] There is no record of anyone saying that which is obvious – that five unexplained deaths was a matter of deep concern.

21.22 On the same day, Letby met Ms Hodkinson, Ms Rees, Ms Kelly and Ms Cooper. Ms Kelly told Letby that the review of clinical cases had been received, but further work was needed. She was told she would see Dr Hawdon's review when it was finalised (see also Chapter 20).[47]

21.23 Dr Hawdon's email exchanges with Mr Harvey are significant. On 5 February 2017, Mr Harvey invited Dr Hawdon's thoughts on skin mottling. She replied: "*Mottling is variable. If transient probably not of significance but if longer lasting reflects peripheral shut down. Reasonable nurses and doctors spot recognise it as a significant finding which if nothing else warrants close observation and additional tests if there are other concerns*."[48]

21.24 On 14 February 2017, Mr Harvey emailed Dr Hawdon again.[49] He attached to his email the consultants' letter to Mr Chambers, sent on 10 February 2017.[50] In the body of the email, Mr Harvey wrote that the paediatricians had made allegations against a member of staff, that the RCPCH was informed about the allegations, and concluded that the consultants' concerns were "*based on coincidence and 'gut-feeling'*".[51] Mr Harvey wrote that the paediatricians were worried that mottling observed in some of the babies was caused by air embolism. Mr Harvey also asked Dr Hawdon how common it was for a neonate to collapse unexpectedly.[52]

21.25 In evidence, Dr Hawdon agreed that, although Mr Harvey did not state it explicitly, the concern about the member of staff and air embolism appeared to be linked.[53] In the letter, the consultants agreed with Dr Hawdon's conclusion that four babies needed broader forensic review. However, they made it plain that they thought more deaths should be in that category.[54] Further, they raised that they did not consider the episodes of identified suboptimal care explained the rise in neonatal mortality and the sudden collapses. Dr Hawdon agreed with the consultants about this.[55]

** In Mr Harvey's oral evidence, he accepted that he made amendments to Dr Hawdon's report.

21.26 As the consultants only referred to four, and not five, deaths that could have been avoided, it is likely that the version of Dr Hawdon's report the consultants saw was the version in which Mr Harvey moved Baby D out of Group 2, leaving Baby A, Baby I, Baby O and Baby P as the deaths that needed a broader forensic review.

21.27 Dr Hawdon responded to Mr Harvey's email of 14 February the same day, saying that "*unexpected collapse in an otherwise stable baby is rare*", and adding that "*there have been more cases than would be expected*".[56] She also made the important point that reviewing the medical records is not, on its own, sufficient to identify a sinister cause. She agreed with Ms Rachel Langdale KC, Counsel to the Inquiry, that obtaining accounts from those involved in the care of the babies around the time of death was necessary and added "*but unexplained and unexpected deaths are unlikely to be from natural causes*".[57]

21.28 On 13 April 2017, Mr Harvey emailed Dr Hawdon, explaining that the Trust had instructed a KC, who had advised the clinicians to set out their:

> "*… best points … which they say most clearly indicate … reasonable grounds for suspecting that a criminal offence has been committed. I'm sure that I know the answer, because I am sure that you would have called it out in your report, but my Chairman has asked to ask the question; were there any concerns that there was anything other than natural causes in your review of the cases?*"[58]

21.29 Dr Hawdon's prompt response to Mr Harvey on 13 April 2017 was: "*Completely unexplained* [death] *on a neonatal unit is rare. So by definition more than one unexplained death does arouse suspicion.*" She also informed him that there was a very clear process to follow and he should obtain it from the local CDOP team if he did not have it.[59] Dr Hawdon said in evidence that Mr Harvey's question demonstrated a complete misunderstanding of her role, because she did not have the ability to identify a criminal act in a case note review.[60] That was an important observation of general application.

21.30 On 18 April 2017 Mr Harvey forwarded Dr Hawdon's email of 13 April to Mr Chambers.[61] By the time Mr Harvey wrote his email to Mr Chambers on 18 April 2017, Dr Hawdon had told him on two separate occasions that unexplained collapses on a neonatal unit are rare. "*So by definition more than one unexplained death does arouse suspicion*" (see paragraphs 21.25 and 21.29). This should have caused Mr Harvey to pause and to acknowledge that the paediatricians' suspicions may be well founded.

21.31 Mr Skelton KC put to Dr Hawdon that, upon receiving Mr Harvey's email on 13 April 2017, she should have raised then that safeguarding action and/or a police referral was required, and shown more curiosity as to whether the actions had been taken. She said that she naively assumed the processes were being actioned. She added, "[k]*nowing what I know now*", she would have raised that safeguarding or police action was required; however, she stopped short of accepting she should have raised it at the time.[62] It was reasonable to assume that the Medical Director and the hospital executives would have taken safeguarding steps, including bringing the matter to the attention of the police, but this was not a safe assumption to make in respect of the Medical Director or the executives of this hospital at that time.

Communication with the families

21.32 The RCPCH report had been published in early February 2017. After several letters to Mr Harvey seeking a meeting, Parents C met Mr Harvey and Ms Kelly on 20 February 2017. Mother C explained that her main aim was to understand what had been found in respect of Baby C's care. Mr Harvey told her that there were some minor learning points; however, nothing had been found that would change Baby C's outcome, and that, after one more meeting, a line would be drawn under the investigation.[63] I accept her evidence about this meeting.

21.33 On 3 March 2017, Mr Harvey wrote to Mother C. He told her a separate independent review of the deaths was recommended by the RCPCH; that this review had been completed, and that the findings would be shared in due course.[64] Mother C said that, when she received this letter, she was "*absolutely devastated because it was completely contrary to what we'd been told in the meeting* [on 20 February 2017] … *we were told that a line was being drawn under the investigation, that it had been completed. So what on earth were they investigating …?*"[65] She said that her understanding of 'a line being drawn under the investigation' was that it was the end of the investigation.[66] This was an entirely reasonable conclusion for her to draw.

21.34 Mr Harvey did not explain to Mother C that the RCPCH had recommended a full, in-depth case review of the babies. Mother C said she learnt this many years later and that, understandably in my view, it "*absolutely horrified me, how misled we'd been in that meeting and how untrue what we had been -- how untrue the information was that we'd been told. I felt completely betrayed on every level.*"[67]

21.35 Mother D was also asked about the letters she received from Mr Harvey during this period. In relation to the letter sent on 3 March 2017, which matched the letter Mother C had received 'appraising of progress' on the reviews, Mother D described it as a "*cop-out. It was just not good enough.*"[68]

21.36 Speaking generally about her attempts to communicate with the Countess, Mother D said "*communication* [was] *poor … there should be no second-guessing. No parents don't want to know. If there is any risk, any problem, anything, we want to know.*"[69] She stated: "*I kept asking, every time I could, and I'd speak to someone at the Countess, no transparency. I don't know what was being done, when it was being done, what came of those conversations, what improvements were -- nothing. Nothing was -- it was very blasé and no information was shared. Nothing*."[70]

21.37 Of Mr Harvey's communication in particular, Mother D said: "*It was upsetting that Ian Harvey was not answering any question at all*."[71]

21.38 Dr Hawdon said that she had assumed Mr Harvey would follow the duty of candour with the parents, in regard to updating them about her report and sharing the report with them.[72] She accepted that she did not raise that the results should be shared with the families, and this was a "*serious omission*", but she thought it would be a "*given*".[73] In my view, that was a reasonable assumption. She was entitled to rely on the Medical Director to be aware of, and to comply with, the duty of candour. He did not do so.

21.39 Excerpts of Dr Hawdon's report were sent to some of the parents of babies who featured in the review, under cover of a letter from Mr Harvey sent in April 2017. This was six months after Dr Hawdon had completed her report.

21.40 Mother C described receiving an excerpt of Dr Hawdon's report, relevant to Baby C, with little context or covering information. She said in evidence to the Inquiry:

> "*There was no introduction to the report, who had written it, what date it was written on. It wasn't signed and dated by anybody. There was no context. And it's, you know, like a bullet-pointed list.*
>
> *So when I received it, I was struck by how superficial it looked. That, you know, we'd been told that these in-depth investigations had taken place and not revealed any cause for concern. Yet, we were sent a couple of pages of a report that didn't look at all in-depth to me …*
>
> *… Or, if it was, then it was woefully inadequate to call that a thorough investigation.*
>
> *So I was upset, distressed, extremely annoyed, perplexed.*"[74]

21.41 Mother C also said:

> "*I really felt that there was something significant going on that we were not being told about.*
>
> …
>
> [I]*t was very evident when we received* [the Hawdon report] *that, again, this was not the complete information. We still had not received the full Royal College report, and this certainly could not possibly have been the full investigation into the death of our son.*
>
> …
>
> *I felt very much that they didn't want us to know anything because they didn't want to be faced with questions that maybe they didn't have the answers to.*"[75]

21.42 Mother E and F also gave evidence to the Inquiry about receiving an excerpt of Dr Hawdon's report. She said:

> "*I was absolutely furious when this arrived. It was -- it's just not meaningful at all. And, you know, I felt the times were wrong on this document. I now know that the times were falsified. But that the timings* [were] *wrong, and the letter itself is -- why on earth would you* [send] *bereaved parents a letter with documentation in about their child from a medical perspective when the parents have no medical training or any medical background? … it's careless. It's not mindful of bereaved parents and I'd go as far as to say it's quite sloppy, to be honest.*"[76]

21.43 Mother E and F's view in relation to Mr Harvey's letters about the reviews was that:

> "*more information should have been given. I know it's a fine line of what they, you know, they can and can't give. I think the way things were presented was really, really poor. And I think they had a duty of care, and -- you know, and that candour and openness. I think it just fell short of the mark, didn't it, really? I mean, when we look at and reflect on the letters that were sent, it's just really, really poor.*"[77]

21.44 Mother G told the Inquiry she was not aware of the RCPCH or Dr Hawdon reviews until the criminal trial. She said: "*The Royal College of Paediatric*[s] *and Child Health review into neonatal services at the Countess of Chester Hospital and the advisory medical report prepared by Dr Hawdon are things I first heard about at the criminal trial of Lucy Letby. We have never received a copy of either report.*"[78]

21.45 Father G gave similar evidence. He said:

> "*I was not aware of the Royal College of Paediatric*[s] *and Child Health review into neonatal services at the Countess of Chester Hospital until the trial, and did not receive a copy of their report.*
>
> *I was also not aware of the advisory medical report prepared by Dr J Hawdon until the criminal trial of Lucy Letby. We were also not told that the Countess of Chester Hospital was investigating or reviewing our daughter's collapse and only became aware of the police investigation in around May 2017.*"[79]

21.46 Father G stated: "*We were also not told that there were concerns about an increasing number of babies dying on the Neonatal Unit, nor that they were looking at the standard of care given to our daughter. It truly came as the biggest shock of my life when I found this out years later.*"[80]

21.47 Parents H were not informed of the RCPCH report or the Dr Hawdon report at the time.

21.48 Mother I said she was not informed of the RCPCH report until February 2017 when the Countess sent her a letter. She stated: "*This was the first I'd heard of any review or investigation, so it was a bit shocking. I had no idea that any investigation or review was being conducted by the Countess of Chester Hospital. Receiving this letter was the first I'd ever heard of the Royal College of Paediatrics and Child Health Review.*"[81] In relation to Dr Hawdon's review, Mother I's evidence to the Inquiry was: "*I also was never aware of an advisory report prepared by Dr J Hawdon. The first I'd heard of this was when my solicitor mentioned it to me while I was making this statement.*"[82]

21.49 Parents J were not aware of the RCPCH report until the police advised them of it. Nor were they aware of the Dr Hawdon report.

21.50 Father K told the Inquiry:

> "*I did not know about the report of the Royal College of Paediatrics and Child Health into neonatal deaths and the NNU* [neonatal unit] *at the Countess of Chester Hospital or the Advisory Report of Dr Hawdon until my solicitors made me aware of them during the disclosure process relating to the upcoming Inquiry hearings. We had no idea about any of the investigations into neonatal services and unexplained collapses until we were contacted by the police. I believe we should have known about these investigations sooner.*"[83]

21.51 Parents L and M said:

> "*We are asked if we were aware that the Royal College of Paediatrics and Child Health undertook a review into neonatal deaths and the neonatal unit at* [the hospital]*. I am informed by my solicitor that this review is dated November 2016. We were not aware of that. We are also asked if we were aware of an Advisory Medical report prepared by Dr Hawdon dated 1 October 2016 in relation to babies who died or had cardiorespiratory collapses in the neonatal period at* [the hospital]*. We were not aware of that.*"[84]

21.52 Mother N said: "*I am asked if I was aware that the Royal College of Paediatrics and Child Health undertook a review into neonatal services at* [the hospital]*. I am informed that this review is dated November 2016. I was not made aware of this report and was not provided with a copy of it.*"[85]

21.53 Dr Hawdon considered that the families were given insufficient covering information when they received excerpts of her report related to their baby. She described a state of "*shock*" when she saw the state in which the excerpts were sent and said that she was not aware of this at the time.[86]

21.54 Mr Harvey was asked about the excerpts of Dr Hawdon's report that were sent to the families. He conceded: "*I fully accept that that communication was both crass and inappropriate … It was done in completely the wrong way. It was unthinking and insensitive*."[87] This was a complete apology for the abject failure of communication with the parents in respect of Dr Hawdon's report. But it was not just communication with parents that was dreadful, although that was the worst of it. In a situation where openness was imperative, if effective investigations were to take place, he was not open. Instead, he drip-fed partial information to Dr Hawdon and then to Dr McPartland.

Dr Hawdon's reflections

21.55 Dr Hawdon said in evidence that "*unexplained and unexpected deaths are unlikely to be from natural causes*"[88] and confirmed murder must therefore be a possibility.[89] Mr Skelton KC put to Dr Hawdon that she should have been more explicit that the causes of the unexplained and unexpected deaths in Group 2 of her report were likely to be unnatural causes, and murder was a possibility. Dr Hawdon rejected that suggestion. She said: "[I]*n my view … for a clinician to be told that a number of deaths are unexpected and unexplained is a message in itself … It means there is something very worrying that needs to be looked into in more detail.*"[90] And later: "*I didn't suspect a crime. Knowing what I know now, I might have suspected a crime. I will -- I will maintain that* [for] *a Medical Director to receive a report that says five deaths are unexpected and unexplained is a sufficient trigger to take the issue seriously.*"[91] I accept her evidence about that.

21.56 Dr Hawdon opined that receipt of her report should have triggered a safeguarding process.[92] When asked by Ms Langdale KC whether the police should have been contacted, Dr Hawdon explained that contacting the safeguarding team would inevitably lead to police involvement.[93] She explained that the benefit of contacting the police indirectly through the safeguarding team is that "*the police that work with safeguarding teams are very, very sensitive to the circumstances and it doesn't -- it doesn't delay things. But it does mean that police would have a certain skillset to become involved.*"[94]

Dr McPartland

21.57 Dr McPartland is a consultant paediatric pathologist at Alder Hey. She has experience of conducting post-mortems and forensic post-mortems. Post-mortems are conducted by a single pathologist, with no police involvement. Forensic post-mortems are conducted by a paediatric pathologist and a forensic pathologist. Dr McPartland explained the two different roles: "[T]*he role of the forensic pathologist is to consider matters of forensic importance and particularly those relating to injuries, and the role of the paediatric pathologist is to consider natural causes of death and look at growth and development and other medical conditions*."[95] Radiology, toxicology and the police are also involved in forensic post-mortems.

21.58 Dr McPartland said that, as a paediatric pathologist, she ordinarily receives clinical information about babies from medical notes, and a letter from the treating consultant. This is provided to her indirectly via the coroner's office.[96] In oral evidence, she described the importance of being provided with a comprehensive history and information about the circumstances of the death at an early stage (including any suspicion of foul play).[97] This ensures that the appropriate professionals are gathered, and a forensic post-mortem can be arranged if necessary.[98]

21.59 The Alder Hey pathologists attended the perinatal mortality and morbidity meetings at the Countess. Dr McPartland explained that the obstetric team presented the antenatal history, the neonatal team presented the neonatal history, and the pathologist would present the findings at post-mortem.[99] There was then a holistic discussion about the case. The meetings occurred periodically throughout the year. In advance of a meeting, the Countess would send the pathologists a list of the cases to be discussed, so that the pathologist could prepare.[100] Baby D was discussed at a meeting on 10 September 2015, but because the pathologists were not told in advance that Baby D was to be discussed, Dr McPartland did not attend.[††] [101] I have dealt in Chapter 3 with Dr McPartland's response to questions about the HMC Authorisation Form in respect of Baby D.

21.60 Dr McPartland was not alerted to the increase in mortality rate via an increase in the number of post-mortems. She explained this was because the coroner's cases were randomly distributed between herself, Dr Kokai and Dr Shukla (both consultant paediatric pathologists at Alder Hey), based on who was on the post-mortem rota at Alder Hey.[102] She believed that it was Dr Shukla who alerted her to the increase, at some point after July 2016 and before December 2016, when Mr Harvey contacted her in respect of deaths on the neonatal unit at the Countess.[103]

21.61 Ms Langdale KC asked whether Dr McPartland should have been suspicious about the mere presence of the cluster. Dr McPartland's evidence was that "*if you want a pathologist to be alerted to something potentially suspicious you would have to highlight that they were sudden and unexpected, unexplained and concerning*".[104]

21.62 As I noted earlier (see Chapter 3), Dr McPartland added that she would require a "*strong clinical steer that it was a suspicious case to warrant insisting on police and forensic pathology involvement*".[105]

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†† Dr McPartland's evidence was that Baby A was the only baby on the indictment who the Alder Hey pathologists were given advance warning would be discussed in a perinatal mortality and morbidity meeting.

21.63 When Dr McPartland was asked what it would require for coronial staff to be concerned about a member of staff's shift commonality to the deaths, she said it would need to be made very clear: "*I think for it to be picked up at the Coronial stage where they are doing a referral someone would have had to have said that there is an increased number of worrying deaths and we are worried that the same staff member has been involved in all of them.*"[106]

Dr McPartland's instructions

21.64 On 6 December 2016, Mr Harvey contacted Dr McPartland by telephone.[107] In an email the same day, informing Dr Shukla and Dr Kokai about the call, Dr McPartland said that Mr Harvey informed her there had been a "*spate of neonatal deaths*".[108] An independent expert, Dr Hawdon, had reviewed 13 deaths and 4 near misses, and "*suggested that in 4 of the PMs* [post-mortems]*, it **might*** [emphasis added] *be worth a follow up discussion with the pathologists to see if further light can be shed on the issues*".[109] This sounds like a typically low-key call from Mr Harvey. The fact that Dr Hawdon has recommended a forensic review becomes that she has suggested that "*it might be worth a follow up discussion with the pathologists*". Mr Harvey said in evidence that, in instructing Dr McPartland, he made reference to an association with a staff member.[110] (In his statement, he had said: "*I think I had discussed with Dr McPartland, verbally, that clinicians had raised concerns over a member of staff and her presence on the ward at relevant times.*"[111])

21.65 Ms Langdale KC informed Dr McPartland of Mr Harvey's written evidence to the Inquiry that he thought he had a phone call with Dr McPartland in which he verbally raised that the clinicians had concerns about a member of staff. Dr McPartland rejected this and stated they only had one phone call and that Mr Harvey did not mention anything about a nurse. She said that, if she had known of those concerns, "*I would have said that the police needed to be immediately involved and a forensic pathologist would need to be instructed*".[112] She also said that it "*hadn't been conveyed*" to her that a broader forensic review had been recommended by Dr Hawdon.[113] I accept her evidence. I think it highly unlikely that Mr Harvey would have taken a different approach with Dr McPartland from the one he had taken with Dr Hawdon.

21.66 On 21 December 2016, Mr Harvey emailed Dr McPartland with a summary of Dr Hawdon's findings about four cases: Baby A, Baby I, Baby O and Baby P. He told her Dr Hawdon had advised a "'*local forensic review*', *to include pathology/histopathology*" of the cases.[114] Mr Harvey did not provide her with a copy of the RCPCH report, or Dr Hawdon's report. Dr McPartland considered she should have been provided with both, and been briefed about the concerns.[115] I agree with her.

Review of Baby A, Baby I, Baby O and Baby P

21.67 Dr McPartland conducted her review with Dr Shukla and Dr Kokai. Prior to their meeting, she sent them the post-mortems of each of the babies and the accompanying clinical information.[116] The three pathologists discussed each of the cases for 20 minutes and Dr McPartland wrote the conclusions. She summarised that "*in three of the four cases we indicated that we didn't know why the babies had collapsed*".[117] These three cases were Baby A, Baby P and Baby O. The trio agreed with the recorded death at post-mortem for Baby I.[118]

21.68 Dr McPartland acknowledged the review was "*an informal process*", which "*hadn't added much to the original reports*".[119] Effectively, they considered the same information as had been available to them when they completed the post-mortems. The only difference was that this time they discussed the cases as a group. Thus, arguably, Dr Hawdon's recommendation for an independent pathology review was not undertaken, as these professionals had all undertaken the original reviews.

21.69 On 25 January 2017, Dr McPartland emailed Mr Harvey a summary of the conclusions from the review. She acknowledged the informal nature of the review and informed him that it was "*not a full and formal medicolegal review*".[120] Dr McPartland advised Mr Harvey that, if he wanted a report of that depth, it would require a four-hour review of each case and that this would be best performed by someone independent at another hospital. This was a clear message to Mr Harvey that the review had been very limited.

21.70 Later on 25 January 2017, Mr Harvey emailed Dr McPartland about a comment she made in her review that "*a very small air embolism might not be detectable at autopsy*".‡‡ [121] Mr Harvey asked whether a significant embolism would be evident. This was, again, a deliberately casual and brief question from Mr Harvey. The following day, Dr McPartland responded equally casually: "*Yes, a significant air embolism should be accompanied by froth in the vessels or lungs.*"[122] In evidence, Dr McPartland said she thought Mr Harvey was asking this question because of concerns about Baby A's long line insertion prior to the collapse.[123] She went on to say, in respect of the detectability of air embolism:

> "*I have done a lot of reading about this and it seems to me, although in some cases in the literature there is a large amount of froth in the heart and in the lungs reported there are other cases reported where there has been a significant amount of air visible on postmortem CT, but then the pathologist couldn't identify it at all at postmortem.*
>
> *So I think from that, and I have had experience of another case since of a postmortem case where a large amount of air was identified on CT and with a forensic pathologist, even with filling the pericardial sac with water and puncturing the heart we couldn't demonstrate it at postmortem.*
>
> *So my experience from that reading and that case I have had since then would lead me to believe that you could have a significant amount of air at postmortem and it could be completely undetectable at autopsy.*
>
> **Q:** *So your statement to Ian Harvey in the email may have been quite genuine based upon your understanding at the time, but since then you have come to learn that in fact you can have a very significant air embolism without froth in the heart or lungs?*
>
> **Dr McPartland:** *Yes, that's correct.*"[124]

21.71 Dr McPartland was asked how she would have responded at the time if Mr Harvey had specified that he was asking about a deliberate air embolism. She stated: "*That would have alerted us* [to] *the fact that this was a concern of a criminal case and that this needed a much more in-depth and forensic pathology opinion. So if he had said that to us, we wouldn't have made a brief email response; it would have been apparent that that needed a very different approach.*"[125]

‡‡ This was in relation to Baby A.

21.72 Dr McPartland informed the Inquiry that, following the verdicts in Letby's criminal trials, she changed her practice on the radiological investigation of neonatal deaths.[126] Now, in addition to whole-body skeletal surveys (X-rays), whole-body CT scans are included in post-mortems for all neonatal deaths that occur in hospitals, with the view to identifying if there has been an administration of intravascular air.[§§] Dr McPartland said that this is not yet current practice around the country.[127]

21.73 It was Dr McPartland's view that she should have received safeguarding training about what to do if there were concerns that a staff member had deliberately harmed a child.[128]

21.74 By the end of 2016/the beginning of 2017, Mr Harvey knew that none of the reports (from the RCPCH, Dr Hawdon or Dr McPartland) had excluded deliberate harm. In evidence, he accepted that "*the reviews hadn't been specifically commissioned to look for a crime*".[129] Below is an extract from the transcript:

> "**Mr Skelton KC:** … *just to be clear, the* [RCPCH] *report that was produced as a result of that review could not be relied on to exclude the possibility that the children had been harmed?*
>
> **Mr Harvey:** *No.*
>
> **Mr Skelton KC:** *The same really must apply to Dr Hawdon's examination and Dr McPartland's examination, because first of all, Dr Hawdon, in respect of five of the deaths … couldn't find an explanation. So, by definition, she hadn't found a crime or excluded a crime … So that had not excluded Lucy Letby harming them?*
>
> **Mr Harvey:** *No.*
>
> **Mr Skelton KC:** *Dr McPartland was not a forensic pathologist, so she, by definition, couldn't investigate a crime and exclude it definitively …*
>
> …
>
> **Mr Skelton KC:** … *she was not briefed to investigate criminal activity?*
>
> **Mr Harvey:** *No, she was a specialist paediatric neonatal pathologist.*
>
> **Mr Skelton KC:** *So as all these investigations are being pursued throughout the course of 2016 and into 2017, the upshot is that none of them, in fact, exclude the possibility that had been raised on 29 and 30 June 2016 that Lucy Letby harmed the children: none of them?*
>
> **Mr Harvey:** *No, which is why we ended up subsequently going to the police.*"[130]

21.75 Mr Harvey added: "*I suppose we had anticipated that, in the event that there had been a malicious act, that there would have been evidence found in the course of those reviews.*"[131] That answer is revealing. The Executive Team all knew by this stage that there had been a marked increase in the number of deaths in the hospital. They also knew that the deaths were unexpected and unexplained. According to Dr Hawdon, and she was not challenged on this on behalf of any of the Core Participants, unexpected and unexplained deaths on a neonatal unit are rare.[132] Here, there were several. That was worrying and required investigation, as Dr Hawdon and the consultant paediatricians said.

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§§ Dr McPartland specified that this happens for all neonatal deaths in hospitals, whether they are coronial SUDIC cases or not.

21.76 After all the reviews, from the Thematic Review to Dr Hawdon's review, by way of the Silver Command process and the RCPCH report, there was still no explanation for several deaths. The worry should have increased exponentially. Instead, it subsequently came to be said that Letby had been exonerated. Had Mr Harvey been neutral with the RCPCH about the doctors' concerns, the RCPCH would have taken a different approach. Had he been frank with Dr Hawdon and Dr McPartland, they, too, would have taken a different approach. It is likely that this would have led to the police being called. Given that this was Mr Harvey's view and that of Ms Kelly in June 2016, and nothing had changed, other than the exclusion of all the obvious non-criminal explanations, the failure to call the police at this stage is impossible to defend.

Endnotes

1 INQ0014365/3

2 INQ0014365/3

3 INQ0012066

4 INQ0003120/2

5 Dr Jane Hawdon 12 November 2024 10/20-22

6 Dr Jane Hawdon 12 November 2024 71/9-12

7 Dr Jane Hawdon 12 November 2024 14/9-13

8 Dr Jane Hawdon 12 November 2024 14/21-22

9 Dr Jane Hawdon 12 November 2024 15/11-13 and 19/6-19

10 INQ0003123/2

11 INQ0003123/1

12 Dr Jane Hawdon 12 November 2024 44/7-13

13 Ian Harvey 29 November 2024 25/6-7

14 INQ0003328/1-2

15 Dr Jane Hawdon 12 November 2024 12/6-13

16 Dr Jane Hawdon 12 November 2024 12/19

17 INQ0014365/3

18 INQ0003120/2

19 INQ0003358/1-2

20 INQ0012066

21 Dr Jane Hawdon 12 November 2024 6/11-13

22 Dr Jane Hawdon 12 November 2024 51/6-10

23 Dr Jane Hawdon 12 November 2024 9/19

24 Dr Jane Hawdon 12 November 2024 16/11

25 Dr Jane Hawdon 12 November 2024 9/7-14 and 10/6-10 and 21/16-20

26 Dr Jane Hawdon 12 November 2024 21/21 to 23/5

27 INQ0003358/2

28 Dr Jane Hawdon 12 November 2024 17/8-16 and 59/7 to 60/6

29 INQ0003172/44

30 INQ0003172/44: "*Cause of death as given in post mortem report should be reviewed given baby stable in air in days preceding collapse.*"

31 INQ0003102/1

32 Ian Harvey 29 November 2024 41/14-24

33 Dr Jane Hawdon 12 November 2024 27/16 to 28/6 and 65/9-18

34 Dr Jane Hawdon 12 November 2024 26/7 to 27/15 and 65/22 to 68/4

35 Dr Jane Hawdon 12 November 2024 65/22 to 67/4 and 26/19-27/15

36 Dr Jane Hawdon 12 November 2024 68/18 to 69/24

37 Dr Jane Hawdon 12 November 2024 67/17-19

38 Dr Jane Hawdon 12 November 2024 67/20 to 68/14

39 Dr Jane Hawdon 12 November 2024 69/4-18

40 Dr Jane Hawdon 12 November 2024 69/19-24

41 Dr Jane Hawdon 12 November 2024 54/1-9

42 Dr Jane Hawdon 12 November 2024 54/16-19

43 Dr Jane Hawdon 12 November 2024 54/20 to 55/1

44 Dr Jane Hawdon 12 November 2024 55/16-25; Ian Harvey 29 November 2024 40/4-25

45 Dr Jane Hawdon 12 November 2024 56/1-11

46 INQ0003383/2

47 INQ0008946/79

48 INQ0099055

49 INQ0014376/2-3

50 INQ0003117

51 INQ0014376/3

52 INQ0014376/3

53 Dr Jane Hawdon 12 November 2024 36/20 to 37/2

54 INQ0003117

55 Dr Jane Hawdon 12 November 2024 31/18-24

56 INQ0014376/2

57 Dr Jane Hawdon 12 November 2024 40/3-4

58 INQ0003124/2

59 INQ0003124/2

60 Dr Jane Hawdon 12 November 2024 39/2-22

61 INQ0003124/1

62 Dr Jane Hawdon 12 November 2024 79/22 to 80/20

63 Mother C 16 September 2024 95/14 to 100/5

64 Mother C 16 September 2024 101/4 to 102/10

65 Mother C 16 September 2024 102/12-21

66 Mother C 16 September 2024 100/6-12

67 Mother C 16 September 2024 100/17-20

68 Mother D 17 September 2024 38/12

69 Mother D 17 September 2024 57/19-25
70 Mother D 17 September 2024 42/7-12
71 Mother D 17 September 2024 54/23-24
72 Dr Jane Hawdon 12 November 2024 49/19 to 51/4
73 Dr Jane Hawdon 12 November 2024 50/23-25
74 Mother C 16 September 2024 108/1-22
75 Mother C 16 September 2024 105/14 to 109/15
76 Mother E and F 18 September 2024 33/22 to 34/9
77 Mother E and F 18 September 2024 38/19-39/1
78 Mother G 18 September 2024 85/7-12
79 Father G 18 September 2024 104/4-13
80 Father G 18 September 2024 99/9-14
81 Mother I 17 September 2024 122/10-16
82 Mother I 17 September 2024 122/16-19
83 Father K 23 September 2024 166/19 to 167/3
84 Father L and M 24 September 2024 17/4-13
85 Mother N 24 September 2024 40/19-24
86 Dr Jane Hawdon 12 November 2024 42/2-5
87 Ian Harvey 29 November 2024 113/11 to 114/5
88 Dr Jane Hawdon 12 November 2024 40/3-4
89 Dr Jane Hawdon 12 November 2024 73/24 to 74/4
90 Dr Jane Hawdon 12 November 2024 74/18-23
91 Dr Jane Hawdon 12 November 2024 75/24 to 76/4
92 Dr Jane Hawdon 12 November 2024 24/17-20
93 Dr Jane Hawdon 12 November 2024 24/21 to 25/1
94 Dr Jane Hawdon 12 November 2024 25/2-6
95 Dr Jo McPartland 12 November 2024 84/10-15
96 Dr Jo McPartland 12 November 2024 98/24 to 99/5
97 Dr Jo McPartland 12 November 2024 134/23 to 135/8
98 Dr Jo McPartland 12 November 2024 84/3 to 85/5
99 Dr Jo McPartland 12 November 2024 103/8-17
100 Dr Jo McPartland 12 November 2024 101/6-9
101 Dr Jo McPartland 12 November 2024 101/10-19 and 103/22-25
102 Dr Jo McPartland 12 November 2024 100/6-23
103 Dr Jo McPartland 12 November 2024 100/12-20

104 Dr Jo McPartland 12 November 2024 109/23 to 110/2

105 Dr Jo McPartland 12 November 2024 156/21-23

106 Dr Jo McPartland 12 November 2024 111/10-14

107 INQ0101999

108 INQ0101999

109 INQ0101999

110 Ian Harvey 29 November 2024 35/10-13

111 INQ0107653/120/para 512

112 Dr Jo McPartland 12 November 2024 127/21-23 and 130/18 to 131/2

113 Dr Jo McPartland 12 November 2024 129/20-23

114 INQ0102002/2

115 Dr Jo McPartland 12 November 2024 122/4 to 123/4

116 Dr Jo McPartland 12 November 2024 125/19-25

117 Dr Jo McPartland 12 November 2024 131/8-9

118 INQ0102007/3

119 Dr Jo McPartland 12 November 2024 126/1-19

120 INQ0003135/1

121 INQ0102010

122 INQ0102011; Dr Jo McPartland 12 November 2024 154/15 to 155/6

123 Dr Jo McPartland 12 November 2024 128/16-23

124 Dr Jo McPartland 12 November 2024 154/15 to 156/12

125 Dr Jo McPartland 12 November 2024 129/5-10

126 Dr Jo McPartland 12 November 2024 129/24 to 130/14

127 Dr Jo McPartland 12 November 2024 130/15-17

128 Dr Jo McPartland 12 November 2024 91/15 to 92/11

129 Ian Harvey 28 November 2024 203/20-21

130 Ian Harvey 28 November 2024 201/1 to 202/15

131 Ian Harvey 28 November 2024 203/21-24

132 Dr Jane Hawdon 12 November 2024 39/8-11

Chapter 22. September 2016 to January 2017: The grievance

22.1 On 1 September 2016, Ms Cooper (Royal College of Nursing (RCN) representative), accompanied Letby to her RCPCH interview. By this time, Ms Cooper knew there were allegations of deliberate harm carried out by Letby. The interviewers did not ask Letby any questions about the allegations, and thus Ms Cooper concluded "*it was quite evident in the interview it wasn't going to solve anything*" for Letby.[1]

22.2 Although it was ultimately Letby's decision to initiate a grievance, Ms Cooper and Mr Millea (also RCN) suggested it to Letby based on the lack of information about her redeployment and the fact that she had been redeployed when the RCPCH review was not investigating her. Ms Cooper was candid that, at times, the RCN put pressure on the hospital: "*we felt or she felt she needed to put pressure on them*" to return Letby to the neonatal unit.[2]

22.3 Ms Cooper gave evidence that she told Ms Kelly and Ms Hodkinson that "*they had a duty to investigate this and … from the RCN point of view … you either need to investigate our member or you need to allow her back on the unit because nothing was happening*".[3] She reflected that this may have been a mis-step: "[T]*his was made into an employment issue and it was never an employment issue and it shouldn't have been*."[4]

22.4 On 2 September 2016, Mr Millea emailed Ms Rees stating that he believed Letby "*has grounds to action a Grievance*".[5] Mr Millea wrote that he had two concerns:

a. First, it had become apparent in the course of Letby's interview with the RCPCH the previous day that, contrary to the indication given in the meetings in July 2016, "[T]*he terms of reference for this investigation does not seem to address the … concerns … in relation to the unacceptable high mortality rate on the NNU* [neonatal unit] *and our* [member's] *involvement*." Instead, the RCPCH review appeared to be focused on more general matters – such as procedure, culture and staffing levels – and therefore "*will not solve the issues for* [Letby] *personally*".[6]

b. Second, Letby's redeployment was as a result "*of the* [Trust's] *response to …* [consultants'] *comments about our* [member's] *practice … I would like to request to see the* [Trust's] *evidence to substantiate their actions.*"[7]

22.5 Ms Kelly knew as of 1 September 2016, when she and Mr Harvey were interviewed, that the RCPCH review team would not be investigating Letby or any link between her and the deaths. By 2 September 2016, Ms Kelly, Ms Hodkinson and Ms Rees knew that Letby and her representatives were aware of this also. As Ms Appleton-Cairns and Ms Hodkinson had been told by their employment lawyers, DAC Beachcroft, as long ago as 5 July and 18 July 2016,[8] investigation was necessary. This advice had been ignored.

The RCPCH review was being presented by the executives as something that would find the reason(s) for the increased number of deaths on the neonatal unit. It could not, and did not.

22.6 Mr Millea said that the allegations made by the hospital "*would have a detrimental effect on our member's career … which may constitute professional slander resulting in our member being constructively dismissed*". He asked that the hospital reinstate Letby to her substantive role on the neonatal unit. Mr Millea's focus was on looking after his union member's interests. He thought the matter needed to be investigated. He raised the seriousness of the allegations against Letby and, later, in the course of the grievance interview with Letby, asked why the police had not been informed about them.[9]

22.7 On 7 September 2016, Ms Cooper had a "*frank*" discussion with Letby about the nature of the allegations against her.[10] Ms Cooper's evidence was that Letby's reaction to being told about the allegations was to calmly walk away and then to go on her phone. She did not show any emotion or ask Ms Cooper questions about the allegations. Speaking about Letby's reaction, Ms Cooper stated: "*I did think it was strange at the time*."[11]

22.8 Dr Green, the Investigator of Letby's grievance, told the Inquiry that at some point during the grievance process he explained to Letby the allegations against her were that she had murdered babies. Dr Green remarked that Letby's "*reaction wasn't quite normal*" and that he was "*struck by how coolly she took it*".[12] He explained he "*didn't get a lot of emotion from Lucy Letby*" and that she "*didn't really react at all*".[13] He remarked that if somebody had made those allegations against him, he would be indignant about it.[14] I accept that Letby reacted in the way described by Ms Cooper and Dr Green, and I note her reaction to Ms Powell on 14 July 2016, when Letby was very upset. Ms Powell said she later realised that Letby was upset because her competence was being doubted and not because she was suspected of harming babies.[15]

The grievance

22.9 Letby filed a written grievance on 7 September 2016 on the grounds of victimisation and discrimination. This echoed the concerns expressed in Mr Millea's email of 2 September 2016, and it raised the following issues:

a. On 14 July 2016, Letby had been informed that she must undertake supervised practice and redo her competencies, and that the same process would apply to other members of staff.

b. On 18 July 2016, she had agreed to be redeployed, as the neonatal unit was unable to offer supervised practice, due to staffing levels. She was informed that the redeployment would last until an external review had taken place regarding the increase in mortalities.

c. Contrary to what Letby had been told, no other member of staff had been placed under supervised practice or redeployed. Letby had been "*singled out*" and felt that she was "*being discriminated against*".

d. The RCPCH reviewers had interviewed Letby and made no mention of any wrongdoing on her part. They had also told her that it would take up to eight weeks for them to complete their report. Letby wished to know what was "*expected to happen with me during this period as I am keen to return to work on the NNU as soon as possible*".[16]

22.10 Consultants had raised concerns about Letby. She wished to know what they were and how the Trust was dealing with them. She wished to know what evidence there was against her "*and if there is to be an investigation into my practice then on what grounds*?".[17] Letby felt that she was being "*targeted and blamed for incidents that even the review report will not address*".[18]

22.11 On the same day, 7 September 2016, the Executive Directors met to discuss the immediate recommendations in the letter of 5 September arising from the RCPCH report. These are set out at paragraphs 20.97 to 20.110, Chapter 20. In brief, they were that the Trust should conduct an HR investigation in respect of Letby and should commission a detailed case note review in respect of "*each of the deaths since July 2015* [this should have been June]".[19]

Legal advice from DAC Beachcroft

22.12 On 8 September 2016, Ms Hodkinson and Ms Appleton-Cairns met Mr Pace to discuss Letby's redeployment and grievance. Mr Pace noted that Letby had been "*removed from the neonatal unit, following a correlation of baby deaths when she was on the unit*" and that the decision to redeploy her to the Risk and Patient Safety Department had been taken "*because it was not possible to place her under close supervision and because of the ongoing concerns that had been raised by the consultant*".[20] She had been redeployed until the external RCPCH review was completed. The review had been completed (final report not received), but its findings were inconclusive and a further forensic review was to be carried out.

22.13 Ms Hodkinson's handwritten notes demonstrate that Mr Pace advised that there was a high risk of constructive dismissal. She also noted: "[J]*ustified in decision to remove … remove risks to babies*". Under a list headed 'Options', it was noted that suspension was "*not appropriate in* [the] *circumstances*".[21]

22.14 The file note of the meeting records that Mr Pace advised the Trust to respond in line with the grievance policy.[22]

22.15 Also on 8 September 2016, Ms Hodkinson compiled an 'NNU Options Appraisal' document marked "*Strictly Private & Confidential*". The options set out in this document were: Letby to remain in redeployment for an extended period; to re-integrate Letby back within the neonatal unit (with various ways of doing so set out); to undertake a disciplinary investigation; to engage the Speak Out Safely Policy; and for Letby to resign. Various "*Considerations*" in respect of each option were set out.[23]

22.16 Option 4 was to "*re-integrate* [Letby] *back within the NNU without ITU/HDU duties whilst competencies reviewed*".[24] Ms Hodkinson recommended this in an Executive meeting on 8 September 2016, and it was initially agreed amongst the Executive Team.[25] However,

this decision was overturned almost immediately by Mr Chambers, Mr Harvey and Mr Cross, who took the view that Letby should not be returned to the unit but should remain redeployed on the basis that the RCPCH review had not been fully completed.[26]

22.17 On 9 September 2016, Ms Hodkinson and Ms Kelly had a further discussion with Mr Pace to discuss the handling of the grievance and provision of support for Letby, noting that constructive dismissal was considered to be high risk. In light of the executives' decision to continue Letby's redeployment, Mr Pace advised that it should be made clear to Letby that her redeployment was not a disciplinary sanction but to allow for a further external investigation to take place. He subsequently provided draft letters to Ms Hodkinson for Letby and the RCN.

22.18 The same day, there was a meeting of the Executive Team. Mr Chambers, Mr Harvey, Ms Kelly, Ms Hodkinson and Mr Cross attended. The consultants' behaviour was discussed. The delayed recruitment of a consultant was also discussed. Mr Harvey had made enquiries and he proposed that recruitment would remain on indefinite hold due to "*major reorg*[anisation]" and until "*final findings of review received*".[27]

22.19 Also on 9 September 2016, Ms Rees met Ms Kelly and Ms Hodkinson. Following the meeting, Ms Rees emailed Ms Kelly and Ms Hodkinson to express serious reservations about the decision to delay Letby's transfer back to the neonatal unit.[28]

HR progress the grievance

22.20 On 20 September 2016, Ms Cooper emailed Ms Hodkinson to chase progress as they had received no acknowledgement of the grievance. On 21 September 2016, Ms Appleton-Cairns emailed Ms Kelly and Ms Hodkinson and said: "[W]*e were going to ask Ian* [Harvey] *to speak to SB* [Dr Brearey] *and ask him to formally voice his concerns under Speak Out Safely. I think we need to do this in parallel – any thoughts?*"[29] Ms Hodkinson and Ms Kelly both replied that they were unsure whether this had happened. There is no evidence that it did, or that they expressed any thoughts.

22.21 On 28 September 2016, Ms Appleton-Cairns considered Letby's case and her grievance. In an email to Ms Hodkinson, Ms Kelly and Mr Harvey, she made a list of the nine "*key questions*" which she believed Letby "*wants answering*". These included:

> "*What are the issues the Consultants have raised …?*
>
> *What is the Trust doing about it?*
>
> *What evidence does the Trust have …?*
>
> *If there is to be an investigation into her practice – then what are the grounds?*
>
> *Does she have to undertake supervised practice and redo competencies?*
>
> *Who else has to … redo competencies?*
>
> *If no one else, why not?*
>
> *Why has she been singled out for redeployment?*
>
> *When can she return to the NNU?*"[30]

22.22 In the same email, Ms Appleton-Cairns wrote that an independent Chair had been appointed (Ms Annette Weatherley, Deputy Chief Nurse, University Hospital of South Manchester NHS Foundation Trust and Independent Chair of Grievance Panel), "*but at the moment we do not have a modicum of a defence for this*".[31] Given the concerns, this was surprising advice. She suggested that an Investigating Officer be appointed and noted that they "*will have to ask very difficult questions of the Consultants*". She also noted, again, that the plan was for the consultants to be asked to "*explain their concerns in writing under Speak Out Safely*". She considered that this would be a "*softer approach*" than questioning the consultants under the grievance procedure. She concluded that, after the consultants had explained their concerns: "[W]*e can investigate any statements of evidence and be able to respond to LL … This is now time pressured as we are failing to respond under our own policy timeframes which is an unnecessary risk should it go further*."[32] The consultants were not asked to explain their concerns in writing under the Speak Out Safely Policy, or at all.

22.23 Ms Hodkinson met Letby on 5 October 2016. She explained that this was an informal meeting and was separate from the grievance process; and that the findings of the formal (RCPCH) review into the neonatal unit were being awaited, and in the meantime Letby's redeployment would continue, with weekly support meetings in place. It was noted that Letby was receiving clinical updates from the neonatal unit, and that the "*best outcome would be to get you back working on the Neonatal Unit*". Letby was asked whether she still required the formal grievance process to continue. Letby confirmed that she did. It was explained that there had been delays in identifying an independent Chair for the grievance hearing, but that Dr Green had been appointed as Investigating Officer.[33]

Dr Green's appointment as Grievance Investigating Officer

22.24 Dr Green was Director of Pharmacy and Medicines Management at the Countess. He said in evidence that Ms Appleton-Cairns had explained that his job "*was to gather the evidence and come up with some suggestions as to whether those grievances could be upheld and it was Annette Weatherley's decision whether the argument that I presented was reasonable or not*".[34]

22.25 Several aspects of Dr Green's appointment were flawed. Dr Green had only been involved with two or three grievances prior to this one. He accepted that this was not many.[35] He had no experience of investigating a complex grievance in the context of serious patient safety concerns.

22.26 Dr Green was not independent. He worked at the hospital, and he had some recent history with Dr Brearey. In April 2016, Dr Green and Dr Brearey had what Dr Green characterised as a "*professional disagreement*" in relation to a junior pharmacist's medication error in respect of a baby (not one of the babies featuring on the indictment).[36] At the time, Dr Brearey and Dr Green met to discuss the matter. Dr Green said that there was no grudge or malice between them arising from the disagreement.

22.27 Dr Green's assertion that there was no grudge is at odds with the way he conducted Dr Brearey's interview for the grievance. Professor Bowers KC, employment law expert, remarked when giving evidence that he was struck by Dr Green's "*hostile questioning*" of Dr Brearey and that it "*didn't look the normal sort of independent questioning*" that he would expect within a grievance interview.[37] I agree.

22.28 Concern about the appointment of Dr Green and his independence was expressed at the time by Ms Hodkinson to Ms Appleton-Cairns. In an email, dated 23 September 2016, Ms Hodkinson asked: "[W]*e are having an independent manager external to the Trust hearing not investigating the case so is this right in line with the policy*?"[38] It is not clear from the contemporaneous documents why Ms Hodkinson, the senior executive, did not intervene.

22.29 Ms Appleton-Cairns said that she was unaware that Dr Green and Dr Brearey had had a disagreement prior to his appointment to the grievance investigation.[39] She maintained that the appointment was appropriate based on the information she had at the time. She described Dr Green as an honourable person.

22.30 Ms Kelly, said that, on reflection, Dr Green should not have been appointed. She said it should have been someone independent because Dr Green was assessing people he had experience of.[40] It should also be borne in mind that gossip was rife in the hospital at this time.

22.31 Leaving aside gossip, Dr Green had been involved in Silver Command (see Chapter 15). He knew that several people in the hospital had been given investigatory tasks because of concerns about foul play. He had been asked to retain a TPN bag in case it was needed for investigation, after the deaths of Baby O and Baby P. He was inexperienced in grievances. After some reflection, Dr Green said in evidence: "[P]*erhaps I should not have been asked to do that grievance; perhaps it should have been someone external to the Trust*."[41] I agree with him.

22.32 It was wrong to appoint Dr Green. If there was no one within the Trust with sufficient experience and independence to conduct such an important grievance investigation fairly, someone from outside the Trust should have been appointed. Dr Green could bring neither independence nor objectivity, as Ms Hodkinson and Ms Lucy Sementa, an HR specialist (who attended the grievance interviews with Dr Green and assisted him on HR matters), should have appreciated.

22.33 Letby's primary complaint was that she had been moved without explanation or proper process. This got lost in the investigation process. Having dressed up the move to the Risk and Patient Safety Department as being for Letby's protection, because of the association between her presence and the deaths, Mr Harvey, Ms Kelly and others did all they could to downplay the consultants' concerns. It is worth remembering that Mr Pace had been very clear in advising that the safety of the babies must come first.

22.34 Dr Green interviewed each of the witnesses and produced his findings in an investigation report that was given to Ms Weatherley. Ms Weatherley did not hear from any witness. Professor Bowers KC said this was not how he would normally expect a grievance to be organised. He stated: "*I would expect the hearing officer who is chosen to be independent here, I think from a different Trust, should hear directly from at least the important people*."[42] I agree. The process adopted obviously risked being a rubber stamp.

That is what happened. Ms Weatherley was a deputy senior nurse in a different hospital. She was sent the papers 48 hours before the hearing.[43] Whilst last-minute instructions may well be a feature of busy hospitals, it is clear that no option was given to extend the time or allow for witnesses to be called. This was to be a review of Dr Green's investigation and approval of it.

Interview of Letby, 14 October 2016

22.35 Dr Green began conducting investigation interviews on 14 October 2016. Letby's was the first. She told Dr Green that she had not received any formal allegations; it was all verbal. Mr Millea is recorded as stating Dr Brearey had bullied and harassed Letby.[44] Dr Green accepted that he did not ask any follow-up questions about this and he should have done.[45] He does not seem even to have asked Letby whether this was true and what form the bullying had taken. Had he done so, he would have learned that Dr Brearey had neither bullied nor harassed her. In her interview, Ms Kelly described the consultants as "*anxious*". She made no reference to them exhibiting bullying or discriminating behaviour.[46] There was none.

22.36 Mr Millea also said in the interview that, whilst Dr Brearey had the right to raise concerns, he had failed to produce evidence regarding the allegations against Letby and that he was "*out to professionally discredit LL*".[47] Dr Green did not probe this statement and he should have done. He stated that, "*in the interview situation, that didn't cross my mind*".[48] The purpose of the interview was to elicit information about the grievance.

22.37 On 20 October 2016, Dr Green interviewed Ms Kelly, Ms Rees and Ms Sian Williams.

Ms Kelly's interview, 20 October 2016

22.38 The record of Ms Kelly's interview reads that she said: "[T]*here were no issues with the individual nurse … SB* [Dr Brearey] *conducted his own mini review of the cases and an analysis of staff on duty at the time of deaths*."[49] She did not say that the Thematic Review was carried out by multidisciplinary professionals including an external reviewer.

22.39 Dr Green gave evidence that he accepted Ms Kelly's comments as fact because they were "*triangulated with other members of the nursing leadership team and there were no pieces of information to suggest otherwise*".[50] He does not seem to have realised that there was a single source of information upon which the senior leadership nurses relied when talking about Letby: Ms Powell. This was not triangulation; it was an echo chamber. He was not to know that there was other conduct of Letby that he was not told about: for example, leaving her allocated baby and intervening repeatedly in the care of another; refusing to look after special care babies as she perceived it as boring; and insisting on looking after the unwell babies. See Chapters 3 and 5.

22.40 Ms Kelly was referred to several parts of her grievance interview with Dr Green in which she failed to state the full extent of the consultants' concerns about Letby. For example, she informed Dr Green that there was no other rationale to the consultants' concerns other than Letby being on shift. She acknowledged the "*wording of how it's articulated there probably doesn't give the full picture*".[51] Ms Kelly went on to concede: "*I should have articulated more detail around that. It is misleading*" and "*You could say potentially false. Yes*."[52] What she said was misleading. That was plainly what she intended.

22.41 Ms Kelly agreed it was also misleading that she told Dr Green the draft RCPCH report had confirmed "*nothing significant as regards Lucy*", when the RCPCH had not investigated Letby.[53] Further, she stated there were no immediate recommendations from the RCPCH report. This was not true; the RCPCH had recommended an investigatory process and a case note review. She told him Letby was only moved off the unit for her protection.[54] She accepted that this was not the true reflection of the circumstances, and that what she told Dr Green could have been more detailed.[55] It should have been more detailed and more accurate. She should have told him what she knew, in full. He should have probed her answers. If she felt the doctors' concerns were not justified, she was entitled to say so. She was not entitled to mislead.

22.42 Ms Kelly told Dr Green: "*The case will be closed when we get LL back on the unit*."[56] She agreed in evidence that it was premature to make that remark.[57] Not only was it premature, it made plain to Dr Green that she thought there was nothing in the concerns – and she had not been frank about what she knew.

22.43 Ms Kelly confirmed that Letby filed her grievance because of the way the executives redeployed Letby from the unit. On 15 October 2016, days before her grievance interview, Ms Kelly met Letby. A letter documenting the meeting records that "*Alison* [Kelly] *explained she had no concerns in you returning back to the Neonatal Unit and that we were going to plan for this with Karen* [Rees] *over the coming weeks*."[58] The grievance hearing was not until 1 December 2016. Ms Kelly conceded that being a witness in the grievance process and meeting with Letby was a "*conflict*".[59] She also acknowledged it was "*premature me having that conversation with her at that time when I knew that all of the other pieces of information hadn't been concluded*".[60] It was a clear signal to all concerned that she believed there was nothing in the concerns. This was irresponsible.

Ms Sian Williams' interview, 20 October 2016

22.44 Ms Sian Williams was also interviewed on 20 October 2016.[61] In evidence, she reflected that, although Dr Green did not ask her, she should have made it clear that the consultants held genuine concerns.[62] She stated that she personally had never heard a doctor say anything along the lines of: "[T]*here is a murderer on the loose*."[63]

22.45 Ms Sian Williams informed Dr Green about her staffing analysis with Ms Fogarty and her conclusions. Despite Dr Green's knowledge of this, he did not ask Ms Sian Williams questions about it. When asked why not, he stated that he did not want to investigate the allegations when there was a separate review process doing this.[64] This is inconsistent with the way he approached the questioning of the consultants.

22.46 Speaking about the timing of the grievance, Ms Sian Williams recalled "*saying to Alison that I didn't feel it was appropriate and would it not be better to wait until we had all the information together because we had not got the Royal College … report*".[65] The response she received from Ms Kelly was that the Trust had taken advice and was carrying on with the grievance.[66]

Ms Rees's interview, 20 October 2016

22.47 Ms Rees was interviewed on 20 October 2016.[67] In her oral evidence, Ms Rees clarified an issue that Letby had included in her grievance: that she was told she could not contact colleagues on the neonatal unit. Ms Rees confirmed that she did not say this to Letby, and that she thought Letby had misunderstood her letter dated 18 July 2016.[68] In fact, the letter stated very clearly that Letby did not need to stop social contact with her neonatal unit colleagues.[69] Rather, she should be mindful of discussing any sensitive matters relevant to the RCPCH report. She had not been told not to contact colleagues.

22.48 Ms Rees told Dr Green: "*In the meeting with Stephen Cross it was mentioned if we call the police the unit will be shut down and people may be arrested*."[70] When asked about this, she told the Inquiry this might have been part of why she did not contact the police. Another reason she gave was because neither the consultants nor the executives had contacted the police.[71] She added: "*I have never been so relieved when they eventually did*."[72]

Ms Hodkinson's interview, 21 October 2016

22.49 Ms Hodkinson was interviewed on 21 October 2016.[73] When giving evidence to the Inquiry, she was referred to a passage of her grievance interview transcript, where she is recorded as saying: "*I am concerned with how open we were with Lucy. The reason we weren't was there being such vehement feeling without substantiation*." She expressed remorse for characterising the consultants' concerns in that way: "*I apologise … I think that it was the emotion myself at that time and that was probably inappropriate*."[74] It was.

22.50 Ms Hodkinson said in evidence that, on 20 October 2016, the day before her interview, she had told Letby that she was not under investigation;[75] also that "[w]*e felt there wasn't enough evidence to investigate formally or contact the police*".[76] She made clear to Dr Green that "[w]*e are working on early December to prepare for LL to go back to the unit*" and that she "*had tried to reassure her that we will be supporting her at the board level appeal*".[77] Like Ms Kelly before her, Ms Hodkinson was undermining the consultants' position before Dr Green had heard from them. The effect of doing this was to risk further harm to babies on the unit.

Ms Powell's interview, 28 October 2016

22.51 In Ms Powell's interview, she alleged that Nurse T had heard Dr Jayaram saying: "[S]*omebody is causing these deaths on this unit*."[78] Dr Green accepted that he did not interrogate the substance of this allegation with Ms Powell, and his reason for this was that he was not there to investigate the consultants.[79] At the same time, he had been asked to investigate the name-calling on the unit. This fell into that category. He accepted Ms Powell's evidence (which was at one remove from the source). This led him into error.

22.52 In oral evidence, Nurse T denied that Dr Jayaram had said this. She explained she had a conversation with Dr Jayaram in which she had said that the RCPCH's preliminary review did not find anything untoward in the deaths, and Dr Jayaram had responded something

along the lines of: "[J]*ust because they didn't find something doesn't mean there isn't something to find*."[80] Dr Jayaram agreed that this was a comment he "*probably*" made to Nurse T.[81]

22.53 Ms Powell spoke highly of Letby in her grievance interview. She did not raise any concerns about Letby's behaviour, the morphine error made by Letby in 2013 or complaints made about her by colleagues. Ms Powell is recorded as saying that she believed Letby "*is 100% innocent*".[82] Her interview evidenced her antipathy towards, and lack of respect for, consultants, with comments such as "[Dr Brearey and Dr Jayaram] *brainwashed other consultants*" and "*equality doesn't run both ways*".[83]

22.54 Ms Powell was asked about her position that there was nothing in the allegations against Letby. She was asked: "*What do you think about that now*?" Ms Powell said: "*Well, looking back at it, it looks obvious … with hindsight, it looks obvious*."[84]

22.55 Ms Powell was taken to a passage in her grievance interview where she alleges Dr McCormack had said to her: "*You are harbouring a murderer.*" She maintained in evidence that this was said to her.[85]

22.56 On 28 October 2016, Ms Powell sent Dr Green an email in which she wrote: "*Hi Chris, This is the article and email that I was alluding to in our discussion*."[86] Below her email was the email chain between the consultants and Ms Powell, in which Dr Jayaram sent a link to an article regarding air embolism. Dr Green permitting Ms Powell to send him a document that was discussed in their interview is in marked contrast to the way he responded when Dr Brearey offered to send him a copy of the Thematic Review. Dr Green did not accept Dr Brearey's offer[87] (see paragraph 22.69).

Mr Harvey's interview, 7 November 2016

22.57 Dr Green conducted the remainder of his grievance investigation interviews in the first half of November 2016. Mr Harvey was interviewed on 7 November 2016.[88] In oral evidence, Mr Harvey stated that he understood that Letby's grievance concerned how the Trust had managed Letby off the unit and the information (or lack thereof) that she had been provided with.[89] Despite this, for much of his interview, Mr Harvey discussed the consultants. When asked why this was, Mr Harvey said his answers were influenced by the questions asked, and "*I, with regret, just answered the questions*".[90]

22.58 During the grievance interview, Mr Harvey told Dr Green:

> "*There has been a number of behaviours on the ward that do not reflect too well. I had to go and speak to RJ* [Dr Jayaram] *that some of the trainees had been making reference to 'Angel of Death', but no specific person was named. There was behaviour in clinic being heard, talking about killing babies on the unit. I had to speak to Ravi about comments about killing babies. This was not denied and RJ did accept that it was inappropriate*."[91]

22.59 Mr Harvey said to Dr Green that if the allegations were made against a doctor, there would be a different process; that the doctor would be placed on supervised practice. However, this was not an option for Letby because "*there was a block to that as the Consultants were not prepared to have the nurse on the unit, and if we do, they said the police will be called*".[92] Mr Harvey conceded in oral evidence: "*I would accept that I have*

put in a factual inaccuracy in saying there was a block by the Consultants. I did not at the time understand that, in fact, there couldn't be supervised practice because the nursing staff didn't have the numbers to be able to support that as an option."[93]

22.60 What Mr Harvey said about the consultants putting a block on supervised practice was not true. The consultants had agreed to it. The plan was abandoned before it started because of a lack of staff to supervise. Mr Chambers, in a Board of Directors meeting on 10 January 2017, made a similar comment, stating: "[S]*upervised practice for the individual … was not supported by clinical colleagues*."[94] This was not true.

22.61 Mr Harvey was asked in the Inquiry whether he accepted that some of his comments in the grievance interview were used against the doctors and ultimately led to outcomes such as an apology and mediation. Mr Harvey responded: "*I accept, as Medical Director, that might carry a degree of weight, but that was never my intention*."[95] As I have said elsewhere in the Report, Mr Harvey said what he wanted to say. It is not easy to see what his intention was other than to undermine the doctors when asserting that the doctors had blocked the supervised practice. Had he checked, he would have found out that they had not. It was not true. This assertion had already found its way into the draft RCPCH report.[96]

Dr Jayaram's interview, 11 November 2016

22.62 Dr Jayaram was interviewed on 11 November 2016.[97] He told the Inquiry that he was surprised and anxious when he received a letter from Dr Green inviting him to an investigative meeting for Letby's grievance.[98] The letter stated that the information Dr Jayaram provided in the interview "*may be presented in a Disciplinary Hearing, should this be necessary*".[99] The letter also said that he could be accompanied by a representative. Dr Jayaram contacted the BMA and Mr Cross for advice, because he was worried that if he raised concerns of deliberate harm he may be putting himself at risk. He was advised by his BMA representative not to say anything speculative in the grievance interview.[100]

22.63 The interview transcript demonstrates that Dr Jayaram explained to Dr Green that there had been an increase in neonatal mortality, that the babies who had died were unexpected deaths, and there did not appear to be anything in terms of clinical practice, equipment or the environment that was relevant to the deaths. Further, there was an association that Letby was either looking after the babies or was present at the times of death.[101] In evidence, Dr Green sought to point out that Dr Jayaram did not explicitly tell him that Letby had deliberately harmed babies. In particular, he stated that Dr Jayaram could have, but did not, mention what he had witnessed in relation to Letby and Baby K.[102] Dr Jayaram explained in his oral evidence why he did not mention his concerns explicitly: "[T]*hese are very sensitive issues and I didn't feel, on the one hand, that Chris Green needed to know and number two, again, because of my concern that explicitly stating my concern in this forum could be used against me*."[103]

22.64 Despite a number of witnesses telling Dr Green that doctors had called Letby names, Dr Jayaram said Dr Green did not ask him about this. Dr Jayaram denied calling Letby a "*baby killer*" or "*Angel of Death*". I am satisfied that he did not. As I have noted above,

he accepted that he may have had a conversation with a colleague about the findings in the RCPCH report: "*I won't deny I probably made a comment around the fact that even if the report doesn't find anything it doesn't mean there isn't anything*."[104]

22.65 Speaking on the grievance generally, Dr Jayaram's view was that "*the desired outcomes of this grievance process -- had possibly been predetermined*".[105]

Dr Brearey's interview, 11 November 2016

22.66 Dr Brearey was also interviewed on 11 November 2016. As with Dr Jayaram, on 19 October 2016, Dr Brearey had received a letter from Dr Green inviting him to a grievance interview.[106] The letter set out he could bring a trade union representative to the interview and that any information provided to Dr Green may ultimately be presented in a disciplinary hearing should it be necessary. Dr Brearey stated upon receiving the letter that he was unsure what the grievance procedure was for, and he felt "*worried*" that the consultants raising concerns about Letby "*seemed to be turning into a narrative against us rather than concentrating on the cause of the deaths*".[107] That was correct.

22.67 Dr Brearey told the Inquiry that, around the time of the grievance, "[T]*here was an escalating amount of pressure I felt along with Dr Jayaram that yes, it was -- it felt intimidating*."[108] The narrative surrounding the grievance process was that the consultants were being "*unreasonable*".[109] He said: "[W]*e were in a grievance procedure where we were made to feel as though we were on trial*."[110]

22.68 In oral evidence, Dr Green said he had reasonably good soft skills.[111] Despite having these skills and knowledge of the background to the consultants' concerns, he failed to detect Dr Jayaram's and Dr Brearey's difficult position and why they may not be forthcoming. He told the Inquiry that he found the consultants' answers to be "*evasive, non-committal*".[112] Further, he felt "*frustrated*" by Dr Brearey's responses to his questions in the grievance interview and said that he "*wasn't getting open and honest answers*".[113] Dr Green accepted it was a "*fair comment*" that there was a mismatch between what he was frustrated by and what he was tasked with investigating in the scope of the grievance.[114]

22.69 Dr Brearey told the Inquiry: "[T]*he myth that we were bashed with during the grievance procedure,* [was] *that we are somehow withholding information*."[115] Dr Brearey said that all the information about their concerns was in the Thematic Review, which he offered to send Dr Green at the interview.[116] Dr Green accepted in his oral evidence that he did not take up Dr Brearey's offer.[117] This was different from the approach he took with Ms Powell.

22.70 Ironically, the meeting of the multidisciplinary team who were conducting the Thematic Review records Dr Green's apologies.[118] When the Thematic Review was finished, Dr Brearey sent a copy to Dr Green. Dr Green did not remember it.[119] There is no reason to think that Dr Green did not receive the Thematic Review. This was another reason why he was unsuitable to investigate the grievance. On the face of it, he was in possession of detailed information from Dr Brearey about important matters relating to the deaths of babies on the neonatal unit, and he had not read it.

22.71 To make matters worse, at the time of the interviews Dr Green misinterpreted why Dr Jayaram and Dr Brearey attended with a representative. Despite the grievance invitation letters clearly stating they were entitled to be accompanied by a representative, Dr Green told the Inquiry he had thought they did this because "*they might think that they have a problem that they need support with in terms of their behaviour*".[120] That answer revealed, again, a partisan approach. It did not seem to occur to Dr Green that he was being unfair.

22.72 Towards the end of Dr Green's evidence, he stated that he wished he had probed Dr Jayaram and Dr Brearey about why they were not sharing the full extent of their concerns with him, and he now recognised that this was partly because of an agreed approach with their trade union representatives.[121]

22.73 Whilst Dr Green now understood why a whistleblower would find the grievance process intimidating, at the time of the investigative meetings he said: "[I]*t did not cross my mind that if they said in those meetings that they had concerns about* [Letby] *they would lose their jobs*."[122]

22.74 Not only was Dr Green not independent, his reflections made it clear that he was not suitably skilled or experienced to investigate Letby's grievance. Dr Green himself conceded that he was not equipped for the task. He said: "[I]*f ever this situation arises again there should be some specialist input to deal with it, not, not a local manager who thinks he's doing the right thing*."[123]

Dr Green's draft investigation report

22.75 Dr Green produced a draft investigation report dated 12 November 2016. He concluded:

a. The Trust genuinely intended to place Letby on a period of supervision of practice and repetition of competencies.

b. Although no such instruction was given to her, Letby felt that she was not permitted to have normal social contact with her colleagues on the neonatal unit.

c. The purpose of the reviews undertaken by the RCPCH and Dr Hawdon was to: "*explore circumstances and detail around patient safety on the Neonatal unit. With respect to the commonality identified between LL being on duty and the collapses/ deaths of the babies on the Neonatal unit, I conclude that the Execs team feel that the Review will provide confirmation and reassurance that there is no direct link between the two*"[124] (see Chapters 20 and 21).

d. The Trust had not been open and honest with Letby regarding the nature of the consultants' "*informal allegations regarding her*".[125]

22.76 Dr Green also wrote: "*The drive to remove LL from the neonatal unit appears to have come from the Consultant SB* [Dr Brearey] *and to a lesser extent RJ* [Dr Jayaram]*. The concept of air embolism also appears to have originated from the Consultant body although this is denied*."[126]

22.77 Significantly, in a section headed 'Recommendations', Dr Green wrote that, pending the outcome of the "*final reports*", Letby should be "*given the opportunity to return to NNU*";[127] he also wrote that the allegations made against Letby, which led to her "*suspension*", warranted "*further investigation, possibly under the Trust's Disciplinary policy and/or under the Trust's Bullying and Harassment policy*".[128] This was a suggestion that the doctors should be investigated under those policies.

22.78 On 16 November 2016, between the draft version and final version of his investigation report, Dr Green met Mr Cross. Dr Green could not recall the meeting but agreed it took place, as there was an entry in his calendar.[129] Mr Cross's note of the meeting with Dr Green refers to:

> "*Pressure on Execs. 'threatened' to go to Police (by Consultants) … Consultants say 'no issue re' Police being called. Not them. Denied any knowledge that they had wanted Police. 'Accusations that Lucy had harmed babies.' Disputed Emails from Eirian – marked 'confidential' – refer to foul play – 'harm'. Advice from Beechcrofts*."[130]

Dr Green gave evidence that "*there was nothing that happened at that meeting that was a red flag to me or crossed any lines, red lines in terms of, you know, how the grievance was being conducted or the outcome or anything like that*".[131]

22.79 Due to being seriously ill, Mr Cross did not give oral evidence.

22.80 In his Inquiry statement, Mr Cross commented on this meeting with Dr Green. He described the meeting as "*informal and as such was not pre-arranged and not formally recorded. The Director of Pharmacy came to see me and I felt that he wanted reassurance as he felt that he was in a difficult position and that I would be a listening ear*."[132] Mr Cross continued: "*I expect that I would have suggested to the Director of Pharmacy that he await the advice from the external legal advisors*."[133] While Mr Cross said the meeting was not formally recorded, he did record it, and the notes speak for themselves. It appears that the two men discussed the issues in the grievance. This inevitably gives the impression of Mr Cross having the opportunity to influence the outcome. It should not have happened. Some of the matters discussed – for example, the allegation that the consultants had threatened to go to the police – were not accurate.

Further legal advice (DAC Beachcroft)

22.81 As the grievance investigation interviews continued, Ms Hodkinson consulted Mr Pace. On 28 October 2016, they had a phone call. The file note shows that they discussed the RCPCH review, the draft RCPCH report and the further forensic review that was going to be undertaken of 13 neonatal cases.[134] Mr Pace advised that Letby continue to be redeployed away from the neonatal unit because his "*ultimate concern was the potential for patient safety*".[135] This was not conveyed to Dr Green. It was a cast-iron reason for the redeployment, about which Letby had brought the grievance, but the executives failed even to mention patient safety.

22.82 On 18 November 2016, Ms Hodkinson spoke again with Mr Pace from DAC Beachcroft. Mr Pace's file note sets out that she told him: "*The external and internal reviews have both been completed and there is nothing to implicate* [Letby] *in any of the events. The board has decided that she should return to the neonatal unit*."[136]

22.83 Mr Pace is recorded as saying that this "*all sounded very positive, and we need to take steps to ensure that proper steps are made to reintegrate her back into the workplace*".[137] He is also noted as explaining that the consultant had raised whistleblowing concerns that needed to be investigated. In Mr Pace's record, Ms Hodkinson replied: "[T]*he Trust had taken the view that the internal and external investigations have been sufficient and that we would provide the outcome of these investigations to the consultant who had raised a whistleblowing concern*."[138]

22.84 Mr Pace informed the Inquiry that he advised the Trust, on a number of occasions, to deal with the consultants' concerns under its Speak Out Safely Policy. He confirmed the consultants did not need to put their concerns in writing to trigger the policy. He explained the value of managing the concerns under the policy was that "*an investigation could be undertaken to consider those concerns and get more detail*".[139]

22.85 Mr Pace also said Ms Appleton-Cairns and Ms Hodkinson had expressed concerns to him about the behaviour of some of the consultants, and that they were considering engineering the removal of a consultant from the neonatal unit. The fact that Ms Hodkinson was considering using a restructure as a pretext to dismiss Dr Brearey is specifically recorded in Mr Pace's file note.[140] This concerned Mr Pace for two reasons:

> "*I was also concerned by the … possibility that there is some sort of restructure or engineering should happen on the grounds of whistleblowing legislation. The concerns that had been raised by the Consultants no doubt would amount to Protected Disclosures under the Protected Disclosure legislation and any detriment or dismissal as a consequence would have resulted no doubt in a successful claim for the tribunal.*"[141]

22.86 Ms Hodkinson confirmed that she was aware that Dr Brearey may have been entitled to whistleblowing protection: "*I think the point that Ian* [Pace] *was saying was that because Dr Brearey had raised those clinical concerns … that was a protected disclosure from a whistleblowing perspective*."[142] Mr Pace's advice and Ms Hodkinson's concession make it even more striking that the Speak Out Safely Policy was not deployed, and that the consultants were deprived of protection, for so long.

Dr Green's final draft and recommendations

22.87 On 22 November 2016, Dr Green produced the final draft of his investigation report, with input from the HR specialist Ms Sementa.[143] This largely mirrored the conclusions in his draft report of 12 November 2016, save that the section dealing with the allegations against Letby was rewritten, including that the phrase "*informal allegations*" was replaced by "*accusations*". The key parts explored in evidence with Dr Green are set out below:

> "*LL's question: 'I would like to know exactly what I have been accused of/what allegations have been made and by who and how the Trust has dealt with this?'*"[144]

> "*No party refutes that concerns were raised by the Consultants in particular SB* [Dr Brearey] *to the Executive Team around a perceived commonality between LL's presence on the NNU* [neonatal unit] *and the collapse/deaths of babies. I acknowledge that these concerns were raised through the appropriate channels in line with both the Trust's Speak Out Safely policy and the guidance proffered by the GMC through the Executive Team. I do not find that the Consultants' concerns when reiterated to the executive team were clear, honest and objective*."[145]

22.88 Dr Green conceded that, despite making the finding that the consultants' concerns, when reiterated to the Executive Team, were not clear, honest and objective, he had not asked Dr Brearey or Dr Jayaram (or anyone else) whether they had been open and honest with the Executive Team.[146] There was no evidence from anywhere that the doctors were holding anything back from the Executive Team or that they were not being open and honest. This finding seemed to be more about the fact that Dr Brearey and Dr Jayaram were reticent when speaking to Dr Green and Ms Sementa, who were frustrated that the two paediatricians did not tell them more. The Executive Team knew what the concerns were. Dr Green said in evidence that this was "*one of the key parts of the document that I look back on now with some regret*".[147] He could not recall whether it was he or Ms Sementa who added this section, but he said: "[M]*y name's on the document so I'm responsible for it*."[148] It is likely that it was Ms Sementa who raised it. She had read the GMC guidance and thought it likely that she had raised it.[149] Both Dr Green and she must have realised what a serious finding this was. It was not justified.

22.89 Ms Kelly confirmed that at this stage the concerns had not been logged and considered under the Speak Out Safely Policy. Ms Kelly could not recall speaking to Dr Green about the Speak Out Safely Policy in her grievance interview, and she did not know why Dr Green had made that finding. She stated that she had read Dr Green's report, but not until some time later. She did not correct the error.[150] Nor did Ms Sementa, who should have ensured that this was correct. It was her role to support Dr Green.

22.90 Dr Green recorded Ms Griffiths saying in her interview that she considered there was a "*witch hunt*" by the consultants and that the doctors had threatened to go to the police. It was not true that the doctors had threatened to do so.[151]

22.91 An additional finding that Dr Green made was in response to Letby's question "*to be informed of any evidence the Trust may have and the process which they have followed*". His finding was:

> "*During the course of this investigation I have not been made aware, nor has there been any allusion to, any evidence relating to any wrongdoing – alleged wrongdoing by LL. There has been repeated reference to a commonality between the dates and times that LL was on duty and the collapse/deaths of a significant number of babies but there is nothing to support that there is additional information or data beyond this that has not been shared with LL*."[152]

22.92 Dr Green confirmed that that was his view at the time he wrote his final report.[153] Dr Jayaram had told Dr Green and Ms Sementa in plain terms that there was a spike in deaths; that there was a commonality with Letby on shift; that most of the deaths were occurring at night; that when she was taken off nights the deaths at nights stopped (and moved to days); and that, although very young babies did die, the babies who were dying were not those where death would be expected. When challenged about that evidence, Ms Sementa asserted, eventually, that this was not evidence as she would recognise it. This is a rather troubling view of evidence from an HR professional. In any event, she defended her approach on the basis that the appendices contained everything that was said, and Ms Weatherley could have found it if she had read them.[154] This was an unfortunate observation.

22.93 Finally, after a very long exchange, Ms Sementa accepted that there were safeguarding issues here and she should have spoken up.[155]

22.94 In summary, Dr Green's recommendations were that:

a. Letby be given the opportunity to return to the neonatal unit in her substantive role, and that this "*should be managed in tandem with the final reports commissioned by the Trust*".

b. If Letby wished for more detail to be explored with regard to the specifics around Dr Brearey and Dr Jayaram and the concerns they raised about her, then she should submit a separate Bullying and Harassment complaint in line with the Trust policy, and an investigation should be undertaken.

c. The evidence gathered from the grievance "*raised significant concerns around the behaviours reported to have been exhibited by SB* [Dr Brearey] *and RJ* [Dr Jayaram] *… I recommend that the Trust takes action to explore this in more detail and to investigate if required, in line with the Trust Disciplinary Policy*."

d. "[T]*he Executive Team should have done more to communicate with LL and could have been more open and honest about why LL had been redeployed, however, their intentions around this appear positively directed*."[156]

Grievance hearing

22.95 Letby's grievance was heard on 1 December 2016.[157] The hearing manager was Ms Weatherley. At the time, Ms Kelly thought the appointment was appropriate. On reflection, she considered that it should not have been a doctor or nurse who was appointed, due to the tension between the two professional groups.[158] Ms Hodkinson did not consider Ms Weatherley's appointment as Chair was inappropriate.[159]

22.96 Given the level of tension between the doctors, the senior nurses and the managers, it was important that the grievance be heard by a person who was independent and was seen to be independent. Whilst it would not always be the case that a senior nurse would not be considered independent, there were particular problems here, which meant that someone wholly independent should have been selected.

22.97 In the event, Ms Weatherley's decision-making was compromised from an early stage. On the morning of the grievance hearing, Ms Weatherley met Ms Appleton-Cairns. Ms Weatherley told the Inquiry that in the meeting, Ms Appleton-Cairns "*asked me what my thoughts were. I said it felt like a witch hunt. She said yes, we all feel the same*."[160] Ms Appleton-Cairns, meanwhile, stated that she had no recollection of their discussing a witch hunt against Letby.[161] I accept Ms Weatherley's evidence about this. She had no reason to fabricate the comments made, which were inappropriate and ill-judged by each of them.

22.98 In the grievance hearing transcript, Dr Green is recorded as saying, about Dr Brearey and Dr Jayaram, "*I was disgusted by their behaviour. It's likely that they lied*."[162] In oral evidence, Dr Green said: "*I am deeply embarrassed that I said that. I really wish I hadn't. That said, I was very frustrated with their behaviour during the investigative interviews*."[163] He described it as an "*off-the-cuff comment*" that was "*borne out of some frustration*".[164] He agreed that it was inappropriate for him to make a comment of that nature in what

should have been a formal discussion.[165] More to the point, he had no basis for saying it was likely that they had lied about anything. It was not just inappropriate. It was offensive and wrong.

22.99 In addition to the informal conversation about a witch hunt in advance of the grievance hearing, the independence of the grievance hearing itself, and the outcome, was compromised by the involvement of others. Although Ms Weatherley was the Chair, she stated: "*The panel was myself and Dee … We collectively were the panel*."[166] Notwithstanding this description of the panel, Ms Weatherley said she maintained her independence, because she was the person responsible for deciding the outcome. However, when it was suggested that her decision-making could have been influenced by Ms Appleton-Cairns, as they had had conversations about the grievance outside of the hearing room, she accepted this as a possibility: "*I guess it's influenced if the hearing manager is prepared to be influenced*."[167] This overlooks the likelihood of confirmation bias and unconscious bias. It appears that Ms Appleton-Cairns and Ms Weatherley were in agreement before the hearing and throughout. It is impossible to tell the extent to which – if at all – the one was influenced by the other.

22.100 Ms Weatherley produced a written outcome of the grievance on 1 December 2016 (the same day as the grievance hearing).[168] Ms Appleton-Cairns confirmed that she was involved in the drafting of the grievance outcome. She stated it was "*standard practice*" for an HR professional to assist the Chair in this way.[169] She told the Inquiry that she typed the grievance outcome and sent it to Ms Weatherley to then make any changes deemed necessary.[170] She said it is usual for the document to go back and forth between the Chair and HR member "*two or three times*", and that the Chair would then have the final sign-off. [171] Ms Weatherley confirmed that Ms Appleton-Cairns was involved in the drafting, but that the outcome was hers: "*When the grievance had concluded, we sat and wrote together what I wanted the outcome to be*."[172]

22.101 Professor Bowers KC's expert opinion was that HR support for the Chair of the grievance/decision-maker can involve reviewing the report to check that it does not contain any factual errors. However, HR should not be involved in the actual decision-making.[173] I accept his view about that.

22.102 In addition to Ms Appleton-Cairns, Ms Kelly and Ms Hodkinson were involved in finalising the grievance outcome letter. Ms Appleton-Cairns confirmed that she sent a draft version of the grievance outcome to Ms Kelly and Ms Hodkinson. She said that both asked to see a draft copy.[174]

22.103 The draft did not include Dr Green's view that the consultants had not been clear, honest and objective. When Ms Kelly received it, she raised whether Dr Green's conclusion about the consultants not being clear, honest and objective should be included. Ms Appleton-Cairns gave evidence that, irrespective of Ms Kelly's view, it was going to be added anyway.[175] Ms Appleton-Cairns was asked whether it was appropriate to involve the executives in the drafting of the outcome. She replied: "*Should I have sent it on reflection? No probably I shouldn't of, because I think you are right, I think that they thought oh … I think I can add something in and it was like, well, no, you can't*."[176] The drafts should not have gone to the executives.

22.104 Ms Kelly recalled being sent a copy of the draft outcome letter. However, she did not have a recollection of commenting on it. She agreed that, if she had, it would be inappropriate given she was a witness and one of the people against whom the grievance had been made out.[177] I find that she did ask that the finding by Dr Green about the consultants be included.

22.105 Ms Hodkinson confirmed that Ms Weatherley's grievance outcome letter was shared with her. She said she made no changes to the document.[178] Despite knowing that the consultants' concerns had not been managed under the Speak Out Safely Policy, Ms Hodkinson did not challenge Ms Weatherley's conclusion that they had.[179] This assertion was the result of Ms Sementa's advice. As I have explained already, there was no basis for saying that the Speak Out Safely Policy had been applied. It should not have appeared in the decision.

22.106 Ms Weatherley concluded in her outcome letter that Letby could have been supervised and her competencies repeated, as she had been told was the plan on 14 July 2016; that the Trust's intention was that Letby cease professional, rather than social, contact with members of the neonatal unit, and miscommunication had resulted in Letby thinking that she was required to cease both; that the remit of the external reviews was not explained to Letby; and that the Trust generally had not been "*as open and honest with* [Letby] *as they could be*".[180]

22.107 The first of these is unsustainable. Ms Weatherley seems to have been unaware that the plan for supervision was abandoned because there were not the staff available to supervise. The second conclusion was correct in part: the Trust's intention was that professional contact between Letby and the neonatal unit should cease. There was no miscommunication. The letter was clear. The third conclusion was correct: the remit of the external review was not explained, and the Trust had not been open and honest with Letby.

22.108 Ms Weatherley went on to conclude: "*I have not seen, nor has there been any allusion to, any evidence relating to any alleged wrongdoing by yourself*." Although reference was made to "*commonality between the dates and times that you were on duty and the collapse/deaths of a significant number of babies … there is nothing to support that there is additional information or data beyond this, that has not been shared with you*".[181]

22.109 Ms Weatherley adopted and quoted extensively from Dr Green's report in respect of the allegations made by Dr Brearey and Dr Jayaram. In particular, that their concerns were not "*clear, honest and objective*".[182] This language was copied from the investigation report. As I have said earlier, it was unjustified.

Ms Weatherley's determination of the next steps

22.110 Ms Weatherley concluded that a number of steps would be taken. First, the CEO and a Non-Executive representative would apologise to Letby in the presence of her parents.[183] Ms Weatherley explained:

> "[T]*he apology was in respect to how she had been treated. The Trust hadn't followed their own policies … which would have been back in 2015 to suspend and then obviously pending investigation.*
>
> *But they hadn't communicated with her. She didn't know what was going on and it was as if they were managing her against the disciplinary policy but they had not actually told her that.*
>
> *So the redeployment she wasn't clear why she was being redeployed. The information that she would redo her competencies and be supervised but nobody else was and she didn't understand why that was, so I think … the whole situation was chaotic and they didn't manage her very well at all and that's why I felt that they needed to apologise for that*."[184]

22.111 Second, after the "*final report*" was received, and provided it contained "*no references made to you* [Letby]", Letby would be informed, in writing, that she had "*no case to answer*".[185] This was a surprising finding, which Ms Weatherley was in no position to make having read only the grievance papers.

22.112 Ms Weatherley explained that the provisional RCPCH report, which she understood did not make any findings about Letby, was a "*significant*" factor in her conclusion that Letby had no case to answer and mediation was required.[186] Ms Kelly agreed that she had told Dr Green that the draft RCPCH report "*confirmed nothing significant as regards Lucy*".[187] Ms Weatherley did not read the RCPCH report; she did not know what the RCPCH's Terms of Reference were, nor what the RCPCH had found. She was not in a position to find that Letby had no case to answer.

22.113 Third, mediation would take place with Dr Brearey and Dr Jayaram, and there would also be an "*apology from both consultants*".[188] Ms Weatherley explained that the consultants' apology was "*in respect of them calling her names without material evidence, when they knew that she wasn't aware of what they were saying or what was going on*".[189] Ms Weatherley was taken through the grievance interview transcripts and shown that there was no evidence that either Dr Brearey or Dr Jayaram had called Letby names. Despite this, she stated: "*I take what is in front of me as the truth*."[190] This is an extraordinary remark. If that really was her approach, then she was unsuitable as a reviewer of a grievance process. That she considered her response to the question reasonable in light of the evidence before her demonstrates unfairness and lack of insight into her own shortcomings.

22.114 In the outcome letter, Ms Weatherley added: "*Failure to comply with these reasonable measures from the named parties, should result in disciplinary action taken by the Trust*."[191] This was unreasonable and ill judged. Dr Jayaram and Dr Brearey were to apologise for name-calling mistakenly attributed to them, which, to the extent that it occurred at all, should have been attributed to the junior doctors. Dr Jayaram had spoken to the junior doctors about it. As to Dr Brearey, I can find no reason for his being chosen other than that he had raised concerns about Letby.

22.115 To take the worst example, Dr McCormack, who knew nothing about any of this, was to be required to apologise for making a remark attributed to him by Ms Powell in her grievance interview (see also Chapter 14). Dr McCormack told the Inquiry: "*I had no idea of the impact of what I'd said to Eirian Powell … the first time I knew that there had been any concern with what I had said was when Ian Harvey rang to ask me to apologise seven months later*."[192] He explained:

> "[I]*t had been disclosed in the HR* [i.e. the grievance] *report from Annette Weatherley that I had called -- it had moved then to say I had called her a murderer. I didn't call her a murderer. I didn't know who she was. And I said this to Ian Harvey. I said, 'Look Ian, I'm very -- this -- this -- this isn't right. You know, that's down in an official document now. I've got a HR document with that in it and I'm a bit unhappy about that.'*"[193]

22.116 Dr McCormack said he told Mr Harvey that "*I shouldn't really have to apologise*" because he did not make the alleged comment.[194] He said Mr Harvey informed him that "*the chairman wants – needs an apology from all those involved to address the issue and that I hadn't* [a] *choice because the paediatricians had already replied*"[195] and that "*Tony Chambers had insisted that an apology letter was written*".[196]

22.117 After his conversation with Mr Harvey, Dr McCormack spoke with Dr Jayaram, who showed him the consultant paediatricians' letter. He decided to write his own letter, as he was not a consultant paediatrician. Dr McCormack stated that he "*was very careful*" with the words he used in his apology letter "*because I didn't actually do it*".[197] He added: "*I was surprised that* [Letby] *accepted it when I -- when I was writing the letter because I was really saying nothing. I didn't actually apologise for calling her a murderer.*"[198]

22.118 Dr McCormack described being asked to write the apology letter as

> "*extremely disappointing, and I said to Ian Harvey at the time -- and I actually specifically asked Ian that, you know, of all the departments* [HR] *in the hospital to be producing the report and -- and not to have confirmed from a person, 'Is that what you said?' Or, 'Are there any truths to it?' I -- I couldn't understand it.*"[199]

He added: "[T]*here seemed to be something amiss with me that … I hadn't got an opportunity to -- to discuss the situation and the sequence* [in] *which the remark was made*."[200] He was entitled to complain about the finding that had been made against him in the grievance process when he had not even been given the opportunity to comment. This was a further failing in the grievance process. That he was required to apologise in writing to Letby was ridiculous. The reason this was required of him was because Mr Chambers had insisted upon it. Mr Chambers had insisted upon it because Letby had insisted upon it.[201]

22.119 It is quite clear that Ms Weatherley read the papers far too quickly, came to a view before she began the hearing, and offered her opinion that this was a witch hunt as soon as she was asked what she thought.

22.120 Ms Weatherley may not have read the interviews carefully. She rubber-stamped the investigator's findings without question. She then made recommendations as to what the doctors should do, that were unjustified, unfair and offensive. That she recommended disciplinary action should they refuse to comply with her recommendations makes it clear that she did not consider the Speak Out Safely Policy.

Had she done so, she would have appreciated that Dr Green's report said the concerns had been considered under the Speak Out Safely Policy when there was no evidence at all to that effect.

Post-grievance

22.121 In accordance with Ms Weatherley's recommendation, a meeting was held on 22 December 2016 with Letby and her parents, Mr and Mrs Letby, Ms Cooper, Ms Rees, Mr Chambers, Mr Harvey, Ms Kelly and Ms Hodkinson.[202] A note of the meeting sets out that Mr Harvey told Letby and her parents that the consultants' behaviour was not appropriate.[203]

22.122 Mr Harvey did not mention that the consultants held genuine concerns that Letby had caused deliberate harm. Mr Harvey was asked by the Inquiry why he was critical of the consultants in the meeting. He responded that it was a "*difficult meeting*", in which he reported the findings of the grievance and was "*influenced by being in a meeting that I hadn't anticipated with Letby and her parents*".[204] This could not justify his failure to say that the consultants had genuine concerns about Letby, and it allowed Letby and her parents to control the meeting.

22.123 Mr Chambers conveyed his support for Letby and his commitment to her returning to the unit. Set out below are the notes of what he said in the meeting of 22 December 2016[205] and his evidence about it in the Inquiry hearing:

a. "*The explanation that the only reasonable cause was mischievous behaviour, but we never accepted this*." Mr Chambers accepted he possibly said this.[206] I find that he did say it.

b. "*We are within our rights to phone the police but we didn't believe it*." Mr Chambers said in evidence that he had said something along the lines of: "[W]*e could have phoned the police but on balance we wanted to try and understand what the other causes might be for the unexplained increase in mortality*."[207] This convoluted sentence was his attempt to explain away the fact that he had said: "[W]*e didn't believe it*." I am satisfied that he did say it, not least because that was what he wanted Letby and her parents to believe.

c. "*Our commitment is now to meet with the consultants to get you back on the unit and meet with you again in the future*." Mr Chambers gave evidence that he made these comments in the spirit of the grievance recommendation outcomes. He accepted "*I was not able to give that commitment*" because the actions from all the reports were not finalised.[208] Mr Chambers' aim was to please Letby and her parents.

d. "*We had unexpected deaths. We have received an explanation by expert reviews*." Mr Chambers admitted this was not correct.[209] He denied trying to trivialise the concerns. He explained he was "*trying to take the heat out of what was increasingly a difficult relationship with her father*".[210] I accept that he was trying to take the heat out of the relationship with Letby's father, but there was no need to do so by telling lies about the outcome of the reviews.

22.124 Mr Chambers provided insight into the pressure he felt from Letby's father during this period: "*Letby's father was very angry. He was making threats. He was making threats that would have just made an already difficult situation even worse by threatening GMC referrals for the doctors*."[211] Mr Chambers conceded that "*the handling of this meeting was perhaps not as good as it could be*".[212] The meeting was very badly handled indeed. Mr Chambers was in charge. It was his responsibility.

22.125 Mr Letby is noted as saying in this meeting that Dr Brearey and Dr Jayaram should be dismissed. Ms Hodkinson told the Inquiry that "*he was adamant about the feeling for the doctors. He was wanting to refer them to the GMC*."[213] The Countess's Speak Out Safely Policy made plain that "*no recriminations will follow reports which are made in good faith about low standards of care or possible abuses*". Staff were encouraged to raise concerns with their line manager. If staff felt unable to do so, the policy listed others to whom staff could raise concerns,* including designated officers Mr Harvey, Ms Kelly and Ms Hodkinson. In contradiction to the Speak Out Safely Policy, in this meeting Ms Hodkinson offered to provide Letby's parents with the hospital disciplinary policy.[214] This undermines the executives' assertion that they accepted that the doctors were acting in good faith.

22.126 On 9 January 2017, Letby emailed Mr Harvey directly. She copied in Ms Cooper and Ms Rees, who was clearly the conduit for the information referred to by Letby:

> "*Dear Ian,*
>
> *There is something that has been playing on my mind since receipt of my grievance statement that I am wondering if you could help me with.*
>
> *Karen Rees was informed that a junior doctor 'openly tabled at a meeting' when discussing the increased mortality rates … my possible connection/involvement with this.*
>
> *When Karen asked, the details of the doctor and the meeting were not provided. Is there an agenda or minutes which could be traced? I am interested to know who tabled this and who was present as they are potentially professionals that I will be working with in the future and feel that it is only fair for me to know.*
>
> *I believe the meeting took place shortly after the deaths of the 2 triplets and involved senior and junior doctors. I would appreciate any help you could offer*."[215]

22.127 Mr Harvey had no memory of this email. The Inquiry has found no evidence of a reply, nor of it being forwarded. Mr Harvey said in evidence that he considered the email inappropriate in both tone and content.[216] I agree with that assessment. It is likely that Mr Harvey did nothing about it. It is revealing that Letby felt sufficiently confident after the meeting on 22 December 2016 to contact Mr Harvey personally with this request. It supports my view that at the meeting the executives had ceded control to the Letby family.

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* Such as the Designated Officers, Staff Representative, Head of Service, Professional Manager/Lead or an Executive Director of the Trust.

Conclusion

22.128 A grievance investigation had been conducted by someone who was not equal to the task; he was inexperienced, lacked independence and was unfair in his approach. He included, in his report, reference to the Speak Out Safely Policy having been applied to the consultants, when he must have known he had heard no evidence about that, not least because there was no evidence to that effect. Ms Sementa should never have suggested it. That having been done, it was then incorporated into the investigation report when it was not true.

22.129 The Chair of the grievance was unfair in her approach, expressed a baseless view that this was a witch hunt, failed to scrutinise any aspect of the investigation report, and approved it without challenge. At the same time, the senior managers were saying that Letby had been exonerated when she had not been, as they knew. The consultants were to be forced to apologise when they had nothing to apologise for and were required to mediate. In a nutshell, the way the grievance and its consequences were handled was deplorable.

Endnotes

1 Hayley Cooper 6 November 2024 22/24 to 23/4
2 Hayley Cooper 6 November 2024 54/16-17
3 Hayley Cooper 6 November 2024 28/22 to 29/2
4 Hayley Cooper 6 November 2024 29/3-5
5 INQ0002746/4
6 INQ0002746/3-4
7 INQ0002746/3-4
8 INQ0101934; INQ0102205/1-2
9 INQ0002746/4
10 Hayley Cooper 6 November 2024 68/13-23
11 Hayley Cooper 6 November 2024 69/19
12 Dr Christopher Green 6 November 2024 191/21 to 192/4
13 Dr Christopher Green 6 November 2024 191/9 and 191/19
14 Dr Christopher Green 6 November 2024 191/16-18
15 INQ0002879/39
16 INQ0002879/3
17 INQ0002879/3
18 INQ0002879/4
19 INQ0003120/1-2
20 INQ0102274/1-2
21 INQ0015640/38
22 INQ0102274/2
23 INQ0004660/1-2
24 INQ0004660
25 Sue Hodkinson 26 November 2024 92/10 to 93/7; INQ0006265
26 INQ0015537/19
27 INQ0015640/40
28 INQ0002860/1
29 INQ0002976/2
30 INQ0003281/2
31 INQ0003281/2
32 INQ0003281/2
33 INQ0003445

34 Dr Christopher Green 6 November 2024 241/16-25
35 Dr Christopher Green 6 November 2024 188/25
36 Dr Christopher Green 6 November 2024 153/12
37 Prof. John Bowers KC 5 December 2024 86/12 to 87/1
38 INQ0052509/1
39 Dee Appleton-Cairns 5 November 2024 223/22 to 224/2
40 Alison Kelly 25 November 2024 179/25 to 180/7
41 Dr Christopher Green 6 November 2024 244/12-14
42 Prof. John Bowers KC 5 December 2024 82/20-23
43 Annette Weatherley 7 November 2024 29/2-6
44 INQ0002879/5
45 Dr Christopher Green 6 November 2024 181/19-21
46 INQ0002879/21-24
47 INQ0002879/7
48 Dr Christopher Green 6 November 2024 184/20-21
49 INQ0002879/21
50 Dr Christopher Green 6 November 2024 193/1-4
51 Alison Kelly 25 November 2024 185/22-23
52 Alison Kelly 25 November 2024 187/4-7
53 Alison Kelly 25 November 2024 187/16 to 188/11
54 INQ0002879/22
55 Alison Kelly 25 November 2024 188/12 to 191/16
56 INQ0002879/24
57 Alison Kelly 25 November 2024 195/7
58 INQ0014313/2
59 Alison Kelly 25 November 2024 197/17
60 Alison Kelly 25 November 2024 199/11-13
61 INQ0002879/29-31
62 Sian Williams 5 November 2024 69/1-14
63 Sian Williams 5 November 2024 70/10-16
64 Dr Christopher Green 6 November 2024 195/9 to 196/13
65 Sian Williams 5 November 2024 64/21-24
66 Sian Williams 5 November 2024 87/20 to 88/1
67 INQ0002879/33-36
68 Karen Rees 21 October 2024 155/20 to 157/5

69 INQ0002458/2
70 Karen Rees 21 October 2024 163/18-24
71 Karen Rees 21 October 2024 164/2-6
72 Karen Rees 21 October 2024 164/7-8
73 INQ0002879/25-27
74 Sue Hodkinson 26 November 2024 108/23 to 110/10
75 Sue Hodkinson 26 November 2024 100/3-25
76 INQ0002879/26
77 INQ0002879/26
78 INQ0002879/38
79 Dr Christopher Green 6 November 2024 197/19 to 198/24
80 Nurse T 14 October 2024 62/13-23
81 Dr Ravi Jayaram 13 November 2024 139/4-14
82 Eirian Powell 17 October 2024 209/23-25
83 Eirian Powell 17 October 2024 210/5-18
84 Eirian Powell 17 October 2024 169/12-14
85 Eirian Powell 17 October 2024 163/2-20
86 INQ0002879/83
87 Dr Christopher Green 6 November 2024 151/17-21
88 INQ0002879/9-12
89 Ian Harvey 29 November 2024 54/6-15
90 Ian Harvey 29 November 2024 69/14
91 INQ0002879/10
92 INQ0002879/10
93 Ian Harvey 29 November 2024 62/11-16
94 INQ0003237/4
95 Ian Harvey 29 November 2024 78/3-4
96 Dr Nicholas Wilson 11 November 2024 236/6-11
97 INQ0002879/47-49
98 Dr Ravi Jayaram 13 November 2024 123/19 to 124/4
99 INQ0107964/43
100 Dr Ravi Jayaram 13 November 2024 129/17-18
101 INQ0002879/47-49
102 Dr Christopher Green 6 November 2024 210/14-20
103 Dr Ravi Jayaram 13 November 2024 134/4-8

104 Dr Ravi Jayaram 13 November 2024 139/12-14
105 Dr Ravi Jayaram 13 November 2024 133/1-2
106 INQ0004349
107 Dr Stephen Brearey 19 November 2024 162/2-11
108 Dr Stephen Brearey 19 November 2024 162/15-17
109 Dr Stephen Brearey 19 November 2024 167/14
110 Dr Stephen Brearey 19 November 2024 259/21-23
111 Dr Christopher Green 6 November 2024 143/5-6
112 Dr Christopher Green 6 November 2024 214/19-20
113 Dr Christopher Green 6 November 2024 215/8-10
114 Dr Christopher Green 6 November 2024 259/8-15
115 Dr Stephen Brearey 19 November 2024 259/18-20
116 INQ0002879/52
117 Dr Christopher Green 6 November 2024 151/17-21
118 INQ0003251
119 Dr Christopher Green 6 November 2024 148/11 to 149/15
120 Dr Christopher Green 6 November 2024 205/22-24
121 Dr Christopher Green 6 November 2024 244/5-11
122 Dr Christopher Green 6 November 2024 252/17-19
123 Dr Christopher Green 6 November 2024 244/24 to 245/2
124 INQ0002879/186
125 INQ0002879/187
126 Dr Christopher Green 6 November 2024 220/21-25
127 INQ0002879/193
128 INQ0002879/194
129 Dr Christopher Green 6 November 2024 174/1-10
130 INQ0003373
131 Dr Christopher Green 6 November 2024 174/12-16
132 INQ0107707/30/para 108
133 INQ0107707/30/para 108
134 INQ0102271
135 INQ0102271; INQ0107651/18
136 INQ0102269
137 INQ0102269
138 INQ0102269

139 Ian Pace 21 November 2024 97/3-11
140 INQ0102274/1-2
141 Ian Pace 21 November 2024 99/21 to 100/4
142 Sue Hodkinson 26 November 2024 92/4-9
143 INQ0002879/208-226
144 INQ0002879/219
145 INQ0002879/221
146 Dr Christopher Green 6 November 2024 216/4-9
147 Dr Christopher Green 6 November 2024 229/24-25
148 Dr Christopher Green 6 November 2024 230/7-9
149 Lucy Sementa 6 November 2024 111/3-20
150 Alison Kelly 25 November 2024 53/15 to 56/24
151 Dr Christopher Green 6 November 2024 185/2-11
152 INQ0002879/218
153 Dr Christopher Green 6 November 2024 220/7-19
154 Lucy Sementa 6 November 2024 122/16 to 124/5
155 Lucy Sementa 6 November 2024 132/17 to 133/24
156 INQ0002879/225-226
157 INQ0003155
158 Alison Kelly 25 November 2024 181/5-9
159 Sue Hodkinson 26 November 2024 111/21 to 112/13
160 Annette Weatherley 7 November 2024 20/9-11
161 Dee Appleton-Cairns 5 November 2024 225/20 to 226/8
162 INQ0003155/17
163 Dr Christopher Green 6 November 2024 237/12-14
164 Dr Christopher Green 6 November 2024 239/25 to 240/1
165 Dr Christopher Green 6 November 2024 240/19 to 241/3
166 Annette Weatherley 7 November 2024 22/14-19
167 Annette Weatherley 7 November 2024 23/15-16
168 INQ0003611
169 Dee Appleton-Cairns 5 November 2024 227/5-17
170 Dee Appleton-Cairns 5 November 2024 227/5 and 233/14 to 234/5
171 Dee Appleton-Cairns 5 November 2024 228/1-15
172 Annette Weatherley 7 November 2024 90/2-3
173 Prof. John Bowers KC 5 December 2024 87/21 to 88/6

174 Dee Appleton-Cairns 5 November 2024 230/17-18

175 Dee Appleton-Cairns 5 November 2024 233/6 to 234/18

176 Dee Appleton-Cairns 5 November 2024 230/20 to 231/1

177 Alison Kelly 25 November 2024 200/8 to 202/16

178 Sue Hodkinson 26 November 2024 113/14-16

179 Sue Hodkinson 26 November 2024 113/6 to 114/7

180 INQ0003611

181 INQ0003611/2

182 INQ0003611/2

183 INQ0003611/3

184 Annette Weatherley 7 November 2024 78/9-24

185 INQ0003611/4

186 Annette Weatherley 7 November 2024 94/7-10

187 Alison Kelly 25 November 2024 187/7 to 188/11

188 INQ0003611/4

189 Annette Weatherley 7 November 2024 79/7-10

190 Annette Weatherley 7 November 2024 89/1

191 INQ0003611/4

192 Dr Jim McCormack 8 October 2024 53/14-20

193 Dr Jim McCormack 8 October 2024 56/14-23

194 Dr Jim McCormack 8 October 2024 57/14

195 Dr Jim McCormack 8 October 2024 59/19-22

196 Dr Jim McCormack 8 October 2024 60/20-21

197 Dr Jim McCormack 8 October 2024 59/3-5

198 Dr Jim McCormack 8 October 2024 58/23 to 59/1

199 Dr Jim McCormack 8 October 2024 61/18-23

200 Dr Jim McCormack 8 October 2024 61/25 to 62/4

201 Letby insisted on four apologies in her meeting with the executives on 6 February 2017 (INQ0014279/2).

202 INQ0003463

203 INQ0003463/3

204 Ian Harvey 29 November 2024 81/10-15

205 INQ0003463/3

206 Tony Chambers 27 November 2024 101/11-15

207 Tony Chambers 27 November 2024 102/24 to 103/4-7

208 Tony Chambers 27 November 2024 107/23 to 108/22

209 Tony Chambers 27 November 2024 106/11-14

210 Tony Chambers 27 November 2024 106/22-23

211 Tony Chambers 27 November 2024 104/13-16

212 Tony Chambers 27 November 2024 108/4-5

213 Sue Hodkinson 26 November 2024 176/21-22

214 Sue Hodkinson 26 November 2024 116/19 to 118/11

215 INQ0057499

216 Ian Harvey 29 November 2024 83/15-23

Chapter 23. January to March 2017

Extraordinary Board meeting, 10 January 2017

23.1 The Board of Directors at the Countess had agreed to meet once the RCPCH report had been received. The final report arrived on 28 November 2016. Letby's grievance had been determined on 1 December 2016.

23.2 The next Extraordinary Board meeting took place on 10 January 2017. Present were Sir Duncan Nichol, Mr Chambers, Mr Harvey, Ms Kelly, Ms Hodkinson, Mr Cross, Ms Burnett, Mr Holden, and Non-Executive Directors Mr Wilkie, Mr Oliver and Ms Hopwood. Non-Executive Directors Ms Fallon and Mr Higgins did not attend.[1]

23.3 The executives' aims for the meeting were:

- for the Board to accept the RCPCH report and its recommendations
- to gain the Board's support for the return of Letby to nursing duties
- for the Board to accept the neonatal unit remaining at Level 1
- for the Board to support the executives in drawing a line under the whole issue of suspicions about Letby, following completion of the RCPCH review and Letby's grievance.

23.4 Papers were not provided in advance of the meeting. Instead, tabled at the meeting were: (a) a paper prepared by Mr Harvey;[2] and (b) a copy of the RCPCH report.

23.5 The RCPCH report was distributed at the start and removed at the end of the meeting. Mr Oliver recalls being told that he could not keep his copy due to HR issues.[3] I infer that this was the full (confidential) report, and the minutes record Mr Chambers informing the Board during the meeting that they had the unredacted version of the report. Whichever version it was, the Non-Executive Directors were not given sufficient time to read it.[4] They were unable to say which version had been tabled. In light of that evidence, I reject Mr Chambers' assertion that the report had been sent to the Non-Executive Directors a few days before the meeting. I also note Mr Chambers' assertion to the Board, when explaining that there were two versions of the report, that the redacted version "*still answers the questions raised*".[5] The redacted version did not include the questions raised by the consultants.

23.6 The Board were not provided with a copy of the outcome of Letby's grievance procedure, nor of Dr Hawdon's report of 29 October 2016, or her subsequent review of post-mortem reports (25 November 2016).

23.7 The Board were being asked for their views and decisions on a series of very important matters. They should have been provided with all relevant papers in advance so they could prepare for the meeting and make informed decisions. Instead, they heard the views of the executives and were urged to agree with them. Sir Duncan should not have permitted this approach. It meant that the Non-Executive Directors – and the Chair – were underprepared. As a result, they were misled and went along with the executives' recommendations.

23.8 Mr Harvey gave an overview of his paper, reminding the Board that the increased rate of mortality had been looked at but no conclusions had been reached, other than that there seemed to be a link to a member of staff. That had led to a detailed review by the RCPCH, which had made a number of recommendations but nothing immediate.[6] That was not correct. The RCPCH had made two immediate recommendations on 5 September 2016, in a letter to Mr Harvey,[7] and had included them in their final report. They were: (a) the recommendation to instigate a formal HR process and investigate the issues raised about Letby; and (b) the recommendation for an in-depth review of each of the babies' deaths.

23.9 As to the recommendation to instigate an investigation into the allegations of misconduct, nothing had been done between 5 September 2016 and the January 2017 meeting. Nothing was done after this meeting either. The recommendation was ignored. Ms Langdale KC asked Mr Harvey why the allegations of misconduct against Letby were not investigated in accordance with the first recommendation in the RCPCH letter to him. Mr Harvey responded that it "*wasn't explicit*"[8] what the precise misconduct or investigation was, and that he did not interpret it as relating to Letby.[9] I cannot accept that Mr Harvey did not interpret it as relating to Letby. First, he had raised the concerns about Letby with Ms Eardley, before the RCPCH review team's visit – and again at the beginning of the visit. Second, the sections of the full, confidential report that had been removed for the dissemination version were, as he knew, all about Letby. Third, he knew, as he accepted in evidence, that the RCPCH report had not exonerated Letby, nor had the grievance. It is impossible to accept that he did not interpret the recommendation as "*relating to Letby*". He could bring himself no further than to say: "*I would accept that I didn't seek clarification … In retrospect, I should have sought clarification*."[10] In my view, Mr Harvey did not seek clarification because none was necessary. I have no doubt that if the question 'Who is to be investigated?' had been asked of any member of the RCPCH review team, or of any member of the Executive Team at the Countess, there would have been unanimity of response: 'Letby'.

23.10 Mr Harvey advised the Board that the in-depth review (Dr Hawdon's review) had been conducted but not circulated to the Board. It had recommended that post-mortem reviews be conducted in a small number of cases, and that these be carried out by Alder Hey. Mr Harvey, however, did not think Alder Hey's review would substantially change the findings. He added that in one case, the cause of death was 'unascertained'. Mr Harvey went on to say that the case reviews very much reinforced the RCPCH's findings that there were issues of leadership, escalation and intervention, but that no individual had been pointed out.[11] He said this, knowing that the RCPCH had been complimentary about the leadership on the neonatal unit, that the reviews had not been investigating a crime, and that the final confidential version of the report referred to the concerns about Letby and the need for an HR investigation. None of the executives told the Board this, nor that the recommendation had been ignored.

23.11 When giving evidence to the Inquiry, Mr Harvey accepted that the information in his paper was "*limited*"[12] and "*wasn't a complete picture*".[13] This was, as he acknowledged, because "*the reviews hadn't been specifically commissioned to look for a crime*"[14] and "*didn't go to the level of a forensic investigation and, in hindsight, that was incorrect*".[15] He accepted that the reviews did not exclude deliberate harm. Strikingly, this was not something that he said to the Board. He and Mr Chambers should have told them.

23.12 Mr Chambers said that once the reviews were received from Alder Hey, the Board should draw a line under the review.[16] He reminded the Board that they had agreed to commission the RCPCH report and to downgrade the neonatal unit in response to the concerns raised. Since then, the neonatal unit had been monitored on a daily basis to assess the impact.[17] Ms Kelly and Ms Burnett had been carrying out weekly and daily monitoring of the neonatal unit, including admissions, transfers, staffing, activity levels and clinical decisions, and there had been no significant incidents over the previous six months.

23.13 The executives' view was that the downgrade appeared to have had little impact on the neonatal unit or across the Cheshire and Merseyside Neonatal Network. The Board were told that it had been a difficult process for staff, but they now felt more in control, and the relationship between obstetrics and neonates was better. Mr Chambers said that the Trust had successfully achieved safety for mothers and babies without adding to pressures on other units. The meeting noted that only a few mothers had required transfer to another unit during this time.[18] Given how small the impact was said to have been, it is surprising that the absence of unexpected collapses and deaths was not at least mentioned. The fact that the unit was now admitting babies from 32 weeks' gestation upwards was not an explanation for the absence of deaths and unexpected collapses.

23.14 Mr Harvey said that he had concerns over the number of cots being used above allocation and the number of low birth-weight and low gestation babies they were dealing with, which strengthened the case that the issues were due to the activity and intensity of cases coming into the neonatal unit.[19] In light of his own findings in respect of activity and acuity, which I have already described, Mr Harvey knew that none of these things explained any of the deaths. He did not tell the Board that Dr Hawdon had written to say she had not been able to fulfil all the RCPCH recommendations. He told the Board that her review found that "*in one of the cases the cause of death is unascertained, which is not uncommon*".[20] He knew that several deaths did not have a cause. Mr Wilkie said in evidence: "[W]*hen I saw the Royal College report, particularly the confidential one and I also saw Dr Hawdon's letter which refers to the unexpected and unexplained deaths, had I seen those bits of information on 10 January, been made aware of those, my view would have been: why are we not calling the police?*"[21] He concluded: "*I feel I was misled at the board meeting on 10 January*."[22] I agree with him.

23.15 Ms Hopwood was concerned over references to shortages of staff, when no such issues had been raised at QSPEC or with the Board. This was a point well made. Mr Wilkie also could not understand why the issue of staffing had arisen in the review but had not arisen prior to that. The executives then changed tack and assured the Board that there had been no significant staffing issues at the time, but that Ms Kelly had now undertaken a review of staffing. Mr Harvey added that staffing was only one of many factors. He did not add that none of the factors taken individually, or in any combination,

explained any of the deaths. Mr Chambers said that the Trust had seen an increase in mortality but not a change in other data. This rather ran counter to the notion that they had any explanation for the deaths. The Board were assured that the Trust had recently appointed another consultant and were in the process of appointing another.[23]

23.16 Mr Chambers reminded the Board that there had been an unsubstantiated claim that the deaths and deteriorations were due to Letby's actions and behaviour. In describing the claim as "*unsubstantiated*", Mr Chambers did not mention to the Board that the concerns had not been investigated by the RCPCH or anyone else.[24] Nor did he mention the RCPCH recommendation for an HR investigation nor the fact that it had been ignored. Mr Chambers said it was not the case that there was a causal link to one individual, but rather the issues were around leadership (this was inaccurate) and timely interventions. This had impacted on Letby, and she had raised a grievance. The Board were told that there were actions to be taken arising from the outcome of the grievance procedure, but it was felt that it would be inappropriate to share the grievance outcome with the Board (although Ms Hodkinson could outline any issues).[25]

23.17 Mr Chambers told the Board that "*clinical colleagues*" had strongly disagreed with their proposals for Letby to remain on duty under supervision. This had led to Letby's redeployment and a grievance being raised.[26] This was not true. Mr Chambers knew that he and the other executives had given the doctors a cast-iron guarantee that Letby would be supervised whenever dealing with babies. The doctors had been reassured by that. The executives had given the guarantees without first establishing that there were staff available to carry out the supervision, as I have set out in Chapter 16. When the lack of staff for this extra duty became clear, within days of the decision, the plan was abandoned, and Letby was moved. That was the genesis of the grievance. That Mr Chambers laid the blame at the doctors' door (as Mr Harvey did when speaking to Dr Green) was a part of the executives' negative narrative about the consultant paediatricians.

23.18 Mr Chambers also told the Board that Letby had now prepared a statement about the effect this had had on her. Ms Hodkinson read out Letby's statement to the Board[27] and told them it would also be read out to the consultants.[28]

23.19 Mr Chambers told the Board that the decisions the executives had made may have been suboptimal but had been made for the right reasons – that is, in the interests of patient safety, staff and the nurse, rather than for reputational reasons. However, it was felt that they had not been as honest with Letby as they should have been. Ms Kelly had felt the same, he said. This was because they were trying to protect Letby.[29] Mr Chambers went on to say (of himself and Ms Kelly) that if they really believed that Letby was the causal factor for the change of survival rates in the unit, they would have called the police. However, they did not feel that was the case.[30] This was a revealing statement. It makes plain that when faced with very serious concerns – made in good faith (as they accepted when giving evidence) – that a member of staff may be causing harm and killing babies, the executives did not believe the concerns were well founded and so ignored safeguarding, patient safety and the need to protect whistleblowers. That may well explain why Ms Eardley had the impression that the executives did not take the concerns of the consultants seriously. The executives failed to follow the RCPCH recommendation to investigate. This was high-handed, contrary to all safeguarding principles and foolhardy. None of the executives seem to have reflected on their decisions and asked

themselves the question: 'What if the paediatricians' concerns are, or at least may be, well founded?' Had they done so, there would have been only one approach to take: safeguarding and calling the police. Instead, they were seeking to bring Letby back onto the neonatal unit.

23.20 Mr Chambers told the Board that they would do their best to manage Letby's transition back into the neonatal unit but that it might be tricky. He explained that the recommendations from the grievance process, and some of the unprofessional behaviour from the consultants, meant that they were seeking an apology from the consultants for their behaviour and verbal statements, which bordered on victimisation.[31] It seems to have passed Mr Chambers by that there was no evidence of victimisation (borderline or otherwise), and that there was no evidence of name-calling by the consultant paediatricians nor evidence that justified seeking an apology from them.

23.21 Ms Hodkinson added that there was also a request for mediation with the consultant paediatricians Dr Brearey and Dr Jayaram. She advised that Letby had been assigned a mentor (Ms Rees) and had the support of herself and Ms Kelly. It was agreed that the next stage was critical not only for the reputation of the Countess but also for the neonatal unit and Letby. The Board agreed that the decisions they had taken were right, albeit, in hindsight, they might have managed the matter differently. No one explained what might have been different.[32] I am confident that they did not mean that they would have removed Letby from the neonatal unit sooner. It is more likely that they meant they would not have removed her at all.

23.22 In the Board meeting, Ms Hodkinson defended the executives' handling of the grievance, saying that they had acted appropriately. Ms Kelly said that Letby's complaint was that they had not been upfront with her from the start and that they could have done this differently. Mr Chambers said they had given a commitment to Letby and her family to deal with the consultants' behaviour and any recurrence of it. He added that Letby was not looking for anything more, and that the grievance had "*exonerated her*".[33]

23.23 Mr Wilkie queried whether the consultants accepted the RCPCH's recommendations. Mr Harvey did not reply directly, instead saying that the draft report had been shared with Dr Brearey and Dr Jayaram in a controlled way, and they would now move forward with the recommendations from the report and the grievance. He added that they needed to be clear on the message from the Board and on the consequences for stepping over the line.[34] Mr Harvey's reply was disingenuous. He should have told the Board that the consultants had not yet been allowed to see the full (confidential) report. They could not have commented on any of the Letby/HR issues in the report, as they had not seen them. Mr Harvey was inviting the Board to be clear about the consequences for the consultants of stepping over the line without being open with the Board about the approach he had taken to them.

23.24 Mr Wilkie asked whether the issues about behaviours were accurate. Ms Kelly confirmed they were, and Mr Chambers added: "[T]*here was a lot of substantiation around these behaviours*."[35] They were both wrong.

23.25 Mr Oliver said in oral evidence he did not consider the Board were given the correct information at the meeting to enable them to make a decision about Letby returning to the neonatal unit.[36] Mr Wilkie said the Non-Executive Directors were not given the full picture[37] and explained: "[T]*he whole outcome of the Royal College report was framed in a way that the inference that I drew was that basically Letby had been exonerated.*"[38]

23.26 Mr Higgins, speaking about the Board's management of concerns over the whole period, said in respect of hearing from the consultant paediatricians once:

> "*I feel that I made a mistake in not personally pursuing a line that may have been open to me. The reason that I hesitated or didn't do that was because* [I was] *aware that the situation was strained and incredibly fraught, and also subject to some formal HR processes … Going through this whole process I think that either as chair of QSPEC or as a member of the Freedom to Speak Up group, I probably had some … standing or ground from which to do that and I should have done that but didn't.*"[39]

23.27 Any concerns expressed by the Non-Executive Directors were deflected or were not pursued, and by the end of the meeting the Non-Executive Directors appear to have accepted the position as presented by the executives. Mr Wilkie considered that he had been misled at that Board meeting. He agreed in evidence that the Non-Executive Directors' safety mechanism had not worked effectively to scrutinise the executives. However, he pointed out that the executives allowed the Non-Executive Directors to draw false inferences in the meeting. He stated: "[A]*s a NED it's not unreasonable that you take credible views that are given to you by the Executives if you do not have any other information sources available to contradict those.*"[40]

23.28 Turning back to the RCPCH report, Mr Chambers explained that the Board had been given an unredacted version (i.e. the RCPCH's final confidential report), which had comments on HR issues/Letby within it, and which it was not appropriate to share. Mr Harvey had discussed with the RCPCH the issuing of two versions of the report – a redacted (dissemination) version and an unredacted (confidential) one – and it was content to do so. Mr Chambers added that the redacted version answered all the questions raised.[41] This was not the case.

23.29 Sir Duncan summarised that the Trust would implement the review's recommendations; that the issue was now the publication of the report; that the neonatal unit would stay at its current level; and that there was a need to engage with the consultants and manage Letby's return to the neonatal unit. Sir Duncan added that they needed to handle communications carefully and ensure the public knew that they took the actions they did for the right reasons. He thought the communications must be clear around leadership, escalation and staffing levels, and that there was no single cause for the deaths and no collective issue. He told the Board that a statement would be issued after they had met the consultants and that the Board should take time to consider the statement before its release.[42]

23.30 Sir Duncan was asked, in evidence to the Inquiry, whether he appreciated that the RCPCH report had not excluded the possibility that Letby was deliberately harming babies. He responded: "[T]*hat didn't – didn't come across to me at that meeting*."[43] He said that the letter from Dr Hawdon about not being able to fulfil all the RCPCH recommendations was "*a critical piece of information*", and "*for me not to be told about*

that was misleading".[44] He considered he had been misled. He said that the omission of the letter deprived the Board of a collective discussion about what the next steps should be.[45] This could have been contacting the police at that stage.

23.31 In short, the executives achieved their aim of misleading the Board.

Meeting with Letby, 10 January 2017

23.32 On 10 January 2017, after the Board meeting which discussed both the RCPCH report and the grievance outcome, Mr Chambers held a meeting with Letby, Ms Kelly, Ms Rees and Ms Cooper. Mr Chambers informed Letby that the Board were clear in their support for her returning to the neonatal unit, in the requirement for the doctors to apologise, and in supporting the recommendations from the grievance. He outlined the key themes from the RCPCH report to Letby and reassured her that there was nothing in the report about her.[46] This confirms that Mr Chambers was using the redacted (dissemination) version of the RCPCH report for this purpose.

23.33 On 11 January 2017, there was an Executive Team meeting attended by Mr Chambers, Mr Harvey, Ms Kelly, Mr Cross and Ms Hodkinson. Mr Cross's handwritten notes refer to a letter of apology which would state explicitly that the review exonerated Letby. In his witness statement, Mr Cross says that the drafting of the letter was taken out of his hands and passed to HR. No letter has been found. The notes also refer to arranging a meeting with the doctors to deal with "*Spk* [Speak] *Out Safely issues*".[47]

Meeting of the executives and consultants, 26 January 2017

23.34 On 26 January 2017, the executives met the consultants to discuss the findings from the RCPCH report, Dr Hawdon's report and the outcome of Letby's grievance. The consultants present were Dr Jayaram, Dr Brearey, Dr Saladi, Dr V, Dr McGuigan, Dr Gibbs and Dr Sean Tighe, the Chair of the BMA Local Negotiating Committee. Mr Chambers, wrongly, asserted that the Speak Out Safely process had been "*professionally managed*"; and Mr Harvey said that there was no problem with "*raising concerns, as that is fine but the review by a high powered team does not call out a criminal act*" and that there was now "*a need to draw a line under the 'Lucy issue'*".[48] Mr Chambers reiterated the need to draw a line under the past. The statement from Letby read out in the Board meeting was now read aloud by Ms Rees.[49] The consultants were expected to issue an apology to Letby and to engage in mediation.[50]

23.35 As to the review by a "*high powered team*", presumably the RCPCH review team, it was in no position to call out a criminal act, as Mr Harvey should have known.

23.36 Dr Jayaram recalled that Mr Chambers said: "*I am drawing the line under it, you will draw a line under it and if you cross that line, there will be consequences*."[51] In fact, both Mr Chambers and Mr Harvey are recorded in the minutes as telling the consultants they "*needed to draw a line*" under the Letby issue.[52] The consultants were also told that Letby would be coming back to the unit, and some of the consultants must apologise to her.[53] This was a continuation of the irresponsible approach whereby the consultants were to be required to stop raising concerns.

23.37 Dr Tighe described the meeting as "*shocking*".[54] He told the Inquiry that Mr Chambers' tone was "*dictatorial*" and that he felt "*deep concern*" for the consultants "*because there was potential that their jobs were at stake*".[55] Dr Tighe's observations appear accurate and measured. On 29 January 2017, he wrote to Dr Jayaram to express his concern about the meeting and to give advice.[56]

23.38 Dr McGuigan, who had very recently joined the Countess in January 2017, said that Mr Chambers had a "*severe, stern tone*". However, he did not recall Mr Chambers striking the table as was suggested by some members of the consultant body.[57] Dr McGuigan opined that the external reviews were not the level of investigation required and that the issue had not been "*investigated in the way that it needs to be investigated*". In those circumstances, he considered the expectation on the consultants to apologise to Letby "*ridiculous*".[58] I agree.

23.39 Ms Hopwood, an accountant by background, was the only Non-Executive Director in attendance. She had been asked to step in to attend the meeting at short notice, in place of Sir Duncan. She described the meeting as "*tense*".[59] However, she did not think the behaviour of anyone in the meeting was aggressive or unreasonable. She thought the consultants' concerns were genuine and in hindsight appreciated that what the consultants were being asked to do was contrary to the Speak Out Safely Policy.[60] It is not clear that she thought about that policy at the time.

23.40 Mr Chambers' account was that he felt the need "*to be fairly clear and direct in terms of the outcome, particularly of the grievance*".[61] However, he denied raising his voice or that he was angry. He asserted: "*I behaved professionally*."[62] Mr Chambers conceded that, although he thought the consultants' concerns had been listened to and action had been taken, "*I am aware that it wasn't necessarily within the policy (Speak Out Safely)*". He was asked whether he should have checked before making the assertion that the concerns were professionally managed under the policy. He accepted "*with regret, I should have done*".[63] His behaviour at the meeting was entirely in keeping with the attitude to the doctors he had displayed in the meeting with Letby and her parents.

23.41 Dr Jayaram gave oral evidence that, following this meeting, the consultants asked for sight of both the RCPCH report and Dr Hawdon's review and were told by Mr Harvey that they could see them after the Trust had made a public statement (in February 2017).[64] Dr Jayaram considered this to be "*completely inappropriate*".[65] This refusal was unreasonable and unprofessional. Dr Brearey and Dr Jayaram then emailed the RCPCH, lobbying Ms Eardley, for sight of the report.[66] Ms Eardley emailed Mr Harvey about this.[67]

Consultants' letter of 30 January 2017

23.42 Following the meeting on 26 January 2017, the consultants sent a joint letter to Mr Chambers on 30 January 2017.[68] The consultants agreed to send a letter of apology to Letby. They asked for written clarification on what the Board understood the reason to be for increased mortality on the neonatal unit, and to be allowed the opportunity to read the RCPCH report and Dr Hawdon's review prior to their publication.[69] These were reasonable questions. There was no response to the consultants' letter until 16 February 2017.

23.43 The consultants should have been provided with the reports in advance of the meeting on 26 January 2017. Mr Chambers accepted this when giving evidence.[70] The failure to provide the reports was another example of the treatment of the consultants, which was, at best, dismissive.

23.44 In contrast to this, on 26 January 2017, at a meeting which had been scheduled to provide an overview to Letby of the meeting between the executives and the consultants, Ms Hodkinson, Ms Kelly and Ms Rees told Letby that they had always wanted her to have sight of the RCPCH report at an early stage and had previously talked about her having sight of it today. However, it was agreed that with everything that had taken place that day, sight of the report would be rearranged for a more convenient time.[71] This is a stark demonstration of the difference in approach taken by the executives to Letby and to the consultants.

Letby's email to all staff, 31 January 2017

23.45 On 31 January 2017, Letby emailed all staff on the neonatal unit, stating:

> "*I was redeployed from the Unit in July 2016 following serious and distressing allegations of a personal and professional nature made by some members of the medical team … After a thorough investigation it was established that all the allegations were unfounded and untrue and I have therefore been fully exonerated. I have received a full apology from the Trust … I will begin making my return to the Unit in the coming weeks*."[72]

23.46 The Countess was aware of Letby's email: on the same day, Ms Hodkinson had emailed Ms Cooper (the RCN Representative) and Letby asking for "*the final copy of the wording sent out today if possible*".[73] Ms Hodkinson's evidence was that she was "*very disappointed*" when she read Letby's email. She stated: "[W]*e had provided the guidance to* [Letby] *and her Union colleague and this was the information that was then distributed*."[74] Despite this expression of disappointment, the Inquiry has seen no evidence to suggest that anyone at the hospital responded to Letby's email to correct her and to clarify that she had not been investigated at all, still less exonerated. This was a further failure of management.

Sharing the RCPCH report

23.47 Also on 31 January 2017, Ms Kelly and Ms Hodkinson met Letby to share the RCPCH report with her.[75] It is not clear from the notes whether that happened.

23.48 The following day, at a meeting of the executives, there was discussion of the recent meeting with the consultant paediatricians and of a plan for sharing the RCPCH report with parents.[76] Mr Cross's note of the meeting additionally records that the consultants' letter of 30 January 2017 was read out and that a response was to be prepared, with hard copies to be sent to each of the consultants.[77]

23.49 On 3 February 2017, each consultant paediatrician was issued with a numbered copy of the dissemination (redacted) version of the RCPCH report, which they had to collect and sign for.[78] Ms Kelly also sent a copy of the dissemination version of the report to NHS England.[79]

23.50 On 6 February 2017, Mr Chambers wrote to staff at the Countess to inform them that the RCPCH report would be published on 9 February 2017. Mr Chambers stated: "*There is no single cause or factor identified as a means of explaining the increase we have seen in our mortality numbers*."[80]

23.51 On 7 February 2017, the consultants were given Dr Hawdon's report.

23.52 On 8 February 2017, the redacted/dissemination version of the RCPCH report was published by the Trust.

Media interest and the hospital's response

23.53 On 3 February 2017, the *Sunday Times* contacted the Countess, seeking information about the unexpected and unexplained deaths of babies in the neonatal unit between January 2015 and July 2016.[81] They requested a copy of the RCPCH report and asked why it had not been published following its receipt by the hospital in October 2016 and for details of any reviews that had been carried out on the deaths. The newspaper had also spoken to the family of one of the babies who had died and asked for a response to the family's concerns about not being kept better informed of the investigations. Finally, they asked what the plans were for the Countess to reopen the intensive care cots in the neonatal unit that had been closed.

23.54 On the same day, the Countess attempted to contact the families of children who had died to tell them of the imminent publication of the RCPCH report and the media interest. Five families were contacted.[82]

23.55 Following the publication of the article by the *Sunday Times*, there was a flurry of activity to manage the media interest prior to the RCPCH report being published on 8 February 2017 (see also Chapter 20). Only those families who had been contacted by telephone were sent a copy of the report on the day of publication. On 8 February 2017, Mr Harvey wrote to the families that the hospital had been unable to contact by telephone, inviting them to get in touch if they wished to have a copy of the report and/or wanted to discuss any matter concerning their babies.[83]

Executives' meeting, 6 February 2017

23.56 At a meeting on 6 February 2017, the executives discussed the *Sunday Times* article and a plan for communicating with families and other stakeholders. They also discussed strengthening the message for the public – that the hospital had invited the RCPCH to review its neonatal services. Mr Chambers confirmed that the report would not be published earlier than Wednesday, 8 February 2017.

23.57 According to Mr Cross's notes, Mr Chambers asked for the consultants' letter of 30 January 2017 to be shared with Mr Harvey and mentioned that Dr Brearey and Dr Jayaram had wanted a private meeting with Sir Duncan (the Chair) but that "*did not happen*". They discussed the need for Mr Harvey to meet both doctors as soon as possible. Mr Chambers asked Mr Harvey to "*discuss with* [Dr Jayaram] *and* [Dr Brearey] *re: other version* [emphasis in original] *incl College observations tomorrow to share those comments*". Ms Kelly commented that some consultants had contacted the RCPCH directly to obtain transcripts of their interviews with the reviewers. Mr Harvey is then

noted as saying "*GMC issue*". Mr Chambers appears to change his mind about sharing the unredacted report and comments with the consultants, saying that "*no copy to be given to paediatric consultants incl comments*".[84]

Meeting with Letby and her parents, 6 February 2017

23.58 Following the executives' meeting, Mr Chambers, Mr Harvey, Ms Kelly, Ms Hodkinson, Ms Rees and Ms Cooper met Mr and Mrs Letby and Letby.[85]

23.59 At this meeting, Mr Chambers described some of the consultants' behaviour as "*not appropriate*".[86] He told Letby she would receive apologies from the consultants, and that Dr Brearey and Dr Jayaram would engage in mediation with her. Letby was also told by Mr Chambers "*all support your transition back*" to the unit. Mr Chambers conceded in evidence to the Inquiry: "[I]*n truth that's definitely an overstatement of the position*."[87] It was untrue.

23.60 The notes of the meeting record Mr Chambers telling Letby that the reviews had vindicated her.[88] In evidence he accepted that the RCPCH had not investigated Letby and stated his words were a "*misinterpretation*" of the outcome of the grievance and Dr Hawdon's review.[89] What he had said was untrue.

23.61 Mr Chambers was referred to overtly supportive comments he made to Letby, such as: "*Lucy, don't worry, we have got your back*." He described this as "[c]*lumsy language*" that he used in an attempt to de-escalate the issues. He accepted that in hindsight he did not get this right.[90] It was not a question of not getting right a difficult, nuanced situation. To say "*we have got your back*" is unambiguous. If it was to "*avoid any possible escalation*", it was misconceived and dishonest.[91] If he said it because he meant it, it was a gross misjudgement.

23.62 Mr Chambers' comment, recorded in the minutes, that the "*last thing we want is sensational press*" was telling. He went on to say to Letby: "*We don't want the story to be about you; that would be horrific*."[92] Fear of a press story was clearly a matter of concern to Mr Chambers. The meeting notes read as an attempt by him to placate Letby and her parents, at the expense of the truth or fairness to the consultants.

23.63 Mr and Mrs Letby confronted Mr Harvey, saying: "*You should have called the police or told them to go away. But you sat on the fence*." Faced with this challenge, Mr Chambers' response was: "*We had a choice to make. One option was a police investigation, the other option was a clinical investigation … Choices to support Lucy to stay in the environment or take Lucy out*."[93] This was a clear acknowledgement that, while recognising that a referral to the police would have been an appropriate course of action, a decision was made to prioritise support of the nurse and pursue a clinical investigation instead.

23.64 There is no indication in the minutes of the meeting that Mr Chambers gave any consideration to the potential risk to patients posed by Letby's return to a clinical role. Ms Hodkinson, attending the meeting as Director of HR, responded to Letby's question "*But what if I leave?*" with the answer: "*There will be nothing on your record; it will not affect your reference or any other matters*."[94]

23.65 Mr Harvey reinforced the message given by Mr Chambers and told Letby and her parents that he had spoken to Dr Brearey and Dr Jayaram about the need to support Letby and engage in the mediation, and that they accepted their actions were inappropriate. Mr Harvey said in his evidence to the Inquiry that in hindsight "*I … accept that they* [the consultants] *might feel forced into making that concession when that actually wasn't appropriate*."[95]

Board meeting, 7 February 2017

23.66 On 7 February 2017, the Board of Directors met. Mr Chambers updated the Board that clinicians had raised concerns about an increase in deaths in the neonatal unit in July 2016, following which the Trust had changed its admission criteria and had invited the RCPCH to undertake a review. The RCPCH had suggested that a further in-depth, independent review be conducted. This (Dr Hawdon's review) had now been completed. It had pointed out some areas for improvement but did not "*identify a single causal factor or raise concerns regarding unnatural causes*".[96] This was not correct. The RCPCH had recommended that the hospital take steps to regularise the position of Letby, who had been taken off nursing duties, and had recommended a forensic review. The review by Dr Hawdon had not settled the concerns raised by consultants, and a further review had been recommended. The Board were given false reassurance.

Consultants' letter of 10 February 2017

23.67 On 10 February 2017, the consultant paediatricians sent a second letter to Mr Chambers, copied to Mr Harvey.[97] The letter asked Mr Chambers to "*urgently ask the Coroner to undertake a full investigation of all the deaths and unexpected collapses that occurred on the neonatal unit between June 2015 and July 2016*". This was because they had not been reassured that "*all these deaths and collapses are explicable by natural causes*".[98]

23.68 The consultants cited five reasons for their request:[99]

a. "*The RCPCH college review undertaken in September 2016 was a service review. Although its recommendations would help to provide a better service, it did not identify a cause for the sudden increase in neonatal mortality*." (This first complaint was consistent with what had been said by Dr Brearey and Dr McCormack on earlier occasions.)

b. They agreed with "*the conclusion of Dr Hawdon's case note review that 4 babies who died require a broader forensic review*". However, they were "*concerned that the cause of death or sudden collapse was uncertain and could not be fully explained by postmortem findings in an additional 4 babies*". They were "*concerned that the cause of collapse was still uncertain for 2 babies who survived*". And they were "*aware of a number of other babies, over the same time period, whose collapses were inadequately explained*".

c. Although a post-mortem diagnosis had been made in a number of cases, there was still "*considerable doubt as to why certain babies collapsed unexpectedly and subsequently did not respond to appropriate resuscitation measures*".

d. "*The internal investigation in July 2016 by paediatric staff of babies, who were transferred from Chester to a neonatal intensive care unit, identified a number of cases in which the babies' deterioration was unexplained or unusual.*"

e. There had been "*no deaths or unexpected collapses on the neonatal unit since July 2016. Unwell babies have been cared for, received intensive care and in some cases transferred to other hospitals, but their clinical courses have been far more predictable and responsive to treatment than previous cases.*" This change could not "*be solely attributed to the re-designation of the neonatal unit or any other changes in practice that have occurred since then*".

23.69 The letter concluded:

> "*Please be assured that we, as a paediatric consultant body, are making this request because patient safety is our absolute priority. We hope that a comprehensive external investigation will be in the best interests of the bereaved families and those affected by these sad events.*"

23.70 On 14 February 2017, Mr Harvey emailed Ms Eardley at the RCPCH, enclosing the clinicians' letter of 10 February 2017. In the email, Mr Harvey tells Ms Eardley that he has shared the RCPCH report with the coroner. Also, he draws her attention to a comment made in the clinicians' letter, which he refers to as "*a dig at the review … this despite the fact that the observations in green text were shared with the two leads*".[100] This was not correct. The clinicians had seen a redacted draft report (in November 2016, in Mr Harvey's office) and then the redacted/dissemination version of the report in February 2017. Apart from this remark by Mr Harvey, there is no evidence that either Dr Jayaram or Dr Brearey had seen the observations in green text (or even knew they existed at that point).

23.71 The sentence which Mr Harvey seemed to resent in the clinicians' letter of 10 February 2017 is: "*The concerns we expressed to the reviewers are not included in the report.*"[101] Given that Mr Chambers had decided that the redacted version was to be disseminated, and that the consultant paediatricians saw no reference to their concerns in the report, it is plain that they had not seen the full report. Mr Harvey had no reason to think otherwise. His continued readiness to undermine the consultants was regrettable. It was consistent over many months, to the detriment of the individual consultants but more importantly to the decision-making about what to do about the unexplained deaths on the neonatal unit.

23.72 At a meeting on 14 February 2017, the executives discussed the consultants' letter of 10 February 2017 in detail.[102] They considered that the consultants had "*gone backwards*" and were "*firmer*" in their position about unnatural causes. Mr Harvey "[w]*ondered what they were plotting*". He accepted when giving evidence that he had said that. He explained that he had a "*degree of frustration*" with the consultants and that his "*frustration was with regard to trying to pull everything together to get a consensus*".[103] The difficulty for Mr Harvey was that the consultants had genuine concerns, as he later accepted. It was unrealistic and unfair of him to complain that they would not abandon their concerns and go along with his views, which seems to have been what he meant by "*consensus*".

23.73 Generally, it was felt by the executives that the consultants had a locked-in collective mindset, and it was agreed that they would all be treated the same but would receive individual letters in response. There was further work to be done on re-looking at the rotas, and discussion about how 'hot' the neonatal unit was running. It was agreed that the consultants' letter of 10 February 2017 would be shared with the coroner, the RCPCH and Dr Hawdon, to enable the executives to reach a collective view. Finally, it was agreed that they should continue their plan to bring Letby back to work in the neonatal unit.[104]

23.74 At a further meeting of the executives, on 15 February 2017, Mr Chambers suggested a narrative which might explain the reasons for the incidents. This was based on a high level of activity in the neonatal unit, a culture of coping, delays in the transfer of children and heroic endeavours of staff, which had culminated in a failure to recognise the risks which they were managing. It was suggested that they should now be "*drilling down*" further into these themes, which could be used to generate a coherent narrative as an explanation.[105]

23.75 On 16 February 2017, Mr Chambers replied to the consultants' letters of 30 January and 10 February 2017.[106] He enclosed the redacted passages from the RCPCH report. He wrote: "*Whilst your specific allegations are not included in the published report, they were referenced in additional comments and observations made by the Reviewers, which are now attached*." He reiterated that there had been a thorough internal and external review into increased neonatal mortality at the Countess and referred to the consultants agreeing to send a letter of apology to Letby. He mentioned also that action was being taken to return Letby to the neonatal unit as soon as possible. The only response this letter makes to the consultants' letter of 30 January 2017 is a reference to a letter of apology being sent to Letby; it does not respond to the question seeking clarification of the Board's understanding of the reasons for the deaths.

Ongoing concerns of the consultants

23.76 On 23 February 2017, at a regular one-to-one meeting, Dr Gibbs and Mr Harvey discussed further investigations and the concerns that the consultant paediatricians still had regarding Letby's involvement in neonatal deaths. In an email sent to Dr Jayaram at 00:30 on 24 February 2017, Dr Gibbs set out the discussions:[107]

a. Dr Gibbs and Ms McGlade had conducted a review at the time of the Silver Command investigations and a follow-up review of "*non-fatal unexpected and unexplained collapses*". Dr Gibbs made the point that the consultants had not seen the results of the analysis of the data, and that it was disappointing that the Board had reached their decision without the doctors seeing the results of the reports. Mr Harvey agreed to send the information to the doctors.

b. Mr Harvey had reassured Dr Gibbs that the consultants' letter (of 10 February 2017) had been given to the coroner and that he had discussed the consultants' concern over Letby with the coroner; also that the coroner had been given both the review by Dr Hawdon and the "*full*" RCPCH report, which contained the consultants' concerns. Mr Harvey reported that the coroner would not reopen any inquests which had taken place but that there would be an opportunity to examine issues associated with the deaths at the upcoming inquests still to be heard.

c. Mr Harvey had agreed that they (the doctors) would be able to look at any issues surrounding the seven cases that remained unexplained and had been identified by Dr Hawdon and Dr Subhedar at a meeting to take place in a week's time.

d. Dr Gibbs made it clear that the consultants remained suspicious about Letby. Although they did not know that Letby had been the cause of unnatural collapses or deaths, they were still worried that this might be the case, and that the problem could resurface at any stage either at the Countess or, if she moved, in another Trust, and they therefore had to do everything they could to find out.

e. Dr Gibbs also reminded Mr Harvey that most of the "*failings*" raised by the RCPCH applied to most neonatal units and did not adequately explain the increase in mortality. Mr Harvey thought this might be explained by the significant increase in workload.

f. Dr Gibbs informed Mr Harvey that the consultants were preparing further letters for Mr Chambers and for Letby.

23.77 Dr Gibbs concluded the email to Dr Jayaram by saying that he thought that they had probably now done all they could and that it may not be appropriate to consider whistleblowing, as it seemed that the Trust had informed two coroners of the consultants' "*dark*" suspicions; that they may need to accept that they may never get a clear answer as to the cause of the increased mortality and unexplained collapses; and that pushing things further may not provide any better answers (although the coroners or parents may decide to "*take things further*").[108]

23.78 On 28 February 2017, Dr Gibbs circulated his email of 24 February 2017 to the consultant group.[109]

23.79 On 1 March 2017, Letby, Ms Kelly, Ms Rees and Ms Hodkinson met to discuss mediation and Letby's return to the neonatal unit. There was discussion of how Letby had gone onto the neonatal unit on her own the previous week.[110] In written evidence to the Inquiry, Ms Jean Peers, a nursery nurse (band 4), wrote that around 12 months after Letby had been removed from the neonatal unit, Mr Chambers called a meeting with all staff and said that Letby would be coming back to the unit. Ms Peers recalled that Ms Griffiths said that Letby was going to come to the unit and "*that we would do a tea party to welcome her. We did cakes and tea*."[111] Letby came to the tea party.

Endnotes

1 INQ0003237
2 INQ0003239
3 Ed Oliver 3 December 2024 104/4-9
4 Rosalind Fallon 3 December 2024 195/1-5; Rachel Hopwood 3 December 2024 138/13-19
5 INQ0003237/5
6 INQ0003237/1-2
7 INQ0002751
8 Ian Harvey 29 November 2024 14/4-9
9 Ian Harvey 29 November 2024 198/24 to 199/6
10 Ian Harvey 29 November 2024 14/12 to 15/16
11 INQ0003237/2
12 Ian Harvey 29 November 2024 184/1-4
13 Ian Harvey 29 November 2024 186/18-19
14 Ian Harvey 28 November 2024 203/20-21
15 Ian Harvey 28 November 2024 208/4-6
16 INQ0003237/2
17 INQ0003237/2
18 INQ0003237/2-3
19 INQ0003237/3
20 INQ0003237/2
21 James Wilkie 2 December 2024 185/23 to 186/3
22 James Wilkie 2 December 2024 208/9-10
23 INQ0003237/3-4
24 INQ0003237/4
25 INQ0003237/2
26 INQ0003237/4
27 INQ0057493/1-2
28 INQ0003237/4
29 INQ0003237/4
30 INQ0003237/5
31 INQ0003237/5
32 INQ0003237/5
33 INQ0003237/5-6

34 INQ0003237/6
35 INQ0003237/6
36 Ed Oliver 3 December 2024 109/11-15
37 James Wilkie 2 December 2024 188/16-23
38 James Wilkie 2 December 2024 182/13-19
39 Andrew Higgins 3 December 2024 41/12-24
40 James Wilkie 2 December 2024 190/10-13
41 INQ0003237/5
42 INQ0003237/6-7
43 Sir Duncan Nichol CBE 2 December 2024 76/20-25
44 Sir Duncan Nichol CBE 2 December 2024 79/18-22
45 Sir Duncan Nichol CBE 2 December 2024 80/7-16
46 INQ0003471/2
47 INQ0004380
48 INQ0003523/2
49 INQ0057493/1-2
50 INQ0003523
51 Dr Ravi Jayaram 13 November 2024 145/19-21
52 INQ0003523/2
53 INQ0003523/2-3
54 Dr Sean Tighe 8 October 2024 197/11
55 Dr Sean Tighe 8 October 2024 197/20-21, 200/16 and 201/9-10
56 INQ0003489
57 Dr Michael McGuigan 8 October 2024 121/20-25
58 Dr Michael McGuigan 8 October 2024 124/16 to 125/5
59 Rachel Hopwood 3 December 2024 147/6-14
60 Rachel Hopwood 3 December 2024 147/15 to 149/23
61 Tony Chambers 27 November 2024 150/3-4
62 Tony Chambers 27 November 2024 150/11-13
63 Tony Chambers 27 November 2024 151/10-25
64 Dr Ravi Jayaram 13 November 2024 151/9-11
65 Dr Ravi Jayaram 13 November 2024 151/12-13
66 Dr Ravi Jayaram 13 November 2024 151/14-19
67 INQ0003132/1
68 INQ0003095/1

69 INQ0003095/1
70 Tony Chambers 27 November 2024 147/14 to 148/13
71 INQ0003471/4
72 INQ0058624
73 INQ0058646
74 Sue Hodkinson 26 November 2024 122/9-12
75 INQ0003471/5
76 INQ0004392/1
77 INQ0107706/159
78 Dr Ravi Jayaram 13 November 2024 151/1-6
79 Witness statement of Prof. Sir Stephen Powis INQ0017495/136/para 540
80 INQ0003060/1-3
81 Witness statement of Gill Galt INQ0102069/20/para 90
82 INQ0106817/32
83 INQ0012628/3
84 INQ0003375
85 INQ0014279
86 INQ0014279/1
87 Tony Chambers 27 November 2024 114/22 to 115/18
88 INQ0014279
89 Tony Chambers 27 November 2024 118/2 to 119/23
90 Tony Chambers 27 November 2024 120/22 to 121/2
91 Tony Chambers 27 November 2024 120/23-25
92 INQ0014279/4
93 INQ0014279/4
94 INQ0014279/5
95 Ian Harvey 29 November 2024 86/21-23
96 INQ0014821/6
97 INQ0003117
98 INQ0003117/1
99 INQ0003117
100 INQ0012756
101 INQ0003117/1
102 INQ0003379/1
103 Ian Harvey 29 November 2024 96/19-22

104 INQ0003379/3

105 Witness statement of Stephen Cross INQ0107707/43/para 167

106 INQ0006073/1-2

107 INQ00014268/1-3

108 INQ00014268/1-3

109 INQ00014268/1-3

110 INQ0003471/8

111 INQ0101330/2

Chapter 24. Referral of deaths to the coroner

Background

24.1 The Coroners and Justice Act 2009 provides the statutory framework for the powers and duties of coroners.[1] So far as relevant to the Inquiry, the Act provides:

Section 1: On report of a death, if there is a reason to suspect a violent or unnatural death, or the cause of death is unknown, a senior coroner must conduct an investigation.

Section 4: Where preliminary inquiries satisfy the coroner that the death was naturally occurring and non-violent, the coroner is required to discontinue the investigation and his involvement will come to an end.

Section 5: The purpose of an investigation is to ascertain who the deceased was; how, when and where the deceased came by his or her death; and the particulars (if any) required to be registered concerning the death.

Section 6: Otherwise, the coroner must hold an inquest.

24.2 A senior coroner has the power to request a post-mortem examination if an investigation is already under way, or for the purpose of deciding whether the death is one where the coroner has a duty to conduct an investigation.[2]

24.3 Mr Nicholas Rheinberg was the Senior Coroner for Cheshire from July 1999 until his retirement on 10 March 2017.

24.4 Mr Rheinberg had published local guidance titled 'Reporting Deaths to the Coroner', which required all child deaths in the area to be reported, irrespective of the circumstance of death.[3]

Sequence of referrals, 8 June 2015 to 27 June 2016

24.5 The death of Baby A was reported to the coroner by Dr Saladi on 8 June 2015[4] (see Chapter 3). No medical cause of death was given in the reporting form.

24.6 A coronial investigation was commenced into Baby A's death on 22 June 2015.[5] The inquest was opened on 23 December 2015, following receipt of the post-mortem report of Dr Shukla dated 18 December 2015, which gave the cause of death as 'unascertained'.[6] The evidence that the Inquiry heard about preparation for the inquest, and about the inquest itself, is set out below.

24.7 On 15 June 2015, the death of Baby C was reported to the coroner by Dr Gibbs (see Chapter 3). No cause of death was given in the reporting form. An investigation was commenced on 16 June 2015.[7] On 14 August 2015, Dr Gibbs spoke with Dr Kokai, who had performed the post-mortem on Baby C. Dr Kokai's view was that Baby C had died from myocardial ischaemia, which led to Baby C's collapse and unsuccessful resuscitation. Dr Gibbs described his view to the Inquiry, following his conversation with Dr Kokai, as follows:

> "*I was satisfied that the death had been partly explained …*
>
> *… because of the unusual nature of the resuscitation … I wasn't sure whether the damage to the heart that was noticed on this postmortem would have all happened at and after the resuscitation or whether it happened before. It was Dr Kokai's view that the damage to the heart had happened before the resuscitation and therefore caused the collapse but it didn't fully explain it because I then asked 'but why did the damage occur to the heart?' So you keep going back one more step. But with Baby C being an at-risk baby, very small, growth retarded, difficulties with perfusion from the placenta, the afterbirth, before the baby was born possibly that might have explained why the heart … had suffered this insult. But it didn't quite fit together.*"[8]

24.8 Speaking more generally about the impact of the post-mortem reports for the babies featured on the indictment, Dr Gibbs said that the fact that the post-mortems performed had not revealed any suggestion of deliberate harm contributed to his "*dithering*" when deciding whether or not to act upon his concerns.[9] Dr Gibbs added:

> "[M]*ost of the babies had an explanation for their deaths from the ones that had had the postmortem …*
>
> *So because we had explanations for all the deaths … I wasn't sure that harm had happened to these babies … They were not completely unexplained; some of them had postmortems that seemed to explain the death but they didn't explain it adequately when we looked back at it.*"[10]

24.9 The coronial investigation into Baby C's death was discontinued on 26 November 2015, following receipt of the post-mortem report of Dr Kokai.

24.10 In oral evidence, Mr Rheinberg said that he would normally hope that a difference in opinion between a clinician and pathologist as to cause of death would be made clear in any post-mortem report.[11] He agreed that, where the difference in opinion was between a specified natural cause of death or an 'unascertained' cause of death, this would be relevant and important information to provide to the coroner and would have been a reason to hold an inquest into the death of Baby C.[12] At the time, Dr Gibbs did not mention his view that "*it didn't quite fit together*" to Dr Kokai and deferred to his view about the cause of death.[13]

24.11 Baby D's death was reported to the coroner by Dr Newby on 22 June 2015[14] (see Chapter 3). In the report, Dr Newby offered no cause of death. Dr Newby did, however, inform the coroner that there had been a cluster of neonatal deaths at the hospital – the 'additional information' section of the reporting form recorded: "*Reported that this had been 3rd death in 12 days for neonatal. Also a further episode of apnoeic event and CPR for previous twin death; surviving twin had successful CPR.*"[15]

24.12 In oral evidence, Mr Rheinberg said that his team would have discussed the cluster of three deaths in June 2015. He explained: "*It was worrying. But as … the results came … out they all seemed to be explicable*."[16] Baby A's death was not explained. Baby C's death was, on the face of it, explained in November 2015.

24.13 As with Baby A and Baby C, an investigation was commenced into the death of Baby D. The report of the post-mortem conducted by Dr McPartland found a natural cause of death. The view of Mr Alan Moore, Assistant Coroner for Cheshire, was that the investigation should be discontinued. After representations from Baby D's family, however, an inquest was opened on 8 January 2016. Mr Rheinberg described the effect of the representations in his oral evidence:

> "*I think I had received a letter from Baby D's parents direct, giving a number of details relating to what they saw as mismanagement in relation to the death and I decided that this did need further investigation, and although a natural cause of death had been given, and so in normal circumstances a discontinuance would be the only course of action appropriate, in these particular circumstances I decided to accede to the request made*."[17]

24.14 The inquest into Baby D's death was scheduled to be heard on 25 May 2017 but was adjourned due to the coroner being informed that a police investigation was to be carried out into neonatal deaths at the hospital. The coronial investigation was subsequently suspended on 27 November 2020.[18] The inquest of Baby D (as well as Baby C, Baby E, Baby I, Baby O and Baby P) was reopened on 4 February 2026 and adjourned again.

24.15 The death of Baby E was reported to the coroner on 4 August 2015 (see Chapter 5). A cause of death was proposed by the consultant paediatrician Dr ZA – namely, necrotising enterocolitis and prematurity. She explained her view to the assistant coroner, who accepted it. Dr ZA accepted in oral evidence that she had not realised the significance of the X-ray of Baby E, which did not show signs of necrotising enterocolitis; she agreed that a post-mortem should have been performed.[19] Instead, there was no post-mortem and no investigation.

24.16 Baby I's death was reported to the coroner on 23 October 2015 by Dr Gibbs[20] (see Chapter 6). No cause of death was offered. An investigation was opened by Dr Janet Napier, Assistant Coroner, on 28 October 2015. The post-mortem was performed by Dr Kokai, who concluded that there was a natural cause of death. The investigation was discontinued on 12 February 2016.

24.17 The deaths of Baby O and Baby P were reported to the coroner by Dr V, on 27 June 2016.[21] No cause of death was offered. Investigations were opened by the assistant coroner, Mr Moore, on 30 June 2016.[22] Post-mortems were conducted by Dr Kokai, who concluded there were natural causes of death in both cases.

24.18 None of the forms reporting deaths of babies on the indictment to the coroner mentioned suspicion or concern about deliberate harm, or the association between neonatal mortality and a specific nurse. Whilst there is no obvious place on the form to enter such information, the coroner should have been informed by telephone about the concerns. By the time of the deaths of Baby O and Baby P, the failure to alert the coroner was inexcusable.

24.19 Ultimately, however, and on behalf of the Countess, Mr Cross and later Mr Harvey were responsible for liaising directly with the coroner about deaths on the neonatal unit. They bear principal responsibility for the failure to make it clear to the coroner that there were suspicions and concerns about deliberate harm being caused on the unit. They should have communicated this long before the inquest hearing in the case of Baby A.

Communications between the Countess and the coroner, 2016

24.20 The inquest into the death of Baby A was opened on 23 December 2015. The hearing of the inquest was initially set for 23 March 2016, to allow for the preparation of witness evidence.

24.21 On 28 January 2016, Ms Yvonne Williams, Coroner's Officer, wrote to the hospital saying that Mr Rheinberg believed the hospital should consider completing a Serious Untoward Incident report.[23]

24.22 On 29 January 2016, Mother A and B wrote to Ms Yvonne Williams, explaining issues with Baby A's care. In evidence she explained that she felt she had no answers: "*I was brought into the room when it was too late. So I was trying to get some answers as to what had led for, in one of the nurse's own words, a perfectly well pre-term baby to suddenly die*." She asked five questions:

a. Why was the long line not put in straight away?

b. How many times was Baby A's monitor allowed to beep without being checked and how long was it before medical staff attended to Baby A?

c. Why was a more senior doctor not called to assist with the insertion of the long line?

d. Why were Parents A and B allowed to believe Baby A's initial post-mortem showed nothing when in actual fact Baby A had a condition?

e. Why were Parents A and B not informed that the long line had been put through Baby A's liver?

The email was forwarded to Dr Jayaram, who answered each of the questions. Mother A and B said she had found his explanation about the crossed pulmonary artery reassuring. Dr Jayaram had explained that the rare variant should not cause any problems with the function of the heart and lungs, "*and the* [post-mortem report] *suggests that there was no issue with the heart and lungs*" arising from that variant. He also explained, in the context of the long line, that he would not normally expect paediatricians to keep parents informed at every stage of the procedure. Mother A and B made the points in evidence that she had not seen her babies when they were born because they were delivered under general anaesthetic, she had not been allowed to go and see Baby A because of the lines, and she felt that the cause of the delay should have been explained to her. I agree with her.[24]

24.23 On 23 February 2016, Ms Harper-Lea wrote to Mr Harvey and Ms Kelly, passing on Mr Rheinberg's view that the Countess should consider completing a Serious Untoward Incident report. Mr Harvey replied the same day, referring to the recent Thematic Review: "*I believe that* [Dr Brearey's] *review is equivalent of a SUI* [Serious Untoward

Incident] – *but we can make a final decision when we see the report*."[25] Mr Harvey had received the draft Thematic Review from Dr Brearey on 15 February 2016.[26] The updated version was sent to him and to Ms Kelly on 21 March 2016.[27] Although Mr Harvey had described it as the equivalent of a Serious Untoward Incident report, he did not discuss it with Ms Kelly until 11 April 2016 (see Chapter 9).

24.24 The inquest was not ready for hearing on 23 March 2016, because the Countess had not provided the required evidence.[28] The hearing was put back to 10 October 2016.

24.25 Mr Rheinberg said that it was after notification of the deaths of Baby O and Baby P in June 2016 that he first became concerned about neonatal mortality at the hospital.[29] He had already been notified of deaths on the unit during 2015 and during 2016. In oral evidence, Mr Rheinberg described talking with Ms Christine Hurst, Coroner's Officer, "*about our concerns about … this being yet another unexpected death of an infant or infants at the Countess of Chester Hospital*".[30] Mr Rheinberg expanded in his witness statement:

> "*We were both concerned about the number of neonatal deaths that had occurred at* [the Countess] *in a comparatively short period of time and the deaths of the two triplets focused our concern. For my part I wondered whether there was some underlying problem with the standard and quality of care at the hospital although there had been no indication previously to show that this might be the case*."[31]

24.26 Mr Rheinberg says in his statement that it was at about the same time as these concerns arose that he learnt that the Countess had commissioned the RCPCH to carry out a review, and that: "*This was something of a comfort as I hoped that if there were deficiencies within the department, I anticipated that these would be revealed*."[32]

24.27 Consistent with Mr Rheinberg's recollection that he became aware of the hospital commissioning a report by the RCPCH, Mr Moore gave evidence to the Inquiry that he received a telephone call in early July 2016 from Mr Cross, intended for Mr Rheinberg, who was unavailable at the time.[33] Mr Moore said that Mr Cross:

> "*indicated that the Countess of Chester Hospital had experienced a number of neonatal deaths … in recent times and that the Trust had therefore commissioned an independent review by the Royal College of Paediatrics and Child Health. He said the review would look at the neonatal unit and he said, 'We will send a copy of the report once it's available through to Mr Rheinberg'*."[34]

24.28 In a supplementary statement to the Inquiry, Mr Cross stated that he "*liaised with the Coroner on a regular basis and it is my strong recollection that on 8 July 2016 I called him and updated the Assistant Coroner in respect of everything I was aware of about the current position, including the suspicions relating to Lucy Letby*".[35] Mr Moore's evidence was that Mr Cross did not raise the consultants' concerns that a member of staff may be responsible for the deaths. When asked what he would have done if that information had been shared with him, Mr Moore replied: "*I would have explored exactly what the concerns were that Mr Cross or the Trust or both had and, if necessary, would have spoken with the police*."[36]

24.29 There was a clear failure by the Countess, through Mr Cross, to discharge the duty of candour to the coroner. Mr Cross was the point of contact and knew about the paediatricians' suspicions and concerns.

24.30 On 4 August 2016, the solicitors acting for Parents A and B wrote to Mr Rheinberg, expressing their frustration at the current duration of the coronial investigation and seeking a date for the inquest.[37] On 11 August 2016, Mr Rheinberg replied in the following terms: "*I share your frustration and that of your clients. Of particular importance to the inquest is the Serious Untoward Incident report which although promised has yet to be completed. In addition two critical statements are outstanding*."[38]

24.31 There was no Serious Untoward Incident report into the death of Baby A. There was, however, the Thematic Review that Mr Harvey had described as the equivalent of a Serious Untoward Incident report.

24.32 On the same day, Mr Rheinberg wrote to Mr Cross:

> "*My Officer has been chasing up missing statements in relation to the above Inquest together with a copy of the Serious Untoward Incident review. Of late I understand that enquiries have been ignored and bearing in mind the fact that it is now over a year ago since this poor infant died I am prepared to wait no longer*."[39]

24.33 Mr Rheinberg required from Mr Cross witness statements from two further junior doctors involved in Baby A's care as well as a copy of any root cause analysis or Serious Untoward Incident report.

24.34 Mr Cross replied the next day, explaining that: the statements of three junior doctors – Dr Teresa MacCarrick (a paediatric specialty trainee), Dr Harkness and Dr Ogden – had now been sent; that a copy of an obstetric secondary review undertaken would also be sent; and that he had asked Mr Harvey to let him know of any other reviews that had been undertaken.[40]

24.35 On 19 August 2016, Mr Joshua Swash, a band 3 legal services assistant at the Countess, sent to the coroner's office a copy of the obstetric secondary review and a version of the Thematic Review in which the details of babies other than Baby A had been removed.[41] The version of the Thematic Review provided to the coroner identified within its listed themes: (a) that babies had suddenly and unexpectedly deteriorated, with no clear cause on post-mortem; and (b) that six out of the nine babies who had died had arrests between midnight and 4am.[42]

24.36 The coroner was also sent the first page from Dr Brearey's 1 July 2015 'Summary of cases'.[43] Whereas the original document had contained discussion of the deaths of Baby A, Baby C and Baby D, the single page sent to the coroner related to Baby A alone. Six weeks later, that single page only was sent, on 28 September 2016, by the coroner's office to the solicitors for Baby A's parents.

24.37 The solicitors for Baby A's parents wrote to Mr Rheinberg on 28 September 2016, to express their disappointment in the document received:

> "*We were of the understanding that a full investigation was taking place at the Trust regarding* [Baby A's] *death, which would result in a report, detailing the chronology of events, the issues involved, whether any errors were made, whether such errors could*

> *have caused or contributed to* [Baby A's] *death, and the lessons learned. We were told in August 2016, that this investigation was on-going and we would be provided with the Serious Untoward Incident Report.*
>
> *We therefore expected to receive, prior to the Inquest hearing, a fairly lengthy and comprehensive document, dated August or September 2016.*
>
> *We are therefore very surprised that the Trust has now provided such a short document, describing only the most superficial investigation, and one that bears the date 1st July 2015. Clearly this document is not the result of the major and detailed investigation we were told was still on-going only a few weeks ago.*"[44]

24.38 The solicitors requested an adjournment of the inquest hearing if a further, fuller report was not received by 3 October 2016.

24.39 Mr Rheinberg replied on 3 October 2016, expressing his disappointment with the documents provided by the Countess but declining to adjourn the inquest:

> "*I too was disappointed with the brevity* [of] *the report which I received. However, I have no power to order a hospital to conduct an investigation and still less give directions as to the nature and extent of any investigation that is undertaken*."[45]

24.40 Mr Rheinberg made no reference, in his letter, to the Thematic Review, which his office had received along with Dr Brearey's 2015 summary of Baby A's case on 19 August 2016. This was a significant omission. It was the Thematic Review that was being presented as the equivalent of a Serious Untoward Incident report. Mr Rheinberg seems to have been unaware that it had been received. This was a serious failing of the administration of the coroner's office. As a result, important evidence was not provided to the parents or their lawyer. When asked by the Inquiry whether there was any reason not to send the Thematic Review to the parents, Mr Rheinberg answered "*absolutely not*".[46] He also said the Thematic Review "*almost certainly*" was not in front of him at the inquest.[47] It should have been before Mr Rheinberg well before and during the inquest.

24.41 In their written closing submissions, Family Group 1 said: "*Certainly Parents A and B were not provided with the Thematic Review*."[48] I am satisfied that had their solicitors received it they would have forwarded it to Parents A and B. It is at the very least possible that, had that been done, Parents A and B, and/or their solicitor, would have taken the time to read it carefully and to note the various themes, and, importantly, ask questions about them.

24.42 On 6 October 2016, Mr Cross sent Mr Rheinberg an email notifying him that he had told Mr Moore that the RCPCH would be conducting a review of the neonatal deaths; that this review had occurred in September 2016; and that the Trust was awaiting the report.[*] [49] Mr Cross informed Mr Rheinberg that the RCPCH review team were entirely satisfied and had raised no concerns; however, they had recommended that a detailed case note review be undertaken. He told Mr Rheinberg that this review was in progress. It is not apparent that Mr Rheinberg, having been told that the review team were entirely satisfied and had raised no concerns, asked why a detailed case note review was

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* Mr Rheinberg had no recollection of being told by Mr Moore he had taken a call that was intended for him about the RCPCH review. However, he stated that Mr Moore was meticulous in that regard, and he thought Mr Moore did tell him, as he was aware that the RCPCH review was taking place.

being undertaken. That said, Mr Cross deliberately withheld the letter of instruction to Dr Hawdon, who conducted the review, which he should have provided. He said nothing about the fact that a nurse, Letby, had been moved off the unit in July 2016 either.

24.43 Mr Rheinberg confirmed in evidence that he did not receive Dr Hawdon's letter of instruction. He was referred to the section of the letter in which Mr Harvey explained to Dr Hawdon that the RCPCH "*review team agreed that the pattern of recent deaths and the mode of deterioration prior to death in some of them appeared unusual and needed further enquiry*".[50] Mr Rheinberg stated it would have been "*extremely helpful*" to have had that information available to him prior to Baby A's inquest, and it may have prompted him to obtain independent expert advice about the death.[51]

24.44 Both Mr Rheinberg and Mr Moore said that the Countess did not inform them that the consultant paediatricians had raised concerns that a member of staff may be responsible for some or all of the deaths.[52] In his witness statement, Mr Cross said he "*fully briefed the Deputy Coroner for Cheshire*".[53] I have set out at paragraph 24.28 Mr Cross's written evidence that he briefed Mr Moore fully.

The Countess's preparations for the inquest into the death of Baby A

24.45 The GMC's guidance *Good Medical Practice* (2013), in effect at the time, required doctors when giving evidence to legal proceedings not to leave out relevant information deliberately.[54]

24.46 At the time, it was standard practice for the hospital to provide all potential witnesses to an inquest with a copy of a document titled 'Guidance on Writing Statements'.[55] In written evidence, Ms Harper-Lea, as Head of Legal Services at the Countess, explained that the purpose of the guidance was "*to assist the witness with how to write a statement in an appropriate format*".[56] Ms Harper-Lea said that the guidance had been drafted prior to her appointment at the hospital.[57]

24.47 All of the witnesses in this inquest were provided with this guidance. It goes well beyond advice on the appropriate format. The first part of the advice, that statements "*should be accurate and complete*" and not "*leave out significant information*", is unobjectionable. The second, to "*avoid criticism of colleagues/other departments*" and not to "*give opinions*" but rather "*just stick to facts*",[58] is plainly objectionable. Much medical evidence is evidence of opinion, not just facts. Advising doctors to 'just stick to the facts' has the effect of causing them not to stray outside known facts. It would, and did, have the effect of discouraging the doctors from saying anything about their suspicions or concerns.

24.48 In oral evidence, Mr Louis Browne KC (who was instructed by the Countess in respect of Baby A's inquest) gave his view on the advice to avoid criticism of colleagues:

> "*I wouldn't regard that as being a sound position to take in all cases. I can understand why that might be included but if … there was a case where a clinician or a nurse was criticising a colleague, or another department, for reasons that were connected with the Inquest I would expect that matter to be addressed.*
>
> *… if a clinician or nurse was reading this document, and landed on that bullet point, then … it would not necessarily encourage openness and transparency.*"[59]

24.49 Mr Cross's note of an Executive Team meeting on 3 August 2016 recorded discussion of Baby A's inquest. The note recorded that statements "*need to* [be] *reviewed by IH* [Mr Harvey] *& AK* [Ms Kelly] – *Coroner pushing for statements. ACTION prepare* [statement] *bundle for AK IH*."[60]

24.50 In oral evidence, Mr Harvey said that he had no recollection of being sent or reviewing statements produced for the inquest.[61] He may not recall, but I see no reason why Mr Cross would not have taken the action required. In the event, there is no evidence of Ms Kelly or Mr Harvey seeking to amend the statements.

Mr Browne KC

24.51 Mr Cross instructed Mr Browne KC, then a junior barrister, to represent the Countess at the inquest on 10 October 2016, as well as to advise in conference pre-inquest on 8 September 2016 and 6 October 2016.

24.52 Mr Browne KC was adamant throughout his oral evidence that at no point was he told there was any suspicion that Baby A or any other babies in the neonatal unit had been, or might have been, deliberately harmed by a nurse.[62] Mr Browne KC said that had he been told about those suspicions: "*I would have taken action and advised in respect of it … I would have told the Trust in no uncertain terms that they must inform their own safeguarding unit of that and the police should be informed*."[63]

24.53 On 8 September 2016, Mr Browne KC attended a pre-inquest conference with Mr Swash, Dr Wood and Dr Ogden. Mr Swash took a handwritten note of the meeting. Near the top of the note was written:

> "*– Was nurse involved in Child A's care? Re: Wider review.*
>
> *– Where Child A's death fits into the sequence.*
>
> *Q. Sequence.*
>
> *Nurse*
>
> *↳ If yes, DISCLOSURE to family*
>
> *– + Spike in deaths, not just nurse = DISCLOSURE*"[64]

24.54 Mr Browne KC explained in oral evidence:

> "*I suspect that I will have been informed that there had been … reviews and that there was an investigation into a cluster of unexpected deaths and one of the matters that the hospital were considering was who was on duty at the time of those deaths and in that context, I suspect that it was said that a nurse … appeared to have been on duty at the time of some of these deaths*."[65]

24.55 Mr Browne KC said that he was "*interested*" in the implicit concern that Letby might have done something or failed to do something in respect of Baby A, and he would have wanted to know whether the nurse was involved in Baby A's care and, if so, why that was relevant.[66] Mr Browne KC accepted that there was no record of his asking Mr Swash or the doctors what the nurse was alleged to have done.[67] Mr Browne KC explained why he did not ask further, specific questions:

> "*At that stage I didn't ask any questions because I was waiting for further information as to whether it was or was not relevant.*
>
> …
>
> *Firstly, I was not instructed to consider issues more widely and with the benefit of hindsight of course the pieces of the jigsaw fit together that Letby was deliberately harming children.*
>
> *But that was not a matter that I was aware of or had ever been told. So the first reason is there was no basis for me to ask the clinicians about particular care given by a particular nurse because of a suspicion … of that nurse deliberately harming.*
>
> *But the second reason is in the context of this Inquest I had no evidence available to me to suggest that that nurse's conduct, whether viewed from a perspective of competence or from any other perspective, was called into question. I wanted to know whether that nurse had been on duty and I will have wanted to have known that so I could explore if it became relevant*."[68]

24.56 In respect of the references to disclosure in Mr Swash's note above, Mr Browne KC said:

> "*That is advice that I will have given and … it will have been based upon information that I had been provided that the wider review had identified a spike in the number of deaths on the unit and that consideration was being given as to nurses who were on duty at the time of those deaths.*
>
> *Without sight of … either the* [RCPCH] *report or the thematic review, at the time I wouldn't have been able to identify an issue regarding any particular nurse but I suspect … for the fact that I've referenced nurse or there is a reference to 'nurse' probably indicates that I had been told there was a nurse where there was consistency of – that nurse being on duty at the time of some of these neonatal deaths. And I will then have advised: well, if that nurse was on duty at the time of Child A's death, then that fact must be disclosed to the Family. And whether or not that nurse was on duty, the fact of a spike … in deaths should be disclosed to the Family*."[69]

24.57 Mr Browne KC reiterated in his oral evidence that the family of Baby A should have been given disclosure about the suspected involvement of a nurse in Baby A's death and the fact that Baby A's death was part of a cluster of unexpected deaths.[70] I agree.

24.58 Speaking generally about independent reviews into the deaths, Mother A and B said: "[T]*here needs to be given more information to the parents, because as I've stressed the whole time, we are people, and that is our child. That's our world. And I was never given an opportunity to sit in an official setting and ask questions that I wanted answered*."[71] The same applies to Baby A's inquest hearing. By not following Mr Browne KC's disclosure advice, the hospital failed to provide Parents A and B with important information about their child. As a result, no questions could be asked about it on their behalf by their lawyers at the inquest, or anywhere else. This was a serious failing.

24.59 At the bottom of Mr Swash's note, written in different ink, was a series of action points, apparently arising from the meeting. The note read:

> "*Actions:* *yes – nursing notes 09 Jun 15 17:56 09 June 15 18:23*
>
> …
>
> → *check through medical notes Re: was nurse involved in care? LUCY LETBY* ↗
>
> + *SPC* [Mr Cross] *to feedback Re: review from neonates*."[72]

24.60 In his written evidence, Mr Swash explained:

> "*I recollect being asked by Stephen Cross to examine the medical records of Child A to establish whether Lucy Letby was involved in the care of Child A. The red writing stating 'Yes' and the associated times and dates are references to the nursing notes which were attributed to Lucy Letby and therefore confirming her involvement in the care of Child A. I would have written the red part of the note on return to the Legal Services Office when I will have reviewed the medical records*."[73]

24.61 Mr Browne KC stated that his understanding was that "*Mr Swash went back to the office, checked the notes and on checking the notes, made that entry*".[74] Mr Browne KC said he had no recollection of knowing Letby's name before it was publicly reported in the media.[75] He pointed out that the dates in red in Mr Swash's notes were obviously wrong.[76] Baby A died on 8 June 2015.

24.62 On 27 September 2016, Mr Swash emailed Mr Browne KC, attaching the hospital policies mentioned in witness statements for the inquest, and asking when Mr Browne KC would be available for a pre-inquest telephone conference. Mr Swash also wrote:

> "*Finally, following on from our conversation prior to the pre-inquest meeting on the 8th of September surrounding the nurses involvement in the care of* [Baby A]*, having investigated the records, I can confirm she was involved in the care of* [Baby A]*. Stephen* [Cross] *has suggested that it would be helpful if he could have a conversation with you regarding this issue this week if possible?*"[77]

24.63 Mr Swash forwarded this email to Mr Harvey the same day, adding: "[Y]*ou will note that the nurse that has recently been moved out of the neonatal unit was involved in the care of* [Baby A]*. You will also note that Stephen Is going to speak with counsel about disclosure to the Coroner on this matter*."[78]

24.64 In oral evidence, Mr Browne KC said that he did not recall being told by Mr Cross, or by anybody else at the hospital, that the nurse had been moved out of the neonatal unit.[79] Mr Browne KC stated: "*Had I been told that, it would have led me to ask questions about it because it would have been important*."[80] Mr Browne KC also did not recall having a conversation with Mr Cross about disclosure. He said: "[I]*f we had a conversation about the issue of disclosure I would have reminded him what I had already said that both the Family and the Coroner should be informed*."[81]

24.65 The same day, Mr Browne KC was sent a number of documents, including the Thematic Review. Mr Browne KC could not remember which version of the Thematic Review he was sent. It was suggested to him that the Thematic Review made it obvious that Letby was the nurse concerned and that she was potentially connected with Baby A's and a number of other deaths. Mr Browne KC responded:

> "*I was not instructed to enquire whether any one or other of these nurses was deliberately harming babies. Had I been told that there was any suspicion of that, my approach to advising and representing the hospital would have been fundamentally different.*
>
> *What I will have taken in part from this Thematic Review, that there's no suggestion of criminality on the part of any person in the review. It specifically says as I recall there was no unifying theme to explain the spikes in deaths. So I will have read all of that. I won't have focused on appendix 1 and looked at that name and thought: well, there may well be some issue there. I was being reassured that there was not an issue about the level of care provided by this nurse or any nurse and nobody indicated that there was a suspicion that that named nurse was deliberately harming babies*."[82]

24.66 On 6 October 2016, Mr Browne KC had a telephone conference with Mr Cross, Mr Swash, Dr Jayaram, Dr Saladi, Dr Harkness and Dr MacCarrick. Again, Mr Swash took a handwritten note.[83] Part of the note recorded: "*Still to this day Ravi* [Jayaram] *doesn't know why this happened. In 27 years in paediatrics, never seen this kind of situation*."[84]

24.67 Under the heading 'Dr Saladi', the note also recorded:

> "*- If review is outside the remit of your knowledge, then say so.*
>
> *↳ Don't say anything unless you know. REVIEW IS ONGOING.*"[85]

24.68 When asked why he did not expressly ask the doctors whether they were concerned about the nursing care provided to Baby A, Mr Browne KC answered:

> "[B]*y virtue of the fact that Dr Jayaram had gone through, if I might put it this way, a checklist of potential causes of a sudden unexpected deterioration, and hadn't highlighted any issue about any other matter on the unit, I wouldn't have felt it necessary to explore with him whether* [he] *felt that a failure of one or more nurses might have been contributory to Child A's death.*"[86]

24.69 Mr Browne KC said that at no stage did Dr Jayaram explain he had any concerns or suspicions.[87] Mr Browne KC said he could not understand why he had not been told by anybody at the hospital that the consultants were concerned that Letby had killed Baby A.[88] Further, Mr Browne KC said:

> "[T]*here was other material that* [Dr Jayaram] *didn't tell me that I have now found out that might have been a matter that I would have wished to have explored.*
>
> *So, for example, issues of discolouration of the child's body. I would have wanted to have known.*"[89]

24.70 The same day, Mr Cross emailed Mr Rheinberg. He advised Mr Rheinberg that the hospital was awaiting the RCPCH report. Mr Cross stated that the RCPCH review team had "*indicated that they were entirely satisfied with the care within the neonatal unit and raised no concerns*". Mr Cross also said that the RCPCH had recommended that a detailed forensic case note review of each of the deaths from July 2015 should be undertaken, and this was a work in progress.[90]

24.71 Mr Cross thus failed in the run-up to Baby A's inquest to provide information and documents to the coroner that would have made the coroner aware of the concerns raised by the consultant paediatricians that Letby may have murdered Baby A.

24.72 Mr Cross forwarded to Mr Browne KC and Dr Jayaram a copy of the email to Mr Rheinberg, about the RCPCH report, along with a copy of Dr Hawdon's letter of instruction.[91] Mr Browne KC was explicitly told that the letter of instruction had not been sent on to Mr Rheinberg. That letter said, among other things:

> "*The* [RCPCH] *review team agreed that the pattern of recent deaths and the mode of deterioration prior to death in some of them appeared unusual and needed further enquiry to try to explain the cluster of deaths … They recommended a detailed forensic case note review of each of the deaths from July 2015 be undertaken. They also recommended that the investigation should include as a minimum the following elements:*
>
> …
>
> *(d) details of all staff with access to the unit from 4 hours before the death of each infant. Ancillary and facilities staff should be included.*"[92]

24.73 Mr Browne KC was asked whether the contents of the letter of instruction had caused him any concern or raised any alerts with him. He said:

> "*Absolutely not. No. I will have read subparagraph D in the context of the Thematic Review that I had been provided with by that date and from recollection I think one of the recommendations of the Thematic Review was that there should be a consideration of precisely that issue.*
>
> …
>
> *I would have interpreted forensic in the context of a letter of instruction as meaning a thorough review, having regard to the records within the scope of what* [Dr Hawdon] *was being asked to do as the expert. But again, this was not a matter that I was being asked to consider expressly. It was part of the wider picture as to what the Trust were doing to deal with the findings as I understood them to be of the* [RCPCH]."[93]

24.74 It was also suggested to Mr Browne KC that the letter of instruction, which he knew had not been sent to the coroner, ought to have been provided to Mr Rheinberg.[94] There was no question that it ought to have been. Mr Browne KC said that the disclosure obligation fell on the hospital and not him.[95] The Countess accepted this in their closing submissions.

24.75 Mr Rheinberg said that the letter of instruction contained the type of information he needed to know when approaching Baby A's inquest:

> "[I]*t would be extremely helpful. I think one of the things that wasn't undertaken in relation to Child A's Inquest was an independent examination of evidence by an expert instructed by me.*
>
> *The more information that I had raising questions, might well have prompted me to get independent expert advice*."[96]

24.76 Mr Rheinberg agreed that the characterisation of the mode of deterioration as unusual and the cluster of deaths were factors of concern and something to be investigated.[97]

24.77 It is important, however, to remember the information that Mr Rheinberg or his office had received by the date of Baby A's inquest:

a. Mr Rheinberg was aware of the cluster of deaths in June 2015, which Baby A was part of. Mr Rheinberg said he discussed this cluster with Ms Hurst contemporaneously.[98]

b. The record of Dr Newby's report of Baby D's death to the coroner's office in June 2015 noted: "*CPR for previous twin death; surviving twin had successful CPR*."[99] Though not explicit, this was a reference to the fact that Baby A's twin, Baby B, had also suffered a collapse.

c. The deaths of Baby A, Baby C, Baby D, Baby E, Baby I, Baby O and Baby P had all been reported to the coroner.

d. Mr Rheinberg had been told that the neonatal unit at the Countess had been downgraded, due to an increase in neonatal deaths.[100]

e. Mr Rheinberg was aware that the RCPCH was conducting a review at the hospital, due to the increase in neonatal deaths. He had also been told that the review team were satisfied and had raised no concerns but had recommended that a case note review be conducted.

f. As set out earlier, the coroner's office had received an edited copy of the Thematic Review. It identified a common theme that: "*Some of the babies suddenly and unexpectedly deteriorated and there was no clear cause for the deterioration/death identified at PM* [post-mortem]."[101] Baby A was, of course, one of the babies in this category, and this suggested that Baby A's death had at least one feature in common with other babies who had died at the neonatal unit.

g. The coroner's office had been told that a forensic case note review of deaths had been recommended by the RCPCH and was under way.[102]

24.78 Dr Jayaram, to whom Mr Cross's email of 6 October 2016 had also been forwarded, said he took the correspondence sent to Mr Rheinberg to mean that the coroner had been told of the consultants' concerns regarding Letby. Dr Jayaram said in oral evidence:

> "*So my impression from this, although I note that Stephen Cross says he hasn't sent the letter of instruction to the Coroner, was that the Coroner, number one, was aware of our specific concern and that's a big assumption, because reading this, it doesn't specifically say that but –*
>
> …
>
> *Aware of our specific concern about an individual.*
>
> *The Coroner was also aware that very detailed forensic level reviews were going on and so my … understanding at the time of the Inquest is that the Coroner was already aware of the concerns that we had*."[103]

This was a reasonable assumption. It was Mr Cross's evidence that I do not accept, that he had fully briefed the coroner. It is likely that he had said as much to the paediatricians.

24.79 In fact, on 6 October 2016, Mr Cross had emailed multiple recipients (including Mr Harvey, Ms Kelly and Ms Hodkinson) and copied in Mr Swash. In the body of the email, Mr Cross stated in respect of Baby A's inquest:

> "*6 of our clinicians are giving evidence at the inquest and I have provided support and held case conferences with the clinicians and all the necessary preparations have been completed.*
>
> *The Coroner is aware of the review undertaken by the RCPCH and the necessary disclosures have been made*."[104]

24.80 Whilst the coroner was aware that the RCPCH had undertaken a review of an increase in neonatal deaths, he was not aware that there were suspicions and concerns about a member of staff harming babies. Reference to 'necessary disclosures' did not include this crucial information. The use of the phrase 'necessary disclosures' may have led others to believe that it did.

The inquest hearing

24.81 The inquest was held on 10 October 2016. The Inquiry has been provided with three notes of the hearing: an attendance note produced by the solicitors of Parents A and B; an attendance note produced by the Countess; and the handwritten notes of Mr Swash.

24.82 Parents A and B, Dr Jayaram, Dr Saladi, Dr Ogden, Dr Harkness, Dr Wood, Dr MacCarrick and Dr Shukla were each called to give oral evidence.

24.83 The focus of the inquest's oral evidence was on the potential relevance of the long line. Dr Shukla is recorded in the attendance note produced by the solicitors of Parents A and B as saying in oral evidence that there was nothing that he could identify to link the death with the long line. He is recorded as going on to say that they "*had not found*

anything to suggest a natural disease but then there was no evidence that there had been anything unnatural either" and that "*it would be very difficult for him to conclude that it was more likely than not natural causes because there was no evidence of it either way*".[105]

24.84 After Dr Shukla gave evidence, Dr Jayaram was recalled "*to try and assist with his paediatric knowledge of the circumstances in … concluding with any kind of cause of death*".[106] The careful and detailed attendance note made by the solicitors of Parents A and B recorded the evidence he gave as follows:

> "*Mr Rheinberg asked Dr* [Jayaram] *whether or not he had seen anything similar. Dr* [Jayaram] *confirmed that normally death in neonates is the end point in a course of events and normally they can be resuscitated. He confirmed that there have been similar cases of neonates dying in similar circumstances on the unit which they have not been able to explain. He confirmed that they have therefore downgraded the unit so that* [they] *do not care currently for preterm babies and they have also requested an independent review and they are still awaiting the formal report. However the initial feedback from this is that nothing can be found that is wrong with any of the training, any of the practises or any of the equipment. However there is a potential issue with staffing. As far as Dr* [Jayaram] *is aware this report is then to go back to the Executive Board and they decide whether or not to release it to the public. Mr Rheinberg asked whether or not it would be possible for the family to receive a copy. Dr* [Jayaram] *said he is of the personal view that it should be made available for the public and he would have no issue with a copy of it being provided to the family, however as he pointed out it is the Executive Board's decision. He has to confirm however that the events that happened to Child A do not make any clinical sense to him at all.*"[107]

24.85 Dr Jayaram said in oral evidence that he thought the note accurately recorded what he had said.[108]

24.86 Neither Dr Jayaram nor Dr Saladi explicitly mentioned their concerns or suspicions about Letby. Dr Jayaram did refer to "*staffing*". Mr Rheinberg may not have realised that this word was used as a euphemism for a member of staff, in this case a nurse, as Dr Jayaram confirmed in evidence. Mr Rheinberg expressed dismay that neither Dr Jayaram nor Dr Saladi informed him about their concerns about the nurse.[109]

24.87 Mother A and B was asked how she felt regarding the concerns about Letby not being verbalised in Baby A's inquest hearing. She replied: "*Very, very concerned. At the inquest, we had no idea, and from the trial we know that by that time they did suspect her, but nobody mentioned it, not once, and they should have*."[110]

24.88 Dr Jayaram accepted in oral evidence that he should have told the coroner that a member of staff may have been responsible for Baby A's death.[111]

24.89 Dr Jayaram said that he understood that the neonatal deaths, bar one, had been referred to the coroner.[112] Dr Jayaram said he was "*cognisant of the fact we had been told: do not speculate*"[113] and he had the impression, reasonably, that the consultants' specific concerns regarding Letby were "*on the Coroner's radar*".[114] Dr Jayaram also said that he had tried to make it clear to the coroner that he did not understand what was going on, could not think of a clinical explanation, and that "*there had been other things like this*

as well".[115] Dr Jayaram said the reference in his evidence at the inquest to "*a potential issue with staffing*" was an "*oblique reference to an individual member of staff*".[116] Finally, Dr Jayaram explained his omission as a lack of courage:

> "[Y]*ou are absolutely correct, you know, I am there in a Coroner's court and I should say what I think and again I didn't.*
>
> *I think … and I … reach out to Baby A and B's parents for this, and maybe I should have done a supplementary statement or talked afterwards – I was trying to in my discussion about the fact I couldn't explain this, you know, in the context that my understanding was that the Coroner knew of our concerns I was trying to sort of throw as many breadcrumbs as possible for the Coroner to pick up without explicitly saying what the suspicion was.*
>
> *Why did I do that? And again – and I appreciate that this was the wrong judgment – I had Baby A's parents sitting 10 feet away from me and, yes, absolutely, duty of candour.*
>
> *I just didn't have the courage to say it and I think part of this, part of this was already from … I guess being influenced by the … pushback that we were getting that, 'There's nothing to see here' and … I regret not explicitly saying that then on many, many levels because it should have been said … and I am not going to make excuses. I did have an understanding that the Coroner was aware of our concerns because I interpreted that's what Stephen Cross had told the Coroner and I should have done better*."[117]

24.90 Dr Saladi also accepted it was a mistake not to have shared his concerns about Letby with the coroner.[118] Like Dr Jayaram, Dr Saladi also explained his omission by reference to advice he had received about giving evidence and the knowledge he believed the coroner already had. Dr Saladi also relied on his inexperience at giving evidence at inquests. Dr Saladi said:

> "[T]*hat was probably my first, maybe first or second appearance of Inquest and I was stressed and advice we got from the solicitors was answer the questions, what is asked, don't answer what you think was asked and keep it brief and do not speculate.*
>
> *So if the Coroner has asked me, I would have probably said. But because it wasn't asked, because what I didn't know is what is speculation at that stage …*
>
> *… as far as I am aware, Coroner is aware of the deaths. I didn't need to tell them …*
>
> *… because we were having these unexpected deaths, we were referring them to the Coroner.*
>
> …
>
> *I was thinking whether rightly or wrongly that I was answering in relation to Baby A my involvement, if I had any suspicion, and my answers were brief and to that point, what he asked.*
>
> *It is probably in retrospect mistake for me to not share my concerns but that is because of inexperience, I think.*"[119]

24.91 Mr Rheinberg thought that if the concerns had come out at the inquest he would have adjourned: "*I wouldn't have gone on any further, and probably sought police involvement*."[120]

24.92 Mr Rheinberg said that he would have expected Mr Cross or someone else to inform him before the inquest of the concerns regarding Letby, the unusual characteristics of Baby A's death and Baby B's subsequent collapse.[121] Mr Rheinberg said it was "*horribly disappointing*" that he was not made aware of these matters.[122] Mr Rheinberg explained:

> "*It was all within the … ethos of the … Sudden Unexpected Infant Death protocol, that we should approach all these tragedies not just in our own ivory towers; that we should share all information because we might individually have pieces of the picture to put together.*
>
> *So police, hospital, everybody that has anything to add should add to the discussion, as I say, to complete a full picture* [of] *what … has happened.*
>
> …
>
> [It is] *very, very disappointing that relevant information is not shared.*
>
> …
>
> *But quite outside the protocol, that … should be the case in any event. I am holding inquests. I need information. I am holding investigations. I need people to be forthcoming with information*."[123]

24.93 Against that background, I return to the failure of the coroner's office to provide the Thematic Review to Parents A and B. Had it been provided, as it should have been, it is likely that it would have prompted questions from their lawyers. That would also have alerted the coroner to the fact that there was a document missing from his papers and, in all likelihood, would have prompted him to ask some questions about the findings of the Thematic Review. In light of Dr Jayaram's "*breadcrumbs*", the coroner may have asked some more probing questions, which might have obtained answers which might have caused him to adjourn. But the Thematic Review was not sent, and I cannot say with confidence what would have been done had it been sent.

24.94 In the event, the inquest of Baby A ended with a narrative conclusion: "*It cannot be determined what caused Child A's collapse and subsequent death and further it cannot be determined whether this was due to a natural or unnatural event*."[124]

24.95 Mother A and B expressed her frustration about the inquest in oral evidence. She said: "*I felt like it was a waste of time, if I was being completely honest, because nothing came of it*."[125] She was right about that.

Further observations on the inquest for Baby A

24.96 Dr Jayaram told the coroner in evidence that there had been similar cases of neonates dying in similar circumstances on the neonatal unit, which the consultant paediatricians had been unable to explain. This was alarming. As Dr Hawdon said in her evidence to the Inquiry, neonates dying without explanation is worrying. That Mr Rheinberg did not seek to explore this further suggests that he did not pick up on the worrying nature of Dr Jayaram's inquest evidence, presumably because this was outside his area of expertise.

24.97 Mr Rheinberg acknowledged in evidence that he knew about the cluster of three neonatal deaths in June 2015 and the general increase in mortality at the Countess; and he had been concerned by the deaths of Baby O and Baby P. By the time of the inquest for Baby A, he had been told of the RCPCH review (September 2016) and the recommendation for a forensic case note review (October 2016).[126]

24.98 In its closing submissions, the Countess accepted that the obligation to make disclosure to the coroner ultimately rested with the hospital.[127] Mr Browne KC had advised the hospital in conference, in September 2016, to disclose to the coroner any involvement of the nurse concerned in the care of Baby A.[128] Mr Browne KC said in oral evidence: "[H]*aving worked with Mr Cross on a number of occasions, there had not been an occasion to my memory where he had failed to follow my advice*."[129] Mr Browne KC's error may have been in assuming that Mr Cross would follow his advice on this occasion.

24.99 Mr Cross's notebook entries are referred to in his supplementary statement. Following Baby A's inquest, there was a discussion between Mr Cross and Mr Swash. Mr Cross records the discussion as follows: "*Narrative Verdict 'Unascertained' No negative comments No press.*"[130] Mr Cross's handwritten notes dated 10 October 2016 record that he briefed "*AK* [Ms Kelly] … *re outcome of* [Baby A] *Inq- Narrative conclusion. No adverse comments. No press present.*"[131] He also had a meeting with the Medical Director, Mr Harvey, to discuss the outcome of the inquest.[132] It is readily apparent that Mr Cross and the executives were following the events and any press interest with care. The notes make no reference to any discussion about, or concern for, the parents of Baby A. The obvious concern of Mr Cross and the executives about "*negative comments*" or "*press*" suggests that the prevention of reputational damage to the hospital was prized more highly than the transparency and inquisitorial function of the coronial process.

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24.100 On 8 February 2017, Mr Rheinberg met Mr Harvey and Mr Cross. Mr Rheinberg had no recollection of the meeting, but a note was taken by Mr Cross.[133] The note recorded that Mr Rheinberg asked whether anything had "*come out*" of the investigations undertaken. Mr Harvey was noted to reply: "*No theme has emerged.*" The note also recorded Mr Rheinberg as saying that the hospital had done the right thing, but Mr Rheinberg was unable to recall what he had meant by this.[134] In his witness statement, Mr Cross said that Mr Harvey "*fully briefed the Coroner on the neonatal matters to date*".[135] Again, Mr Cross did not explain what a full brief involved. Mr Rheinberg's evidence was that he was not told of any suspicions or concerns relating to the involvement of a nurse in the deaths of babies at the hospital.[136] Given the way matters developed, this is likely to be correct. Mr Cross should have told Mr Rheinberg.

24.101 On 15 February 2017, Mr Rheinberg and Mr Moore met Mr Harvey and Mr Cross. During the course of the meeting, Mr Rheinberg was handed a letter which enclosed: (a) a document comprising the portions of the RCPCH report that had been redacted for the dissemination version; (b) Dr Hawdon's report; and (c) the 10 February 2017 letter from the consultant paediatricians to Mr Chambers.[137]

24.102 Mr Rheinberg could not recall the meeting, but did take a note of it.[138] The note recorded discussion of Dr Hawdon's report and the consultant paediatricians' letter. The bulk of the meeting appeared to have involved discussion of the request that the coroner undertake an investigation into the deaths and unexpected collapses that had occurred

on the neonatal unit. In oral evidence, Mr Rheinberg summarised his response to that as follows:

> "*I … got the idea that what the Consultants wanted was* [for] *me to have an overall review of all the cases to see if there were any … mistakes or any common themes or whatever. And I was explaining that … I don't have jurisdiction … to do that, no authority to carry out a general review, as it were, only to hold an Inquest into a specific death.*"[139]

24.103 Mr Rheinberg's note contained no reference to the document containing the extracts from the unredacted RCPCH report. Mr Rheinberg explained in his witness statement:

> "*I am confident that the document which is dated November 2016 was not referred to at the meeting. If it had been the subject of any discussion, I would have made reference to this in my note. The first time that I saw this note was when I received the bundle of relevant papers in order to assist in the preparation of my statement of evidence to the inquiry. There are two possible explanations for the document not coming to my attention. The first is that it was not in fact included with the other papers accompanying the letter of 15th February 2017. The other and more likely explanation is that it was included but that I overlooked it. Although the note is less than an accusation of wrongdoing the suggestion that one nurse had been 'rostered on shift for all the deaths' would have prompted me to ask for the identity of the nurse in question and I believe that I would probably have spoken to DI Mark Tasker or another police officer and have asked if the officer was aware of the fact that, coincident with all the deaths, was the presence on each occasion of one nurse.*"[140]

24.104 In oral evidence, Mr Rheinberg repeated that he had not read the RCPCH report extracts during the meeting and there was no reference to that document during the meeting.[141] Mr Rheinberg accepted, however, that it would have been a fair assumption for Mr Harvey and Mr Cross to make that he had and could look at the information contained in the bundle of documents provided to him.[142] He did not look at the other documents and did not tell Mr Harvey or Mr Cross that he would not do so.

24.105 The final paragraph in Mr Rheinberg's note of the meeting on 15 February 2017 recorded a question asked by Mr Moore: "*AGM* [Mr Moore] *asked what the clinicians hoped to achieve by seeking Inquests and wondered whether there were reputational motives* [for] *there being no right of 'appeal' from the Royal College's findings*."[143]

24.106 Mr Moore was asked in oral evidence what he meant by this. He explained:

> "[T]*his is a letter from the Consultants to the … Chief Executive at the hospital and I thought this is somewhat unusual. Why would the Consultants be writing to the Chief Executive? So in my own mind I am asking the question: what could be the motivation behind that? There must be some reason. And I asked that question at the meeting. And I … think … I asked: is it perhaps they have suffered reputationally from something in that report which of course I hadn't seen.*"[144]

24.107 When it was suggested that one interpretation of the note was that Mr Moore was ascribing to the consultant paediatricians a bad faith motivation of concern about self-reputation, Mr Moore denied that that was what he had meant.[145] He suggested he meant the opposite. Mr Rheinberg, however, working on the basis of his note rather than his

recollection, did interpret Mr Moore as saying just that: "*By that I suppose he meant, was it a question of trying to deflect blame and find some other factor that could exonerate.*"[146] Given the state of the evidence I make no finding. I say only that there was no basis upon which bad faith could have been attributed to the paediatricians in writing the letter. What they were asking for was a review of the cases by the coroner. That would be an odd request to make in bad faith.

24.108 Both Mr Moore and Mr Rheinberg were adamant that at no point during the meeting were they told of any suspicions or concerns relating to the involvement of a nurse in the deaths of babies at the hospital.[147] Mr Moore said in oral evidence: "*There was no mention whatsoever of anything of that kind. If there had been, the outcome of this meeting would have been very different, I assure you*."[148]

24.109 Mr Rheinberg's note of the meeting did not record mention of any such concerns. Similarly, Mr Moore's email of 3 May 2017 to Detective Superintendent Aaron Duggan, recalling the meeting of 15 February 2017, also made no mention of Mr Rheinberg and Mr Moore being told about concerns regarding a member of staff.[149]

24.110 Mr Harvey differed in his recollection. In his oral evidence, Mr Harvey said of the meeting on 15 February 2017:

> "*I recall that either Mr Cross or I, in passing the paediatricians' letter across to Mr Rheinberg explained the background to the letter and the paediatricians' concerns. I am also aware that there is documentation within the Inquiry that confirms part of the bundle that Mr Rheinberg received was actually the full RCPCH report, which included reference to the paediatricians' concerns.*"[150]

24.111 Mr Harvey said there was absolutely no doubt in his mind that such a conversation took place.[151] Again, Mr Cross's written evidence was that Mr Harvey "*fully briefed the Coroner on all matters*", without explanation of what a full brief involved.[152] I think it unlikely that had Mr Rheinberg been told of the concerns about the nurse he would not have written it down. However, in handing over the redacted report, albeit buried amongst other documents, Mr Harvey and Mr Cross were able to say, as they did, to the paediatricians and others, that they had briefed the coroner about all matters.

24.112 Given the unusual nature of the visit from Mr Harvey and Mr Cross and the unusual letter to Mr Chambers from the consultants, it is surprising that Mr Rheinberg did not read the sections of the unredacted report. Had he done so he would have realised immediately that until that stage he had not been given the full story and had been given no clue that this was the case. Realising this was the case would have caused him irritation and consternation. He would then, I have no doubt, acted in accordance with his normal process and contacted the police.

24.113 I recognise that Mr Rheinberg was under enormous pressure of work at that time and was about to retire. He had made it very clear to Mr Harvey and Mr Cross that he would not be carrying out the investigation being asked for. At that point Mr Harvey and Mr Cross should have pointed out the additional material in the bundle and explained why the consultants were making the request in the letter.

24.114 Mr Rheinberg retired on 10 March 2017 and Mr Moore became Senior Coroner the same day.[153]

Endnotes

1 Coroners and Justice Act 2009 (https://www.legislation.gov.uk/ukpga/2009/25/contents)

2 Coroners and Justice Act 2009, section 14 (https://www.legislation.gov.uk/ukpga/2009/25/section/14)

3 INQ0017840/3

4 INQ0002042/4

5 INQ0002042/9

6 Nicholas Rheinberg 6 December 2024 14/17-18

7 INQ0002047/3-5; INQ0002047/11

8 Dr John Gibbs 1 October 2025 65/9 to 66/12

9 Dr John Gibbs 1 October 2024 123/12-23

10 Dr John Gibbs 1 October 2024 98/13 to 99/5

11 Nicholas Rheinberg 6 December 2025 90/3-7

12 Nicholas Rheinberg 6 December 2024 91/2-11

13 Dr John Gibbs 1 October 2024 66/11-12

14 INQ0002045/4

15 INQ0002045/8

16 Nicholas Rheinberg 6 December 2024 100/19-21

17 Nicholas Rheinberg 6 December 2024 29/14-22

18 Alan Moore 4 December 2024 116/18 to 119/4

19 Dr ZA 7 October 2024 24/6 to 26/21

20 INQ0002043/3

21 INQ0002046/3; INQ0002044/3

22 Alan Moore 4 December 2024 118/6-8

23 INQ0008927/7

24 Mother A and B 16 September 2024 17/1-24

25 INQ0008927/7

26 Dr Stephen Brearey 19 November 2024 204/21 to 205/3

27 INQ0003089/1-2

28 INQ0008927/5

29 Witness statement of Nicholas Rheinberg INQ0017842/26/para 84

30 Nicholas Rheinberg 6 December 2024 33/23-25

31 Witness statement of Nicholas Rheinberg INQ0017842/23/para 74

32 Witness statement of Nicholas Rheinberg INQ0017842/26/para 84

33 Alan Moore 4 December 2024 119/8-22

34 Alan Moore 4 December 2024 120/5-12
35 Witness statement of Stephen Cross INQ0108952/4/para 19
36 Alan Moore 4 December 2024 120/25 to 121/3
37 INQ0002042/174
38 INQ0002042/173
39 INQ0002042/167
40 INQ0002042/186
41 INQ0050707; INQ0008841
42 INQ0008841/3
43 INQ0002042/777
44 INQ0002042/155
45 INQ0002042/154
46 Nicholas Rheinberg 6 December 2024 105/16-18
47 Nicholas Rheinberg 6 December 2024 107/17-19
48 Written Closing Submissions on Behalf of Family Group 1 4 March 2025 71/footnote 237
49 INQ0053069
50 INQ0012066/1
51 Nicholas Rheinberg 6 December 2024 68/12-18
52 Nicholas Rheinberg 6 December 2024 23/18 to 24/6; Alan Moore 4 December 2024 120/13-20
53 Witness statement of Stephen Cross INQ0107707/19/para 66
54 INQ0007314/25
55 INQ0008638/1-3
56 Witness statement of Sarah Harper-Lea INQ0108403/2/para 5
57 Witness statement of Sarah Harper-Lea INQ0108403/1/para 2
58 INQ0008638/1-3
59 Louis Browne KC 4 December 2024 16/23 to 17/13
60 INQ0007197/138
61 Ian Harvey 28 November 2024 124/24 to 125/3
62 Louis Browne KC 4 December 2024 9/3-7
63 Louis Browne KC 4 December 2024 9/9-18
64 INQ0108406/3
65 Louis Browne KC 4 December 2024 45/23 to 46/5
66 Louis Browne KC 4 December 2024 46/6 to 47/6
67 Louis Browne KC 4 December 2024 48/2-8

68 Louis Browne KC 4 December 2024 47/19 to 48/1 and 52/16 to 53/7
69 Louis Browne KC 4 December 2024 12/20 to 13/14
70 Louis Browne KC 4 December 2024 13/9-14, 49/22-25 and 55/4-15
71 Mother A and B 16 September 2024 45/7-12
72 INQ0108406/7
73 Witness statement of Joshua Swash INQ0108481/6/para 24
74 Louis Browne KC 4 December 2024 77/1-5
75 Louis Browne KC 4 December 2024 38/25 to 39/7
76 Louis Browne KC 4 December 2024 25/9-11
77 INQ0052593/2
78 INQ0052593/1
79 Louis Browne KC 4 December 2024 28/14-17
80 Louis Browne KC 4 December 2024 28/18-19
81 Louis Browne KC 4 December 2024 55/21-24
82 Louis Browne KC 4 December 2024 58/16 to 59/7
83 INQ0108406/9-12
84 INQ0108406/11
85 INQ0108406/12
86 Louis Browne KC 4 December 2024 61/15-22
87 Louis Browne KC 4 December 2024 61/24-25
88 Louis Browne KC 4 December 2024 62/21-22
89 Louis Browne KC 4 December 2024 62/1-5
90 INQ0053069
91 INQ0053069
92 INQ0012066
93 Louis Browne KC 4 December 2024 37/19-24 and 40/2-10
94 Louis Browne KC 4 December 2024 66/8-14
95 Louis Browne KC 4 December 2024 66/17-19
96 Nicholas Rheinberg 6 December 2024 68/12-15
97 Nicholas Rheinberg 6 December 2024 68/19 to 69/1
98 Nicholas Rheinberg 6 December 2024 99/21 to 100/10
99 INQ0002045/8
100 INQ0107909/8
101 INQ0008841/3
102 INQ0053069

103 Dr Ravi Jayaram 13 November 2024 65/19-24 and 66/3-8. Also see Dr Ravi Jayaram 13 November 2024 239/9-13

104 INQ0108480/17

105 INQ0107909/8

106 INQ0107909/8

107 INQ0107909/8

108 Dr Ravi Jayaram 13 November 2024 68/7-11

109 Nicholas Rheinberg 6 December 2024 72/2-18

110 Mother A and B 16 September 2024 52/7-10

111 Dr Ravi Jayaram 13 November 2024 246/11-14

112 Dr Ravi Jayaram 13 November 2024 68/17-19

113 Dr Ravi Jayaram 13 November 2024 68/19-20

114 Dr Ravi Jayaram 13 November 2024 69/7-9

115 Dr Ravi Jayaram 13 November 2024 68/24 to 69/2

116 Dr Ravi Jayaram 13 November 2024 244/12 and 244/21-22

117 Dr Ravi Jayaram 13 November 2024 245/9 to 246/10

118 Dr Murthy Saladi 3 October 2024 118/14-16

119 Dr Murthy Saladi 3 October 2024 117/3 to 118/16

120 Nicholas Rheinberg 6 December 2024 72/11-13

121 Nicholas Rheinberg 6 December 2024 60/12-15

122 Nicholas Rheinberg 6 December 2024 60/8

123 Nicholas Rheinberg 6 December 2024 60/15-23, 61/9-10 and 62/18-22

124 Nicholas Rheinberg 6 December 2026 25/22-25

125 Mother A and B 16 September 2024 25/9-11

126 Nicholas Rheinberg 6 December 2024 24/1-14 and 33/7-25

127 Written Closing Submissions on Behalf of the Countess of Chester Hospital NHS Foundation Trust 4 March 2025 58/para 227

128 INQ0108406/3

129 Louis Browne KC 4 December 2024 50/5-7

130 INQ0108953/5

131 INQ0108953/6

132 INQ0108953/6

133 Nicholas Rheinberg 6 December 2024 45/17-20; INQ0106817/34

134 Nicholas Rheinberg 6 December 2024 108/15-23

135 Witness statement of Stephen Cross INQ0107707/42 para 160

136 Witness statement of Nicholas Rheinberg INQ0017842/32 para 100

137 INQ0002048/34; Witness statement of Nicholas Rheinberg INQ0017842/30 para 95

138 INQ0002048/102

139 Nicholas Rheinberg 6 December 2024 57/9-15

140 Witness statement of Nicholas Rheinberg INQ0017842/32 para 98

141 Nicholas Rheinberg 6 December 2026 99/10-11 and 112/2-9

142 Nicholas Rheinberg 6 December 2024 58/2-7

143 INQ002048/102

144 Alan Moore 4 December 2024 132/8-17

145 Alan Moore 4 December 2024 133/18-23

146 Nicholas Rheinberg 6 December 2024 111/15-18

147 Witness statement of Nicholas Rheinberg INQ0017842/32 para 100

148 Alan Moore 4 December 2024 136/22-24

149 INQ0002048/110

150 Ian Harvey 29 November 2024 117/17-24

151 Ian Harvey 29 November 2024 219/15-17

152 Witness statement of Stephen Cross INQ0107707/44 para 168

153 Alan Moore 4 December 2024 108/10-12

Chapter 25. NHS England and the Countess

Background

25.1 During the period 2015 to 2017, certain functions that currently sit within NHS England were split between different organisations:

a. Until 1 April 2016, Monitor was the independent regulator of NHS Foundation Trusts. It licensed them and enforced the conditions of their licences.[1] (The NHS Trust Development Authority provided a similar regulatory function for Non-Foundation Trusts.)

b. From 1 April 2016, the functions of Monitor and the NHS Trust Development Authority were brought together to create NHS Improvement.[2]

NHS England remained responsible for oversight of local CCGs and had direct responsibility for commissioning of specialised services.[3]

25.2 Throughout the relevant period, and until 31 March 2025, NHS England was directly responsible for the commissioning of neonatal services at the Countess as a specialised service.[4]

25.3 Management of the neonatal services commissioning contract with the hospital was delegated to the NHS England Regional Specialised Commissioning Team for the North (Specialised Commissioning (North)).[5] In practice, this meant that Specialised Commissioning (North) was responsible for quality-assuring neonatal services at the Countess.[6]

25.4 The NHS North regional team also had oversight and surveillance of Serious Incident management within NHS-funded care in its area. In his first witness statement, Sir Stephen Powis described NHS England at a regional level as having a "*dual role*" in relation to Serious Incident management:

> "*(a) As the commissioner, it would discharge the duties set out in the Serious Incident Framework and monitor contractual performance … If issues were detected around serious incidents that had an impact on the quality and safety of commissioned services, then NHS England regional teams could take action as the commissioner* [emphasis in original] … *This action might include enhanced monitoring and reporting of temporary changes in commissioned services. The downgrading of the neonatal unit at the … Hospital was an example of this …*

(b) An oversight role (consistent with its patient safety responsibilities at the time) to ensure there was effective serious incident reporting and subsequent management of incidents by the lead commissioner."[7]

25.5 NHS Improvement also had a regional team for the North, headed by Ms Lyn Simpson, Executive Regional Managing Director (North).

25.6 Annex 6 of the first witness statement of Sir Stephen contains a list of persons holding key regional positions in NHS England and NHS Improvement since 2015.[8]

25.7 The organisational split between NHS England and NHS Improvement, and the regional/national separation within each organisation, led to a mismatch of data flow and information:

a. The NRLS was monitored by NHS England at a national level only. The regional team would only become aware of issues or concerns identified through the NRLS database if the national team brought them to its attention.[9] This is surprising. If something merits a report to the national system, surely it merits consideration and action at a regional level. That said, it is not apparent that this made any difference to events at the Countess. CQC did have access to the database.

b. StEIS, on which Serious Incidents were reported and logged, could be accessed by NHS England regional teams but not by NHS Improvement/Monitor. The latter "*instead relied on either commissioners sharing relevant information with them or on matters of relevance being directly reported to them by providers*".[10]

25.8 Operational Delivery Networks were established by NHS England in 2014. In its written closing submissions, NHS England described Operational Delivery Networks as being "*still in their infancy*" during the period 2015 to 2017.[11]

25.9 In her witness statement, Ms Julie McCabe (née Maddocks), Director, North West Neonatal Operational Delivery Network (NWNODN), described the Network's function as being "*to coordinate patient pathways between providers over a wide area to ensure access and egress of specialist neonatal care. The NWNODN supported neonatal services in the region to meet national, local and Trust standards and optimise resources to improve services*."[12]

25.10 The Cheshire and Merseyside Neonatal Network Clinical Effectiveness Group was a sub-group within the Operational Delivery Network, which provided clinically driven governance and "*an assurance framework*" in its locality. Its role included "*co-ordinat*[ing] *incident reporting across the Network as well as reviewing mortality cases across the network, sharing learning gained from these reviews where appropriate*".[13]

Interactions between Monitor and the Countess

25.11 In his first witness statement, Sir Stephen said:

"*During the period up until 30 June 2016, Monitor had no contact with the Countess of Chester Hospital outside of its routine review of the annual submissions provided by the Hospital … No by-exception reporting requirements were required by Monitor or proactively made by the Countess*."[14]

25.12 Two possible explanations for the lack of contact are found in Sir Stephen's evidence.

25.13 First, the Countess was considered by Monitor and NHS Improvement to be a high-performing organisation. As such:

> "*It was not on Monitor's radar as a Foundation Trust requiring additional support or intervention and its reported performance did not suggest it was an outlier in any respect.*
>
> *This was in contrast to the challenges that other providers were experiencing, both in the North Region and nationally.*"[15]

25.14 Second, in respect of NHS Improvement, Sir Stephen said:

> "*NHS Improvement's primary focus shortly after it was established was on financial management at a provider level, due to the concerns that existed at the time around financial performance. Whilst NHS Improvement did have several workstreams that related to quality, it relied primarily on the oversight provided by the Care Quality Commission and commissioners when it came to assessing the quality and safety of particular services.*"[16]

25.15 Sir Stephen described how Monitor/NHS Improvement placed considerable reliance on inspections and assessments performed by CQC.[17] It appears that Monitor/NHS Improvement was not informed in advance of CQC's inspection of the Countess in February 2016.[18] Further, the June 2016 report produced following that inspection was not thought to raise any particular concerns.[19] My analysis of CQC's inspection is at Chapter 7. It is likely that CQC's approach, too, was affected by the fact that the Countess was high-performing.

The reporting of neonatal deaths to NHS England and associated bodies

25.16 As set out in Chapter 4, on 3 July 2015, a report was filed to StEIS in relation to the death of Baby D.[20] Baby D was the only baby to feature on the indictment whose collapse or death was the subject of a Serious Incident report during this period until the deaths of Baby O and Baby P a year later.

25.17 The Wirral CCG was the lead commissioning organisation with oversight of the investigation into the Serious Incident reported in relation to Baby D. As described in paragraphs 4.58 to 4.60, Chapter 4, the Countess finalised the case review after the post-mortem, and it was reviewed by the CCG on 14 December 2015. The CCG agreed that the Serious Incident had been adequately investigated, and it closed the investigation.

25.18 It was the shared view of NHS England[21] and Family Group 1[22] that each of the clusters, as well as each of the individual unexpected collapses or deaths, should have been reported to StEIS as Serious Incidents. What difference that would have made is not clear, given that the report of Baby D led to nothing. Separately, the deaths of Baby A, Baby C, Baby D, Baby E and Baby I were each reported to the NRLS. Nothing seems to have happened as a result of this. The clinicians were not hiding anything.

25.19 The North West Neonatal Operational Delivery Network knew about the deaths on the Countess's neonatal unit: see the meetings of the Cheshire and Merseyside Neonatal Network Clinical Effectiveness Group to which I refer in Chapters 5 and 6.

25.20 The minutes of the meeting of the Clinical Effectiveness Group on 16 September 2015 recorded: "*Mortality: 3 deaths under review will be discussed at subsequent CEGs*."[23] The meeting was chaired by Dr Subhedar, and attended by Dr Brearey.

25.21 Baby E's death was noted in the minutes of the meeting held on 12 November 2015.[24] The meeting was chaired by Dr Subhedar and attended by Dr Brearey and Ms Powell.

25.22 Baby I's death was noted in the minutes of the meeting held on 21 January 2016.[25] The meeting was chaired by Dr Subhedar and attended by Dr Brearey.

25.23 At none of the meetings were concerns about an increase in neonatal mortality discussed.

25.24 On 8 February 2016, Dr Subhedar participated in the meeting on the Thematic Review at the Countess (see Chapter 7). It was Dr Subhedar who, following the meeting, suggested adding to the Thematic Review report a theme of "*some of the cases involving babies that suddenly and unexpectedly deteriorated and in whom there was no clear cause for the deterioration/death identified at PM* [post-mortem]".[26]

25.25 NHS England maintained that those concerns were not passed on to them, nor to NHS Improvement, so that these organisations were unaware of the concerns regarding neonatal deaths and collapses at the hospital during the period of Letby's offending.[27] It is, however, right to say that NHS England did have reports on the NRLS about every death on the neonatal unit. So far as I can tell, NHS England did nothing with these reports. In particular, NHS England did not raise the deaths with the regional team.

Interactions between NHS England and the Countess following the deaths of Baby O and Baby P, July 2016

25.26 NHS West Cheshire CCG was designated as lead commissioner overseeing the Serious Incident investigation.

25.27 On the day the deaths of Baby O and Baby P were reported to StEIS, Mr Peter Groggins, Quality and Experience Lead, NHS England (Cheshire and Merseyside), emailed Mr John Grant, Incidents and Assurance Co-ordinator, NHS West Cheshire CCG, to ask whether one of the two Serious Incidents was a duplicate and needed to be removed. Mr Grant confirmed that they were not duplicates but related to the deaths of two different babies.

25.28 On 5 July 2016, Ms Lisa Cooper, Deputy Director Quality and Safeguarding (Cheshire and Merseyside) and Regional Lead Safeguarding (NHS North), emailed Mr Groggins and others at NHS North asking to meet "*to discuss a number of serious incidents that have occurred at the Countess of Chester on the Neonatal unit and a potential review we will need to do this week*".[28] Mr Groggins was asked to collate any incidents reported in the previous 12 months.

25.29 The same day, Ms Patel sent an email to a number of addressees, including directors and senior clinicians within the NHS North regional team, senior directors of operations within NHS England, NHS West Cheshire CCG and NHS Improvement. In the email, Ms Patel wrote:

> "*We have been made aware of some serious issues relating to the Countess of Chester Hospital Neonatal Services …*
>
> *The Unit was under some local scrutiny earlier in the year following an infection concern. A thematic review was undertaken 10 cases reviewed with no clinical issues identified. The Director of Nursing has confirmed that there are other issues coming to light now, that have not necessarily been reported, the recent deaths identify the Trust as an outlier.*
>
> …
>
> *The Countess also has some work to do in order for the system to be assured regarding their internal governance within this speciality.*"[29]

25.30 Ms Patel also outlined the hospital's plan to downgrade the neonatal unit and for an "[e]*xternal review*" to be carried out.[30]

25.31 On 6 July 2016, Mr Groggins contacted the hospital to request the 72-hour review of the deaths of Baby O and Baby P (in line with the Serious Incident Framework).[31] The same day, Ms Millward emailed Mr Groggins, stating:

> "*I understand that you have been in touch regarding the 72 hour review of the triplets who died.*
>
> *I can confirm that the initial review was held yesterday and that this has triggered a number of areas for deeper dive, including peer review of the x-rays undertaken on* [Baby O] *and a further review by obstetricians regarding delivery and the possibility that the liver sub-capsular haematoma [identified on PM] occurred in perinatal period. No clear cause of death was identified for* [Baby P] *from the initial review*."[32]

25.32 Ms Millward did not mention in her email the concerns that had been raised by the consultant paediatricians regarding Letby. This was despite Ms Millward having attended a meeting on 29 June 2016 during which those concerns were discussed.*

25.33 On 7 July 2016, the neonatal unit at the Countess was downgraded from Level 2 to Level 1. The hospital made another Serious Incident report to StEIS in respect of this downgrade. The report gave the reason for the downgrade as "*an increase in neonatal mortality rates for 2015 and 2016 compared to previous years*".[33] The report made no mention of the fact that concerns had been raised about a particular member of staff's association with neonatal mortality.

25.34 The same day, Mr Harvey, Ms Kelly, Ms Townsend and Ms Millward met Ms Lisa Cooper, Ms Paula Wedd (Director, Quality and Safeguarding, NHS West Cheshire CCG) and Ms Sue McGorry (Quality Lead, North West Hub, Specialised Commissioning, NHS England). Mr Bibby sent his apologies for absence.

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* See notes of the meeting on 29 June 2016 at **INQ0098334** and **INQ0106816/5-7**.

25.35 The purpose of the meeting was to discuss the downgrading of the Countess's neonatal unit and the Trust's plan to commission a review of neonatology services by the RCPCH. The Inquiry has not seen any formal note of the meeting but was provided with Ms Kelly's handwritten note.[34]

25.36 Ms Kelly recorded the following main points as being discussed at the meeting:

- Whether the "*system*" was safe today and tomorrow. Side notes were "*thematic review*" and "*safeguarding referral*".
- The "*external review Jan 15 – June 16*" (i.e. the proposed RCPCH review), "*data set*" and "*tick box re StEIS*". Side note: "*clarify and agree*".
- "*Duty of Candour – (?)19 mums*".
- "*Special Care Baby Unit*" and "*small, sicker babies*". Side note: "*KT* [Karen Townsend] *d/w* [dealing with] *NNU network re: comms & other units*."
- "*CDOP – no knowledge of all deaths*" and "*PW* [Paula Wedd] *to look at CDOP list*". Side note: "*CDOP list and notification*".
- "*Gill* [Frame]" (Chair) and "*Sian* [Jones]" (Business Manager) of the Cheshire West and Chester "*LSCB* [Local Safeguarding Children Board]", next to whose names is written: "*Email Re Welcome a discussion with you of all deaths*".
- "*Nigel Wenham* [Detective Superintendent] – *Police Route*".[35]

25.37 Ms Kelly also recorded the actions to be taken.[36] They included: "*AK* [Alison Kelly] *safeguarding team*" and, later, "*consider any safeguarding concerns*" and "*Sue* [McGorry]*, Lisa* [Cooper]*, Paula* [Wedd] – *call re: Police re: or any further*".

25.38 In oral evidence Ms Kelly said she could not remember the context of the discussion which led to her noting "*safeguarding referral*". She was pressed on whether this was an action for her, but said she could not recall agreeing to make any safeguarding referral. That she did not recall this is likely to be true. In her witness statement, she said that the reference to "*safeguarding referral*" was so that she could consider whether any of the deaths gave rise to any safeguarding concerns.[37] She told the Inquiry that she did not recall telling the NHS England attendees about the consultants' concerns or about any suspicion that there was deliberate harm. Her reason for not doing so was out of consideration for Letby's welfare. She agreed that she had not made a formal safeguarding referral at that time but had spoken to Ms Gill Frame, Independent Chair, Cheshire West and Chester Local Safeguarding Children Board, to inform her of the downgrade and action being taken (just as she informed other "*partners*").[38]

25.39 The note of this telephone call in Ms Kelly's notebook is on the page before this 7 July 2016 meeting; but, given the content of the note on 7 July 2016 (regarding emailing about a discussion), it is likely that the phone call occurred the following day. But whenever the call was made, it was not a safeguarding referral, formal or otherwise. She sought – and, it appears, received – reassurance that the hospital was "*doing the right things*". There is a further reference to the police: "*?Police action may be required*."[39] Ms Kelly did not make a safeguarding referral for another year.

25.40 Mr Harvey, Ms Kelly and Ms Millward were well aware of the seriousness of the doctors' concerns. They should have informed NHS England about them. This was another significant failure by managers, two of them at Board level.

25.41 It is clear that, at the meeting on 7 July 2016, the NHS England attendees raised the possibility of a safeguarding referral and whether the police may need to be involved, along with the possibility of doing so via DCS Wenham, who was the police member on the Local Safeguarding Children Board and the CDOP. It is also clear that none of those attending from the Countess acted upon that possibility, even though they did know about the concerns. This was a serious failure by Ms Kelly, Mr Harvey and Ms Millward. The failure by Ms Kelly was particularly egregious.

25.42 Thus, Mr Bibby described his understanding at the time as follows:

> "*In July 2016, the Trust was telling the Specialised Commissioning (North West) Team that they did not know what had caused the run of outcomes on the Unit. While I now understand that clinicians at the Hospital had raised concerns about LL this was not disclosed to me and, as far as I am aware, this was not disclosed to anyone else in the Specialised Commissioning (North West) Team at NHS England.*"[40]

25.43 Dr Gregory similarly said:

> "*I am now aware that LL was moved from the Neonatal Unit to the risk team in July 2016 because of concerns raised about her. I was not made aware of this by the Hospital. If the Hospital had informed us at the time, then that would have prompted a series of questions from Specialised Commissioning and might have led to a recalibration of the risk profile for the Hospital. Clinicians raising concerns about an individual in relation to increased mortality is something that I would expect Specialised Commissioning to have been informed of by the Medical or Nursing Director at a hospital*."[41]

25.44 Ms Kitching said she was not aware "*that the finger was being pointed at any particular individual at this time*", nor did she believe anyone else in NHS North was aware.[42]

25.45 Despite the close organisational connection between NHS England and the North West Neonatal Operational Delivery Network, there is no evidence that this information was passed on to NHS England. Ms McCabe, then Director of the Network, had been told by Dr Subhedar and Dr Brearey that the clinicians had "*concerns about a particular member of staff in connection with the increased mortality rate*" at about the time the hospital was downgraded. She did not pass this on to NHS England. She said:

> "*In my role it would not have been professional to make enquiries about any concerns with the capability or performance of a particular employee. My understanding was that an individual had been moved from the unit and was going through a HR process at the Trust. Following her removal from the Neonatal Unit, I was subsequently informed that the mortality rate and number of collapses had reduced significantly*."[43]

25.46 The first meeting of the Local Safeguarding Children Board held after the 7 July 2016 meeting was on 26 September 2016. Amongst a large cast of board members were Ms Gill Frame, Ms Wedd, Ms Lisa Cooper, DCS Wenham (on behalf of Cheshire Police

Public Protection Unit) and Ms Kelly. In attendance also was the consultant paediatrician Dr Mittal as Designated Doctor for Safeguarding and Child Deaths, ICB. Even at that stage, there was no discussion of any issues concerning the Countess.

RCPCH review, further investigations and failure to share information

25.47 On 12 August 2016, Mr Bibby, as Assistant Regional Director, Specialised Commissioning (North), chaired a teleconference with the hospital and other stakeholders. Attendees included Ms Kelly and representatives from the NHS West Cheshire CCG, NHS Improvement and NHS England's Cheshire and Merseyside team. In his witness statement, Mr Bibby said that the meeting served two purposes: "[F]*irst, to seek updates and assurances around the steps that the Hospital was taking to understand what was driving the run of poor outcomes on the Unit, and secondly, to discuss the operational management of the Unit following the downgrade of that Unit*."[44]

25.48 An email sent by Mr Bibby following the meeting summarised what was discussed, including:

> "*Nothing significant to report either in Neonatal or Maternity since the last call. Further analysis of the data that was discussed last time has not identified anything in terms of trends.*
>
> …
>
> *External Review has been delayed to 1 and 2 Sept* …
>
> *Briefing paper going to Trust Quality and Safety Committee next week summarising the analysis of the data of the internal review.*"[45]

The plan was for a face-to-face meeting to take place once the RCPCH review was completed and the report received.

25.49 There appears to have been no mention of the case review performed by Dr Gibbs and Ms McGlade, nor the staff review completed by Ms Sian Williams, and Ms Fogarty, in July 2016. The case review had identified six babies in whose cases something unexpected or unusual had occurred;[46] and the staff review had identified that Letby was on duty more often than other staff when a baby had died or collapsed.[47]

25.50 In August 2016, the hospital was also on enhanced surveillance. Mr Bibby explained in his witness statement: "*The threshold for enhanced surveillance is relatively low, and enhanced surveillance may be triggered by any level of concern our quality team had about patient care, quality, or outcomes, with any of our providers*."[48]

25.51 On 14 September 2016, commissioners from Specialised Commissioning (North West) requested a copy of the RCPCH report from the hospital.[49] The response from the hospital was to inform the Specialised Commissioning Team that the RCPCH had recommended a "*forensic deep dive into the clinical cases*" but had not advised any immediate actions in terms of safety.[50]

25.52 By this time, Mr Harvey had received the letter from Ms Eardley, RCPCH, dated 5 September 2016. In that letter, Ms Eardley recommended that the hospital take immediate steps to formalise the actions it had taken with Letby, including by commencing a process of investigation, setting out the nature of the allegations against her. The letter also stated that "*the pattern of recent deaths and the mode of deterioration prior to death in some of them appears unusual and needs further enquiry*" and recommended a "*detailed forensic casenote review of each of the deaths*".[51]

25.53 There is no evidence that NHS England or NHS Improvement were provided by the hospital with a copy of Ms Eardley's letter or told about the recommendation to take "*immediate steps*" to commence an investigation into the allegations made against Letby. To the contrary, Ms Kitching said in her evidence: "*My recollection is that Specialised Commissioning only had a verbal update and reassurance from the Hospital at this time*."[52]

25.54 On 16 September 2016, the North Regional Quality and Surveillance Group met. The meeting was chaired by Ms Kitching. The minutes of the meeting noted that the RCPCH review had gone "*well*" and therefore it had been agreed that "*the level of surveillance should be downgraded to 'routine'*".[53]

25.55 In her witness statement, Ms Kitching explained: "*I understood the comment that the review 'went well' to mean that there were no urgent patient safety issues being identified and the review had happened with full cooperation from the Hospital, and I recollect that it was reported that no single issue or individual was identified as a causal factor*."[54]

25.56 Dr Gregory, who also attended the meeting, gave similar written evidence.[55]

25.57 As at 2 December 2016, the Countess's surveillance level was "*enhanced*".[56]

25.58 On 16 December 2016, Mr Bibby wrote to Ms Kelly in the following terms:

> "*During our conversations, you offered to keep us updated on the situation and as such I am writing to request the following:*
>
> - *An update regarding progress towards the reinstatement of the neonatal cots*
> - *A copy of internal and external reports relating to the neonatal unit*
> - *Immediate risks or concerns*
> - *Formal action plan*
> - *Communications plan*
>
> *I am seeking your assurance that the Trust are progressing with the investigations and resulting outcomes are being actioned*."[57]

25.59 Ms Kelly replied by letter, dated 21 December 2016. She wrote: "*You will be aware that I sent an update to Sue McGorry via email on the 14th November 2016 explaining that the draft report had been received and was being checked by us for factual accuracy, this was sent back accordingly and we have only just received the final approved document from the Royal College*."[58]

25.60 This was untrue. The final report had been received three weeks earlier, on 28 November 2016. The letter continued:

> "*One of the recommendations of the report was that a further independent case review was required of relevant cases. This is being undertaken by a Neonatologist from London and they require a second pathology review on a small number of cases before their final report is completed. As a consequence, we currently do not have a final report of this part of the review and therefore are not comfortable in sharing the Royal College report until we have the details of the case review.*
>
> …
>
> *Obviously the safety of our unit is paramount and on the day the review team left the Trust, they assured us that there were no immediate actions or concerns.*
>
> …
>
> … [t]*he relevant report will be shared with you as part of our communication plan and until that time, we will not be able to comment on the timescales or plan going forward*."[59]

25.61 In oral evidence, Ms Kelly was asked about the basis on which she said to Mr Bibby that the RCPCH review team had "*assured*" the hospital "*there were no immediate actions or concerns*", when the RCPCH report had in fact made a number of what it termed "*immediate recommendations*". Ms Kelly accepted that her language was "*potentially*" misleading but said it was "*not intentionally*" so.[60] Ms Kelly also accepted that, had she told NHS England that there were a number of immediate actions recommended by the RCPCH, that might have provoked NHS England to demand to see the report immediately.[61] She said, on more than one occasion, that she did not intend to mislead. What she said on this occasion can be read quite easily. It was not written by accident. It was misleading. I am satisfied that its purpose was to mislead, just as she intended to mislead about the date on which the RCPCH report had been received.

25.62 As to the decision not to provide the RCPCH report to NHS England at this time, Ms Kelly said that it was "*a collective decision from the Executives*".[62]

25.63 In his witness statement, Mr Bibby described his reaction to Ms Kelly's letter:

> "*I was concerned, although not necessarily surprised, by this response to my letter. As commissioners, our expectation is that providers of specialised services would share a report of this nature with us as soon as the final report was received. Indeed, NHS trusts and foundation trusts are contractually obliged to share these with us. However, it was clear to me that the Trust regarded us as analogous to any other stakeholder and felt that we needed to be managed as part of their wider communication plan*."[63]

25.64 Mr Bibby said that Specialised Commissioning (North) considered using its "*contractual levers*" to obtain a copy of the RCPCH report but felt that NHS Improvement and CQC were more likely to be successful in obtaining a copy.[64] Mr Cornall explained in his witness statement that this was because "*contact from NHS Improvement could have more impact in conveying the seriousness of an issue because they could ultimately take regulatory action if they considered this necessary, including placing an organisation in special measures*".[65]

25.65 To that end, on 21 December 2016, three weeks later Specialised Commissioning (North) asked NHS Improvement for help in obtaining the RCPCH report from the hospital.[66]

25.66 Mr Vince Connolly, Medical Director (North), NHS Improvement, met Mr Harvey on 3 January 2017. Mr Connolly recorded what Mr Harvey told him as follows:

> "*Paediatricians concerned re ↑ in neonatal deaths over 18/12.*
>
> *V. low nos.*
> *Not an outlier* } → *Preliminary Investigation*
>
> *Meantime downgrade to unit* $L_3 \rightarrow L_2$
>
> *Review by RCPCH, v. thorough*
>
> *report in due course, no immediate concerns.*
>
> …
>
> *anticipate full report in 2/12*
>
> …
>
> *Ask for using this as example of*
>
> *'how to do it if needed in future'*
>
> *Example of governance*
>
> *Have involved families*
>
> *Met* [with] *CCG, Spec Comm* [Specialised Commissioning]*, NHSE* [NHS England] + *informed them of what has been happening*
>
> *Mtg* [meeting] *with staff + parents ~ 10 Jan*"[67]

25.67 This was misleading in a number of respects. First, the families of babies who had died were not 'involved' at all in the RCPCH review, nor in the review by Dr Hawdon, nor the pathology review by Dr McPartland. Second, the CCG, Specialised Commissioning and NHS England had not been fully informed of what was happening, as they had not been told of the concerns regarding Letby. Third, while the hospital's Board of Directors met on 10 January 2017, no parents were contacted that day, and the only member of staff who seems to have been given the benefit of a meeting was Letby herself. The note was also factually incorrect, as the downgrade was from a Level 2 to a Level 1 unit, not, as the note erroneously records, from a Level 3 to a Level 2.

25.68 The message passed on by Mr Connolly to NHS Improvement was that the hospital had said that the issues it was dealing with were complex and it needed more time.

25.69 On 3 February 2017, the *Sunday Times* contacted the Countess, seeking a copy of the RCPCH report. On the same day, Ms Kelly sent a copy of the embargoed RCPCH report to NHS England and other relevant organisations.[68] It was the (redacted) dissemination version.

25.70 Mr Harvey, Mr Bibby, Dr Gregory and Ms Patel met on 23 February 2017. The typed note of the meeting records that Mr Harvey confirmed that the hospital had completed the external review of babies who had died in the period January 2015 to July 2016.[69] In

his written evidence, Dr Gregory said that the meeting arose because Ms Patel was raising concerns about the hospital's response to the RCPCH report. Dr Gregory further explained: "*It was at this point my concerns around the failure of the Hospital to volunteer information and its reluctance to share the RCPCH report first arose*."[70]

25.71 In their written statements, Mr Bibby and Dr Gregory both said that they were in the dark about the number of reviews that had been carried out and about which were still ongoing.[71] Dr Gregory stated:

> "*The notes prepared for the meeting do not refer to the completion of Dr Jane Hawdon's review. At that time the assumption was that the external review referred to the one carried out by the RCPCH … The language we were receiving from the Hospital throughout this period was ambiguous and I was confused as to the number of reviews being conducted and what the recommendations were.*"[72]

25.72 Similarly, Mr Bibby said that he was "*not sure*" he fully understood what Mr Harvey was saying about further reviews at the time.[73] It is unclear what steps, if any, Mr Bibby and Dr Gregory took at the time to clarify their understanding of the reviews commissioned by the Countess.

25.73 Again, there was no discussion of Letby, or of any individual members of staff, during the meeting.

25.74 In light of growing concerns about the hospital, a quality risk profile of the Countess was performed by Specialised Commissioning (North) on 25 February 2017. Mr Cornall said in his witness statement: "*This was a step we would take where we were managing a situation of this nature as a way of bringing together known risks and ensuring we had an agreed approach to managing them*."[74] Mr Bibby explained in more detail:

> "*A quality risk profile is usually undertaken where a specific concern or issue is identified that might indicate wider failings within an NHS Trust or Foundation Trust. It involves trawling the data submitted by providers across a wide range of metrics and using this data to analyse risk across the piece to understand whether an individual trust is an outlier across that wide range of metrics against a spectrum … While this quality risk profile … did not highlight any specific risks around neonatal services, it did reaffirm our view that the Trust were not in a good place in respect of their governance. The Specialised Commissioning (North West) Team would not have taken any action around this, as such action would have been done more broadly by colleagues in NHS England and NHS Improvement*."[75]

25.75 Given that Mr Bibby and Dr Gregory considered that they were in the dark and did not understand what Mr Harvey was telling them, before spending effort on a quality risk profile it would have been more useful to require answers from Mr Harvey to their reasonable questions. Up to this point, the NHS England regional team were curiously diffident in their dealings with the hospital. It was easy for the executives at the Countess to keep them out.

NHS England's response to the concerns about Letby

25.76 On 27 March 2017, Ms McCabe and Dr Subhedar attended a meeting with Mr Chambers, Mr Harvey, Ms Hodkinson, Dr Jayaram and Dr Brearey. A typed note of the meeting was taken.[76] The note recorded Ms McCabe saying: "*Given the information, on the balance of probability, illegal activity has caused the deaths*."[77] In her witness statement, Ms McCabe said this was a question she was putting to the clinicians. Dr Jayaram and Dr Brearey "*responded by nodding their heads, indicating that they believed that this was the case*".[78]

25.77 Ms McCabe also gave her account of the atmosphere at the meeting:

> "*There was a high level of anxiety at the meeting. I remember it being challenging and intense and there was a palpable sense of tension in the room. I remember the clinicians trembling, my recollection was that the clinicians felt like they were putting their careers on the line, but they were prepared to do this given the seriousness of the matter …*
>
> *After the clinicians confirmed that they suspected criminality, Tony Chambers said that if that was the case then they should call the police. This sounded like an ultimatum and that there were negative connotations to involving the police. The clinicians wanted the Trust to go to the police as an organisation, and although I think that they would have done so if that was the only option, they believed that the right thing to do was for the Executive to take this action*."[79]

25.78 Ms McCabe said that, as a result of the meeting, she believed that the hospital had made the decision to contact the police.[80]

25.79 On 28 March 2017,† Ms McCabe told Mr Bibby about the recent meeting and that the hospital "*had concerns relating to additional cases paediatricians felt required review. ? Whether a police investigation required.*"[81] In her written evidence, Ms McCabe provided further detail:

> "[Mr Bibby] *said he had spoken to the Trust, and they thought that the clinicians had gone maverick. I told Andrew very firmly that my impression from the meeting was that the clinicians had not gone maverick, but rather they were concerned about deaths and collapses to which they could find no clinical explanation. I said that they believed that the next step was to bring the police in, which was something that the Trust was going to action.*"[82]

† The timeline at **INQ0014692/4** gives a date of 29 March 2017. Ms McCabe's witness statement at **Witness statement of Julie McCabe INQ0107030/21/para 82** suggests a date of 28 March 2017.

25.80 In turn, this appears to have prompted Dr Gregory to contact Mr Harvey on 29 March 2017. They spoke over the phone.[83] What precisely was said between them is a matter of dispute. The paragraphs of Dr Gregory's witness statement dealing with this are worth setting out in full:

> "*I raised the possibility that an individual member of staff might be involved with the increased mortality on the Neonatal Unit. I had no information or evidence that this was the case, but I felt it was a question that needed to be asked. At this stage I was considering all possibilities. I recall also discussing the question of an individual having a disproportionate involvement in a meeting with Ian Harvey and Lesley Patel on 23rd February* [2017]*, however I did not document it. When I asked in the 29th March meeting, Ian Harvey stated that they had looked into an individual being involved as a possibility but, due to a combination of skill mixture and rotas, it had been discounted. He explained that, given the severity of the cases, the rotas meant that certain, more experienced members of staff would naturally have greater contact with those babies. In addition, some babies had fallen ill after being transferred off the unit and this would seem to discount an individual on the unit having involvement …*
>
> *When I pushed Ian Harvey on the involvement of an individual staff member, he stated that he did not want to go into any more detail until the Hospital had made a significant announcement about the decision they had taken to speak to an 'appropriate body' on the following Monday. He did not indicate what that announcement was, nor what 'appropriate body' he was referring to. I do not believe that an announcement was ever made on the Monday. Ian Harvey told me that he was handling a very difficult situation and was asking for more time so that he could handle matters within the Hospital. When I pressed Ian Harvey as to what this difficult situation was, he indicated that the Hospital were having some issues with the paediatricians.*
>
> *It was at this meeting that I first learnt about a clinician that raised concerns about the babies that had died or needed resuscitation in the Hospital or other units. My understanding was that the clinicians were picking up signs and symptoms that they didn't understand. Ian Harvey mentioned that the clinicians were confused about the signs that they were seeing and that they had observed mottling of skin, which they had not seen before. However, Ian Harvey also seemed to suggest that one clinician had some other sort of agenda. I got the impression that there was a complex situation going on and Ian Harvey was trying to piece together a consistent thread in the unexpected mortalities and illnesses. At no point during my involvement was I informed by the Hospital that two clinicians were concerned about an individual nurse, and I was not aware that this individual was LL until her arrest in 2018.*"[84]

25.81 In oral evidence, Mr Harvey disagreed with this characterisation of the meeting:

> "*In the first instance, I would dispute* [Dr Gregory's] *assertion that he had raised the possibility of an individual with disproportionate involvement. I believe that, in the meeting with him, I had described that. I have accepted that we were slow in sharing information with Specialised Commissioning but I would also dispute the degree to which they pressured to obtain information.*
>
> …
>
> *I believe that, given that a more senior Medical Director working with Specialised Commissioning was involved, he was in a position to actually enforce if he felt it was appropriate and … I wouldn't have, in those circumstances, withheld anything.*"[85]

25.82 The contention that Mr Harvey was reluctant to share information with Dr Gregory during the telephone call finds support in the contemporaneous email that Dr Gregory sent to Mr Bibby, Ms Patel and Mr Cornall after the meeting. He wrote:

> "*To summarise our conversation:*
>
> …
>
> *There is a member of staff whose presence has been seemingly disproportionate but (as was discussed when we met) this was originally accounted for by rotas and skill level. However, when pushed about staff members Ian stated that this matter was best dealt with when they make the significant announcement about the decision they have taken to speak to an 'appropriate body' on Monday.*
>
> *Clearly something very serious is going on and they must have their hands tied somewhere – but it would be speculation to guess what …*
>
> *He will keep me briefed about that announcement.*
>
> *Not sure if we can do any more until Monday unless we wish to escalate further.*"[86]

25.83 In his witness statement, Dr Gregory explained how he felt at the time:

> "*At this time it felt like we were going to the Hospital repeatedly and having to ask questions, rather than them volunteering the information and giving us updates. This lack of co-operation from the Hospital meant that I did not feel as though Specialised Commissioning North was able to fulfil its assurance role. Every time that we went to the Hospital we were met with obscure terminology and a lack of explanations. As Ian Harvey was the Medical Director of a Hospital, I did not feel that I had the clear lines of escalation which I would if I was regional Medical Director. I escalated my concerns within NHS England who could escalate to NHS Improvement, who had the power to take regulatory action.*"[87]

25.84 Mr Cornall forwarded Dr Gregory's email to Ms Kitching for discussion.[88] Ms Kitching, however, was on annual leave.

25.85 On 30 March 2017, Dr Gregory spoke with Mr Harvey and Mr Chambers. Mr Harvey took a note of the meeting.[89] The note recorded an apparent query by Dr Gregory as to what the hospital meant by "*appropriate authority*". Mr Chambers was recorded to have said that there were no immediate concerns regarding safety and the major concern was for families and the effects on them. The RCPCH review and "*case reviews*" were said not to identify a single cause, but the clinicians remained "*unconvinced that excluded unnatural causes*". The intention was said to be to consult the police the following week.

25.86 In his witness statement, Dr Gregory said he recalled Mr Harvey and Mr Chambers saying that "*they did not want to go public as there would be a media backlash and this would cause concerns with the families*". Dr Gregory did not recall any mention of "*unnatural causes*" during the meeting.[90]

25.87 In his written evidence, Dr Gregory again said that he was "*becoming increasingly concerned by the lack of answers from the Hospital*". As to Mr Chambers' reference to case reviews, Dr Gregory said he was "*personally confused as to which review was being referred to as they were mentioned without specific nomenclature in emails*". The end effect, Dr Gregory said, was that he was "*growing increasingly concerned as to why the Hospital was not being open*".[91]

25.88 The NHS England North Regional Specialised Leadership Group met on 4 April 2017.[92] Attendees included Mr Bibby, Mr Cornall and Dr Gregory. An email was circulated summarising the "*key messages*" from the meeting, which included: "*There are still concerns in relation to the Neo Natal Service following the review into the high numbers of patient deaths. Members of the RLG [regional leadership group] are working with the Trust and members of the North Regional Team to understand these more fully*."[93]

25.89 On 5 April 2017, Dr Gregory sent Mr Harvey an email requesting a copy of the "*external review report*" that had been undertaken. In the email, Dr Gregory also requested: (a) a copy of the brief given to Mr Simon Medland KC; (b) "*a written record of the concerns expressed by the two paediatricians*"; and (c) a "*proposed timeline of events*" going forward.[94]

25.90 As to the instruction of a barrister by the hospital, Dr Gregory commented in his written evidence: "*We couldn't understand this decision to involve a QC* [Queen's Counsel] *as they do not have the investigative powers of the police. It increasingly felt like the Hospital were making a concerted effort to avoid going to the police*."[95]

25.91 On 6 April 2017, Ms Kitching emailed Dr Gregory, stating that she had had "*a very helpful conversation*" with Mr Chambers, who had given her "*the run down re the Trust's position and potential future actions*".[96] Ms Kitching said that Mr Chambers would provide a further brief when the process with the paediatricians had concluded. Dr Gregory replied in more urgent terms, writing:

> "*It has been difficult to manage internally because we don't have all the information from the Trust. I understand the situation is delicate and should hopefully lead to nothing but if it does escalate we need to be prepared and have good documentation.*
>
> *There have been a number of separate phone calls and conversations but nothing is written at the moment and we have no understanding of timescales – hence my request to Ian* [Harvey] …
>
> *I am not sure what we should do if I don't get a response.*"[97]

25.92 Ms Kitching replied to say that she agreed and would chase Mr Chambers the following week.[98]

25.93 The chasing, however, appears to have been done by Dr Gregory. On 19 April 2017, Dr Gregory emailed Mr Harvey asking for an update as to any decision made following the Board meeting held on 13 April 2017.[99] Mr Harvey replied the same day, advising that four deaths remained unexplained and the hospital intended to approach the CDOP.

Dr Gregory responded to ask whether the clinicians still had concerns, and to request a timeframe as to when "*the report of the external reviewer*" would be available. Mr Harvey replied once more, writing:

> "*We are going through this process because there isn't yet a complete and definitive answer in all cases … Re the process, I shall appraise you after my conversation tomorrow. I don't think that there was ever an agreement that the individual case report would be shared – this contains identifiable data – this would need a conversation*."[100]

25.94 This prompted Dr Gregory to forward the email chain to Mr Cornall, Ms Patel and Ms Kitching, with the comment: "*Still no response as to whether the clinicians have had their concerns addressed*."[101] Mr Cornall replied, stating: "*It all feels a little evasive again*." Ms Kitching said that she would contact the hospital.

25.95 In his witness statement, Mr Cornall commented: "*I felt that Ian Harvey's response was evasive, because we did not get straight answers to questions and he was still unwilling to share information*."[102] Dr Gregory gave similar evidence.[103]

25.96 Ms Kitching recalled having a conversation with Mr Chambers or Mr Harvey at some point between 19 April and 25 April 2017. Ms Kitching said in her witness statement:

> "*At this stage, I was not aware that an individual had been implicated, just that two consultants had concerns that the external investigation didn't go far enough, and they hadn't received sufficient assurance. That of itself was not uncommon … I understood from this call that the Hospital was going back to the chair of the Child Death Overview Panel to look at the cluster of neonatal deaths*."[104]

25.97 Ms Kitching seems to have forgotten, when making her statement, that she had received Dr Gregory's email referring to the disproportionate presence of a member of staff.[105]

25.98 A Regional Management Team meeting was held on 25 April 2017. Attendees of the meeting included Ms Kitching and Dr Gregory. During the meeting, Ms Kitching advised Dr Gregory that they should give the hospital a bit more time to respond. In her written evidence, Ms Kitching said that by this she meant "*a week or so*" and that the suggestion was "*supported by those present*".[106]

25.99 The same day, Mr Harvey emailed Dr Gregory, stating that he and Mr Chambers would be happy to meet Dr Gregory once the hospital had "*completed* [its] *process*".[107] Dr Gregory forwarded the email to Mr Cornall, Ms Patel and Mr Bibby the following day, with a list of suggested further requests for information for Mr Harvey.[108] In the ensuing email chain, Ms Patel expressed concern that the CDOP process could take weeks, and so a meeting after it had finished would not be timely enough.[109] Mr Cornall said that he had spoken with both the Nursing Director and Medical Director from the National Specialised Commissioning Team (Ms Teresa French and Professor James Palmer respectively), and all thought that a referral to police should be made now.[110]

25.100 Ms Kitching, however, again preferred a more cautious approach. She copied the email chain to Mr Richard Barker, Regional Director (North), and wrote:

> "*I believe a call with their CE* [Chief Executive] *should happen tomorrow to clarify our position and if we are still concerned we should give them the opportunity to seek advice from the police first.*
>
> *In my experience CDOP processes are not lengthy and I suggested at the RMT* [Regional Management Team meeting] *that we would give them to the end of this week if we have not received any further assurance.*"[111]

25.101 In a later email, sent just to Mr Barker, Ms Kitching wrote:

> "*When I last spoke with Tony* [Chambers]*, he explained that the independent* [investigations] *did not identify any criminality, two of their paediatricians are disputing and casting doubt on the findings*
>
> *Hence them taking further steps, we did discuss involving the police which they intend to do if full assurance is not gained, the two paeds could be the problem but we need to be sure Tony and the team want to exhaust internal processes first as they recognise that involving the police could cause further significant distress to the families*
>
> *… Michael* [Gregory] *is worried that he believes they are being evasive hence escalation to the national leads, Tony is not happy at this accusation as he believes that they have been fully transparent,*
>
> *I don't think we should involve the police without appraising the Trust and giving them the opportunity to explain and contact the police if needed*
>
> *The unit is safe as it has continued to stop admitting complex cases*."[112]

25.102 Mr Barker agreed to this approach and commented, presciently: "*However, if it transpires that we do need to subsequently involve the Police then the delay will not look good and lead to further concerns from the families*."[113]

25.103 In her witness statement, Ms Kitching said that she recalled speaking with Mr Chambers around this time. Ms Kitching's full account of that conversation is set out at paragraphs 110 to 112 of her witness statement. The most important parts are set out below:

> "*He* [Mr Chambers] *explained that there was no evidence to suggest any criminality or unnatural causes had occurred …*
>
> *Mr Chambers expressed that it was his view that the two consultants were the problem as they were not happy with the findings of the investigations … It was at* [this] *point when I learnt that the consultant paediatricians were pointing the finger at a member of staff, and I pressed Mr Chambers to engage the police immediately for advice as I was not sure how long their internal process would take. He stated that there was no evidence to support the* [consultants'] *view but I explained that just the accusation alone was sufficient to seek police advice as they are the experts in this field. Mr Chambers assured me he would do it that day.*

> *… I said Specialised Commissioning were really concerned about the Hospital's lack of transparency. He was upset by that … Mr Chambers agreed to expedite his conversation with the police, and I agreed to set up an urgent meeting the following day with the Hospital so we could be briefed fully and determine appropriate actions and escalations*."[114]

25.104 On 27 April 2017, Mr Connolly and Ms Kitching had a teleconference with Mr Harvey and Mr Cross. Ms Kitching took a note of the meeting.[115] Ms Kitching again raised the concerns of Specialised Commissioning, that they had not been provided sufficient information, and communicated their view that the police should now be involved.

25.105 The note of the meeting records that Mr Harvey said that he was "*unsure why the commissioners felt this way as he had met with MG* [Dr Gregory]*, AB* [Mr Bibby] *and LP* [Ms Patel] *and updated them accordingly*". He referred to "*the sensationalist story reported in the* Sunday Times *which caused further upset to some of the families and therefore it was important information sharing was kept to a minimum*". This was ironic, given his own failure to share information with parents in a timely fashion. He did not refer to the fact that the nurse about whom the paediatricians had expressed concern had not been on duty since the unit was downgraded. He explained: "[T]*he incident occurred as a result of a Paediatrician and neonatologist alert re numbers of deaths in the unit, which resulted in the Trust commissioning the RCPCH to undertake an investigation into all the deaths*." This was not accurate. There was no doubt by April 2017 that the RCPCH had not conducted an investigation into all the deaths, and it was never going to. Mr Harvey continued: "*There was no single factor identified rather it was multi factorial.*"[116] Nowhere did the RCPCH report suggest that the cause of the deaths was multifactorial. Its findings explained none of the deaths.

25.106 In Mr Harvey's outline of events given at the meeting, the note records no mention by him that the consultant paediatricians were concerned about deliberate harm. He said: "*It was determined that a single member of the nursing staff was on duty and attended to most of the cases, but not all, and her full time status meant that this was probably not unusual*."[117] He informed the meeting that the Trust had sought an independent legal opinion, on evidence so far, and the findings were that they could not see any evidence of criminality. He did not mention what the lawyer had been asked to consider, nor that the lawyer had said that, if reasonable people were concerned, the police should be consulted. He said that the hospital had shared everything with the coroner. It had not. Nor had the Countess kept families involved in the process, contrary to what Mr Harvey asserted.

25.107 It was agreed that Ms Kitching would act as the single point of contact for the hospital, and that Mr Harvey would keep her informed of the outcome of discussions with the police.

25.108 Mr Chambers wrote to Chief Constable Simon Byrne QPM of Cheshire Constabulary, and meetings were held between the hospital and the police on 5 May and 12 May 2017. Following the latter meeting, Mr Harvey emailed Ms Kitching in the following terms:

> "*They* [the police] *are minded not* [emphasis in original] *to hold an investigation – firstly they don't feel that there is evidence of criminal activity and secondly they are mindful of the effects on families. However, our Paediatricians sent a document to them that was a listing of their concerns which was a very prejudiced view, effectively pointing the finger at one nurse … It does not contain anything new that multiple people and agencies haven't heard despite the Paediatricians assertion that they haven't been listened to …*
>
> *My own feeling is that unless there is something that the Paediatricians haven't disclosed previously that evidences criminal activity there will not be an investigation and the police will assist us in a message that will allow us to close down the speculation here and deal with the issues of culture etc*."[118]

25.109 In her written evidence, Ms Kitching said that she "*was very concerned*" with Mr Harvey's email. She explained:

> "[C]*learly the Hospital was still focused on the two consultants being the problem. I was worried that the police may be listening to the Hospital view without looking at all the facts and speaking with the two consultants. I recollect speaking to Ian and asking him to make sure the police did speak to the two consultants before any decisions were made*."[119]

25.110 The police met Dr Brearey, Dr Jayaram and Dr Holt on 15 May 2017. The commencement of a police investigation was announced the same day.

Lack of candour

25.111 Mr Bibby was severely critical of the information provided to him and to Specialised Commissioning (North). He said:

> "*I was never fully informed about the concerns at the Hospital, and I felt my attempts to obtain the information I needed to take informed decisions were repeatedly frustrated. By way of example, I was unaware that clinicians at the Hospital had raised concerns about a possible link between the increased mortality rate on the Unit and a particular individual until the end of March 2017, I did not know that that the Trust had removed LL from the Unit, and I did not learn of LL's identity until the day of her arrest*."[120]

25.112 The effect, Mr Bibby said, was to delay the involvement of the police:

> "*I do strongly believe that the Trust should have been significantly more transparent with us … about the concerns that had been raised by clinicians regarding LL, and about what the Trust knew. There were key pieces of information that should have been shared with the Specialised Commissioning (North West) Team, that would, I think, have led us to the conclusion that the police should be involved much sooner. We were of the understanding throughout the Relevant Period that there was no known or suspected cause for the poor outcomes of the Unit. Consequently, we followed our*

standard processes and worked to eliminate the most likely causes, before turning to consider the alternatives. Had we known that consultant paediatricians had identified a link between the poor outcomes on the Unit and a particular individual, that individual had been moved off the Unit in June/July 2016, and outcomes had thereafter improved, we would likely have viewed the Royal College of Paediatrics and Child Health's findings very differently and I am certain that we would have pushed for greater scrutiny of LL's performance and conduct on the Unit, and for the police to be involved, much sooner."[121]

25.113 As to the reasons behind the hospital's reluctance to share information with NHS England, Mr Bibby thought:

"*The Trust's apparent unwillingness to be open and transparent with the Specialised Commissioning (North West) Team seemed to me to reflect a broader culture within the Trust, and a desire to avoid or mitigate adverse assumptions about what might be causing the increased local mortality rate on the Unit until a definitive cause had been determined. I suspect that reputational considerations may have been a factor in this*."[122]

25.114 Dr Gregory gave evidence of his frustration with the hospital's interactions with the Specialised Commissioning (North) team:

"*During this period it was my responsibility to get a full understanding of what had happened and to have assurance through the action plan that the Hospital was addressing the situation. I was asking questions and seeking clarification on the reviews and the actions the Hospital had taken. I did not consider that the responses I was getting from the Hospital helped with that clarification. If anything they served to create more confusion. I asked direct questions about concerns that were being raised by clinicians and they were not answered. I should not have needed to keep pressing the Hospital for answers. In the interests of patient safety I should have expected that Ian Harvey, as Medical Director, be open and transparent with me and disclose that information that, as Commissioner for the service, we needed to know. The answers that we received never made reference to an individual or the specific concerns that the paediatricians had about LL. The Hospital alluded to clinical issues and the internal processes that they were following but were not clear on what these were … I have not since experienced the same level of lack of cooperation from a hospital.*

…

I felt that there was a lack of transparency from the Hospital, avoidance of answers and wanting to defer the issues we raised … My sense was that the Hospital was intent on conducting its own process through their board and were evasive in response to our questions."[123]

25.115 Like Mr Bibby, Dr Gregory thought that the hospital's failure to share information impeded the ability of NHS England to take further action in respect of the hospital, including involving the police.[124]

25.116 Ultimately, Dr Gregory was at a loss as to why the senior management of the hospital acted as it did: "*I personally do not understand why this important information was not being shared and what the Hospital thought would be gained by avoiding our questions and asking for more time to respond to our requests*."[125]

25.117 Ms Kitching was similarly critical of the failure of the hospital to share information with NHS England. She said:

> "*I do believe that the Hospital was being evasive and uncooperative with NHS England and NHS Improvement, particularly in early 2017. I do not understand why the Hospital didn't just give the information requested and do not consider that concerns around confidentiality were justified; these concerns are standard and can be addressed by redacting any identifiable patient information if necessary.*
>
> *It is my firm view that the Hospital should have informed NHS England much sooner regarding the suspicions held by clinicians that an individual (LL) was the reason for the increased mortality rate within the neonatal unit*."[126]

25.118 Mr Cornall summarised the actions of the senior management of the hospital as follows: "*I think the Trust executive team controlled the narrative so that it was hard to unravel the issues*."[127]

25.119 Mr Chambers was asked whether he accepted that NHS England should have been told about the concerns regarding Letby when those concerns first arose. The answer to the question was plainly 'yes'. Instead of giving that answer, Mr Chambers did not engage with the question and said that he did not know. He was the Chief Executive of the hospital. He had had years to reflect on what had happened. He suggested that the Inquiry make recommendations that provided greater clarity as to when concerns should be shared[128] (see Chapter 41). He explained: "*The … answer is I don't know, it's just that balance between … duty of candour and duty of care … I'm not sure at that time whether we had got the balance right.*"[129] This balance was referred to by several people. In this example there is no question of balancing two duties. There was a clear duty to inform NHS England about what was happening in the hospital. There was no counterbalance to that.

25.120 Mr Harvey accepted in oral evidence that the hospital was "*slow in sharing information with Specialised Commissioning*". He explained that this was not because of "*a desire to hide anything from them*" but rather due to "*perhaps an inappropriate degree of concern about documents leaking into the public domain*".[130] Mr Harvey also went on to suggest that NHS England ought to have imposed more pressure on the hospital to obtain information.[131] Had Mr Harvey and the other executives been open with NHS England, no pressure would have been needed.

25.121 NHS England should not have agreed to repeated delays with regard to the provision of documents and information by the hospital. Ms Kitching was too ready to accept Mr Chambers' reassurances. At the end of the meeting on 27 April 2017, Ms Kitching records that she thanked Mr Harvey for his time and briefing, and recognised that the

Trust was doing all it could in order to resolve this. Given the way matters had emerged, this was an overly generous assessment. She added: "*The involvement of CDOP and the police is welcomed*."[132]

25.122 It remains the case, however, that the principal responsibility was upon the Countess to tell NHS England that it was dealing with a situation of possible deliberate harm of babies by a neonatal nurse, and that this had been the position for many months. The hospital's failure was that of Mr Chambers, Mr Harvey and Ms Kelly.

Endnotes

1 Witness statement of Prof. Sir Stephen Powis INQ0017495/43/para 167 and 45/paras 174-175
2 Witness statement of Prof. Sir Stephen Powis INQ0017495/62/para 239
3 Witness statement of Prof. Sir Stephen Powis INQ0017495/20/para 65
4 Witness statement of Prof. Sir Stephen Powis INQ0017495/26/para 96
5 Witness statement of Prof. Sir Stephen Powis INQ0017495/121/para 478
6 Witness statement of Andrew Bibby INQ0106970/14/para 47
7 Witness statement of Prof. Sir Stephen Powis INQ0017495/121/para 480
8 Witness statement of Prof. Sir Stephen Powis INQ0017495/309-311
9 Witness statement of Prof. Sir Stephen Powis INQ0017495/116/para 456
10 Witness statement of Prof. Sir Stephen Powis INQ0017495/117/paras 457-458
11 Written Closing Submissions on Behalf of NHS England 4 March 2025 7/para 21
12 Witness statement of Julie McCabe INQ0107030/5/para 13
13 Witness statement of Julie McCabe INQ0107030/9/para 30
14 Witness statement of Prof. Sir Stephen Powis INQ0017495/125/para 495
15 Witness statement of Prof. Sir Stephen Powis INQ0017495/125/paras 496-497
16 Witness statement of Prof. Sir Stephen Powis INQ0017495/119/para 469
17 Witness statement of Prof. Sir Stephen Powis INQ0017495/122/para 484
18 Witness statement of Prof. Sir Stephen Powis INQ0017495/124/para 492
19 Witness statement of Prof. Sir Stephen Powis INQ0017495/124/para 493
20 INQ0108752
21 Witness statement of Prof. Sir Stephen Powis INQ0017495/128/para 508
22 Written Closing Statement on Behalf of Family Group 1 4 March 2025 33-34/paras 116-117
23 INQ0005531/5
24 INQ0009762/6
25 INQ0005559/8
26 INQ0103111/1
27 Written Opening Statement on Behalf of NHS England 30 August 2024 5/para 26
28 INQ0014634
29 INQ0102984
30 INQ0102984
31 INQ0009236/33
32 INQ0014635
33 INQ0014636/1

34 INQ0004320
35 INQ0106930/126
36 INQ0106930/127
37 Witness statement of Alison Kelly INQ0107704/41/paras 126-127
38 Alison Kelly 25 November 2024 25/3 to 30/25
39 INQ0106930/125
40 Witness statement of Andrew Bibby INQ0106970/41/para 146
41 Witness statement of Dr Michael Gregory INQ0107034/22/para 84
42 Witness statement of Margaret Kitching INQ0107036/18/para 72
43 Witness statement of Julie McCabe INQ0107030/14/para 50
44 Witness statement of Andrew Bibby INQ0106970/41/para 147
45 INQ0014679
46 INQ0005336/6
47 Witness statement of Sian Williams INQ0101320/9/para 54
48 Witness statement of Andrew Bibby INQ0106970/42/para 149
49 Witness statement of Andrew Bibby INQ0106970/44/para 158
50 INQ0014642/103
51 INQ0002751
52 Witness statement of Margaret Kitching INQ0107036/20/para 78
53 INQ0014687/7
54 Witness statement of Margaret Kitching INQ0107036/20/para 78
55 Witness statement of Dr Michael Gregory INQ0107034/23/para 87
56 INQ0106988/3; INQ0106992/9
57 INQ0102994
58 INQ0008077/1
59 INQ0008077/1-2
60 Alison Kelly 25 November 2024 175/21 to 176/25
61 Alison Kelly 25 November 2024 177/13-17
62 Alison Kelly 25 November 2024 177/1-12
63 Witness statement of Andrew Bibby INQ0106970/46/para 170
64 Witness statement of Andrew Bibby INQ0106970/47/para 176
65 Witness statement of Robert Cornall INQ0107032/31/para 101
66 Witness statement of Andrew Bibby INQ0106970/47/para 177
67 INQ0014771/4
68 INQ0103029/3-4

69 INQ0006081
70 Witness statement of Dr Michael Gregory INQ0107034/25/para 99
71 Witness statement of Andrew Bibby INQ0106970/50/para 187
72 Witness statement of Dr Michael Gregory INQ0107034/25/para 100
73 Witness statement of Andrew Bibby INQ0106970/50/para 187
74 Witness statement of Robert Cornall INQ0107032/32/para 109
75 Witness statement of Andrew Bibby INQ0106970/51/para 191
76 INQ0003150/1-6
77 INQ0003150/2
78 Witness statement of Julie McCabe INQ0107030/20/para 77
79 Witness statement of Julie McCabe INQ0107030/21/paras 79-80
80 Witness statement of Julie McCabe INQ0107030/21/para 81
81 INQ0014692/4; Witness statement of Andrew Bibby INQ0106970/53/para 200
82 Witness statement of Julie McCabe INQ0107030/22/para 83
83 Witness statement of Dr Michael Gregory INQ0107034/26/para 103
84 Witness statement of Dr Michael Gregory INQ0107034/26-27/paras 103-105
85 Ian Harvey 29 November 2024 134/2 to 135/24
86 INQ0014651
87 Witness statement of Dr Michael Gregory INQ0107034/27/para 106
88 INQ0014651/1
89 INQ0003245
90 Witness statement of Dr Michael Gregory INQ0107034/28/para 108
91 Witness statement of Dr Michael Gregory INQ0107034/28/paras 109-110
92 Witness statement of Andrew Bibby INQ0106970/54/para 204
93 Witness statement of Prof. Sir Stephen Powis INQ0017495/139/para 553
94 INQ0003126
95 Witness statement of Dr Michael Gregory INQ0107034/29/para 111
96 INQ0103067/2
97 INQ0103067/1
98 INQ0103067/1
99 INQ0014667/4
100 INQ0014667/3
101 INQ0014667/2
102 Witness statement of Robert Cornall INQ0107032/34/para 115
103 Witness statement of Dr Michael Gregory INQ0107034/32/para 123

104 Witness statement of Margaret Kitching INQ0107036/27/para 107
105 INQ0014651
106 Witness statement of Margaret Kitching INQ0107036/27/para 106
107 INQ0014673/5
108 INQ0014673/4
109 INQ0014673/3
110 Witness statement of Robert Cornall INQ0107032/34/para 117; INQ0014673/3
111 INQ0014673/2
112 INQ0014673/1
113 INQ0014673/1
114 Witness statement of Margaret Kitching INQ0107036/28/paras 110-112
115 INQ0003193
116 INQ0003193
117 INQ0003193
118 INQ0014678
119 Witness statement of Margaret Kitching INQ0107036/35/para 130
120 Witness statement of Andrew Bibby INQ0106970/3/para 4d
121 Witness statement of Andrew Bibby INQ0106970/57/para 217
122 Witness statement of Andrew Bibby INQ0106970/58/para 220
123 Witness statement of Dr Michael Gregory INQ0107034/34/para 132 and 36/para 140
124 Witness statement of Dr Michael Gregory INQ0107034/38/para 151 and 39/para 153
125 Witness statement of Dr Michael Gregory INQ0107034/39/para 152
126 Witness statement of Margaret Kitching INQ0107036/36/paras 135-136
127 Witness statement of Robert Cornall INQ0107032/37/para 133
128 Tony Chambers 27 November 2024 209/4 to 210/4
129 Tony Chambers 27 November 2024 211/7-11
130 Ian Harvey 29 November 2024 134/6-14
131 Ian Harvey 29 November 2024 134/7-9
132 INQ0003193

Chapter 26. Contacting the police

26.1 On 28 February 2017, Mr Harvey held a meeting with Dr Subhedar, Dr Brearey, Dr Jayaram and Dr Gibbs to discuss the hospital's response to the concerns raised by the consultants, the issue of mediation with Letby, the case note review undertaken by Dr Hawdon and the need for further investigation.

26.2 The consultants informed Mr Harvey that they had reviewed Dr Hawdon's report and considered there were eight cases that required further forensic review.[1] Dr Subhedar could not remember whether a staff member was discussed in this meeting, but he recalled there was a tone of concern expressed by the paediatricians.[2] Mr Harvey explained that the consultants:

> "*presented those babies for whom they still had major concerns and, on the back of* [this] *meeting, I felt that there was nothing further for us to do, short of speaking to the police. And I believe it was on the back of me describing that situation to my Executive colleagues and to Tony Chambers that this meeting, with some of the paediatricians and with the network, was scheduled.*"[3]

26.3 On 6 March 2017, Dr Brearey emailed Mr Harvey with a summary of the meeting of 28 February 2017.[4] This included that the consultants were dissatisfied with the way the Trust had handled their concerns, since they had been escalated, and considered that their professional opinions had been disregarded. They considered also that any attempt at mediation with Letby was too early, given that there was still uncertainty over the cause of the rise in mortality and the unexpected collapses. They agreed that further investigation was needed into the cases of eight babies, but that there were six other babies who had collapsed unexpectedly and were transferred to another hospital, and others who had collapsed but were not transferred from the Countess, whose cases had not been examined. Of Dr Subhedar's contribution, Dr Brearey wrote:

> "*Nim* [Subhedar] *also emphasised the Network's position that the observed excess in neonatal mortality at COCH* [the Countess] *could not be explained merely as a consequence of medical or nursing workforce deficits or increased activity and occupancy levels. Other network local neonatal units are working at similar levels of occupancy and staffing and COCH is not an outlier in this regard. Since these units are not reporting an excess in neonatal mortality, it suggests that there is a different explanation for our increased number of unexplained deaths.*"[5]

Dr Subhedar confirmed this in oral evidence.[6]

26.4 Mr Harvey replied by email. He wrote that he had not seen any evidence to support Dr Subhedar's suggestions. This was remarkable given, as Dr Subhedar said in evidence, that the data was included in the Cheshire and Merseyside Neonatal Network's annual report. Mr Harvey did not ask to see it.[7] Had he maintained an open mind, he would have looked at the report. He did not do so. This cannot have been accidental. In truth, he was still not interested in information that ran counter to his narrative that the consultants' concerns were unfounded.

Concerns increase and relationships deteriorate

26.5 On 1 March 2017, the consultants sent a further letter to Mr Chambers thanking him for escalating their concerns to the coroner.[8] They reiterated their concerns that the events had not been fully investigated, as recommended by the RCPCH and by Dr Hawdon. They agreed that there were areas which could be improved, and offered to work with the Trust on an action plan to implement those recommendations. They explained that they had noticed a temporal association with the unexplained and unexpected collapses and the presence of one member of staff at the time of the events. Also, that they were concerned by the unusual nature of collapses and responses to efforts to resuscitate. The consultants considered it their professional obligation to highlight why they thought these matters had not been fully investigated, and cited three reasons as: (a) the RCPCH review was a service review which did not specifically investigate the deaths or causes for collapse; (b) the external case note review (by Dr Hawdon) was not the multidisciplinary review recommended by the RCPCH (but did identify four cases needing further review, to which another four had now been added); and (c) the review of activity, acuity and staffing by Mr Harvey did not identify a reason for the increase in mortality, and was neither independent nor external. Finally, the consultants thanked Mr Chambers for "*sharing the additional comments and observations made by the RCPCH reviewers in the original report*".[9] They added that some of the comments contained factual inaccuracies, which they were willing to discuss. The consultants also confirmed that they had sent a letter of apology to Letby.

Pressure on the consultants

26.6 The consultants' concerns over the mediation process led to Mr Harvey emailing both Dr Brearey and Dr Jayaram separately, on 1 March 2017, to encourage each to attend the meeting with the mediation facilitator scheduled for the following week.[10] Mr Harvey warned both doctors that "*this gesture would also go a long way to protect you from a possible referral to the GMC from other parties which, having supported many doctors who have done no wrong through, even then isn't a comfortable process*".[11]

26.7 Dr Jayaram responded to Mr Harvey's email on 2 March 2017.[12] He sought to clarify why he and Dr Brearey were the only two consultants being required to enter mediation, given that:

a. During the grievance process, someone had reported that he (Dr Jayaram) and two other consultants had been heard to say potentially slanderous things about Letby.

b. Mr Harvey did not know what was alleged to have been said, did not know who had reported it, nor who had fed it back to Letby. It was not known if the remarks had been made in a formal setting or in private. It was also not known how accurate the reports were, and how much had been lost or exaggerated.

c. If the process was to help re-integrate Letby into the neonatal unit, then all seven consultants should take part, as all had expressed the same sentiments. Mr Harvey had explained that the recommendation for only Dr Brearey and Dr Jayaram to participate in mediation had come from the grievance (that is, Ms Weatherley).

26.8 Dr Jayaram also reminded Mr Harvey that the concerns they had raised eight months ago were still present, and that it may be inappropriate to proceed with mediation at this stage. He noted Mr Harvey's reference to 'self-protection' and said he would attend the preliminary meeting after taking advice from his BMA representative.[13]

26.9 The first of Dr Brearey's preliminary mediation meetings was arranged for 7 March 2017, when Dr Brearey was unavailable, being away on RCPCH duties. He alerted Ms Hodkinson to this by email on 6 March 2017. Ms Hodkinson then arranged for Dr Brearey's two preliminary meetings to take place on 16 March 2017, with a meeting with Letby later. On 6 March 2017, Ms Rees emailed Ms Kelly and Ms Hodkinson, copying Mr Harvey, Ms Cooper and Letby.[14] Although Ms Rees knew that the meeting had been rescheduled to 16 March 2017, she seems to have been unaware that Dr Brearey had never been available on 7 March. She wrote that she found it "*hard to believe that Steve* [Brearey] *cannot make an effort* ***for 1 hour*** [emphasis in original]".[15] She expressed concern that Dr Brearey was "*not fully committed to resolving this issue*" and she sought assurances that he should not be allowed to cancel again, given that "*Lucy has suffered enough over the last eight months*".[16] In evidence, she said that she was frustrated that mediation was not insisted upon, and she thought that the consultants were being treated more favourably than a nurse in the same position would have been treated. She was unaware, I infer, that mediation is not compulsory. There was no difference in treatment.

26.10 On 9 March 2017, Dr Brearey accepted the date for the mediation meeting with Letby. In his email to Ms Hodkinson, he reiterated his view that it was inappropriate to proceed with mediation when the hospital was still investigating the cause of the increase in mortality.[17] Dr Jayaram also emailed Ms Hodkinson, on 13 March, to say he was "*extremely uncomfortable*" with the mediation process, given the significant concerns of the consultants.[18] He pointed out that the mediator had said that the process was entirely voluntary, with no obligation to engage, but that Mr Chambers had said (on 26 January 2017) that the Board had a plan which the consultants would be expected to follow, or they would be "*crossing a line*".[19] Also, that Mr Harvey had intimated that "*not engaging … could increase* [his] *chances of being reported to the GMC for whatever I am alleged to have done*".[20]

Meeting between Ms Hodkinson and Dr Jayaram, 15 March 2017

26.11 Dr Jayaram and Ms Hodkinson met on 15 March 2017.[21] At the meeting, Dr Jayaram expressed concerns about the mediation process (as described above), but he was reassured enough to agree to provide his availability for the next mediation meeting. He told Ms Hodkinson he could not understand how the Board had been reassured, and had reached their decisions on the basis of the RCPCH and Dr Hawdon's report, including their decision to reinstate Letby. He felt that the Board had been misled by Mr Harvey.

26.12 The meeting then took an unexpected turn. Ms Hodkinson's file note of the meeting records that Dr Jayaram "*remembers 3 occasions when there were concerns. One* [where] *baby deteriorated and LL at end of cot* [this was Baby K]. *Another* [where] *valve at different setting and it was explained that it was a mistake when she was looking after the baby* [this was Baby H]. *And a third.*"[22]

26.13 Ms Hodkinson was asked in evidence to the Inquiry when she considered the police should have been contacted. She replied: "*For definite 15 March 2017.*"[23] The reason she gave for not contacting the police immediately after this meeting was that she was confused as to why Dr Jayaram waited this long to tell her, and that she needed to triangulate with others what she had been told.[24] I would put it another way. She could not quite believe what she had been told and wanted to see if there was any support for it. She and her colleagues would have to decide what to do with this new information. In the event, they did nothing.

Meeting of executives, 16 March 2017

26.14 At the executives' meeting the following day, 16 March 2017, they discussed Dr Jayaram's suggestion for a meeting of all parties. Mr Chambers was against this, and it was agreed instead that he, Mr Harvey and Ms Hodkinson would meet with Dr Brearey, Dr Jayaram and Dr Subhedar.[25]

26.15 Ms Hodkinson also informed the Executive Team that Dr Jayaram had told her the consultants felt bullied, intimidated and victimised, and that they did not feel reassured by how Mr Chambers and Mr Harvey were managing the process.[26] In evidence to the Inquiry, Ms Hodkinson said that the relationship between the consultants and executives "*had broken down at that stage*".[27] Ms Hodkinson also informed the executives about the three incidents Dr Jayaram had disclosed. Below is an excerpt from her meeting notes:

> "*Sue* [Hodkinson]. *3 deaths. Lucy at cot. Real concerns. Lucy moved valves.*
>
> *AK* [Ms Kelly]. *Why not before? Serious allegations.*
>
> …
>
> *TC* [Mr Chambers]. *Lucy cannot go back to Unit. They want us to throw Lucy under a bus.*
>
> *AK. Challenge – she* [should] *go back.*
>
> *TC. Okay she goes back and something happens. New comments – deal with a Speak Out Safely. Part of me says ring police and GMC*."[28]

26.16 This is the first record of Mr Chambers referring to ringing the GMC. This could only have been about the paediatricians, as became clear two weeks later (see paragraph 26.36).

26.17 When asked about the reaction to the new information from Dr Jayaram, Ms Kelly described the executives as "*shocked and horrified*".[29] Yet her response in the meeting is recorded as being one of challenge, and that Letby "[should] *go back*".[30] Earlier, she had said: "*Why not before? Serious allegations.*"[31] This suggests that Ms Kelly did not believe the serious allegations because they had not been reported to the executives before that time. Whilst it was a natural and reasonable human reaction to wonder why Dr Jayaram was giving this information now, rather than earlier, it was not reasonable to do nothing at all about what he was saying. I am satisfied that Ms Kelly did not believe it. But that was not the issue. This was evidence that absolutely required safeguarding action. Her response, to which she could have given no thought, was that Letby "[should] *go back*".[32]

26.18 Mr Chambers thought he ought to speak to Dr Jayaram, and a meeting was arranged for 17 March 2017 between Mr Chambers, Ms Hodkinson and Dr Jayaram. Mr Chambers described this as a very short meeting that "*felt very rushed, it was a two-minute thing*".[33] He gave evidence that there were two parts to the conversation. First, Ms Hodkinson set out Letby's conduct in relation to Baby K, as described by Dr Jayaram. Second, he recalled Dr Jayaram's high level of anxiety at the meeting.[34]

26.19 Mr Chambers did not ask Dr Jayaram any direct questions about his disclosures; instead, he asked him general questions about whether there was anything else he needed to know, and how he was feeling. Mr Chambers accepted in evidence that he should have been more direct. He explained that he did not want to intimidate Dr Jayaram.[35]

Meetings between 27 and 30 March 2017

26.20 As agreed at the executives' meeting on 16 March 2017, a further meeting took place on 27 March 2017.[36] Present were Mr Chambers, Mr Harvey, Ms Hodkinson, Dr Brearey, Dr Jayaram, Ms McCabe and Dr Subhedar.[37]

26.21 Dr Subhedar told the Inquiry that, by the time of this meeting, he had learnt that the consultants suspected deliberate harm was involved in the deaths. He explained that by attending the meeting, he and Ms McCabe were "*supporting the paediatricians in their view that this needed to be escalated to a police matter*".[38] Dr Subhedar recalled that a "*referral to the police was discussed at that meeting*".[39] He did not remember that a decision was made.

26.22 Dr Brearey explained that, by this meeting, concern had grown because the consultants began to appreciate that some of the morbidity cases (i.e. non-fatal collapses) may have also been attributable to Letby. Dr Brearey read out an email that Dr McGuigan had sent him regarding his thoughts on the neonatal deaths. Dr McGuigan wrote: "*My opinion is that this can never be investigated properly without a police led investigation … I have read the RCPCH service review and I don't see anything that explains the high mortality rates*."[40] Dr Brearey said in this meeting: "[W]*e directly told or asked the Chief Executive Tony Chambers to go to the police.*"[41] He said that Mr Chambers and Mr Harvey had agreed to go to the police.[42]

26.23 Dr Jayaram told the Inquiry that he thought the meeting was set up in response to the concerns he raised in his meeting with Ms Hodkinson earlier that month. He stated that, by this time, he had "*lost faith*" in Mr Harvey's judgement.[43] Dr Jayaram said: "*We decided before this meeting* [having] *discussed with colleagues that we would explicitly say we just need to talk to the people who are the only people who can look at this forensically which are the police.*"[44] Dr Jayaram understood from this meeting that it was agreed the police would be contacted.[45]

26.24 Ms Hodkinson explained she attended the meeting at Dr Jayaram's request, in case she needed to mediate. She gave evidence that, in this meeting, "*there was collective agreement around going to the police*".[46]

26.25 The minutes of the meeting recorded Dr Brearey expressing his dissatisfaction about the depth of the reviews thus far and raising that "[t]*his needs to escalate to the police*".[47] Mr Chambers said: "*I need to know if we do an individual case note review, or phone the police.*"[48] Ms McCabe was recorded as saying: "*Given the information, on the balance of probability, illegal activity has caused the deaths.*"[49] See also Chapter 25.

26.26 Mr Chambers was then recorded as saying: "*If that is where we are, then phone the police. You can call the police.*"[50] This was the Chief Executive speaking. It was for him to act. Instead, he attempted to pass responsibility to the doctors. On the same theme, later in the meeting, Mr Chambers asked: "*Why have you not phoned the police?*"[51] Dr Jayaram's reply was recorded as being: "*Our career would be on the line if we contact* [the] *police, it would be whistleblowing. Following BMA advice, if there is an alternative of a deeper dive, we should go for it. But this is a worry.*"[52] It is not apparent that Dr Jayaram was given any reassurance about his position, about which he was entitled to be concerned given the conduct of the executives up to that point, in particular the insistence on a line being drawn.

26.27 Discussion about involving the police continued. In relation to criminal and non-criminal behaviour, Mr Chambers was recorded as saying: "*To get the distinction, the only thing to do is a police investigation.*"[53] Mr Chambers accepted in evidence that he had said this, and added that "*the decision was that we would go to the police*".[54]

26.28 Later that evening, Mr Chambers telephoned Mr Cross. Mr Cross's handwritten note of their conversation recorded: "*TC's* [Mr Chambers'] *mtg with Ian Harvey, Steve B and Ravi* [Jayaram]*. No alternative but to report to police – that's what Steve B wants supported by Ravi. Need to discuss a plan.*"[55]

26.29 Mr Chambers' understanding was that, after this meeting, Mr Cross and Sir Duncan Nichol had a conversation about obtaining independent support to assist the Trust to manage the escalation to the police, which resulted in Mr Simon Medland KC being instructed.[56]

26.30 There was a further meeting on the following day, 28 March 2017. Present were Sir Duncan, Mr Chambers, Mr Harvey, Ms Kelly, Ms Hodkinson and Mr Cross. Mr Cross's handwritten notes of this meeting summarised the position of the consultants as follows: "*Position now only independent robust investigation is police investigation according to docs.*"[57] An unattributed comment in the notes also reads: "*Not when but how do we manage police.*"[58] There is reference to a bundle of documents being prepared for Friday 31 March 2017 and "*SPC* [Mr Cross] *to contact police … suggest making*

appointment with police for Mon 3 April 17".[59] Mr Cross also noted an action for "*clarity on Steve B's and Ravi's position on reporting matter to police. Direct question to them.*"[60] In other words, the intention was to put the consultants on the spot about whether the matter should be reported to the police. The executives had known the position of the consultants on this matter for some time.

26.31 The notes also recorded Ms Hodkinson and Ms Kelly telling the group that the plan was to return Letby to the neonatal unit, the week commencing 3 April 2017, for one hour a day. This makes plain that Dr Jayaram's disclosure about Baby K and the other incidents had been ignored. Mr Cross was noted as telling the group that Letby could not return to the unit the following week, as had been the plan, because of the potential police investigation.[61]

26.32 In Ms Hodkinson's oral evidence, she was referred to her Inquiry statement, dated 14 August 2024, in which she wrote: "*It remained a collective decision … to work towards Letby's return to the NNU* [neonatal unit] *on 3 April 2017. Whilst there were ongoing clinical concerns and the potential for a police referral, there continued to be no substantive evidence.*"[62] Ms Hodkinson confirmed that the purpose of this was to ensure the executives were "*maintaining a status quo with Letby*".[63] She explained that "*there was not a plan at that stage for her* [Letby] *to go back but I think in terms of communication with her we were stating that there was still that plan to happen. But internally from an Exec perspective that was not going to proceed*."[64] Whether Ms Hodkinson believed the consultants' concerns or not, she had received a very clear disclosure from Dr Jayaram. She could not simply pretend this had not happened. Nor could the other executives.

26.33 Ms Hodkinson was asked why, despite her saying that she thought the police should be called from 15 March 2017, she was still involved in efforts to return Letby to the neonatal unit, and, in August 2024, had written that there was no substantive evidence. Ms Hodkinson responded: "*I apologise for that.*"[65] The apology did not begin to explain why she had continued with her efforts to return Letby to the neonatal unit.

26.34 On 29 March 2017, at an executives' meeting, there was a brief discussion about "*an additional external review*" which Mr Cross was coordinating.[66] Mr Chambers explained that this was a "*pragmatic option*" and referred to it as "*my*" review arising from his concern for all the parents involved. Mr Chambers also asked Ms Hodkinson, Ms Kelly and Mr Harvey to produce a single timeline, which "*would help explain the process and actions undertaken to my external review*".[67] Two other documents were being produced: (a) 'Rationale' by Mr Cross (see below), and (b) 'Neonatal Services at the Countess of Chester Hospital NHS FT [Foundation Trust]', which was produced by Mr Harvey and included that "*we can demonstrate that we have taken the concerns raised seriously … However, despite extensive and intensive review, the Paediatric Consultants still feel that there are questions to be answered and we feel that we need to share the details and discuss with the police.*"[68] It appears that Mr Chambers' "*external review*" would be conducted while preparations were made to report matters to the police, if necessary. Nothing seems to have come from Mr Chambers' "*external review*".

26.35 On 30 March 2017, Ms Hodkinson spoke by telephone to Ms Slingo.[69] Ms Slingo's handwritten note of this meeting, and the typed summary sent on 4 April 2017 (set out further below), gives some insight into what Ms Hodkinson told her about the thoughts of the executives in relation to contacting the police:

a. "*neonatal unit – particular individual linked by association*" and "*cons* [consultants] *think Trust should call police*"

b. "*? Avoidable deaths – 5 unexplained deaths*"

c. "*if decide to go to police? → consider not much option*"

d. "*? Consider refer cons* [consultants] *to GMC*"

e. "*MD* [Medical Director] *has also* [reviewed] *cases against shift patterns – she* [Letby] *is there a lot but MD considers satisfied incident investigated*"

f. On the final page:"**meet with police*".[70]

26.36 The excerpt at (d) above makes it plain that, in light of the consultants' persistent concerns, a referral to the GMC was to be considered. This underscores, were such underscoring necessary, that there was no question in the minds of the executives of protection for the doctors. The contrary was the case. They were at risk of referral by their employer to the regulator.

26.37 In Ms Slingo's email, she advises Ms Hodkinson based on the information she was provided with during their call on 30 March 2017. She wrote:

> "*The question the Trust is now considering is whether, and if so how, to liaise with the police in this matter, with particular pressure being brought by a consultant and others, about the desire to continue to investigate the issue in the neonatal unit. The motivation for this concern is unclear, but may spring from the personal accountability and involvement of the consultant in the care of the neonates on the unit.*"[71]

The motivation was clear. The doctors were concerned that the investigations had not revealed why the babies had died. The suggestion that the motivation may spring from "*the personal accountability and involvement of the consultant in the care of the neonates on the unit*" is baffling. Unsurprisingly, the advice was that, if the police were to be called, rather than wait for a whistleblower, the hospital should bring in the police first.

26.38 Ms Slingo advised: "*If the matter is to be referred to the police, it is more helpful on balance for this to be the Trust's decision than for the Trust to await a potential whistleblower situation.*"[72] She then sets out various ways the Trust may wish to contact the police. Ms Hodkinson forwarded the email on to Mr Cross.

26.39 Ms Slingo accepted that she did not advise the Trust in direct terms that they should contact the police and in hindsight wishes she had done that. In the event, the wheels were already in motion.

26.40 Ms Slingo reflected on the provision of information provided to her by Ms Hodkinson. She said: "*I do think that I didn't have all the information that you suggest it was available by the July period in 2016, because it doesn't feature in my note as part of the narrative and the history given by Sue … if I wasn't told everything that was known, then that would be disappointing to find out and that appears to be the case.*"[73]

26.41 When Ms Hodkinson was informed of Ms Slingo's evidence that she was not told everything that was known at the time, Ms Hodkinson responded: "[S]*he should have asked me more questions … I felt that I was giving a clear and honest and truthful overview at that time.*"[74]

Preparing for possible referral to the police

'Rationale' document

26.42 On 3 April 2017, Mr Cross prepared a document titled 'Rationale'. The opening paragraph reads: "*In our view, there is no evidence to justify a criminal investigation. However, in the spirit of openness and transparency, the matter is being reported to the Police, having regard to the fact that a number of Consultant Paediatricians are not satisfied with the very thorough investigations and reviews undertaken*."[75]

26.43 The document also included (not exhaustive):

> "*2. A Trust review had highlighted increasing levels of activity and acuity, cross-referenced to lower than recommended staffing levels.*
>
> *…*
>
> *5. Whilst the RCPCH Review identified a number of areas for improvement, they did not identify a single causal factor for the deaths.*
>
> *6. A further independent in-depth review … highlighted some areas for improvement. It did not identify a single causal factor or raise concerns regarding unnatural causes of death.*
>
> *7. Twelve of the deaths have been subject to post mortem, but there have been no suspicious findings.*
>
> *8. A secondary review of four deaths, by Pathologists at Alder Hey Children's Hospital, did not raise any concerns regarding unnatural causes of deaths.*
>
> *9. HM Coroner for Cheshire has been kept fully informed of this matter from the beginning and has not directed any further action, save for holding three inquests …*
>
> *10. The allegation against the nurse was based on her having been present on the unit disproportionately frequently, not necessarily caring for the baby, at the time of the collapse.*
>
> *11. The nurse is one of a few who are full time and regularly worked overtime.*
>
> *12. The nurse is highly qualified, so tended to look after the sicker babies.*
>
> *13. There were no concerns regarding the nurse's performance – she had not been involved in any other incidents.*
>
> *14. The Trust has demonstrated that it has taken the concerns raised seriously and has been open and transparent with the Coroner, its regulators, parents and the public.*"[76]

26.44 In his witness statement to the Inquiry, Mr Cross said that this document was a record of a discussion between Mr Chambers and Sir Duncan, and was a reflection of their views as at 3 April 2017.[77] Mr Chambers said that the purpose of this note was unclear. Whatever its purpose, its title and content make plain the executives' views at that time, with which Sir Duncan was in agreement.

26.45 The 'Rationale' document can only be viewed as a last-ditch attempt to persuade independent people such as Mr Medland KC and the police that the suspicions harboured by the clinicians had no foundation.

26.46 The first paragraph said it all.[78] The document was highly selective. It contained none of the points which the consultants had made repeatedly, and omitted all the information which might have caused an open-minded, independent person to pause and seek more information. It did not refer, for example, to the RCPCH's immediate recommendations, or the temporal association of collapses with Letby's shifts, and it was economical with the truth elsewhere. I accept that the information describing Letby as an exemplary nurse was genuinely believed by the Executive Directors – they had accepted the word of Ms Powell and the other nurses on this.

26.47 The same day, Mr Harvey produced a document headed 'Neonatal Services at the Countess of Chester Hospital NHS FT'.[79] This was prepared in anticipation of the review by Mr Medland KC. It sets out a sequence of events in relation to the increased neonatal mortality. It includes a summary of the various reviews that took place and actions taken in relation to Letby. For example, it details that Letby's shifts had been moved from nights to days "*to ensure that she was supported*",[80] but makes no reference to the fact that deaths at night then stopped. Mr Harvey makes a point of saying, again, that Letby worked full time, had a Qualified in Specialty (QIS) qualification and worked overtime. As before, this information was not put in the context of the other nurses on the neonatal unit. It was a straight repetition of what Ms Powell had said back in the meeting on 11 May 2016 (see Chapter 9). It is not apparent that Mr Harvey ever sought to scrutinise it.

26.48 There was no reference in the document to the staffing analysis done by Ms Sian Williams, or the concerns she had expressed about the need to go to the police. Nor was there mention of the disclosure from Dr Jayaram three weeks earlier.

26.49 Mr Harvey concluded thus:

> "*In summary, we can demonstrate that we have taken the concerns raised seriously and have been open and transparent with the Coroner, our regulators, parents and the public. However, despite extensive and intensive review, the Paediatric Consultants still feel that there are questions to be answered and we feel that we need to share the details and discuss with the police.*"[81]

The first sentence overstates by some margin the openness and transparency shown by the hospital, via the executives.

26.50 In a telephone call, Mr Medland KC received instructions from Mr Cross to advise the hospital. Mr Medland KC said, and I accept, that he knew Mr Cross professionally, as he had been instructed by him to act on about half a dozen inquests over a couple of years. He was a criminal barrister. When asked, he said that he was a Freemason and has never

made a secret of that fact. Mr Cross was also a Freemason, but they were members of different lodges and did not see each other socially, save in passing. I am satisfied that Freemasonry is irrelevant to the Inquiry and say no more about it.[82]

26.51 Mr Medland KC told the Inquiry that, during his initial phone call with Mr Cross, Mr Cross explained: "*There was concern amongst some of the Consultants about serious occurrences in their part of the hospital and he wanted me to consider those and then report to the board.*"[83]

26.52 On 3 April 2017, Mr Medland KC was sent a bundle of papers via email from the Countess. This included the RCPCH 'Final Confidential Report', the summary document 'Neonatal Services at the Countess of Chester Hospital NHS FT' by Mr Harvey, 'Rationale' by Mr Cross, and a timeline created by Ms Hodkinson.[84] Although it was not attached to the email of 3 April 2017, Mr Medland KC thought that he had been sent Dr Hawdon's report. At the very least, he said he was aware of Dr Hawdon's name in April 2017.

26.53 Mr Medland KC said that he took a degree of reassurance from the documents that were sent, because they had not determined that a crime had been committed. However, he also acknowledged that the "*people who had investigated the matters were not tasked with putting together a criminal case*".[85]

26.54 The following day, 4 April 2017, Sir Duncan, Mr Harvey and Mr Chambers met Mr Medland KC. Mr Cross was in attendance and made notes. They discussed the 'Rationale' document.[86] Mr Medland KC advised against the use of the first paragraph "*re: no evidence to justify a criminal investigation*".[87] At the end of the meeting, Mr Medland KC understood that he was being asked to advise the hospital about whether there was sufficient evidence to justify a report to the police. He advised that, before giving that advice, he should hear from the consultants. A meeting was arranged for 12 April 2017.

26.55 On 5 April 2017, Letby was informed that she would not be returning immediately to the neonatal unit.[88]

26.56 Mr Medland KC prepared typed notes of the meeting with the consultants on 12 April 2017. He explained that the purpose of the meeting was "*to bring an independent objective view to present situation and see if formal report to police was presently merited, in other words whether there is presently information giving rise to reasonable grounds for suspecting that a criminal offence has been committed in respect of any one of the neonatal deaths in question*".[89] In Mr Cross's statement, he confirmed that this was the purpose of Mr Medland KC's instruction.[90] This coincided with Sir Duncan's understanding. He explained: "[W]*e had sought Mr Medland's independent view at a meeting with the paediatricians, which he held, as to whether there was sufficient evidence of criminality.*"[91]

26.57 Mr Harvey's and Mr Chambers' recollection of why Mr Medland KC was instructed was at odds with the recollections of Mr Cross, Mr Medland KC and Sir Duncan. Mr Harvey stated Mr Medland KC's role was to "*collate all the information, meet with the paediatricians and advise us the best approach and with what information to go to the police*". In oral evidence, Mr Harvey affirmed this, saying: "*My view was that, prior to the instruction of Mr Medland, we were going to the police.*"[92] Mr Chambers told the Inquiry that Mr Medland KC was instructed to assist the Trust to prepare how to approach the police.[93]

26.58 Mr Medland KC acknowledged that, from the outset, "*there was a difference in understanding*" as to the purpose of the meeting between him and the consultants.[94] Whilst he understood that his instructions were to determine whether there was merit in contacting the police, contrastingly, the consultants were under the impression the decision had already been made to contact the police, and Mr Medland KC was there to help frame their concerns. It is not necessary to resolve this issue. Mr Medland KC answered both questions.

26.59 Mr Medland KC confirmed that, during the meeting, the consultants set out clinical details regarding the neonatal deaths that concerned them. This included examples of specific babies, such as Baby I's frequent transfers to other hospitals, where she stabilised, was returned to the Countess and then rapidly declined; the highly unexpected outcomes for twins; and that rashes were seen on some of the babies. The key passages of the minutes explored with Mr Medland KC in oral evidence were:[95]

a. Paragraph 5 – there was a reference to Beverly Allitt. Mr Medland KC was not sure who raised Allitt's name. However, he stated that in the meeting he drew a distinction between the Allitt and Letby cases. His view was that, at the time, the common thread linking Allitt to the deaths was clearer than the Letby case.[96]

b. Paragraph 6 – Mr Medland KC wrote: "[T]*he police, being strapped for resources and in any event, can only sensibly investigate cases where there is – at the very least – reasonable grounds for suspecting that a criminal offence has been committed. He* [Mr Medland KC] *emphasised that this was very different from there being mere suspicion.*"[97] Mr Medland KC corrected this in evidence. He said that the police do investigate when mere suspicions are brought to them, and that he had overstated the threshold. He said that he should have told the consultants that mere suspicion was enough, and that the police would have been interested in the level of information that the consultants had.[98] Mr Medland KC conceded: "[O]*n the basis of the information I was given I got it very badly wrong*."[99]

c. Also at paragraph 6 – Mr Medland KC wrote: "[R]*eporting any matter to the police was a condign step which was effectively a public action and would incur adverse publicity.*"[100] Mr Medland KC explained in evidence that, in his view, he and the executives were not concerned about the headlines from a superficial point of view, and that the principal consideration was the impact on the families if a criminal investigation took place and it turned out a crime had not been committed.[101]

26.60 Mr Medland KC strongly encouraged the consultants and the Countess to work together to resolve this, rather than take entrenched or opposed positions. He concluded the meeting by saying: "[A]*s things stand he* [Mr Medland KC] *did not see that there was such material as might give rise to reasonable grounds for suspecting that a criminal offence had been committed.*"[102]

26.61 Nevertheless, Mr Medland KC encouraged the consultants to make a list of their "*best points*".[103] He also raised the possibility of a private discussion with Detective Chief Superintendent (DCS) Nigel Wenham (Cheshire Constabulary), on the basis that DCS Wenham sat on the CDOP.[104] It was this advice that led, in short order, to contact with the police.

Extraordinary Board meeting, 13 April 2017

26.62 The following day, on 13 April 2017, an Extraordinary Board meeting was convened.[105] Mr Medland KC was invited to attend. The minutes recorded that Mr Medland KC said:

> "[I]*n his* [Mr Medland KC's] *view there is no evidence of a crime but the consultant view is to go to the police. He suggested that an alternative approach would be to approach the police member of the Child Death Overview Panel … We cannot say no to a further review of some sort as* [Dr Hawdon's] *report says a broader forensic review is needed of the class 2 cases.*"

Mr Medland KC added: "**[Y]*ou need to accept that if something is still unanswered or there are still genuine concerns in well minded people, you should go to the police*** [emphasis added]."[106]

26.63 Mr Medland KC also suggested that it would be helpful to ask Dr Hawdon what she meant by forensic review.[107] He considered that the executives should work with the consultants and involve them in the commissioning of any further forensic review, so that everyone would understand what was being done, and there would be no secrets between them. He urged the executives to let the consultants decide which cases should be included in the further forensic review. The agreed action from the meeting was that the Trust would ask Dr Hawdon to take the forensic review forward.[108] Mr Medland KC explained that this meeting was his last involvement in the case.

26.64 During the course of the meeting, Sir Duncan referred to the Beverly Allitt case.[109] There was no reference to the case of Allitt in the minutes of previous Board meetings. As far as Sir Duncan could recall, this was the first time he mentioned the Allitt case to the Board.[110] This was despite Sir Duncan's professional involvement in the circulation of the Allitt Inquiry and despite Dr McCormack having raised the names of Shipman and Allitt at the meeting on 30 June 2016, which Sir Duncan attended[111] – a matter Sir Duncan was unable to recall in oral evidence.[112]

26.65 Sir Duncan described Mr Medland KC's input as follows:

> "*Mr Medland reported back to us as I recall that he didn't find any evidence of -- of criminality. But he used an expression that stayed in my memory arguably since then, along the lines that: if events are still unexplained and if well-minded people still have concerns, then the police should be called and I wish we had had that advice in July 16.*"[113]

26.66 I don't accept that the Board needed a KC to make that point. I do accept, as I have said elsewhere in the Report, that running a hospital is a complex and demanding task. However, Mr Harvey, Mr Chambers, Ms Kelly and Ms Hodkinson had all known for about a year that the consultants had concerns. Had they treated the consultants with the respect due to any colleague, the executives would have acknowledged that these were concerns of well-minded people, and that action was required. Instead, they disregarded the consultants' views, and, in the end, were thinking of ways to deal with them. The Non-Executive Directors and the Chair should have scrutinised what they were being told by the executives at this meeting, and challenged them about their attitude to the whistleblowers.

26.67 The Board also discussed communications with the parents of babies who had died. Mr Harvey told the Board that the hospital had endeavoured to keep the families up to date, although there were things to be learned.[114] Mr Chambers stated that the hospital had written to the families, advising them in an open and transparent way of what the hospital knew. In oral evidence, Ms Hopwood said she wished she had asked for sight of a written communications plan.[115] Sir Duncan accepted the hospital did not exercise the "*appropriate duty of candour*" towards parents, which was a "*serious failure*".[116]

Involvement of the CDOP

26.68 At a meeting of the executives on 19 April 2017, "[i]*t was agreed that the way forward was through the Child Death Overview Panel (CDOP) and IH* [Mr Harvey] *agreed to arrange a meeting with representatives of CDOP as soon as possible*".[117]

26.69 On 20 April 2017, a telephone call took place between Mr Harvey and Ms Hayley Frame, Independent Chair of the Pan Cheshire CDOP. Mr Harvey's note of the meeting is brief. It contains one line: "*?notified rapid response process for any ? deaths.*"[118] Ms Hayley Frame's evidence was that the call was brief and the reason Mr Harvey contacted her was based on the RCPCH's observation that the SUDIC protocol for unexpected deaths was not always followed, and that the Countess "*hadn't raised any concerns with the CDOP around the cluster of deaths in a timely way and that they would do so going forward*".[119] There was no reference at that time to an intention to call the police.

26.70 On 27 April 2017, a meeting was held with DCS Wenham, the police representative on the CDOP. This was two weeks after the Extraordinary Board meeting. The meeting was attended by Ms Hayley Frame, DCS Wenham, Mr Harvey, Mr Cross, Dr Jayaram and Dr Holt. Dr Mittal was also there at Ms Hayley Frame's request. The beginning of the meeting focused on the investigations into the deaths. However, at some point, the meeting "*shifted*" when they began to discuss "*staff rotas and that there was one member of staff who was on shift during each collapse*". Ms Hayley Frame recalled thinking: "*What, what are we being told here? This, this is gravely concerning.*"[120] This was the first time she became aware there were suspicions a staff member was involved in the deaths.

26.71 Ms Hayley Frame said that it became "*clear that the reviews that had taken place so far hadn't ruled out anything untoward*". She remarked it was "*clear coming out of that meeting that there was something very worrying and Nigel* [Wenham] *had the same view*".[121] Ms Hayley Frame said in evidence that, during the meeting, DCS Wenham said this was a matter for his officers.

The police

26.72 The same day, on 27 April 2017, DCS Wenham emailed Mr Harvey, informing him that he had:

> "*provided initial high level briefing to ACC Darren Martland (Crime)* [Assistant Chief Constable Darren Martland, Cheshire Constabulary]
>
> …
>
> *Mr Martland is in agreement with the approach outlined*
>
> - *COCH to provide me copies of all reports completed to date (my email is secure)*
> - *COCH to prepare more detailed briefing and arrange a briefing session with key people (Including myself, Det Supt Duggan, and ACC Martland) …*
>
> …
>
> - *As discussed, please progress with addressing a formal letter to Chief Constable Simon Byrne,* [formally] *requesting that Cheshire Police conduct a forensic investigation into the circumstances surrounding the deaths*
>
> *Following the formal briefing*[,] *the Constabulary will make a formal decision as to conduct a forensic investigation or not.*"[122]

26.73 Mr Harvey responded to DCS Wenham on 28 April 2017,[123] attaching documents for his review: Mr Harvey's summary document dated 3 April 2017 ('Neonatal Services at the Countess of Chester Hospital NHS FT'), the timeline of events produced by Ms Hodkinson and Dr Hawdon's review.

26.74 On 2 May 2017, after an Extraordinary Board meeting,[124] Mr Chambers wrote to CC Byrne: "[O]*n the advice of T/Detective Chief Superintendent Nigel Wenham … I am writing formally requesting that Cheshire Police conduct a forensic investigation into the circumstances surrounding the deaths with a view to excluding any unnatural causes*."[125] This was an unusual approach to the police. Their role is to prevent and investigate crime. DCS Wenham considered that the clause "*with a view to excluding any unnatural causes*" had no place in the letter, and it appeared Mr Chambers was trying to "*direct a mindset*".[126] For almost a year, Mr Chambers had sought 'to exclude unnatural causes'. This was more of the same.

26.75 Mr Chambers was asked why the police were not contacted earlier. He explained that the decision from 27 March 2017 to contact the police still stood. However, "*we needed help and advice … this was a serious escalation of matters and we needed to be clear around how we would manage that next step.*"[127] This echoes what he said in July 2016 about "*pressing the doomsday button*".[128]

26.76 On 5 May 2017, there was a meeting between Assistant Chief Constable (ACC) Martland and DCS Wenham, of Cheshire Constabulary, and Mr Chambers, Mr Cross and Mr Harvey.[129] The executives told the police, in summary, that:

- a criminal KC had advised that there was no evidence to suggest criminal activity
- there was a shift commonality between Letby and the deaths
- there was no evidence against Letby other than coincidence

- Letby was moved from night shifts to day shifts for her protection
- Letby was QIS, and was therefore more likely to be caring for sicker babies.[130]

26.77 In oral evidence, Mr Chambers agreed it was important that he explained the consultants' concerns at their highest when he communicated with the police.[131] It was put to Mr Chambers that he did not present the full extent of the consultants' concerns in the meeting on 5 May 2017. He conceded that "*it's not been articulated*" in the minutes that:[132]

- Letby was moved from night shifts to day shifts and then the pattern of deaths changed
- when Letby was moved off the neonatal unit, the unexpected deaths and collapses stopped.

26.78 The closing statement of the meeting minutes read: "*There are no significant concerns to suggest any unlawful acts, it appears a series of anomalies that needs to be investigated further.*" It was noted that ACC Martland and DCS Wenham would draft investigative Terms of Reference. A further meeting was arranged for 12 May 2017.[133]

26.79 On 10 May 2017, Dr Jayaram sent a number of documents to DCS Wenham by email. He explained that the documents "*attempt to articulate the clinical concerns of the 7 consultant hospital paediatricians*".[134] Two of the documents were headed:

- 'Reasons for concerns regarding a possible criminal cause for increased neonatal mortality at the Countess of Chester Hospital NHS Foundation Trust, June 2015–July 2016'[135]
- 'Mortality, Acuity and Staffing 2015–2016'.[136]

26.80 DCS Wenham said in evidence to the Inquiry that, collectively, the documents sent to him were "*incredibly powerful and important in terms of how we moved forward*".[137]

Meeting between Mr Chambers, Mr Cross, Mr Harvey and Cheshire Constabulary, 12 May 2017

26.81 On 12 May 2017, Mr Chambers, Mr Cross and Mr Harvey met Cheshire Constabulary: ACC Martland, DCS Wenham (Deputy Head, Public Protection Directorate), Detective Inspector (DI) Paul Hughes (Major Investigation Team), Mr David Bryan (Head of Legal) and Ms Laura Fox (secretary). The minutes of this meeting record that the documents which Dr Jayaram had sent to DCS Wenham on 10 May 2017 had now been shared with the Executive Team. The minutes record that the executives made the following comments.[138]

26.82 Mr Chambers stated:

- There was nothing new in the documents that Dr Jayaram had not already shared with the RCPCH and others.
- It was disappointing that the group of clinicians had not moved on, despite all of the reviews and enquiries that had been completed.

- He would be "*comfortable to pause at this point, but equally would be comfortable to see what level of enquiry could be done*".
- The matter may "*become a wider GMC issue as there becomes a point where a group of clinicians who are not prepared to take the recommendations of RCPCH are blocking the ability to move forward*".[139]

26.83 The minutes record Mr Harvey noted that the concerns were considered an "*HR issue*" and "*nothing that could potentially be evidence of a criminal investigation*".[140]

26.84 DCS Wenham was invited in evidence to the Inquiry to comment on the remarks made by Mr Chambers and Mr Harvey. He opined: "[W]*hen you read some of the comments now it's like indeed doors are trying to be shut.*"[141] Mr Chambers was asked about this. He rejected DCS Wenham's interpretation of what he and Mr Harvey said.[142] He denied that he was discouraging, as opposed to encouraging, an investigation.[143] Mr Harvey also disagreed with DCS Wenham's view. Mr Harvey said: "*I wouldn't accept that I was actually trying to dissuade them.*"[144]

26.85 At the meeting, ACC Martland is recorded as saying: "[T]*here is no specific allegation at this point to suggest a criminal act. We do not have any reasonable grounds to suspect or believe that this may have been the case.*"[145] ACC Martland questioned whether there was an independent body that could consider all the reviews and note any issues, including possible evidence of criminal wrongdoing, prior to a criminal investigation. The executives might reasonably have inferred that the police were not unanimously in favour of starting an investigation. The minutes also record Mr Chambers' agreement that the police needed to speak with the consultants about their concerns.

26.86 An agreed outcome from the meeting was that the police would meet Dr Jayaram. In the event, they met Dr Jayaram, Dr Brearey and Dr Holt.[146]

Mr Chambers' plan for the consultants

26.87 Mr Chambers drove back to the Countess after the meeting. By that stage, he must have realised there was a prospect that the police would investigate, but there was also a realistic prospect that they would not. If they did investigate, matters would be out of his hands. If they did not, he would have to decide what to do. He and Ms Hodkinson had a pre-arranged one-to-one meeting in their diaries. He used the meeting to work through a plan for what to do if the police did not investigate. As was her usual practice, Ms Hodkinson made a note. It reads:

> "*RJ/SB* [Dr Jayaram/Dr Brearey] *plan re management.*
> *1. GMC*
> *2. Actions from grievance*
> *3. Mitigation from SoS* [Speak Out Safely]*/whistleblowing*
> *4. Action plan to manage out.*"[147]

26.88 This note is one of the most revealing documents in the Inquiry. It sets out an aggressive plan to get rid of Dr Brearey and Dr Jayaram, the consultants. When asked about it in evidence, Ms Hodkinson began by saying that Mr Chambers did not really mean it.

She then said that Mr Chambers was frustrated and had discussed plans with her to manage Dr Brearey and Dr Jayaram out of the Trust around the Speak Out Safely Policy. However, she had challenged him, and the plan ultimately did not go ahead.[148]

26.89 Initially, Mr Chambers said he did not recollect this meeting. He later gave the following explanation:

> "*For me, what this meeting was all about was patient safety and insomuch as if we have a scenario that there is a breakdown in relationship between the leaders of our services and the nurses in those services, then that's never going to be good for patient safety. So I kind of was thinking if there isn't a police investigation, what are we going to do? So this was just almost a -- a -- well, we can do this, we can do that. 'Sue, guide me.'*"[149]

26.90 This explanation, which I reject, is on a par with his wordy and untruthful explanation of what he had actually said to Mr Letby, when he was recorded as saying, "*we didn't believe it*" (see Chapter 22).[150] There is no evidence in the note to suggest that patient safety crossed his mind. Nor that he considered there was anything to be done in respect of anyone other than Dr Jayaram and Dr Brearey. He could not explain, when answering questions, why the GMC was an option, nor why the question of mitigation around Speak Out Safely and whistleblowing was noted.

26.91 In my view, his obvious intention was to punish the consultants for persisting with their concerns. I do not accept that he was seeking Ms Hodkinson's advice (and she did not suggest he was). His plan was clear. Each point on the management plan noted by Ms Hodkinson needs no explanation. The two consultants were to be reported to the GMC. The punitive recommendations from the grievance were to be actioned. A way was to be found around the Speak out Safely/Freedom to Speak Up/whistleblowing provisions. There would be an action plan to manage the doctors out. It could not have been clearer. I note that, having said he had no recollection of the meeting (and he had heard Ms Hodkinson's evidence the previous day, so the questions about the meeting cannot have been a surprise), he went on apparently to confirm Ms Hodkinson's evidence that she had pushed back on his suggestions, saying: "[A]*s Sue said yesterday, her advice was: well, that wouldn't be sensible.*"[151] Whether or not she advised against such a misconceived and unfair plan matters not. What caused him to change course was the fact that, a few days later, contrary to his expectation, the police announced they were going to investigate.

26.92 What Mr Chambers is recorded as saying is entirely consistent with his approach for nearly 11 months, that the problem here was the doctors. He never once scrutinised his own thought processes. He failed to consider whether his views were unfair or unfounded. If the doctors' concerns turned out to be well founded, the consequences for patients and their families were catastrophic. Mr Chambers was so determined to bring down the doctors that he lost objectivity and made poor decisions.

26.93 On 15 May 2017, Dr Brearey, Dr Holt and Dr Jayaram met DCS Wenham and DI Hughes of Cheshire Constabulary.[152] In the meeting, the consultants set out the detail of their concerns about the clinical features and circumstances of the deaths, Letby's association, and their concerns that she may be deliberately harming babies. They pointed out that, before she was moved onto day shifts, the sudden collapses occurred between midnight and 04:00. Once she was on day shifts, there were no sudden collapses during the night. After Letby had been moved off the neonatal unit, there

were no neonatal deaths, and no unexpected or unexplained sudden deteriorations. The redesignation of the unit could not be the only reason for this, since it was now permitted to care only for babies of over 32 weeks' gestation. Six of the babies who died in 2015 to 2016 while Letby was working on the unit were over 32 weeks' gestation. Three of these babies featured on the indictment, three did not.

26.94 DCS Wenham stated:

> "*The meeting was -- I can't describe how powerful it was. They were knowledgeable, they spoke from the points of view whereby they were dealing with these things real-time and the -- they have had -- they have had -- they have lived and* [have] *breathed these events …*
>
> *I think we all owe them a great deal for coming forward and speaking out the way they did.*"[153]

26.95 Following the meeting with the doctors, DCS Wenham had a telephone meeting with the executives and informed them that he had decided to begin a police investigation.

Should the police have been notified earlier?

26.96 DCS Wenham was asked about whether the police should have been notified at an earlier stage. His response was: "[C]*learly with hindsight and looking back the obvious answer to that is yes. You know, we should have been notified and engaged with earlier.*"[154]

26.97 Almost everything the consultants raised with the police in May 2017 was known to the executives after the deaths of Baby O and Baby P. Much of what was known in May 2017 was known in June 2016. Had they gone to the police then, the investigation would have begun then. That would have significantly reduced the time that parents had to endure uncertainty about what had happened to their babies. Responsibility for that delay lies squarely at the door of the executives.

26.98 As I have mentioned already, Mr Harvey said in evidence: "[I]*n June/July 2016 I had expressed an opinion that we should approach the police and I sincerely regret that we didn't at that time.*"[155] I accept his evidence about that. This was a major error of judgement, which was the result of a refusal to acknowledge that the doctors' concerns might be well founded, and a failure to act on concerns and suspicions that the executives all accepted were held in good faith.

26.99 Mr Chambers accepted that the concerns featured in the consultants' 'best points' document (the 'Reasons for concerns' document given to the police in May 2017) were shared with the executives in 2016.[156] He denied seeking to stall or obstruct a police investigation.[157] That was precisely what he had done and was what he intended to do. The effect of the actions of the executives from June 2016 was that the police investigation was delayed by a year.

26.100 As I have set out in Chapter 11, Ms Kelly and Mr Harvey exchanged emails on 29 June 2016, stating that they both thought that the police should be contacted.[158] However, on 30 June 2016, Ms Kelly changed her position. When asked about this, she stated: "[O]*n reflection maybe we could have gone to the police then.*"[159] Even at the Inquiry, Ms Kelly could not bring herself to say that the executives should have gone to the police then. Her misguided determination to protect her nurse had overridden her judgement.

26.101 Ms Hodkinson was asked when she considered that the police should have been contacted. She replied: "*For definite 15 March 2017.*"[160] She said: "*I was disappointed that they took so long for the police to be instigated from that 15 March meeting.*"[161] I think it unlikely that was her view at the time. She was still in favour of introducing Letby back onto the neonatal unit after 15 March 2017.

26.102 As I have set out in paragraph 26.65, Sir Duncan reflected:

> "*Mr Medland reported back to us as I recall that he didn't find any evidence of -- of criminality. But he used an expression that stayed in my memory arguably since then, along the lines that: if events are still unexplained and if well-minded people still have concerns, then the police should be called and I wish we had had that advice in July 16.*"[162]

26.103 A number of witnesses, including Non-Executive Directors, recalled that Mr Cross had explained the level of disruption that would occur if the police were called (closure of the unit, blue tape everywhere). I accept that this was something Mr Cross said, more than once. I doubt it was said in the hope of encouraging a referral to the police. It should not have been said, not least because it was wholly inaccurate. Whatever was said did not constitute a reason not to call the police. In the event, the reality of the approach of the police to the investigation was nothing like Mr Cross's description. The unit was not closed, there was no blue tape and disruption was kept to a minimum. Doctors reviewed the medical records over many hours and days, at the request of the police. They prepared statements. Nurses too provided statements. This was all done after working hours, which were spent treating and caring for patients.

Parents

26.104 The Inquiry heard evidence from parents that they were not aware of any concerns that an individual had caused neonatal deaths and collapses at the Countess until Letby was arrested and the police contacted them in July 2018.

26.105 Mother A and B said she had "*mixed feelings*" when she learnt that the consultants had made allegations about a nurse which had resulted in the nurse being removed from the unit.[163] She said: "*I'm forever grateful because the consultants did speak up and did say something, but it's also very sad that nothing was ever shared with us.*"[164]

26.106 Some of the babies who had non-fatal collapses were still being seen as outpatients at the Countess, long after the consultants raised concerns to the executives about Letby and the police had become involved. One of those babies was Baby G. Mother G's evidence was:

> "*Dr Brearey never said anything about an investigation at the Countess of Chester or about concerns over Lucy Letby's care of our daughter. It really upset me to think that he might have helped cover it all up …*
>
> *I did receive a call from Dr Brearey on the day when the police came to see us in July 2018. He apologised and said he had been unable to tell us about any of the concerns while the police were investigating.*"[165]

26.107 Mother C told the Inquiry: "*We had absolutely no idea that there had been layer upon layer upon layer of concern voiced by various people within the hospital about the conduct of Lucy Letby and her association with these deaths.*"[166] She said:

> "*The first time that we knew that there was anybody linked to our son's death was on 3 July 2018, when we were phoned by Cheshire Police to inform us that somebody had been arrested on suspicion of murdering our son. That was the first time that we had any information linking an individual.*
>
> …
>
> *… to not inform us at all until somebody is arrested is unforgivable.*"[167]

26.108 I agree. It is clear from the evidence of Dr Brearey and Dr Gilby that the executives did not expect the investigation to lead anywhere. This was consistent with their earlier conduct. To the extent that they thought about the parents at all at this point, it is likely that they considered it better to say nothing, since nothing was to come out of it. This, as it had been in respect of disclosure to the Board and then to the police, was the wrong call. Once the police were investigating the circumstances of the deaths of and injuries to their babies, the parents had the right to know about it. They should have been informed.

Children's Champion

26.109 The RCPCH had recommended in its report that an executive member of the Board should take on the role of Children's Champion. Nothing was done about that for many months. Ultimately, it was imposed on Ms Hopwood, a Non-Executive Director. This was done, without warning, at a meeting of QSPEC on 17 July 2017. She recalled there were more than 20 people in attendance and she was taken by surprise when Mr Harvey, in the course of an update on the neonatal unit action plan, announced that she had been appointed Children's Champion and that she would be attending monthly meetings. She had no relevant experience, she was not an Executive Director, and the issue had not been raised with her.[168] When she challenged Mr Harvey after the meeting, he said that it was necessary to appoint a Non-Executive Director to the role because of the breakdown of relationships with the consultants.[169]

26.110 Leaving aside the discourtesy to Ms Hopwood, the failure to implement this part of the RCPCH report for many months, followed by the appointment of a Non-Executive Director, was a box-ticking exercise at best. Ms Hopwood complained to Sir Duncan the next morning.[170] In due course, she introduced herself at QSPEC in October 2017, but nothing seems to have come of this.

Endnotes

1 INQ0003395/2-3
2 Dr Nim Subhedar 20 November 2024 47/21 to 48/5
3 Ian Harvey 29 November 2024 120/5-12
4 INQ0003395/2-3
5 INQ0003395/3
6 Dr Nim Subhedar 20 November 2024 48/13 to 49/7
7 Dr Nim Subhedar 20 November 2024 50/11-13
8 INQ0006816
9 INQ0006816
10 INQ0103207
11 INQ0103207
12 INQ0003119
13 INQ0003119
14 INQ0060447/2
15 INQ0060447/2
16 INQ0060447/2
17 INQ0006432/1
18 INQ0011870/1
19 INQ0011870/1
20 INQ0011870/1
21 INQ0003219/1
22 INQ0003219/4
23 Sue Hodkinson 26 November 2024 89/17 to 90/1
24 Sue Hodkinson 26 November 2024 72/14 to 73/6
25 INQ0003344/1
26 INQ0003344/1
27 Sue Hodkinson 26 November 2024 77/4
28 INQ0003344/3
29 Alison Kelly 25 November 2024 208/1
30 INQ0003344/3
31 INQ0003344/3
32 INQ0003344/3
33 Tony Chambers 27 November 2024 127/25

34 Tony Chambers 27 November 2024 126/6-23
35 Tony Chambers 27 November 2024 126/21 to 129/8
36 Tony Chambers 27 November 2024 127/3-4
37 INQ0003150/1
38 Dr Nim Subhedar 20 November 2024 61/25 to 62/2
39 Dr Nim Subhedar 20 November 2024 51/12-13
40 INQ0101093/2
41 Dr Stephen Brearey 19 November 2024 179/9-10
42 Dr Stephen Brearey 19 November 2024 180/2-3
43 Dr Ravi Jayaram 13 November 2024 170/15
44 Dr Ravi Jayaram 13 November 2024 172/3-7
45 Dr Ravi Jayaram 13 November 2024 222/25 to 223/3
46 Sue Hodkinson 26 November 2024 156/12-13
47 INQ0003150/1
48 INQ0003150/2
49 INQ0003150/2
50 INQ0003150/2
51 INQ0003150/3
52 INQ0003150/3
53 INQ0003150/6
54 Tony Chambers 27 November 2024 157/5-6
55 INQ0003384
56 Tony Chambers 27 November 2024 44/21 to 45/11
57 INQ0014281
58 INQ0014281
59 INQ0014281
60 INQ0107706/169
61 INQ0014281
62 Sue Hodkinson 26 November 2024 182/23 to 183/2
63 Sue Hodkinson 26 November 2024 182/9-10
64 Sue Hodkinson 26 November 2024 182/10-14
65 Sue Hodkinson 26 November 2024 180/13 to 183/13
66 INQ0004409/1
67 INQ0004409/1
68 INQ0014378/4

69 INQ0101944
70 INQ0101944
71 INQ0003088/2
72 INQ0003088/2
73 Corinne Slingo 21 November 2024 157/18-22 and 158/10-12
74 Sue Hodkinson 26 November 2024 64/20-24
75 INQ0003226
76 INQ0003226
77 Witness statement of Stephen Cross INQ0107707/47/para 183
78 INQ0003226
79 INQ0014378/1
80 INQ0014378/1
81 INQ0014378/4
82 Simon Medland KC 21 November 2024 160/21 to 162/25 and 195/22 to 197/24
83 Simon Medland KC 21 November 2024 160/10-13
84 INQ0014378; INQ0006123; INQ0002927
85 Simon Medland KC 21 November 2024 201/7-9
86 INQ0006123
87 INQ0003351
88 INQ0003477
89 INQ0005857/1
90 Witness statement of Stephen Cross INQ0107707/48/para 185
91 Sir Duncan Nichol CBE 2 December 2024 80/25 to 81/3
92 Ian Harvey 29 November 2024 126/16-17
93 Tony Chambers 27 November 2024 45/2-5
94 Simon Medland KC 21 November 2024 181/7-11
95 INQ0005857/1
96 Simon Medland KC 21 November 2024 183/23 to 184/11
97 INQ0005857/2/para 6
98 Simon Medland KC 21 November 2024 185/3 to 188/5
99 Simon Medland KC 21 November 2024/204/13-14
100 INQ0005857/2/para 6
101 Simon Medland KC 21 November 2024 186/19 to 187/1
102 INQ0005857/4/para 12
103 INQ0005857/5/para 14

104 INQ0005857/5/paras 14-15
105 INQ0003236/1
106 INQ0003236/2
107 INQ0003236/2
108 INQ0003236/5
109 INQ0003236/4
110 Sir Duncan Nichol CBE 2 December 2024 133/8-12
111 INQ0015639/55
112 Sir Duncan Nichol CBE 2 December 2024 46/19 to 47/1
113 Sir Duncan Nichol CBE 2 December 2024 81/8-14
114 INQ0003236/5
115 Rachel Hopwood 3 December 2024 152/2-17
116 Sir Duncan Nichol CBE 2 December 2024 138/5-8
117 INQ0004414/1
118 INQ0003244
119 Hayley Frame 18 November 2024 122/6-8
120 Hayley Frame 18 November 2024 123/24 to 124/4
121 Hayley Frame 18 November 2024 124/9 to 125/2
122 INQ0003340
123 INQ0003337
124 INQ0004221/1
125 INQ0102319/2
126 DCS Nigel Wenham 20 November 2024 204/24 to 205/20
127 Tony Chambers 27 November 2024 44/18-20
128 INQ0003365/8
129 INQ0003077
130 INQ0003077/2
131 Tony Chambers 27 November 2024 46/3-6
132 Tony Chambers 27 November 2024 49/7-13
133 INQ0003077/2
134 INQ0102300/3
135 INQ0003671/1-3
136 INQ0102303/2-4
137 DCS Nigel Wenham 20 November 2024 165/17-18
138 INQ0003076/1-6

139 INQ0003076/1-6
140 INQ0003076/5-6
141 DCS Nigel Wenham 20 November 2024 172/5-6
142 Tony Chambers 28 November 2024 42/12-20
143 Tony Chambers 27 November 2024 50/21-24
144 Ian Harvey 29 November 2024 130/17-18
145 INQ0003076/2
146 INQ0003076/10
147 INQ0015642/48
148 Sue Hodkinson 26 November 2024 169/7-14
149 Tony Chambers 27 November 2024 180/2-10
150 INQ0003463/2
151 Tony Chambers 27 November 2024 184/5-6
152 INQ0102309/2-7
153 DCS Nigel Wenham 20 November 2024 179/22 to 180/12
154 DCS Nigel Wenham 20 November 2024 191/11-15
155 Ian Harvey 28 November 2024 72/5-7
156 Tony Chambers 27 November 2024 221/18 to 224/15
157 Tony Chambers 28 November 2024 23/5-14
158 INQ0047571/1
159 Alison Kelly 25 November 2024 145/18-19
160 Sue Hodkinson 26 November 2024 89/24 to 90/1
161 Sue Hodkinson 26 November 2024 174/9-11
162 Sir Duncan Nichol CBE 2 December 2024 81/8-14
163 Mother A and B 16 September 2024 54/10
164 Mother A and B 16 September 2024 54/10-13
165 Mother G 18 September 2024 85/13-22
166 Mother C 16 September 2024 115/3-6
167 Mother C 16 September 2024 114/9-115/3
168 Rachel Hopwood 3 December 2024 155/2-11
169 Rachel Hopwood 3 December 2024 156/6-22
170 INQ0003122

Chapter 27. Events in 2018; the Inquiry 2023 to 2026

Contact with Professor Modi, February 2018

27.1 On 5 February 2018, Dr Brearey emailed Professor Neena Modi, who was then the President of the RCPCH. In his email, he stated that the police investigation continued to cause "*a great deal of stress and upset for the nursing and medical staff on the neonatal unit and for the affected parents*". He raised a number of concerns about the involvement of the RCPCH, including: that the modified report (that is, the redacted dissemination report) did not include any of the paediatricians' concerns and "*was utilised by the Trust to follow a plan*" that had caused them "*considerable patient safety concerns*"; that the paediatricians and affected parents could have been "*supported in a more positive way*"; and that it was unacceptable to "*produce a report and then have no further involvement in the case*". He offered to discuss the problems in person at the RCPCH headquarters, so that they could reflect on their role and be ready to respond to media interest.[1]

27.2 In her witness statement, Professor Modi described this email as the first communication she had received directly from the Countess regarding the RCPCH report.[2] She had not been involved in any aspect of the review and had not seen the final report until it was provided to her by the Inquiry.[3]

27.3 She emailed Ms Eardley, asking for her comments. Ms Eardley responded that the review was confidential. Ms Eardley suggested that the RCPCH did not have an obligation to support the paediatricians beyond providing an independent review.[4] This was a surprising observation from someone representing the RCPCH.

27.4 Professor Modi replied to Dr Brearey on 8 February 2018. She stated that the review manager (Ms Eardley) had maintained contact and follow-up with the client (Mr Harvey) and that the RCPCH did not have authority over what action was taken by the Trust. She asked Dr Brearey if he had something specific in mind when he referred to "*supported by the college in a more positive way*". She stated that it would not be appropriate for the RCPCH to intervene due to the police investigation.[5]

27.5 Whilst this was not, as Professor Turner accepted, the most supportive response, given the issues that had been raised by Dr Brearey, Dr Brearey was undeterred and replied to Professor Modi the same day. He explained that the paediatricians had concerns "*regarding the integrity and competence of the … medical director*", so the RCPCH maintaining sole contact with him was making their problems worse. He stated that he was not sure who they "*should turn to for help*". He explained that the purpose of his email was not to request that the RCPCH intervene in the police investigation but to make Professor Modi aware of what was happening at the Countess and point out

the problem of the RCPCH's redacted report, which had misled the public and affected families and delayed the onset of the police investigation. He asked whether the RCPCH "*could have done more*".[6]

27.6 Professor Modi emailed colleagues at the RCPCH saying: "*There is clearly a real problem here, a cry for help which we cannot ignore.*"[7] An internal RCPCH email dated 9 February 2018 notes that Professor Modi spoke to Dr Brearey that day.[8] A chronology of events was prepared in light of Dr Brearey's contact with Professor Modi.[9] She then wrote to Dr Brearey on 20 February 2018. She advised him to write to "*the Chair of the Trust Board of Directors and the Lead Governor*", copied to Professor Ted Baker (Chief Inspector of Hospitals for CQC), summarising his concerns about the senior leadership of the Trust and the distressing impact their actions were having on the morale of the staff.[10]

27.7 Professor Modi did not have any further communication with Dr Brearey, ceasing to be President of the RCPCH on 14 March 2018. Dr Brearey had contact with her successor (see paragraph 20.153, Chapter 20).

Senior management changes – Mr Harvey retires, Dr Gilby arrives

Dr Gilby arrives

27.8 Dr Gilby qualified as a doctor in 1992. She became a consultant in critical care and anaesthesia and is a Fellow of the Royal College of Anaesthetists and a Fellow of the Faculty of Intensive Care Medicine. From 2012 onwards, Dr Gilby held leadership roles while maintaining a clinical practice. Between 2013 and March 2015, she was the Associate Medical Director at Mid Cheshire Hospitals NHS Foundation Trust. Between March 2015 and January 2017, she was the Executive Medical Director at Wye Valley NHS Trust, and from January 2017 to July 2018, she was the Executive Medical Director of the Wirral University Teaching Hospital NHS Foundation Trust. She was recruited to each of these posts to turn the hospital around.

27.9 In March 2018, Sir Duncan Nichol approached Dr Gilby to ask her to apply for the Medical Director role at the Countess. Mr Harvey had announced his intention to retire later that summer. Dr Gilby applied for the role, and on 1 August 2018, she took up her post as the Medical Director and Deputy Chief Executive. Mr Harvey retired on 21 August 2018.

27.10 Seven weeks after Dr Gilby joined the Countess, Mr Chambers stepped down as Chief Executive. At Sir Duncan's request, Dr Gilby became the acting Chief Executive from 18 September 2018. She occupied the role substantively from 1 April 2019 and, at the same time, stepped down from the post of Medical Director. On 2 December 2022, Dr Gilby was unlawfully excluded from the hospital by the successor Chair to Sir Duncan and stepped down from the role of Chief Executive but remained employed as the Chief Executive until 5 June 2023. Dr Gilby was later successful in claims against the hospital for constructive unfair dismissal, automatic unfair dismissal and detriment on the ground that she had made protected disclosures between April and December 2022. The events surrounding Dr Gilby's departure are outside the Terms of Reference of this Inquiry. The judgment of the Employment Tribunal, distributed on 13 February 2025, was provided to the Inquiry. It is in the public domain on the Courts and Tribunals Judiciary website.[11] The judgment contains excoriating criticism of the leadership of the Countess at that time.

What Dr Gilby knew prior to her appointment

27.11 Dr Gilby told the Inquiry that, prior to joining the Countess in 2018, her understanding was that the Countess had "*a solid reputation externally as an organisation which provided good medical care and attracted high calibre clinicians*".[12] She considered the Chair, Sir Duncan, to be an "*eminent NHS leader*" and, after reviewing the CQC report from June 2016, she felt it appeared to "*imply that it was a good organisation … delivering good care … So I was expecting to go into a high performing organisation.*"[13]

27.12 She had, however, spoken to Mr Harvey in February 2017, when they were both working as Medical Directors. In a discussion about neonatal care at the Countess, he said:

> "'*This issue we've got with the paediatricians is how it started' and he went on to discuss how they had asked for more numbers in the consultant body in the department, that it had been not approved, it was unaffordable. But they had kept on making the point and stamping their feet, is how he put it, until they got what they wanted and, you know, they were the problem. So I left feeling that in the back of my mind, thinking: I wonder what that was about? But it wasn't until later that I discovered that he was referring to the concerns they had about the deaths and unexpected collapses on the neonatal unit, and his irritation at their persistence in that.*"[14]

27.13 A year later, in a meeting with Mr Chambers after her successful application for the role of Medical Director but prior to her joining the Countess, Mr Chambers told Dr Gilby he might be leaving the hospital. He also discussed with her "*the problems that they were having with the paediatricians and Duncan was meeting with the paediatricians at the time to try to broker some sort of improvement in relations*".[15] She said that, in her initial discussions, Mr Chambers was "*very concerned about the breakdown in the relationship, and he emphasised the need to address that, to fix it*".[16]

27.14 On 30 April 2018, Mr Harvey and Mr Cross met all eight consultant paediatricians. This was the first such meeting for nearly a year. At the end of the meeting, Dr Jayaram handed a long letter to Mr Cross from the paediatricians.[17] It was then given to Mr Chambers. The consultants challenged the approach taken by Mr Harvey to their concerns and said that they felt that "*the chief executive and some members of the board treated the consultants' views with contempt*". The letter set out a number of searching questions about the approach taken by Mr Harvey and the rest of the Board to the paediatricians' concerns. On 24 May 2018 (also after Dr Gilby's successful application but before her appointment), Sir Duncan telephoned Dr Gilby to discuss the situation with the neonatal unit and asked for her help. He sent her the list of questions that the consultants had submitted to Mr Chambers. He also provided her with Mr Chambers' draft response.[18] Dr Gilby described the draft response as "*tone deaf*".[19] I would add that it was incorrect in parts and ignored most of the questions. The final paragraph concludes with the following words: "*He … genuinely and sincerely wants the neonatal team to know they have his full support and respect and that of the entire board and colleagues throughout the trust and this has never changed*." This was untrue.

27.15 Dr Gilby explained that, when she read the paediatricians' list of questions, she could "*see the anguish coming off the page, and yet the response that had been formulated read … very defensive, it was dry, it didn't acknowledge their experience*".[20] She thought the response would be "*detrimental*" to what Sir Duncan was trying to achieve.[21]

27.16 Although at that time Dr Gilby had not met the paediatricians, in conversations with both Mr Harvey and Mr Chambers they had expressed to her:

> "*their frustration about the ongoing behaviours and about the reputation issues of the police investigation because this was prior to Letby's arrest, and they were fairly confident, I would say very confident in some cases that, you know, they've been investigating for X number of months, I'm sure they're going to tell us soon that it's all over and, you know, the problem is the paediatricians and their department. And that was the mantra I was given right up until the day I started.*"[22]

27.17 Dr Gilby told the Inquiry: "*I felt they* [Mr Harvey and Mr Chambers] *did believe they* [the paediatricians] *were being genuine, but they were wrong.*"[23] She then described Mr Harvey and Mr Chambers as "*very dismissive of the paediatricians and on a number of occasions it was said to me that they were just looking for somebody to blame* [for poor patient outcomes]".[24] This observation was consistent with the response of Mr Chambers to the paediatricians in the summer of 2016 upon being told that they had concerns about a nurse ("*that would be convenient*").[25] Neither of those views was consistent with Mr Chambers' assertion that he believed the paediatricians were being genuine but were wrong.

27.18 Dr Gilby joined the Countess as the Medical Director and Deputy Chief Executive three weeks after Letby had been arrested.* When she started, she found that there "*were individuals who were shocked, but there was a denial that this meant that it needed to be taken really seriously … it certainly seemed to me that even at that point, it was believed that nothing would come of this, and the focus continued to be on the people who had raised concerns. So that was a surprise.*"[26]

27.19 She was particularly surprised to find that what "*Tony* [Chambers] *wanted to discuss with me, was his concern that actually, he still believed, in spite of the arrest, that no deliberate harm had been caused. He kept repeating that there was no single cause found*"[27] and that he was concerned about a "*wrongful conviction*".[28] In response, Dr Gilby told Mr Chambers: "[I]*t's not for you to find the cause. You have unexpected and unexplained collapses and deaths of patients, and that even one of those, is a cause of concern.*"[29] She was correct.

27.20 In oral evidence, Dr Gilby confirmed:

> "*Ian* [Harvey] *described the Royal College of Paediatrics and Child Health review to me, saying that the Terms of Reference could have been better and didn't include a review of the cases. He told me that pathologists at Alder Hey Children's Hospital had discussed the cases and that an expert review by Dr Jane Hawdon had found all but two of the deaths explained. He said that the pathologists at Alder Hey had felt that they were all explained. He also told me that the Coroner Alan Moore* [now in post after the retirement of Mr Rheinberg] *had no concerns.*"[30]

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* Dr Gilby described the police and their investigation as "*very respectful and discreet*" and "*there was no disruption to services whatsoever*": **Dr Susan Gilby 24 February 2025 112/13 to 113/3-4**.

27.21 She added: "*I was told the Royal College review had not found any evidence of deliberate harm. I was told that there had been a detailed specialist review of the cases and that had not come up with any evidence of deliberate harm.*"[31] Dr Gilby's evidence was that Mr Harvey had not given her the opportunity to read any of the reports at this stage and that she did not see them until after he had retired in August 2018.[32] This was the same approach as he had taken with the Board of Directors, NHS England and, to some extent at least, the coroner and the police. He sought to control the narrative. He presented the case, as he saw it, to others and made sure that only documents that supported his case were seen (if necessary, writing them himself).

27.22 After Mr Harvey retired, Dr Gilby met Dr Brearey to discuss the neonatal deaths and collapses. He asked to meet her away from the main executive offices at the Countess so that he would not run into Mr Chambers or any of the other executives. That simple fact demonstrates the breakdown of relationships in the hospital. Initially, Dr Brearey told Dr Gilby that the consultants "*had been raising concerns about … unexplained, unexpected clinical collapses and deaths in the unit, and they hadn't been listened to, and it hadn't been addressed appropriately*".[33]

27.23 Dr Gilby said that, as Dr Brearey shared the pattern of events: "[I]*t became obvious to me as a clinician, never mind as an Executive, that these just -- it was most unlikely that these were clinically explainable collapses.*"[34] Dr Gilby and Dr Brearey spent three hours together going through the timeline of each collapse and fatality.

27.24 Dr Gilby told the Inquiry: "*I learnt clinical information that gave me great concern.*"[35] She expanded:

> "*I discovered clinical histories of patients who were doing well, who were expected to go home, who perhaps even the day they were due to go home, suddenly having a cardiorespiratory collapse and being refactored to resuscitation in a way that you would never expect with a child or especially with a baby. Perhaps it was helpful that my background is critical care and I have spent quite a bit of time in paediatric critical care and in surgical neonatal and critical care … What Dr Brearey was describing to me was something that I have never ever seen or heard of in my clinical practice, and just one of those for me would have been enough as a Medical Director or a director of nursing to absolutely want to get to the bottom of what has happened here.*"[36]

27.25 Dr Gilby also reflected: "*I did have in my mind questions about why were they not able just to go to the police themselves? … But later … I learned that the interactions had been so threatening that they were fearful.*"[37]

Mr Harvey's departure

27.26 Dr Gilby told the Inquiry that, at the end of a handover meeting with Mr Harvey in the summer of 2018, he made a "*shocking statement*".[38] She explained: "[A]*s we walked out the door, he's packing his things away and he said to me 'You need to refer those paediatricians to the GMC'.*"[39] In response, Dr Gilby asked Mr Harvey why he had not already done so. Mr Harvey told her that he did not want to break his record of not referring anyone. Dr Gilby was surprised that Mr Harvey had not referred any doctor to the GMC given that he had been in position as Medical Director for several years. Ms Langdale KC asked Dr Gilby whether Mr Harvey's statement was a flippant remark. She said it was not. Dr Gilby described Mr Harvey as "*serious … I was being given the*

impression that I had some problem doctors that needed dealing with".[40] Mr Harvey denied this emphatically when he was asked about it.[41] For Dr Gilby, this was a striking and memorable remark. There is no reason to think she fabricated it. As a matter of fact, Mr Harvey had not referred any doctor to the GMC, so her memory of that part of what he said is consistent with what he had done. That suggests her memory is accurate. I accept Dr Gilby's evidence about it. I also take into account that the remark is consistent with Mr Harvey's view of the paediatricians and the fact that, by this stage, he was very busy, clearing the decks before his departure. It seemed to me that, by the time of the Inquiry hearing, he had forgotten about this conversation and cannot now accept that he said it.

Paediatricians report Mr Harvey to the GMC

27.27 On 29 August 2018, well after she had discovered a box of documents about the neonatal unit in Mr Harvey's former office,[42] Dr Gilby received a text message from Mr Harvey:

> "*Hi Susan - Hope all ok. Just had a long chat with Tony. Rumour has it that I can expect to hear from the GMC – alleged paeds* [paediatricians] *have referred. I left a file of neonates documents for you locked in desk drawer in T block office. Please could you get Claire Raggett to copy them for future reference?*"[43]

27.28 Dr Brearey, supported by other consultant paediatricians at the Countess, referred Mr Harvey to the GMC for misconduct. They complained about the way he had treated them in response to their concerns about the neonatal deaths and the possibility that they had not been of natural causes. They complained about his probity and said that he had made misleading statements to internal and external stakeholders and to the public. Mr Harvey denied the allegations. The GMC investigated that matter. Ultimately, they concluded that, whilst some of Mr Harvey's communication with the paediatricians fell below the standard expected, there was no realistic prospect of establishing that Mr Harvey's fitness to practise was impaired to a degree that justified action on his registration. The case was closed with no further action in May 2022.[44] Mr Harvey reapplied for voluntary erasure from the Medical Register and this was granted in June 2022.[45]

Mr Chambers' resignation

27.29 After meeting with Dr Brearey and Dr Jayaram and finding the box file of documents concerning the neonatal unit in Mr Harvey's old office, Dr Gilby told Sir Duncan that she "*had come to* [the] *conclusion that the board, and in particular the Executive team, had got this wrong, and I explained to him why*".[46] Dr Gilby's explanation to Sir Duncan included:

a. explaining the difference between an RCPCH invited service review and a forensic review

b. that Dr Hawdon had written to Mr Harvey explaining that she could not complete the review requested

c. that Dr Hawdon's brief synopsis of each case "*did not give any assurance whatsoever*".[47]

27.30 Dr Gilby said that Sir Duncan "*fully accepted what I was saying, which was in stark contrast to the same conversation that I had with Mr Chambers*".[48] She described Sir Duncan as "*very visibly upset. And he asked me if I would tell what I just told to him to the rest of the Non-Executive Directors.*"[49] Dr Gilby explained her conclusion to the Non-Executive Directors in a meeting on 13 September 2018, at which she was accompanied by Dr Paul Jameson, Chair of the Medical Staff Committee. She said that the response from the Non-Executive Directors was the same as from Sir Duncan: "*visibly upset, horrified*".[50]

27.31 Dr Gilby said:

> "*I don't think they had really understood everything that was being said and hadn't been sighted on the various reports in detail, hadn't had time to consider them. Had very much deferred to the medical expertise of the Medical Director, and I think they had a lot of pieces of the jigsaw that just weren't there and so my discussion with them kind of was starting to fill those gaps, and it was an awful dawning realisation.*"[51]

27.32 By this stage, the consultant paediatricians had made a request of Dr Jameson for a meeting of the Medical Staff Committee, at which they wanted to share their experience of Mr Chambers and hear the experience of other doctors in order to decide whether to consider a vote of no confidence in Mr Chambers. Dr Jameson said in evidence that he thought it was highly likely that the Medical Staff Committee (which includes all senior doctors) might ask for a vote of no confidence in Mr Chambers.[52] At a meeting with the Non-Executive Directors, he told them that the consultant paediatricians were intending to ask for a vote of no confidence in Mr Chambers.[53]

27.33 Mr Chambers was made aware of Dr Jameson's intentions and asked Dr Gilby what she could do to persuade the paediatricians against this. Dr Gilby's evidence was that Mr Chambers said he could not have a vote of no confidence against him because he had not done anything wrong and did not want it on his record.[54]

27.34 Dr Gilby said the paediatricians were intent on having the vote of no confidence and that nothing could deter them.

27.35 Sir Duncan said that, between 2013 and 2017, Mr Chambers had exceeded expectations as Chief Executive. In 2017/18, that changed. Sir Duncan said: "[I]*n 17/18, it was judged by me that he had not met expectations … It was also agreed at that meeting that I had, which was a one-on-one performance review, that -- that he would -- he would be looking for a new -- a new job, the best years possibly behind him.*"[55] Sir Duncan explained that his assessment was due to operational issues, such as the underperformance of the Accident and Emergency Department. He said that the breakdown in relationships with the paediatricians did not form part of the appraisal. This is surprising. By way of example, I note that on 14 February 2018 Dr Gibbs, Dr Jayaram, Dr V, Dr McGuigan, Dr Brearey, Dr Saladi, Dr ZA and Dr Holt wrote to Mr Chambers to express their concerns about the inaccuracy of public statements he had made in relation to the neonatal unit. Mr Chambers responded to say he had "*received and noted*" the letter but he did not directly acknowledge their concerns.[56]

NHS Improvement's role in facilitating senior leaders to find another NHS role

27.36 Sir Duncan explained his position:

> "*I think that we had reached the point where Mr Chambers was looking for a placement outside the Countess of Chester, and Mr Chambers had talked to Ian Dalton* [Chief Executive of NHS Improvement]*, Ian Dalton was open to helping him and he delegated that responsibility to Lyn Simpson to … put things in motion.*"[57]

27.37 Ms Lyn Simpson had a background as a nurse, midwife and health visitor. In 1987, she began working in hospital management. In 1998, she became the Executive Director of a hospital Trust and stayed in the role for approximately six years. She subsequently held various director positions within the NHS, including for one year as Deputy Chief Executive of South London Healthcare NHS Trust. Between 2013 and 2016, she was the NHS Trust Development Authority Director of Delivery and Development (North), and between 2016 and 2019, she was the Regional Managing Director (North) at NHS Improvement (which became part of NHS England).

27.38 One of Ms Lyn Simpson's roles was to assist senior leadership staff to leave a Trust and find a new position within the NHS. Ms Lyn Simpson explained that, over time, "*NHSI* [NHS Improvement] *had taken on that role as a facilitator … to secure the retention of highly skilled, highly trained managers in the service if that was appropriate. It was sometimes to enable a board to move on, where it had become dysfunctional.*"[58]

27.39 On 17 September 2018, Mr Dalton gave Ms Lyn Simpson the task of finding a position for Mr Chambers. She said in evidence that he told her that there was to be a vote of no confidence in Mr Chambers and that "*there were concerns about the relationships in the board which was becoming dysfunctional at the Countess of Chester and … to work with Sir Duncan Nichol to resolve that, which was to facilitate a move of Tony Chambers*".[59] Ms Lyn Simpson said she understood this to mean that the Board were not working well together and Mr Chambers was thought to be the problem.[60]

27.40 Sir Duncan was informed of Ms Lyn Simpson's evidence that she understood the issues at the centre of the facilitated exit were issues between Mr Chambers and the Board.† Sir Duncan responded: "*I don't accept that. The -- the issue was the breakdown of relationships between Mr Chambers and the paediatricians.*"[61] Sir Duncan could not remember exactly what he told Ms Lyn Simpson; however, he stated she was aware that the breakdown in relationships with the paediatricians was the reason he was looking for a placement. It is in light of that evidence that I am surprised Sir Duncan had not raised the issue with Mr Chambers, since the difficulties between him and the consultant paediatricians had been evident for some time. By the time Ms Lyn Simpson was involved, not only had relationships broken down, but matters had escalated to the consultant body as a whole and a vote of no confidence was in prospect within days. Mr Chambers was clear that he did not want this to adversely affect his prospects of obtaining future employment at Chief Executive level.

† Sir Duncan was informed of Ms Lyn Simpson's description of these moves as 'rehabilitation'. This was not a description he was familiar with.

27.41 Ms Lyn Simpson did not know Mr Chambers before she began helping him in September 2018. However, she had the following impression of him from her colleagues: "*Mr Chambers had quite a strong personality and was known for being very demanding and at times could be perceived as somewhat arrogant.*"[62] She stated this did not cause her to be concerned about moving Mr Chambers to another leadership role because there were metrics which demonstrated that he was a good leader. She pointed out that, in 2016, CQC graded the Countess as 'good'. That was more than two years earlier, and in the intervening two years, it had become clear that there were serious problems within the hospital, as I have set out in this Report.

27.42 Facilitated moves can be beneficial where issues have arisen that are not the fault of the person for whom a move is being arranged. This is particularly the case where the loss of an individual's skills and experience would be detrimental to an organisation. But such moves cost public money and should never be used to avoid going through normal employment procedures in circumstances where there are concerns about the individual's skills, conduct or behaviour. In this case, it is not clear that there was any focus on the reasons for proposing such a move. Ms Lyn Simpson did not consider it necessary to question whether a facilitated move was justified, even though she knew that the purpose of it was to avoid the proposed vote of no confidence. She was also aware of the police investigation.

27.43 Mr Chambers confirmed he told Ms Lyn Simpson that he wanted to maintain his status as a Chief Executive and that he did not want the facilitated move to be at the expense of his career. He remarked he had heard facilitated moves for senior leaders described as "*rehabilitation*" or a "*donkey sanctuary*". From his perspective, he considered that he and Sir Duncan were taking a pragmatic step to help the organisation move forward. Mr Chambers asserted: "*I had done nothing that was in breach of that contract, so therefore I had a contractual right as a minimum to serve six months' notice.*"[63] He explained he could not be made redundant because the Chief Executive role still existed and he could not stay at the Countess under the circumstances; thus he sought a move to another organisation where he could reset and rebuild his career.[64] He denied leaving in the circumstances he did to avoid scrutiny of his leadership via the vote of no confidence. That was undoubtedly the effect of what he did, and I am satisfied that was at least part of his motivation for leaving.

27.44 I can see that there may be situations where a senior manager needs support and help to move on when problems have arisen for which they have little responsibility. That was not the case here. The breakdown in relationships and the impending vote of no confidence were the result of Mr Chambers' approach to the consultant paediatricians. Moving elsewhere was an appealing prospect to Mr Chambers and appealing to the Trust, which avoided a vote of no confidence and could then recruit a new Chief Executive. Ms Lyn Simpson discussed various posts in various NHS settings and pursued them on Mr Chambers' behalf. He accepted a role that she had identified in another NHS Trust. She was unaware that Mr Chambers was also pursuing alternative placements on his own account. In the event, he secured a position with the Northern Care Alliance NHS Foundation Trust. His salary was paid by the Countess at the rate for a Chief Executive for six months (his notice period). In the three months before that, he was paid his salary by the Countess while he sought an alternative post – again, something to which he was contractually entitled.

27.45 This meant that, for nine months, the Countess, part of a Foundation Trust, funded Mr Chambers' salary package, which, including pension contributions and benefits in kind, amounted to £290,000 to £295,000, with no benefit to the Countess.

27.46 Sir Duncan confirmed the financial arrangement. He said that this "*was approved by the RemCo* [Remuneration Committee] *of the Countess* [which Sir Duncan chaired] *and also by the national body, which has to approve exceptional provider remuneration*".[65]

27.47 Whilst I can understand that the organisation that had the benefit of Mr Chambers' services may have been pleased to have an extra pair of hands at no cost for a period of six months, it is troubling that public money could be spent in this way without any analysis of the reason for the move from the Countess and no consideration of whether Mr Chambers was a fit and proper person for the role he was taking on.

Announcement of Mr Chambers' departure

27.48 On 19 September 2018, the Countess published an announcement about Mr Chambers' departure from the Trust. Mr Chambers said in evidence that he and Sir Duncan had collaborated on this.[66] The third paragraph read: "*Tony's stepping down as CEO at the Countess is as a result of extraordinary circumstances and is not a judgement on his ability as a CEO but more a reflection of his integrity as a leader.*"[67] That was pure spin. He was stepping down to avoid a vote of no confidence. It had nothing to do with integrity and everything to do with the preservation of his career.

27.49 Mr Chambers' view was that this was an accurate statement. He asserted that events on the neonatal unit did not define his time at the Countess and expressed the view that, overall, he had a successful tenure. The announcement contained a sentence that "*investigations into neonatal deaths at the trust have escalated over the past 2 years and inevitably put relations between senior management and paediatricians under exceptional strain*".[68] There was nothing inevitable about the "*exceptional strain*", as Sir Duncan accepted. Mr Chambers was invited to reflect on whether relations would have "*inevitably*" worsened or whether that was as a result of the way he had managed the concerns of the consultant paediatricians. Mr Chambers said: "*I stand by the decisions that we made.*"[69] That is quite a statement, failing, as it does, to acknowledge any responsibility for any part of the deterioration in relationships. It is striking that he characterised his tenure as successful overall. The lack of reflection and the failure to take any responsibility for the delays and the heartache caused to the families by his approach to the concerns about the deaths of their children is indefensible.

27.50 Mr Chambers stepped down from his role on 19 September 2018 and his employment at the Countess terminated on 30 June 2019 (he was on secondment to his new role for six months from 1 January 2019 to 30 June 2019).

27.51 Dr Gilby stated that it was "*something of a shock*" to find herself having to act as interim Chief Executive "*after only seven weeks in the role*" of Medical Director and Deputy Chief Executive.[70]

27.52 She explained that Sir Duncan had asked her to step up to the Chief Executive role: "*I did say to him that I felt that my time in the organisation hadn't been long enough to act into the role, and I didn't feel confident that in such a challenged organisation, I would be able to deliver what they needed.*"[71] However, she felt she had the full support of Sir Duncan and the Non-Executive Directors.[72]

27.53 Sir Duncan stood down as Chair of the Trust on 31 March 2020.

27.54 Ms Kelly remained in her role as Director of Nursing until she left the Trust in June 2021.

Charges

27.55 Letby was charged with offences of murder and attempted murder in November 2020. She was later removed from the Nursing Register (see Chapter 18).

Events since August 2023

27.56 The Inquiry was announced in Parliament by the then Secretary of State for Health and Social Care, the Rt Hon. Steve Barclay MP, on 4 September 2023. The Terms of Reference were set by the Secretary of State, after consultation with me and with the parents of the babies who died or were injured and with other Core Participants. The Inquiry was formally established on 19 October 2023. The Terms of Reference are on the Inquiry's website and in Appendix 1.

The Inquiry hearings

27.57 The Inquiry's process is set out in Appendix 6. The substantive hearings began on 1 September 2024. All Core Participants were invited to contribute to the selection of witnesses to be called and to indicate whether they wished to ask questions in addition to the questions asked by Counsel to the Inquiry, Ms Langdale KC, and her team. Wherever Counsel for Core Participants wished to ask questions themselves, they were permitted to do so. All witnesses who gave evidence, 134 in total, are listed in Appendix 4. All the oral evidence was transcribed and is on the Inquiry's website. I am grateful to all Counsel for observing all time limits and for sitting extended hours whenever necessary to complete the evidence within five months, with closing submissions a month later in March 2025.

27.58 As I indicated I would, when giving my ruling in March 2025, I have kept under review the question of whether I should pause the publication of my Report. I have read the documents from those speaking at the press conference in February 2025, provided to me by Counsel for the executives and by a journalist. I was urged by Counsel for the executives not to consider them but to acknowledge there was a real possibility that the convictions may be referred to the Court of Appeal and there quashed. Counsel for the families agreed that I should not consider the documents and they set out the countervailing position in respect of the effect of the documents produced at the press conference. Mr Baker KC subjected them to a rigorous and detailed critique. Mr Skelton KC referred to them as "*full of analytical holes*".[73]

27.59 I have endeavoured to keep abreast of press reporting, documentaries and other programmes about the criminal case.

27.60 I have also kept myself informed of the numerous developments within the NHS since the Inquiry began in September 2024, as set out in my Report.

27.61 At the end of the last Parliamentary session in July 2026, the Rt Hon. Sir David Davis MP suggested that the Terms of Reference of this Inquiry should be widened in light of the findings of a number of reviews of maternity care in hospitals in England, some of them recent. I have not thought it appropriate to ask the Secretary of State either to pause the Inquiry or to extend the Terms of Reference.

Endnotes

1 INQ0012734/4-5

2 Witness statement of Prof. Neena Modi INQ0102753/3/para 5.1

3 Witness statement of Prof. Neena Modi INQ0102753/2/para 4.1

4 Witness statement of Prof. Neena Modi INQ0102753/3/para 5.1

5 INQ0012734/2

6 INQ0012734/2

7 Witness statement of Professor Neena Modi INQ0102753/3/para 5.1

8 INQ0012734/1

9 INQ0012748/1

10 Witness statement of Professor Neena Modi INQ0102753/3/para 5.1

11 Employment Tribunals Judgment Case Number: 2402398/2023; 2408654/2023 (https://www.judiciary.uk/wp-content/uploads/2025/02/Dr-Susan-Gilby-v-Countess-of-Chester-Hospital-NHS-Foundation-Trust.pdf)

12 Dr Susan Gilby 24 February 2025 28/1-3

13 CQC, *The Countess of Chester Hospital: Quality Report*, 29 June 2016 (https://api.cqc.org.uk/public/v1/reports/7d84e4fd-bdbe-4e99-839d-df83baa36adc?20210123080129#page=4); Dr Susan Gilby 24 February 2025 29/7-19

14 Dr Susan Gilby 24 February 2025 26/10-22

15 Dr Susan Gilby 24 February 2025 64/7-10

16 Dr Susan Gilby 24 February 2025 71/16-18

17 INQ0102361/78-81

18 INQ0102361/78-81; INQ0006198

19 Dr Susan Gilby 24 February 2025 65/2-3

20 Dr Susan Gilby 24 February 2025 65/14-18

21 Dr Susan Gilby 24 February 2025 65/21

22 Dr Susan Gilby 24 February 2025 66/8-16

23 Dr Susan Gilby 24 February 2025 67/1-2

24 Dr Susan Gilby 24 February 2025 67/2-4

25 Dr Stephen Brearey 19 November 2024 217/13-14

26 Dr Susan Gilby 24 February 2025 17/19 to 18/5

27 Dr Susan Gilby 24 February 2025 72/21-24

28 Dr Susan Gilby 24 February 2025 73/6

29 Dr Susan Gilby 24 February 2025 72/25 to 73/3

30 Dr Susan Gilby 24 February 2025 92/22 to 93/6

31 Dr Susan Gilby 24 February 2025 67/9-13

32 Dr Susan Gilby 24 February 2025 93/14-16

33 Dr Susan Gilby 24 February 2025 77/6-10

34 Dr Susan Gilby 24 February 2025 77/19-22

35 Dr Susan Gilby 24 February 2025 81/10-11

36 Dr Susan Gilby 24 February 2025 78/6 to 79/6

37 Dr Susan Gilby 24 February 2025 81/1-7

38 Dr Susan Gilby 24 February 2025 70/8

39 Dr Susan Gilby 24 February 2025 69/19-21

40 Dr Susan Gilby 24 February 2025 70/16-25

41 Ian Harvey 29 November 2024 50/18 to 51/4

42 Dr Susan Gilby 24 February 2025 74/16-23

43 INQ0099064/18

44 Ian Harvey 28 November 2024 75/10-14

45 Voluntary erasure is the process by which a doctor applies to the GMC to be removed from the Medical Register, ending their registration and ability to practise medicine in the UK, subject to the GMC's approval.

46 Dr Susan Gilby 24 February 2025 90/13-15

47 Dr Susan Gilby 24 February 2025 91/4

48 Dr Susan Gilby 24 February 2025 91/5-7

49 Dr Susan Gilby 24 February 2025 91/20-22

50 Dr Susan Gilby 24 February 2025 92/4-5

51 Dr Susan Gilby 24 February 2025 92/11-19

52 Dr Paul Jameson 8 October 2024 174/3-6

53 Dr Susan Gilby 24 February 2025 94/1-21

54 Dr Susan Gilby 24 February 2025 95/14 to 96/25

55 Sir Duncan Nichol CBE 2 December 2024 110/22 to 111/2

56 INQ0002935

57 Sir Duncan Nichol CBE 2 December 2024 105/8-13

58 Lyn Simpson 21 November 2024 19/1 and 18/21-24

59 Lyn Simpson 21 November 2024 14/15-19

60 Lyn Simpson 21 November 2024 64/13-17

61 Sir Duncan Nichol CBE 2 December 2024 105/24 to 106/1

62 Witness statement of Lyn Simpson INQ0101414/7/para 22

63 Tony Chambers 27 November 2024 189/8-11 and 190/12-13

64 Tony Chambers 27 November 2024 189/15 to 190/3

65 Sir Duncan Nichol CBE 2 December 2024 108/19-21

66 Tony Chambers 27 November 2024 192/11-12

67 INQ0015683/31

68 INQ0015683/31

69 Tony Chambers 27 November 2024 194/8

70 Dr Susan Gilby 24 February 2025 16/2-6

71 Dr Susan Gilby 24 February 2025 16/7-11

72 Dr Susan Gilby 24 February 2025 16/15-17

73 Closing Submissions on Behalf of Family Group 1 18 March 2025 90/8

Chapter 28. The current position

The Countess

28.1 In 2025, the Countess opened an impressive combined facility for women and children.[1] It includes the current neonatal unit, opened in 2021, which occupies a larger, modern space where family integrated care is facilitated. Parents are encouraged to attend ward rounds, and babies' cots are next to their mothers.

28.2 The Countess has a Safeguarding and Promoting the Welfare of Children Policy dated 1 September 2022, in which the Trust commits to ensuring that all staff have access to expert advice, support and safeguarding supervision and training in relation to safeguarding children. The policy specifically addresses the possibility that a member of staff may have harmed a child, committed a criminal offence or displayed behaviours which may indicate that they are not suited to work with children. In such circumstances, the Associate Director of Safeguarding is directed to ensure that a referral is made to the LADO.[2] This is a welcome and important improvement.

Intercollegiate document

28.3 In their submissions, the RCPCH refer to a framework for all healthcare staff published in November 2025: 'Intercollegiate document (2025) Safeguarding children and young people & children and young people in care: Competencies for health care staff'.[3] The document brings together and updates two previous documents (*Safeguarding Children and Young People: Roles and competencies for healthcare staff* (2019) and *Looked After Children: Roles and competencies of healthcare staff* (2020)).[4] It sets out a system of five different levels, with an additional level for senior managers and executives. Level 3, the level for healthcare professionals and staff who deliver a clinical service to children, makes a passing reference under the heading 'Underlying knowledge' to the LADO process.[5] The streamlining of guidance is welcome, but the resulting document is neither clear nor concise.

28.4 The competencies, resources and references do not include mention of learning from previous inquiries or reference to the possibility that healthcare professionals may be responsible for deliberate harm to a child.

28.5 Appendix 5 provides a 'comprehensive' list of safeguarding and children and young people in care legislation, and statutory and non-statutory guidance.[6] There are 13 links to separate documents for England alone.

28.6 I am sorry to say that, whilst the effort taken to produce this guidance is no doubt significant, the effort and time required to read it are too great when something needs to be dealt with immediately. What is needed is clear, focused guidance. The guidance produced for use in schools, as well as providing background and context, is clearer and more direct about what to do in a variety of situations, including when a member of staff has a safeguarding concern or an allegation about another staff member.[7]

28.7 I acknowledge that, whilst it is useful for everyone working in healthcare to have an understanding of the importance of safeguarding and the reasons for it, including an understanding of the context in which children live, such information should be provided by way of guidance. What is at least as important is that there be a short, clear document setting out what to do when there are suspicions that a member of staff may be harming children.

28.8 In a world bristling with frameworks and competencies, short, clear documents are essential.

Memorandum of Understanding

28.9 On 17 December 2024, guidance was issued on investigating healthcare incidents where suspected criminal activity may have contributed to death or serious life-changing harm. This Memorandum of Understanding between regulatory, investigatory and prosecutorial bodies sets out at paragraph 2.1 that it "*has been produced to help deliver early, co-ordinated and effective action following incidents where there is* ***reasonable suspicion*** *that a patient/service user's death or* ***serious life-changing harm*** *occurred as a result of an incident where there is suspected criminal activity in the course of healthcare delivery*".[8]

Neonatal Mortality Governance framework

28.10 BAPM published its Neonatal Mortality Governance framework in February 2026.[9] Dr Eleri Adams, BAPM President and NHS England Getting It Right First Time (GIRFT) Clinical Lead for Neonatology, led the development of the guidance with a working group that includes Dr Ngozi Edi-Osagie, NHS England National Clinical Director for Neonatology, who has also provided a statement to the Inquiry.[10]

28.11 The framework provides detailed operational guidance on mortality governance in neonatal services and is intended to complement existing national and statutory guidance across the UK. It is aimed primarily at perinatal healthcare professionals but may also be relevant to coroners and medical examiners. It sets out expectations for: governance and leadership of neonatal mortality processes; investigation and review of deaths; engagement with coronial and statutory processes; and support for families and staff, including bereavement care. It does not include a mechanism for ensuring the guidance is complied with nor the consequences of failure to do so. I address these issues later in this chapter.

Post-mortem examinations

28.12 The Neonatal Mortality Governance framework makes clear that all parents should be offered a post-mortem examination following a neonatal death, recognising its importance for establishing cause of death, supporting learning, and providing understanding and closure for families. In coronial cases, a post-mortem examination may be mandated.

28.13 There are only 43 paediatric pathologists in England.[11] Without a successful programme to train more, this cannot be achieved (see paragraphs 28.24 to 28.31). I acknowledge the commitment that has already been made. I recommend a clear timetable (see paragraph 28.31).

28.14 The framework links post-mortems to:

- Child Death Review meetings
- escalation to coronial authorities where appropriate
- thematic review and system-wide learning, including identification of clusters, trends or unexpected findings.[12]

Sudden unexpected neonatal deaths (SUDIC, SUDI, PRUDiC, SUPC)

28.15 This section of the framework points out that, whilst such deaths are uncommon in neonatal units and post-natal wards, they require prompt and structured responses.

28.16 BAPM states that deaths meeting the criteria for sudden unexpected death (including SUDIC, SUDI, Procedural Response to Unexpected Death in Childhood (PRUDiC) and Sudden and Unexpected Postnatal Collapse (SUPC)) should follow national statutory guidance, including a JAR/PRUDiC process or equivalent, with arrangements tailored to the circumstances and place of collapse or death.[13]

28.17 The framework gives neonatal-specific examples of circumstances in which a JAR/PRUDiC process should be considered, including:

- a baby born at home without medical professionals present who subsequently dies or is thought to be stillborn
- a sudden death with no immediately apparent medical cause
- circumstances raising suspicion that the death may not have been natural.

28.18 BAPM states that a JAR/PRUDiC should also be triggered where a baby is successfully resuscitated following such events but is expected to die in the following days. In those circumstances, the response should be considered at the point of presentation, rather than at the point of death, to allow accurate history-taking and, where necessary, a ‘scene of collapse’ visit. Where there is uncertainty, the framework advises discussion with the designated doctor for child death as part of immediate decision-making.

28.19 The framework also refers to BAPM guidance on SUPC, noting that it may be used in specific circumstances but should not be relied upon in isolation.

Child Death Review meetings and investigation

28.20 The framework sets out expectations for Child Death Review meetings and associated investigations, including:

- review of individual neonatal deaths and identification of unexpected findings
- investigation of clusters of deaths or increased mortality rates
- notification to the coroner where investigations identify previously unknown factors that may have contributed to a death
- alignment between local reviews, statutory notifications and national safety processes.

28.21 The framework includes a timeline covering actions before death and up to at least three months afterwards, encompassing family-focused actions, staff actions, investigations, and statutory or national notifications.[14]

Governance, leadership and resourcing

28.22 BAPM sets out expectations for leadership and resourcing of neonatal mortality governance, including:

- designated consultant and senior nursing leads for mortality governance with dedicated time for Child Death Reviews, thematic reviews and dissemination of learning[15]
- recommended job planned time for medical, nursing and administrative roles, scaled according to unit type (NICU, LNU or SCU)[16]
- training requirements for staff involved in end-of-life care, including completion of Sands neonatal bereavement care pathway training as a minimum standard.[17]

28.23 All of this may be very sound. It should be assessed by Trusts and NHS England. It should inform decisions about leadership and resourcing. Compliance should be part of any CQC inspection.

Paediatric and perinatal pathologists

28.24 A long-standing staffing shortage has become a crisis in the provision of paediatric and perinatal pathology services.

28.25 Perinatal pathologists study disorders of the placenta, problems affecting unborn babies' development, and causes of miscarriage, stillbirth and neonatal (newborn) death. Paediatric pathologists investigate illnesses affecting children up to 18 years of age. Paediatric pathologists are experts in unique childhood diseases. Their work includes post-mortems.[18]

28.26 The latest *Paediatric and Perinatal Pathology Workforce Report*, published by the Royal College of Pathologists in November 2025, declares a "*workforce crisis*".[19] The key findings are:

- The UK has 52 paediatric and perinatal pathology consultants working as 46.35 whole-time equivalents (WTE): 43 in England, 7 in Scotland and 2 in Wales.[20]
- There are no paediatric and perinatal pathology consultants working in Northern Ireland. In England, in the South West and the Midlands, shortages have led to total service collapses.[21]
- Currently, 37% of paediatric and perinatal pathology consultant posts in the UK are vacant.
- Recruitment is almost impossible due to a national shortage of qualified candidates; 83% of paediatric and perinatal pathology consultants report issues with recruitment in their departments.
- Only 3% of paediatric and perinatal pathology consultants believe that current staffing levels are adequate to ensure the long-term sustainability of their service.
- Within the next five years, 25% of the consultant paediatric and perinatal pathology workforce are expected to retire.
- There are only 13 resident doctors in paediatric and perinatal pathology specialist training posts. This is insufficient to address any of the following: long-standing vacancies, expected retirements and future needs.
- Bereaved families are facing a major increase in waiting times – and/or transfer out of their region – for post-mortem examination of their babies and children: 1 in 5 are now waiting 6 months or more, and some longer than 12 months.

28.27 As well as a shortage of consultants, demand for services has increased over the last six years.[22]

28.28 In its report, the Royal College of Pathologists calls for:

- phased expansion of training posts up to 37 by 2030
- increased protected time and funding for professional development so consultants have time to train the next generation
- increased workforce support with dedicated biomedical scientists, anatomical pathology technicians (assistants to pathologists) and administrative support in each paediatric pathology unit
- committed funding and resources to continue the development of ten fellowships
- an advanced training programme in paediatric and perinatal pathology
- upskilling of scientists and histopathologists to engage in placenta reporting and commitment to resource and prioritisation of the specialty.

28.29 The government response in April 2026 points to the launch of a national programme to strengthen perinatal and paediatric pathology services and to improve service capacity and resilience. A £20,000 recruitment incentive for new trainees has been introduced, a fully funded international recruitment campaign has been launched, and a new National Training Programme Director has been appointed.[23]

28.30 On 8 April 2026, in answer to a written question in Parliament about the steps taken in light of the report, Gillian Merron, Baroness Merron, said this:

> "*The paediatric and perinatal pathology workforce report highlights the extent of the workforce crisis in paediatric and perinatal pathology and the impact this can have on turnaround times and families.*
>
> *NHS England has launched a national programme to strengthen perinatal and paediatric pathology services and to improve service capacity and resilience.*
>
> *A £20,000 recruitment incentive for new trainees has been introduced, a fully funded international recruitment campaign has launched, and a new National Training Programme Director has been appointed.*
>
> *Further initiatives are underway to review the training pathway, develop advanced practitioner roles, and implement a retention strategy for existing staff.*"[24]

This looks very promising, assuming the £20,000 incentive will attach to all new trainees with the aim of reaching 37 training posts by 2030. There is no commitment to the latter number. I also note that the Royal College of Pathologists' workforce report stated: "*A previously introduced £20,000 recruitment incentive for new residents has now stopped.*" The incentive must not be removed until workforce requirements are met.

28.31 I recommend that a clear timetable is set and followed so that, by June 2030, there are 37 doctors in training posts as paediatric and perinatal pathologists. Funding must be made available to pay the £20,000 incentive to each new trainee with clear provision for the repayment of the £20,000 in the event the trainee does not complete training or takes up employment outside the NHS within five years of completing training.

Endnotes

1 **Written Closing Submissions on Behalf of the Countess of Chester Hospital NHS Foundation Trust 17 March 2025 83/para 313(d)**

2 **Written Closing Submissions on Behalf of the Countess of Chester Hospital NHS Foundation Trust 4 March 2025 89/paras 334 and 338**

3 RCPCH, 'Intercollegiate document (2025) Safeguarding children and young people & children and young people in care: Competencies for health care staff', November 2025 (**https://child-health-safeguarding.rcpch.ac.uk**)

4 RCN, *Safeguarding Children and Young People: Roles and competencies for healthcare staff*, January 2019 (archived) (**https://child-health-safeguarding.rcpch.ac.uk/wp-content/uploads/sites/25/2025/09/ARCHIVED-Safeguarding-Children-and-Young-People-Roles-and-Competencies-for-Healthcare-Staff.pdf**); RCN and RCPCH, *Looked After Children: Roles and competencies of healthcare staff*, December 2020 (archived) (**https://child-health-safeguarding.rcpch.ac.uk/wp-content/uploads/sites/25/2025/09/ARCHIVED-Looked-After-Children-Roles-and-Competencies-of-Healthcare-Staff.pdf**)

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9 BAPM, *Neonatal Mortality Governance: A BAPM framework for practice*, February 2026 (**https://hubble-live-assets.s3.eu-west-1.amazonaws.com/bapm/file_asset/file/4060/BAPM_Neonatal_Mortality_Governance_FEB_26.pdf**)

10 **INQ0108888**

11 Royal College of Pathologists, *Paediatric and Perinatal Pathology Workforce Report*, November 2025, page 10 (**https://www.rcpath.org/static/798eba01-0ddc-47ce-90b3932f7a3d9312/fce53e2f-178f-474c-9f0c92c3d403590c/Report-RCPath-paediatric-and-perinatal-pathology-2025.pdf#page=10**)

12 BAPM, *Neonatal Mortality Governance: A BAPM framework for practice*, February 2026, page 15 (**https://hubble-live-assets.s3.eu-west-1.amazonaws.com/bapm/file_asset/file/4060/BAPM_Neonatal_Mortality_Governance_FEB_26.pdf#page=15**)

13 BAPM, *Neonatal Mortality Governance: A BAPM framework for practice*, February 2026, page 15 (**https://hubble-live-assets.s3.eu-west-1.amazonaws.com/bapm/file_asset/file/4060/BAPM_Neonatal_Mortality_Governance_FEB_26.pdf#page=15**)

14 BAPM, *Neonatal Mortality Governance: A BAPM framework for practice*, February 2026, page 8 (**https://hubble-live-assets.s3.eu-west-1.amazonaws.com/bapm/file_asset/file/4060/BAPM_Neonatal_Mortality_Governance_FEB_26.pdf#page=8**)

15 BAPM, *Neonatal Mortality Governance: A BAPM framework for practice*, February 2026, page 24 (**https://hubble-live-assets.s3.eu-west-1.amazonaws.com/bapm/file_asset/file/4060/BAPM_Neonatal_Mortality_Governance_FEB_26.pdf#page=24**)

16 BAPM, *Neonatal Mortality Governance: A BAPM framework for practice*, February 2026, page 25 (**https://hubble-live-assets.s3.eu-west-1.amazonaws.com/bapm/file_asset/file/4060/BAPM_Neonatal_Mortality_Governance_FEB_26.pdf#page=25**)

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18 Royal College of Pathologists, 'Become a paediatric and perinatal pathologist' (**https://www.rcpath.org/discover-pathology/careers-in-pathology/careers-in-medicine/become-a-paediatric-and-perinatal-pathologist.html**)

19 Royal College of Pathologists, *Paediatric and Perinatal Pathology Workforce Report*, November 2025 (**https://www.rcpath.org/static/798eba01-0ddc-47ce-90b3932f7a3d9312/fce53e2f-178f-474c-9f0c92c3d403590c/Report-RCPath-paediatric-and-perinatal-pathology-2025.pdf**)

20 Royal College of Pathologists, *Paediatric and Perinatal Pathology Workforce Report*, November 2025, page 7 (**https://www.rcpath.org/static/798eba01-0ddc-47ce-90b3932f7a3d9312/fce53e2f-178f-474c-9f0c92c3d403590c/Report-RCPath-paediatric-and-perinatal-pathology-2025.pdf#page=7**)

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23 Baroness Merron, Written answer, Pathology: Vacancies, UIN HL15826, 8 April 2026 (**https://questions-statements.parliament.uk/written-questions/detail/2026-03-20/hl15826**)

24 Baroness Merron, Written answer, Pathology: Vacancies, UIN HL15826, 8 April 2026 (**https://questions-statements.parliament.uk/written-questions/detail/2026-03-20/hl15826**)

Chapter 29. Bereavement care

Accommodation

29.1 Mother D and Mother E and F both acknowledged that after their babies had died they were given the use of a family room; it is likely that this room was intended to give grieving parents some privacy in the aftermath of the death of their child. Mother D commented on the location of the room; it was situated within the labour ward,[1] and from here the deliveries and cries of newborn babies could be heard. This was, although wholly unintended, further cause of desperate heartache, emphasising in the most intense way possible the loss of their babies. Mother E and F expressed regret that she was not given the opportunity to take her baby, Baby E, to the family room and spend some time with him.[2] I acknowledge that arrangements are now different, but the accounts of these mothers should serve as a reminder to all those in the NHS who deal with the allocation of space to particular uses always to be informed of, and take into account, the circumstances of the people likely to be using the room – and adjacent rooms and corridors. These are matters upon which the expert advice of bereavement support groups should also be sought.

Support

29.2 A number of parents spoke about their experiences immediately after the death of their babies. I have set out in detail the experience of Mother and Father C in the time leading up to and after the death of their son. I do not repeat it. That Ms Taylor was acting empathetically and with kindness is not in doubt. In contrast, Letby's interventions following the death of Baby C were unnecessary and unprofessional, intervening when she should have been caring for her allocated baby. Her behaviour was the subject of criticism from the nursing shift leader at the time.[3]

29.3 Mother E and F spoke of Letby's actions at the time of Baby E's death, bathing him and preparing a memory box which is now tainted by the knowledge that Letby killed Baby E and tried to kill Baby F. She was unkind in the way she responded to Mother E and F's shock at seeing her child still in his incubator next to his brother: "*You haven't told us to take him*."[4]

29.4 As I observed elsewhere (see paragraph 10.59, Chapter 10), having endured the excited behaviour of Letby as she prepared a memory box, Parents O, P and R were left with no support and their son Baby R was moved to another hospital, with all the additional anxiety that entailed. I repeat: this was unacceptable. Even though two of their babies had died at the Countess, Mother O, P and R informed the Inquiry that she and her

husband "*were not offered any support or counselling by the Countess of Chester Hospital. The possibility of support was never even mentioned. The first time we were offered support was through Homicide Support and then at the criminal trial*."[5]

29.5 Several parents spoke about being given leaflets about bereavement, but no other recognition of their situation, and no support. This was not good enough. Ms Jane Tomkinson OBE, the current Chief Executive of the Countess, acknowledged that bereavement care in 2015 and 2016 was inadequate.[6] It may well be that some of the nurses had not previously experienced the death of a baby and did not know what to say. Some people, nurses amongst them, have a high level of empathy, and some have had personal experience of bereavement. They often know what to do. Help is needed for those who do not know what to do, particularly when facing the death of a patient for the first time.

29.6 Mother E and F's experiences of bereavement care at the Countess led her to re-train as a bereavement counsellor. One can only admire and pay tribute to her capacity for generosity and her determination to make sure that the experience of bereavement for others is quite different from hers.

29.7 Sir Stephen Powis said in his statement to the Inquiry:

> "[I]*n 2022/23 NHS England provided £2.26m of national funding to support trusts to expand the number of staff being trained in bereavement care and increase access to specialist bereavement services.*
>
> *In 2023/24, NHS England are investing £5.9m in bereavement care to enable all trusts to implement a seven-day provision … and increase the number of staff trained in bereavement care.*"[7]

These are relatively modest sums of money and very good intentions. What is essential is that bereaved parents have access to support that goes beyond the mere handing out of a leaflet, and that nurses and medical professionals have access to training where this is relevant to their roles.

29.8 It was encouraging to hear the evidence of Ms Fiona Murphy MBE, former Corporate Director of Nursing for End-of-Life Care and Bereavement at the Northern Care Alliance. One of the first points she raised was the need for nurses (for it is usually nurses) who may deal with bereavement to have training in advanced communication. It is training in how to communicate with people whose loved one has died. Here, a baby. The responsibility on medical and nursing staff is first not to make a terrible situation worse, and second for the parents to know that they care. The SWAN (Sign/Words/Actions/Needs) model of care used by the Northern Care Alliance starts from the premise that there is no harm in simply asking a family what matters to them at a particular moment, rather than following any lengthy written protocol. The model sets out four key principles: the provision of private space, sensitive communication with the family, stepping 'outside the box' to facilitate what is important to the family, and considering whether the needs of the family are being met, documented and reviewed regularly. Ms Murphy spoke about the SWAN model providing prompts to help nursing staff with "*difficult*" conversations where they are "*fearful … to distress the family further*".[8] In fact, having conversations about what matters to the family

(such as spending time with their baby after death) can help give the family some control. Ms Murphy said that about another 50 organisations in the UK now use the SWAN model.[9]

29.9 I heard evidence from a number of witnesses in person and also received witness statements, including from Sands, a UK charity that leads the National Bereavement Care Pathway. This includes specific guidance related to neonatal deaths. A summary of that evidence was read into the transcript.[10] I am encouraged that all Trusts have signed up to the National Bereavement Care Pathway. I recommend the National Bereavement Care Pathway for neonatal death should be implemented nationally and in all Trusts by 31 August 2027.

29.10 The evidence from the Countess about the current position is that there are now two full-time bereavement midwives whose job is to support the bereaved for as long as and to the extent that they wish, and wherever they wish – home, hospital or elsewhere, as the bereaved person prefers. It is a service that can be accessed at any point following the death of a baby and referrals can be made to other support services, such as talking (psychological) therapies.[11]

Medical records

29.11 It is relatively recently that there has been acknowledgement of the importance of marking the medical records of parents with a note of the fact of (as here) the death of their baby. The death of a new baby is relevant both to the parents' mental health and the care of a further pregnancy, to name two obvious points. No one should have to repeat their experience each time they go to the GP. It should be flagged in the notes with an alert on the outside of the file, digital or otherwise, so that the GP (or other professional) sees it before they see the patient. This will make a difference to bereaved parents.

29.12 It would appear that both nationally and at the Countess steps have been taken to improve bereavement care. However, perhaps most important is this final reflection, one that was well made in the statement submitted to the Inquiry by the UK charity Sands: "*Listening to the voices and experiences of bereaved parents will help to drive a change in culture and must be at the heart of all policies developed to save babies lives and improve future care*."[12]

Endnotes

1 Mother D 17 September 2024 24/5-7

2 Mother E and F 18 September 2024 54/9-11

3 Nurse W 14 October 2024 93/4 to 94/9

4 Mother E and F 18 September 2024 54/1

5 Mother O, P and R 25 September 2024 27/6-10

6 Jane Tomkinson OBE 13 January 2025 24/10-12

7 Witness statement of Prof. Sir Stephen Powis INQ0017495/242/para 905 to 906

8 Fiona Murphy MBE 14 January 2025 147/12-13

9 Fiona Murphy MBE 14 January 2025 148/7-10

10 Clare Brown 14 January 2025 185/18 to 193/21

11 Written Closing Submissions on Behalf of the Countess of Chester Hospital 4 March 2025 81/paras 305-307

12 Witness statement of Clea Harmer INQ0017151/17/para 55

Chapter 30. Parents' further recollections

Mother A and B: "*After losing* [Baby A]*, not only were we absolutely traumatised at what had happened, we were riddled with fear for* [Baby B]*.*"

"*What should have been the happiest time of our lives had become our worst nightmare.*"

Mother C: "*We have suffered immeasurably from the moment our son collapsed. The trauma that we faced from then until now is thoroughly incomprehensible to anyone who has not endured it.*"

"*Following on from the criminal trial, we have had to try to rebuild life and regain some normality for ourselves and our family, which has been very difficult. As a family, we have endured years of anxiety and stress … the knowledge we gained about the events leading up to our son's murder and the methods that were used by Lucy Letby has been indescribably traumatising*."

Mother D: "*I feel not only I lost* [my daughter]*, but lost all those years of my life too. Since* [she] *passed away I live beside my own shadow.*"

"[T]*he heavy load constantly on my mind has deeply changed me. My heart broke into a million pieces the second* [our daughter] *lost her battle against evil and that is when hell broke loose for us … I felt so guilty and questioned if any of this was my fault. Did I miss something? Did I do something wrong? Did I fail my daughter?*"

Mother E and F: "*The impact that* [the murder and the attempted murder of our sons] *has had on us has been enormous. It changed the course of our life completely and we've had to try and grieve in so many different ways. We tried to grieve at the time, and then we had to endure what was going to be happening when that* [the RCPCH] *report arrived on our doorstep, and then that brought everything back up. We had to grieve for the life that we thought we were going to have with* [our son]*, with his learning difficulties.*"

"*I feel guilty for not requesting* [a post-mortem]*, because if that had been requested and that had come back and something -- well, something would have been on it, you know, there's a lot of babies that could have not been involved in this case and it could have stopped there. And that weighs very, very heavily on me, because that decision was ultimately ours, and that's painful to think about. So I carry our grief, but the sadness of the other families, because it should never have gone past that point.*"

Mother G: "*I feel that Lucy Letby has ruined our lives. She has ruined everything. Our daughter needs 24-hour care because of Letby, we don't know how long she will live and it affects every single minute of all of our days.*"

Father G: "*We now know that the attacks by Lucy Letby caused a lack of oxygen to our daughter's brain resulting in a massive brain injury. Prior to the attacks, our daughter's repeated brain scans looked OK and her brain was seen to be maturing well. After the attacks, her brain scans changed and clearly showed a brain injury which had wiped out most of her brain … She can't be left unattended due to the risk of her choking. My wife and I provide the majority of her care and do so lovingly. She is our little girl, our fighter and our star. I have been besotted with her ever since the day she was born.*"

Mother H: "[The impact of the events at the Countess] *changed me fundamentally as a person. Among other things, it has really affected my ability to trust people, especially when it comes to anyone taking care of my children. You know, they -- the people in the neonatal ward, you put your whole trust into them to take care of your baby and that has been completely abused.*"

Mother I: "*The first year after our baby's death was a blur and I don't know how we as a family got through it. I wore sunglasses constantly to hide the pain and tears from my other kids as I didn't want to upset them as they were also struggling.*"

"[My husband] *wished that he was dead instead of our baby. We even separated for a while as neither of us could deal with what happened. Our other kids also suffered. They gave up things they enjoyed and my older daughter stop*[ped] *speaking. Eventually we got back together.*"

Mother J: "*I cannot emphasise enough the impact of this on our whole family. Who we are as people, parents, work life, spouses, children.*"

Father J: "*It's hard to describe the stress of any parent on a neonatal unit when they've got a sick child and when things aren't quite happening in the way you expect them and then you have an unexplained collapse, that stress is almost incomprehensible.*"

Mother K: "*For myself and my husband, the ripples are unbelievable … it's scarred your life, it's changed you. You look back and you don't only just grieve your daughter, you're grieving who you were. I grieve who we were as a husband and a wife. It just completely destroys what's around you and you have to pick yourself up and find out who you are again in this new world and it just doesn't stop, it doesn't go away and we live with it every single day.*"

"*I only hope that I do* [our daughter] *proud within this statement and raising her siblings with her name continuing to be a great part of our family unit and remembering that she was the one that made me a mum for the first time.*"

Father K: "*They brought our baby in and handed her to me as she took a breath. That was it. Child K only took one breath on her own in my arms … I wish I could have held Child K for a little bit longer. I can't talk about what happened to Child K, it scares me to remember and I don't want to think about it.*"

Father L and M: "*I was first on the scene when one of my* [sons] *had his collapse, and that image has been forever etched in my mind and this case has been going on for … years.*

…

The stress and strain has been unbearable at times and my mental health has suffered as a consequence of this case. I had to take time out of work and seek counselling. I have also had to take a course of anti-depressants to help me cope with this … I had a seizure for the first time in my life as we approached the criminal trial. This happened in front of my children and was very distressing for them. The doctors attributed this to the stress and the pressure of what had happened to our children."

Mother N: "*The day we were called to the neonatal unit was the worst day of our lives, from waking up that morning being prepared to take home our son to the utter catastrophic scene we arrived at has left a lasting imprint on us, seeing our tiny baby fighting for his life, medics doing CPR on his tiny body and not knowing if he was going to live or die with no obvious cause. We have often heard of people dying from a broken heart, this is how we can describe how we felt that day the pain was immeasurable and we didn't want to leave that hospital without our son, we both relive this every day because not a day goes by without thinking about that day.*"

Father N: "*This has caused us massive trust issues which have remained with us to this day and we don't think will ever leave us.*"

Father O, P and R: "*I have visions and nightmares approximately every other night, of being back at the hospital. They tend to be about the boys and are a mix of specific things I remember happening at the time; the colour of their veins and their veins pulsating is something which comes up a lot. I also have angry dreams where I tell myself I should have done more. I have also had angry dreams about Lucy Letby. I cry a lot more than I used to before all of this happened.*"

Mother Q: "*This has left me with massive anxiety with a lot of health professionals … over the years the slightest thing with Child Q has caused me to panic … I am in* [a] *constant state of worry and panic, I'm hyper vigilant with him and it is so draining for me.*"

I repeat my thanks to all the parents. As is plain from the pages of this Report, their insightful and thoughtful contributions to the Inquiry have been invaluable.

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